

PassReady Prep — NCE Question Bank

Generated: 2026-10-04 · Showing: In review · Total items: 400

How to review

✓ **KEY** marks the correct option. Every option shows why it is right or wrong; check both the key and each distractor.

Mark each item **Approve**, **Revise** (note what to change) or **Retire**. Approved items are published from Admin → NCE Questions.

▮ **Check** notes mark specific facts or citations the writer was less than certain of — verify those first.

Check: one clearly best answer · factual accuracy (theorists, dates, test properties, code sections) · distractors plausible but defensibly wrong · no answer given away by length or wording · references support the key.

NCE domains (NBCC 2023 content outline)

Professional Practice and Ethics 19 · Intake, Assessment, and Diagnosis 19 · Areas of Clinical Focus 47 · Treatment Planning 14 · Counseling Skills and Interventions 48 · Core Counseling Attributes 13 — of 160 scored items

In this copy

Professional Practice and Ethics **48** · Intake, Assessment, and Diagnosis **48** · Areas of Clinical Focus **118** · Treatment Planning **34** · Counseling Skills and Interventions **120** · Core Counseling Attributes **32** · **71** flagged ▮

Professional Practice and Ethics

48 items · NCE weight 12% (19 of 160 scored items)

1. nce-s-eth-001 · Professional Counseling Orientation and Ethical Practice · informed consent and its required elements · easy · In review

A newly licensed counselor is drafting an informed consent document for a private practice. Which element is a required part of informed consent under the ACA Code of Ethics?

- A. A guarantee that the presenting problem will improve within a set number of sessions
Counselors explain potential benefits and risks; promising a specific outcome is misleading.
- B. An explanation of the limits of confidentiality and how fees and billing are handled ✓ **KEY**
A.2.b lists confidentiality limits and fees and billing arrangements among required disclosures.
- C. A summary of the counselor's personal religious and political values for the client
Counselors avoid imposing personal values (A.4.b); disclosing them is not a consent element.
- D. A signed waiver releasing the counselor from liability for any outcome of treatment
Liability waivers are not part of informed consent and do not serve client welfare.

Rationale: Informed consent gives clients the information needed to choose whether to enter or continue counseling. ACA A.2.b names elements such as the purposes, goals, techniques, risks and benefits of services, the counselor's credentials, fees and billing, the limits of confidentiality, and the client's right to refuse services. Consent is an ongoing process, not a single signature.

References: ACA Code of Ethics — A.2.b Types of Information Needed

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 12-year-old is brought to counseling by his custodial grandmother, who holds legal guardianship. The boy says he does not want to be there. How should the counselor BEST approach consent?

- A. Treat the grandmother's signature as sufficient and skip any discussion with the boy
Legal consent from a guardian does not remove the duty to seek the minor's assent.
- B. Decline to begin services until the boy is old enough to provide legal consent alone
Minors routinely receive counseling with guardian consent; refusing service is not required.
- C. Let the boy decide alone whether to continue, since his preference overrides consent
A minor generally cannot give legal consent; the guardian's role cannot simply be set aside.
- D. Obtain the guardian's consent and seek the boy's assent in language he can understand ✓ KEY**
A.2.d directs counselors to seek the assent of clients who cannot give consent and to include guardians.

Rationale: When a client cannot legally consent, counselors obtain consent from the parent or guardian and still seek the client's assent, explaining services in developmentally appropriate terms. This balances the guardian's legal rights with the minor's autonomy and welfare. Age-of-consent rules for minors vary by state, so counselors also check state law.

References: ACA Code of Ethics — A.2.d Inability to Give Consent

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is starting a grief group for adults. During the first session, what should the counselor tell members about confidentiality?

- A. That members are asked to keep what is shared private, but this cannot be guaranteed ✓ KEY**
B.4.a asks counselors to explain the importance and parameters of confidentiality in group work.
- B. That group confidentiality is protected by the same law that binds the counselor
Group members are generally not bound by the counselor's legal duties; this overstates protection.
- C. That confidentiality applies only to the counselor, so members may share freely
Members should be asked to respect each other's privacy; this undermines group safety.
- D. That any member who discloses outside the group will face legal action by the counselor
This is an inaccurate threat; the counselor cannot enforce group confidentiality this way.

Rationale: In group work the counselor can promise to protect confidentiality but cannot guarantee that other members will. ACA B.4.a requires counselors to clearly explain the importance and parameters of confidentiality for the specific group. Setting this expectation at the outset supports informed consent and group trust.

References: ACA Code of Ethics — B.4.a Group Work

☐ Approve ☐ Revise ☐ Retire Notes:

A client discloses a new diagnosis of a contagious, life-threatening disease and says she has not told her long-term partner, with whom she continues to have unprotected sex. State law permits disclosure in this situation. What should the counselor do FIRST?

- A. Contact the partner right away and inform him of the diagnosis without telling the client
B.2.c calls for assessing the client's intent to inform the partner before any disclosure.
- B. Keep the information confidential, since health conditions are outside the limits
B.2.c recognizes that disclosure to an identifiable third party at high risk may be justified.
- C. Assess whether the client intends to inform her partner or to change the risky behavior ✓ KEY
B.2.c directs counselors to assess client intent before disclosing to third parties at risk.
- D. Refer the client to a medical provider and end counseling until she discloses to him
Ending services as leverage is coercive and may amount to abandonment (A.12).

Rationale: ACA B.2.c states that when clients disclose a contagious, life-threatening disease, counselors may be justified in disclosing to identifiable third parties at serious and foreseeable risk if laws permit. Before doing so, counselors assess the client's intent to inform those parties or to stop harmful behavior. State law varies, so counselors confirm what their jurisdiction permits or requires.

References: ACA Code of Ethics — B.2.c Contagious, Life-Threatening Diseases

☐ Approve ☐ Revise ☐ Retire Notes:

During a session, a client names a former coworker, states he has bought a handgun, and describes a plan to shoot her at her home this weekend. Which action is MOST consistent with the counselor's ethical duties?

- A. Keep the threat confidential and explore the client's anger at the next session
A specific, imminent threat to an identifiable person is a recognized limit of confidentiality.
- B. Ask the client to sign a no-harm contract and continue with the planned session work
No-harm contracts do not reduce risk and do not meet the duty to protect an identified victim.
- C. Take protective steps such as warning the victim or police, as state law directs ✓ KEY
B.2.a permits disclosure to protect identified others from serious and foreseeable harm.
- D. End the session early and refer the client to a psychiatrist for medication review
Referral alone does nothing to protect the identified person from imminent harm.

Rationale: Following Tarasoff v. Regents of the University of California (1976), counselors may have a duty to protect identifiable victims of serious threats. ACA B.2.a states that confidentiality does not apply when disclosure is required to protect identified others from serious and foreseeable harm. Whether the duty is mandatory or permissive, and what steps satisfy it, varies by state law.

References: ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements · State law — Duty-to-warn/protect statutes (mandatory vs permissive, Tarasoff)

☐ Approve ☐ Revise ☐ Retire Notes:

A client frustrated about a custody dispute says, "Sometimes I could just kill my ex." He names no plan, has no weapons, has no history of violence, and laughs it off. What is the counselor's BEST response?

A. Explore the statement further to assess intent, plan, means, and history of violence ✓ KEY

An ambiguous statement calls for violence risk assessment before any decision to disclose.

B. Notify the ex-spouse and local police immediately that a threat has been made

Breaching confidentiality without assessing seriousness or foreseeability is premature.

C. Ignore the remark, since venting about an ex-spouse is common in custody conflicts

Dismissing a statement of violence without assessment risks missing real danger.

D. Terminate services because the client has made a threat against another person

Termination does not address risk and may leave the client and others less safe.

Rationale: The duty to protect is triggered by serious and foreseeable harm, not by every angry remark. The counselor should assess intent, plan, means, target, and history, consult as needed, and document the reasoning. If assessment reveals a serious threat, protective action under B.2.a and state law follows.

References: ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes:

7. nce-s-eth-007 · Professional Counseling Orientation and Ethical Practice · mandated reporting of abuse and neglect · medium · In review

A school-based counselor notices bruises in the shape of a hand on a 7-year-old's arm. The child says, "Dad gets mad," and then stops talking. The counselor is unsure whether abuse occurred. What should the counselor do?

A. Gather more evidence by questioning the parents before deciding whether to report

Investigating is the role of child protective services and may place the child at risk.

B. Wait to see whether new injuries appear before making a report to the authorities

Delay is not permitted once reasonable suspicion exists; certainty is not required.

C. Ask the child's teacher to make the report so the counseling relationship is protected

In most states the duty to report is personal and cannot be handed off to a colleague.

D. Make a report to child protective services based on reasonable suspicion of abuse ✓ KEY

Mandated reporters report reasonable suspicion; proof or certainty is not required.

Rationale: All U.S. states have mandated reporting laws for suspected child abuse and neglect that cover counselors, though definitions, thresholds, and procedures vary by state. The threshold is reasonable suspicion or cause to believe, not proof, and the counselor should not investigate. Specific timelines, reporting agencies, and procedures vary by state law, and many school policies add internal notification steps.

References: State law — Child abuse reporting statutes: reasonable suspicion standard · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in a HIPAA-covered agency keeps process notes about session dynamics and her own impressions in a file separate from the medical record. Under the HIPAA Privacy Rule, how are these notes treated?

- A. They are part of the designated record set and may be released for any payment purpose
Separately kept psychotherapy notes are excluded from routine payment disclosures.
- B. They are psychotherapy notes, and most disclosures require a specific authorization ✓ KEY
HIPAA defines psychotherapy notes and requires specific authorization for most uses.
- C. They lose all protection once a client signs a general release of medical information
A general release does not suffice; psychotherapy notes need their own authorization.
- D. They must be given to the client on request under the HIPAA right of access to records
Psychotherapy notes are excluded from the HIPAA right of access, though state law may differ.

Rationale: HIPAA gives psychotherapy notes, meaning a counselor's notes analyzing session content that are kept separate from the medical record, special protection. Most uses and disclosures require a specific authorization, and they are excluded from the client's general right of access. Items such as diagnosis, medications, session times, and treatment plans are not psychotherapy notes.

References: HIPAA — Privacy Rule: psychotherapy notes definition and authorization (45 CFR 164.501, 164.508)

☐ Approve ☐ Revise ☐ Retire Notes:

A competent adult client asks to see a copy of her counseling records. What does the ACA Code of Ethics direct the counselor to do?

- A. Provide reasonable access, limiting it only with compelling evidence of harm to her ✓ KEY
B.6.e directs reasonable access, limited only when access would cause the client harm.
- B. Refuse the request, since counseling records belong only to the counselor and agency
Clients have a right of reasonable access to their own records under B.6.e.
- C. Release the records only after she obtains a court order or a lawyer makes a request
No court order is needed for a competent client to access her own records.
- D. Give her a summary only, because clients may not see the counselor's written records
The Code does not limit clients to summaries; it supports access and help interpreting records.

Rationale: ACA B.6.e states that counselors provide reasonable access to records and copies when requested by competent clients. Access is limited only when there is compelling evidence that it would cause harm to the client, and counselors document the request and their reasoning. Counselors also offer assistance in interpreting records (B.6.f).

References: ACA Code of Ethics — B.6.e Client Access

☐ Approve ☐ Revise ☐ Retire Notes:

Two years after ending counseling with a client, a counselor runs into the former client at a community event, and the former client asks the counselor out on a date. Under the ACA Code of Ethics, what applies?

A. A romantic relationship is allowed now, since more than one year has passed since the end

The ACA standard is five years, not one year.

B. A romantic relationship is allowed if a colleague agrees there is no risk of exploitation

Consultation does not override the five-year prohibition.

C. A romantic relationship is prohibited for five years after the last professional contact ✓ **KEY**

A.5.c prohibits such relationships for five years following the last professional contact.

D. A romantic relationship is permitted as long as the former client initiated the request

Who initiates does not change the prohibition; responsibility rests with the counselor.

Rationale: ACA A.5.c prohibits sexual or romantic relationships with former clients, their romantic partners, or family members for five years after the last professional contact, in person or electronic. Even after five years, counselors must show forethought and document that the relationship is not exploitative. The responsibility rests with the counselor regardless of who initiates.

References: ACA Code of Ethics — A.5.c Sexual and/or Romantic Relationships With Former Clients

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in a small rural town is invited by a long-term client to attend the client's high school graduation, which the client has worked toward despite major barriers. The counselor believes attending could benefit the client. What should the counselor do?

A. Decline, because any contact outside the office is an ethical violation for counselors

The Code permits boundary extensions that benefit clients when handled with safeguards.

B. Weigh benefits and risks, discuss it with the client, and document the rationale ✓ **KEY**

A.6.b asks counselors to document rationale, benefit, and anticipated consequences.

C. Attend without discussion, since a graduation is a public event open to everyone

A boundary extension should be discussed and documented, not done casually.

D. Attend only if the client agrees to end counseling before the graduation ceremony

Terminating services to permit contact is unnecessary and harms the client.

Rationale: ACA A.6.b recognizes that extending boundaries, such as attending a formal ceremony, can benefit clients. Before doing so, counselors document the rationale, potential benefit, and anticipated consequences, and take precautions such as informed consent, consultation, and supervision. Rural settings make such decisions more common, which increases the need for careful reasoning.

References: ACA Code of Ethics — A.6.b Extending Counseling Boundaries

☐ Approve ☐ Revise ☐ Retire Notes:

A licensed counselor completes a one-day introductory workshop on a specialized trauma therapy and wants to start using it with several clients who have complex trauma. Which course of action BEST meets the counselor's ethical obligations?

A. Use the method now, since the counselor's license covers all trauma treatments

A license defines legal scope, not competence in every specialized method.

B. Use the method now but tell clients it is experimental and get written consent

Consent does not replace the training needed to use a new specialty competently.

C. Use the method now, since a workshop certificate shows enough competence to practice

A single introductory workshop rarely provides the training and supervision C.2.b requires.

D. Pursue further training and supervised practice before using it, and refer if needed ✓ **KEY**

C.2.b requires education, training, and supervised experience before practicing a new specialty.

Rationale: ACA C.2.a limits practice to the boundaries of competence based on education, training, supervised experience, and credentials. When entering a new specialty area, C.2.b requires appropriate education, training, and supervised experience, and steps to protect clients from harm while skills develop. Referral is appropriate when the counselor cannot yet provide competent care.

References: ACA Code of Ethics — C.2.b New Specialty Areas of Practice · ACA Code of Ethics — C.2.a Boundaries of Competence

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor wants guidance on keeping separate professional and personal social media profiles. Which section of the 2014 ACA Code of Ethics MOST directly addresses this issue?

A. Section B, Confidentiality and Privacy

Section B covers privacy broadly but social media is addressed in Section H.

B. Section D, Relationships With Others

Section D addresses relationships with colleagues and other professionals.

C. Section C, Professional Responsibility

Section C covers competence, advertising, and credentials, not social media profiles.

D. Section H, Distance Counseling ✓ **KEY**

Section H covers distance counseling, technology, and social media, including H.6.

Rationale: The 2014 ACA Code has nine sections, A through I. Section H, Distance Counseling, Technology, and Social Media, was new in 2014; H.6.a directs counselors who keep both a professional and a personal presence on social media to create separate profiles. Knowing the section layout helps counselors find guidance quickly.

References: ACA Code of Ethics — Section H; H.6.a Virtual Professional Presence

☐ Approve ☐ Revise ☐ Retire Notes:

A licensed counselor who also holds the National Certified Counselor (NCC) credential is accused of an ethics violation. Which statement about the NBCC Code of Ethics is accurate?

A. It replaces the state licensing board, so state law no longer applies to the counselor

State licensure law applies independently of national certification.

B. It binds NCCs, and NBCC can act on the certification apart from any state board action ✓ **KEY**

NBCC enforces its Code for its certificants; boards enforce licensure separately.

C. It applies only to counselor educators who teach in CACREP-accredited graduate programs

The NBCC Code applies to NCCs and applicants, not only to educators.

D. It is advisory only, so NBCC cannot suspend or revoke a credential for a violation

NBCC can impose sanctions on certification, including revocation.

Rationale: The NBCC Code of Ethics applies to National Certified Counselors and applicants for NBCC credentials. NBCC can investigate complaints and sanction or revoke certification, which is separate from any action a state licensing board takes on a license. A counselor may therefore answer to more than one body for the same conduct.

References: NBCC Code of Ethics — Scope and application to NCCs and applicants; enforcement of certification

☐ Approve ☐ Revise ☐ Retire Notes:

While evaluating a chosen course of action, a counselor asks, "Would I be comfortable if this decision were reported in the local newspaper?" Which test from Stadler's model is the counselor applying?

A. The test of justice

Justice asks whether the counselor would treat others in the same situation the same way.

B. The test of universality

Universality asks whether the counselor would recommend the action to others in the same situation.

C. The test of publicity ✓ **KEY**

Publicity asks whether the counselor would be comfortable having the action made public.

D. The test of fidelity

Fidelity is a moral principle about keeping commitments, not one of Stadler's tests.

Rationale: Many ethical decision-making models, including the Forester-Miller and Davis practitioner's guide, include Stadler's three tests for evaluating a selected action. Justice asks whether others would be treated the same, publicity asks whether the counselor would accept public reporting, and universality asks whether the counselor would recommend it to colleagues. Passing all three supports the soundness of the decision.

References: Forester-Miller & Davis (Practitioner's Guide to Ethical Decision Making) — Step 7, evaluating the selected course of action: Stadler's tests of justice, publicity, and universality

☐ Approve ☐ Revise ☐ Retire Notes:

A clinical supervisor learns that a post-master's supervisee has not told clients that she is under supervision or that their cases are discussed with the supervisor. What is the supervisor's MOST appropriate response?

A. Require the supervisee to inform clients of her status and how supervision affects privacy ✓ KEY

F.1.c requires that clients know supervisee qualifications and supervision's effect on confidentiality.

B. Allow it, since supervision is internal and does not change confidentiality for clients

Case discussion with a supervisor is a disclosure clients must be told about.

C. Report the supervisee to the licensing board before discussing the problem with her

Corrective supervision is the first step; a board report is premature here.

D. End the supervision contract at once and tell the supervisee to find a new supervisor

Abrupt termination leaves clients unprotected and skips remediation.

Rationale: A supervisor's primary obligation is to monitor the welfare of the clients served by the supervisee (F.1.a). Supervisors ensure that clients are aware of the supervisee's qualifications and of how the supervision process affects the limits of confidentiality (F.1.c). Supervisors share responsibility for client care and should correct the problem promptly through supervision.

References: ACA Code of Ethics — F.1.c Informed Consent and Client Rights · ACA Code of Ethics — F.1.a Client Welfare

☐ Approve ☐ Revise ☐ Retire Notes:

17. nce-s-eth-017 · Professional Counseling Orientation and Ethical Practice · licensure, credentialing and professional identity · easy · In review

Which statement BEST describes the difference between state licensure and national certification for counselors?

A. Licensure is legal permission to practice in a state; certification is a voluntary credential ✓ KEY

Licensure is state regulation; certifications such as the NCC are voluntary and national.

B. Certification is required by law in every state; licensure is optional for private practice

This reverses the two; licensure is the legal requirement for independent practice.

C. Licensure and certification are the same credential, issued by different agencies by region

They are distinct credentials with different legal weight and issuing bodies.

D. Certification allows practice in all states; licensure applies only to school counselors

National certification does not authorize practice in any state.

Rationale: Licensure is a state legal credential that regulates the practice and title of professional counseling within that jurisdiction. National certification, such as the NBCC's National Certified Counselor credential, is voluntary and shows that a counselor met national standards but does not by itself permit practice. Interstate practice is addressed through state law and mechanisms such as the Counseling Compact.

References: Gladding (Counseling: A Comprehensive Profession) — Credentialing: licensure, certification, and accreditation

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor learns that a client who uses a wheelchair is being denied a reasonable accommodation by her landlord. The counselor wants to contact the housing authority about this client's case. What must the counselor do FIRST?

A. Contact the housing authority right away, since advocacy is a core professional duty

Advocating for an identifiable client without consent breaches confidentiality.

B. Avoid advocacy entirely, because it falls outside the counselor's professional role

A.7.a affirms advocacy to address barriers to client growth and development.

C. Write a public letter about the landlord to raise awareness of housing discrimination

Publicizing the case exposes the client and still lacks her consent.

D. Obtain the client's consent before advocating on her behalf as an identifiable client ✓ KEY

A.7.b requires client consent before advocating on behalf of an identifiable client.

Rationale: ACA A.7.a supports advocacy at individual, group, institutional, and societal levels to address barriers that inhibit client growth. A.7.b requires counselors to obtain client consent before engaging in advocacy on behalf of an identifiable client. Seeking consent also respects the client's autonomy and may support her own self-advocacy.

References: ACA Code of Ethics — A.7.b Confidentiality and Advocacy

☐ Approve ☐ Revise ☐ Retire Notes: _____

19. nce-s-eth-019 · Professional Counseling Orientation and Ethical Practice · telehealth and technology ethics · medium · In review

A counselor licensed in one state is asked to provide video counseling to a client who lives in another state. Which consideration is MOST important before beginning?

A. Only the laws of the counselor's home state apply, since that is where the license is

H.1.b notes that the client's state laws may also apply.

B. The counselor may be subject to the laws of both the counselor's and client's locations ✓ KEY

H.1.b directs counselors to know the laws of both locations in distance counseling.

C. Licensure rules do not apply to video sessions, since no in-person contact takes place

Distance counseling is still the practice of counseling and is regulated.

D. Any license is valid nationwide for telehealth as long as the platform is encrypted

Encryption addresses security, not legal authority to practice across states.

Rationale: ACA H.1.b states that counselors providing distance services understand they may be subject to the laws and regulations of both their practicing location and the client's place of residence. In practice, many states regulate based on where the client is located, and rules vary by state. Interstate options such as the Counseling Compact apply only among member states.

References: ACA Code of Ethics — H.1.b Laws and Statutes · Counseling Compact — Privilege to practice in member states

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor is starting video sessions with a new client who has a history of suicidal ideation. Which step is MOST important to complete during the first telehealth session?

- A.** Ask the client to keep the camera off so that the session feels less intrusive
Turning off video reduces the counselor's ability to assess and does not address safety.
- B.** Record the session so it can be reviewed later if any emergency happens
Recording requires consent and does nothing to prepare for an emergency.
- C.** Confirm the client's location and a local emergency plan and contact person ✓ KEY
Knowing the client's location and local resources is essential for responding to a crisis.
- D.** Use any free video app the client prefers so that the client feels comfortable
Platforms must protect confidentiality; preference alone is not the standard (H.2.d).

Rationale: Distance counseling requires planning for emergencies the counselor cannot handle in person. Counselors confirm the client's physical location, identify local emergency services, and agree on a plan as part of telehealth informed consent (H.2.a). This is especially important for clients with elevated risk.

References: ACA Code of Ethics — H.2.a Informed Consent and Disclosure (includes emergency procedures when the counselor is unavailable)

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor with strong religious beliefs is assigned a client who wants help with stress in her same-sex marriage. The counselor feels uncomfortable and wants to refer the client elsewhere. What does the ACA Code of Ethics state?

- A.** Referral is appropriate whenever a client's issue conflicts with the counselor's faith
A.11.b prohibits referral based solely on the counselor's personally held values.
- B.** Referral is required, since counselors must avoid any client whose values differ
The Code calls for respecting diversity and avoiding value imposition, not avoiding clients.
- C.** Referral based only on the counselor's personal values is not permitted by the Code ✓ KEY
A.11.b prohibits values-based referral; counselors seek training to work with diverse clients.
- D.** Referral is acceptable if the counselor explains the religious reason to the client
Explaining the reason does not make a values-based referral ethical and may harm the client.

Rationale: ACAA.11.b states that counselors refrain from referring prospective and current clients based solely on the counselor's personally held values, attitudes, beliefs, and behaviors. A.4.b directs counselors to avoid imposing their own values. Counselors seek training and supervision when values affect their work, and referral is reserved for genuine gaps in competence.

References: ACA Code of Ethics — A.11.b Values Within Termination and Referral · ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor notices that a colleague has been arriving late, smelling of alcohol, and falling asleep in team meetings. The colleague carries a full caseload. What should the counselor do FIRST, if it can be done safely?

A. Speak privately with the colleague about the concerns and encourage seeking help ✓ KEY

C.2.g and I.2.a support helping colleagues recognize impairment and informal resolution first.

B. File a complaint with the state licensing board without speaking to the colleague

Formal reports follow when informal steps fail or clients face imminent harm.

C. Tell the colleague's clients about the concerns so that they can make choices

This breaches the colleague's privacy and is not the counselor's role.

D. Say nothing, since a colleague's personal life is outside the counselor's concern

The Code expects counselors to act when colleague impairment may harm clients.

Rationale: ACA C.2.g asks counselors to monitor themselves for impairment and to assist colleagues or supervisors in recognizing their own impairment, providing consultation and help when warranted. I.2.a directs counselors to first attempt informal resolution with a colleague when feasible. If the problem persists or clients are at substantial risk, counselors take further action, such as reporting to supervisors or the licensing board.

References: ACA Code of Ethics — C.2.g Impairment · ACA Code of Ethics — I.2.a Informal Resolution

☐ Approve ☐ Revise ☐ Retire Notes:

23. nce-s-eth-023 · Professional Counseling Orientation and Ethical Practice · professional organizations and their roles · easy · In review

A student comparing master's programs in clinical mental health counseling asks which organization accredits counseling graduate programs. Which organization is it?

A. NBCC

The National Board for Certified Counselors (NBCC) grants national certification (such as the NCC) and administers the NCE.

B. CACREP ✓ KEY

The Council for Accreditation of Counseling and Related Educational Programs (CACREP) accredits master's and doctoral counseling programs.

C. ACA

The American Counseling Association (ACA) is a professional membership association that publishes the Code of Ethics.

D. AMHCA

The American Mental Health Counselors Association (AMHCA) is a professional association for clinical mental health counselors.

Rationale: CACREP, the Council for Accreditation of Counseling and Related Educational Programs, accredits counseling graduate programs. NBCC certifies individual counselors and administers the NCE, while ACA and its divisions and affiliates, such as AMHCA and ASCA, are membership associations. State licensing boards regulate practice within each state.

References: CACREP 2024 Standards — Accreditation of counseling master's and doctoral programs · Gladding (Counseling: A Comprehensive Profession) — Professional organizations and accreditation

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor researcher wants to study help-seeking and plans to briefly mislead participants about the study's true aim, because full disclosure would bias their answers. Under the ACA Code of Ethics, when is this acceptable?

- A.** Whenever participants have signed a general consent form for the overall study
General consent does not by itself justify deception.
- B.** Whenever the researcher believes the study results will be published widely
Publication plans do not justify deceiving participants.
- C.** Whenever participants are students, since course credit offsets any harm done
Students need added protection, and credit does not offset deception.
- D.** Only if no alternative is feasible, the value justifies it, and debriefing follows ✓ **KEY**
G.2 permits deception only when alternatives are not feasible, value justifies it, and debriefing occurs.

Rationale: The ACA Code allows deception in research only when alternative procedures are not feasible and the prospective value of the research justifies it. Deception that is likely to cause physical or emotional harm is not permitted, and researchers explain the deception during debriefing as soon as feasible. Institutional review boards also review such designs.

References: ACA Code of Ethics — G.2 Rights of Research Participants (deception and debriefing)

► **Check:** Cited as ACA G.2 generally; confirm subsection letters for deception and debriefing.

☐ Approve ☐ Revise ☐ Retire Notes:

25. nce-s-eth-025 · Professional Counseling Orientation and Ethical Practice · informed consent and its required elements · medium · In review

A client in private practice stopped attending sessions and has an unpaid balance of several hundred dollars. The counselor is considering turning the account over to a collection agency. What should the counselor do FIRST?

- A.** Inform the client of the intended collection step and give a chance to pay the balance ✓ **KEY**
A.10.d requires counselors to inform clients of intended collection actions and offer an opportunity to pay first.
- B.** Send the account to the agency right away, since the client has already ended counseling
Ending services does not remove the duty to notify the client before using collection measures.
- C.** Write off the debt, because counselors may not pursue payment from any current or past client
The Code permits collection agencies or legal measures when clients are informed first.
- D.** Give the agency the client's diagnosis and session notes so it can justify the charges
Only the minimum information needed for collection is disclosed; clinical details are not shared.

Rationale: Under A.10.d, counselors who intend to use collection agencies or legal measures for nonpayment first tell the client what they intend and give the client an opportunity to pay. Fee and billing arrangements, including how nonpayment is handled, also belong in the initial informed consent (A.2.b). Any disclosure to a collector is limited to what is needed.

References: ACA Code of Ethics — A.10.d Nonpayment of Fees

☐ Approve ☐ Revise ☐ Retire Notes:

In a rural farming community where trading services is common, a client who recently lost his job asks whether he could repair the counselor's office roof in exchange for sessions. Under the ACA Code of Ethics, when is bartering acceptable?

A. Whenever the counselor sets the value of the work, since the counselor bears responsibility

One-sided valuation by the counselor increases the risk of exploitation.

B. Only when the counselor proposes it first, so that the client does not feel pressured to offer

The Code says the client should request the arrangement, not the counselor.

C. If the client requests it, it is not exploitive, it is accepted locally, and it is in writing ✓ **KEY**

A.10.e lists client request, no exploitation or harm, and community acceptance, with a written contract.

D. Only for clients with no insurance, as long as a supervisor signs off on the trade in advance

Insurance status and supervisor approval are not the conditions the Code sets for bartering.

Rationale: A.10.e allows bartering only when the client requests it, the arrangement does not result in exploitation or harm, and such arrangements are an accepted practice among professionals in the community. Counselors consider cultural implications, discuss concerns with the client, and document the agreement in a clear written contract.

References: ACA Code of Ethics — A.10.e Bartering

☐ Approve

☐ Revise

☐ Retire

Notes:

27. nce-s-eth-027 · Professional Counseling Orientation and Ethical Practice · termination and referral · easy · In review

A counselor accepts a job in another state and will close her practice in two months. Several of her clients are still working on active treatment goals. Which action BEST avoids client abandonment?

A. Stop scheduling sessions now and mail each client a letter after the office has closed

Ending contact abruptly without preparation or referral is a form of abandonment.

B. Keep seeing clients until the last day and leave referrals to the clients' own judgment

Counselors help arrange continued care rather than leaving clients to find it alone.

C. Ask clients to keep texting her after the move so that their counseling can continue informally

Informal contact across state lines raises licensure and boundary problems, not continuity.

D. Tell clients early, provide pretermination counseling, and help arrange suitable referrals ✓ **KEY**

A.11.c and A.12 call for timely notice, pretermination counseling, and referral to other providers.

Rationale: A.12 prohibits abandoning or neglecting clients and requires arrangements for continuation of treatment during interruptions such as relocation. A.11.c and A.11.d direct counselors to provide pretermination counseling, recommend other providers, and coordinate an appropriate transfer of services.

References: ACA Code of Ethics — A.12 Abandonment and Client Neglect · ACA Code of Ethics — A.11.c Appropriate Termination

☐ Approve

☐ Revise

☐ Retire

Notes:

A counselor receives a subpoena from the attorney for a client's former spouse, demanding the client's full counseling file for a custody hearing. The client has not signed a release. What is the counselor's BEST course of action?

A. Send the complete file at once, since a subpoena carries the same force as a court order

An attorney-issued subpoena generally does not by itself authorize release of privileged records.

B. Contact the client, seek legal advice, and release nothing without consent or a court order ✓ **KEY**

B.2.d and state privilege law favor client consent or a judicial order, with disclosure limited.

C. Ignore the subpoena, because counseling records are privileged and cannot be required

Ignoring a subpoena can bring legal penalties; privilege must be asserted, not assumed.

D. Call the former spouse's attorney to discuss the client's progress and resolve it informally

Discussing the client with the opposing attorney is itself a breach of confidentiality.

Rationale: A subpoena is a demand that must be answered, but it is not the same as a judge's order to disclose. Counselors notify the client, consult an attorney, and assert privilege when appropriate. Under B.2.d, even when a court orders disclosure, counselors seek the client's written consent or ask that disclosure be prohibited or limited as narrowly as possible. State law varies.

References: ACA Code of Ethics — B.2.d Court-Ordered Disclosure · State law — Privileged communication and response to subpoenas vs court orders

► **Check:** Verify that the subpoena-vs-court-order distinction is stated accurately enough for a general (non-state-specific) NCE item.

☐ Approve ☐ Revise ☐ Retire Notes:

Three months after a long-term client dies of natural causes, her adult son calls the counselor and asks for copies of his mother's session notes so he can "understand her better." What should guide the counselor's response?

A. Release the notes, since confidentiality obligations end once a client has died

B.3.f states that counselors protect the confidentiality of deceased clients.

B. Release the notes, since any adult child is the legal heir to a parent's own records

Family relationship alone does not confer legal authority over a decedent's records.

C. Protect her confidentiality, following state law on who may act for the deceased ✓ **KEY**

B.3.f protects deceased clients' confidentiality consistent with law and agency policy.

D. Share a verbal summary instead, since a summary does not breach confidentiality

A summary is still a disclosure of confidential information without valid authorization.

Rationale: B.3.f directs counselors to protect the confidentiality of deceased clients, consistent with legal requirements and agency or setting policies. Typically only a legally authorized personal representative, such as an executor, may authorize release, and who qualifies is set by state law, which varies.

References: ACA Code of Ethics — B.3.f Deceased Clients · State law — Personal representative authority over a deceased person's records

☐ Approve ☐ Revise ☐ Retire Notes:

A licensed counselor wants advice from a peer consultation group about a difficult case involving a well-known local business owner. The client has not given consent to discuss the case. How should the counselor proceed?

A. Present the case without details that could reasonably identify the client to the group ✓ KEY

B.7 bars identifying disclosures in consultation without consent, sharing only what is needed.

B. Share the client's name, since members of a consultation group are bound by confidentiality

Peer consultants do not share the counselor's duty in a way that permits identifying the client.

C. Avoid consultation altogether, since any discussion of a case breaches client confidentiality

Consultation is encouraged; it is done with de-identified information or with consent.

D. Share full details but ask the group to sign a confidentiality agreement before the meeting

A group agreement does not replace client consent for identifying disclosures.

Rationale: In case consultation, counselors do not disclose information that could reasonably lead to the client's identification unless the client has consented or disclosure cannot be avoided, and they share only what is needed for the consultation. With a well-known client, extra care is needed to remove identifying details.

References: ACA Code of Ethics — B.7 Case Consultation

☐ Approve

☐ Revise

☐ Retire

Notes:

31. nce-s-eth-031 · Professional Counseling Orientation and Ethical Practice · confidentiality and its limits · medium · In review

A counselor is about to begin couples counseling with two partners in their fifties. To prevent problems later, what should the counselor do regarding confidentiality at the outset?

A. Promise each partner that anything said in an individual call will stay secret from the other

Blanket secret-keeping promises can undermine the couple work and should not be made by default.

B. Tell both partners that the counselor will decide case by case what to reveal between them

An unannounced, case-by-case approach leaves clients unable to give informed consent.

C. Skip the topic, since confidentiality rules do not apply when both partners are present

B.4.b requires discussing confidentiality expectations and limits in couples work.

D. Define who the client is and get a written agreement on how confidences will be handled ✓ KEY

B.4.b asks counselors to define the client and document agreement on confidentiality.

Rationale: In couples and family counseling, B.4.b directs counselors to clearly define who is considered the client and to discuss expectations and limits of confidentiality. Counselors seek agreement among all parties, document it in writing, and A.8 adds that the nature of each relationship is clarified at the outset.

References: ACA Code of Ethics — B.4.b Couples and Family Counseling · ACA Code of Ethics — A.8 Multiple Clients

☐ Approve

☐ Revise

☐ Retire

Notes:

A counselor is forming a 10-week psychoeducational group on managing workplace stress. According to the ACA Code of Ethics, what is the main purpose of screening prospective members?

A. To make sure the group is full before the first meeting so the program stays funded

Enrollment targets are not the ethical purpose of screening.

B. To select members whose goals fit the group and whose well-being will not be at risk ✓ **KEY**

A.9.a calls for selecting compatible members who will not impede the group or be harmed.

C. To collect enough personal history that members need not share anything in sessions

Screening is about fit and safety, not replacing disclosure during the group.

D. To exclude any applicant who has a mental health diagnosis from a psychoeducation group

A diagnosis alone is not a reason to exclude; fit with group goals is the standard.

Rationale: A.9.a directs counselors to screen prospective group members to the extent possible, selecting members whose needs and goals are compatible with the group's goals, who will not impede the group process, and whose well-being will not be jeopardized by the group experience.

References: ACA Code of Ethics — A.9.a Screening · Yalom & Leszcz (Group Psychotherapy) — Selection of clients for group therapy

☐ Approve ☐ Revise ☐ Retire Notes: _____

During the fourth session of an adolescent anger-management group, several members begin mocking a quieter member's accent, and she starts to cry. What is the leader's MOST appropriate response?

A. Intervene at once to stop the mocking and protect the member, then process what happened ✓ **KEY**

A.9.b requires reasonable precautions to protect group members from emotional harm.

B. Let the group handle it alone, since peer feedback is how adolescent groups mature over time

Allowing ridicule to continue fails the leader's duty to protect members.

C. Ask the upset member to leave until she is calm enough to rejoin the group discussion

Removing the targeted member punishes her rather than addressing the harmful behavior.

D. Note the incident and raise it with each member's parents after the session has ended

Delaying action leaves the member unprotected during the session.

Rationale: In a group setting, A.9.b directs counselors to take reasonable precautions to protect clients from physical, emotional, or psychological trauma. Blocking harmful interactions is a core leader skill; once safety is restored, the incident can be processed, including the bias it reflects.

References: ACA Code of Ethics — A.9.b Protecting Clients · Yalom & Leszcz (Group Psychotherapy) — Leader role in establishing group norms and safety

☐ Approve ☐ Revise ☐ Retire Notes: _____

An agency director asks a newly licensed counselor to administer and interpret an individually administered intelligence test. The counselor has never been trained on this instrument. What should the counselor do?

A. Administer it after reading the manual, since the license covers all forms of assessment

Reading a manual does not establish competence with a complex individual test.

B. Administer it but have the computer scoring report serve as the clinical interpretation

Counselors remain responsible for interpretation even when scoring is automated (E.9).

C. Decline until trained and supervised in its use, and refer the testing to a qualified peer ✓ KEY

E.2.a limits counselors to assessments they are trained and competent to use.

D. Administer it, since the director has the authority to approve staff for any test that is needed

An employer cannot confer competence; the ethical duty rests with the counselor.

Rationale: E.2.a states that counselors use only those testing and assessment services for which they have been trained and are competent. Counselors who rely on technology-assisted interpretation are trained in the construct and the instrument before using it. Until competent, the counselor refers the assessment.

References: ACA Code of Ethics — E.2.a Limits of Competence · Hays (Assessment) — Test user qualifications

☐ Approve ☐ Revise ☐ Retire Notes: _____

A client involved in a disability claim signs a release asking the counselor to send her raw personality test responses and scores directly to her attorney, who has no training in psychological testing. How should the counselor respond under the ACA Code of Ethics?

A. Send the raw data, since a signed release obligates the counselor to send anything asked for

Consent is necessary but E.4 also limits release to qualified recipients.

B. Refuse any release, since test results may not leave the counselor's office in any form

Results can be released with consent; the issue is who receives raw data.

C. Send the raw data only after the attorney promises not to share it with the opposing side

A promise of nondisclosure does not make an untrained recipient qualified to interpret.

D. Offer a written interpretive report, or send raw data to a professional qualified to use it ✓ KEY

E.4 releases identified data with consent only to persons qualified to interpret them.

Rationale: E.4 states that counselors release assessment data in which the client is identified only with consent of the client or a legal representative, and only to persons the counselor recognizes as qualified to interpret the data. A report written for lay readers, or release to a qualified professional, meets both the client's need and the standard.

References: ACA Code of Ethics — E.4 Release of Data to Qualified Personnel · HIPAA — Individual right of access to the designated record set

► **Check:** Verify how E.4 interacts with HIPAA and state client-access rights to raw test data, which may require release to the client.

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor plans to use a depression inventory with a client who immigrated from Vietnam a year ago and is learning English. The inventory was normed mainly on U.S.-born English speakers. What is the BEST ethical approach?

A. Use the inventory as usual, since depression symptoms appear the same across all cultures
Symptom expression and item meaning can differ across cultures and languages.

B. Use it with caution, note its norm limits, and weigh results with interviews and context ✓ KEY
E.8 calls for cautious use of measures normed on other groups and cultural context.

C. Translate the items on the spot and score the client's answers with the published norms
An informal translation changes the test, so the published norms no longer apply.

D. Skip all formal assessment, since no standardized test can be fair to a recent immigrant
Assessment can be used with appropriate caution; avoiding it entirely is not required.

Rationale: E.8 directs counselors to select and use with caution assessment techniques normed on populations other than the client's, and to recognize the effects of culture, language, and other factors on administration and interpretation. Results are placed in the context of other information, and limits are noted in the report.

References: ACA Code of Ethics — E.8 Multicultural Issues/Diversity in Assessment · Hays (Assessment) — Culturally fair assessment and norm-group relevance

☐ Approve ☐ Revise ☐ Retire Notes:

37. nce-s-eth-037 · Professional Counseling Orientation and Ethical Practice · supervision and consultation responsibilities · hard · In review

A site supervisor has given a post-master's supervisee written feedback for six months about missed risk assessments and late notes. Despite a written remediation plan, the problems continue. What is the supervisor's MOST appropriate next step?

A. Document the deficits, consult, and decline to endorse licensure while following due process ✓ KEY
F.6.b supports remediation, consultation, and withholding endorsement with due process.

B. Endorse the supervisee for licensure anyway, since all of the required hours have been completed
Hours alone do not justify endorsing a supervisee who is not competent.

C. Quietly end supervision and let the supervisee find another supervisor without a record
Ending without documentation or due process sidesteps the gatekeeping role.

D. Report the supervisee to the licensing board for misconduct without telling the supervisee
Performance deficits call for documented gatekeeping steps, not a secret misconduct report.

Rationale: Supervisors act as gatekeepers. Under F.6.b, they identify limitations that could impede performance, help secure remediation, and when needed recommend dismissal or decline to endorse supervisees who are unable to provide competent services. They consult, document their decisions, and ensure the supervisee knows the options for addressing the decision.

References: ACA Code of Ethics — F.6.b Gatekeeping and Remediation

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor educator teaching a group counseling course requires students to take part in an experiential group as members. How should the educator handle evaluation of students in this experience?

- A. Grade each student on how much personal material the student discloses in the group
Basing grades on depth of self-disclosure is what the Code prohibits.
- B. Share notable disclosures with faculty so they can assess students' fitness for practice
Students should know the risks of self-disclosure; casual sharing of it misuses the group.
- C. Keep grades apart from the level of self-disclosure and explain the risks of disclosing ✓ KEY
Section F requires that evaluation not depend on self-disclosure and that risks be explained.
- D. Require that students disclose a past trauma, since depth is needed for group learning
Requiring specific personal disclosures exploits students and is not required to learn.

Rationale: When educational programs include self-growth or self-disclosure experiences, counselor educators inform students of the ethical ramifications of self-disclosure, and evaluation is not dependent on the student's level of self-disclosure. Many programs also use outside facilitators so the instructor does not evaluate what students reveal.

References: ACA Code of Ethics — Section F, self-growth experiences in counselor education · CACREP 2024 Standards — Experiential group learning in the group counseling core area

► **Check:** Confirm the exact ACA 2014 subsection number for self-growth experiences in Section F (believed to be F.8.c) before citing it.

☐ Approve ☐ Revise ☐ Retire Notes:

39. nce-s-eth-039 · Professional Counseling Orientation and Ethical Practice · telehealth and technology ethics · easy · In review

After a session in which a new client mentions a popular online video channel she runs, the counselor is tempted to look up the channel and the client's public social media posts. What does the ACA Code of Ethics advise?

- A. Viewing is fine, since anything posted publicly by a client is open to the counselor
Public availability does not override the Code's respect for client privacy online.
- B. Viewing is fine if the counselor does not comment, like, or follow the client's posts
Passive viewing without consent still intrudes on the client's virtual presence.
- C. Viewing is required, since it gives collateral information useful for diagnosis
There is no duty to search clients online, and doing so needs consent.
- D. Respect the client's online privacy and view it only if the client gives consent ✓ KEY
H.6.c directs counselors to respect clients' social media privacy unless given consent.

Rationale: H.6.c directs counselors to respect the privacy of clients' presence on social media unless given consent to view such information. Counselors also explain their own social media policies as part of informed consent (H.6.b).

References: ACA Code of Ethics — H.6.c Client Virtual Presence

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in solo practice is retiring and has paper files for clients seen over the past 15 years. How should the counselor handle retention and disposal of these records?

A. Discard all files in the office recycling bin once the practice has officially closed

Recycling bins expose confidential records and ignore retention requirements.

B. Keep files for the period state law requires, then destroy them in a secure manner ✓ **KEY**

B.6.h links retention to law and requires disposal that protects confidentiality.

C. Give each file to a family member to store at home until the clients ask for them

Untrained family members lack authority and safeguards for client records.

D. Shred all files at once, since a closed practice has no further duty for its records

Retention duties continue after closing; immediate destruction may violate law.

Rationale: B.6.h directs counselors to store records after termination to ensure reasonable future access, to maintain them in accordance with federal and state law and licensure rules, and to dispose of them in a way that protects confidentiality, such as shredding. Retention periods are set by state law and vary. Counselors also plan for records in case of retirement or incapacity.

References: ACA Code of Ethics — B.6.h Storage and Disposal After Termination · State law — Record retention periods for licensed counselors

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor at a state agency is told that a new agency rule requires sharing full session notes with a client's employer whenever the employer pays for services. The counselor believes this conflicts with the ACA Code of Ethics. What should the counselor do FIRST?

A. Follow the rule without comment, since a legal requirement replaces any ethical code

I.1.c first calls for stating the ethical commitment and trying to resolve the conflict.

B. Make known the commitment to the Code and take steps to resolve the conflict ✓ **KEY**

I.1.c directs counselors to state their commitment and work to resolve the conflict.

C. Refuse to follow the rule, since ethical codes take legal priority over agency rules

Ethical codes do not override law; outright refusal skips the steps the Code sets out.

D. Stop taking notes so that there is no record that could be given to employers

Failing to document violates record-keeping duties and does not resolve the conflict.

Rationale: Under I.1.c, when ethical responsibilities conflict with laws, regulations, or other legal authority, counselors make known their commitment to the ACA Code and take steps to resolve the conflict. If it cannot be resolved that way, counselors, acting in the client's best interest, may adhere to the legal requirement.

References: ACA Code of Ethics — I.1.c Conflicts Between Ethics and Laws

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor learns that a colleague has been billing insurers for family sessions that never took place. When she raised it privately, the colleague dismissed her concern and the billing continued. What should the counselor do NEXT?

A. Report it to the licensing board or another proper authority, since informal efforts failed ✓ KEY

I.2.b calls for formal reporting when informal resolution fails or harm is likely.

B. Raise it with the colleague again every month, since informal resolution must still be continued

The Code does not require repeating failed informal efforts while harm continues.

C. Warn the colleague's clients about the billing so that they can decide to change counselors

Contacting the colleague's clients is not an appropriate reporting channel.

D. Drop the matter, since billing disputes are between insurers and providers, not counselors

Fraud is an ethical violation counselors are expected to address.

Rationale: I.2.a supports informal resolution first when appropriate. When an apparent violation has substantially harmed or is likely to harm others and is not appropriate for, or not resolved by, informal resolution, I.2.b directs counselors to take further action such as referral to state licensure boards, ethics committees, certification bodies, or institutional authorities.

References: ACA Code of Ethics — I.2.b Reporting Ethical Violations · ACA Code of Ethics — I.2.a Informal Resolution

☐ Approve ☐ Revise ☐ Retire Notes:

A licensed professional counselor holds a master's in counseling and a doctorate in music history. On her practice website she calls herself "Dr." and lists her credentials. What does the ACA Code of Ethics say about this?

A. It is acceptable, since any earned doctorate allows the title "Dr." in every context

C.4.d restricts using "Dr." in a counseling context when the doctorate is unrelated.

B. It is acceptable if the website also lists her current state counseling license number too

A license number does not correct a misleading implication of doctoral-level competence.

C. It is acceptable as long as no client has ever complained that the title was misleading to them

Misrepresentation is unethical whether or not anyone complains.

D. It is misleading, because the doctorate is not in counseling or a closely related field ✓ KEY

C.4.d bars implying doctoral-level competence with an unrelated doctorate.

Rationale: C.4.a requires accurate representation of credentials, and C.4.d states that counselors do not imply doctoral-level competence by using "Dr." in a counseling context when their doctorate is not in counseling or a related field. C.3.a likewise requires advertising to be accurate and not misleading.

References: ACA Code of Ethics — C.4.d Implying Doctoral-Level Competence · ACA Code of Ethics — C.3.a Accurate Advertising

☐ Approve ☐ Revise ☐ Retire Notes:

A 78-year-old client with a terminal illness tells his counselor he is considering ending his life through his state's legal medical aid-in-dying process. He has no other risk factors. Under the ACA Code of Ethics, how should the counselor approach confidentiality?

A. Disclose the plan to his family at once, since any talk of dying requires a breach of trust

B.2.b gives counselors discretion; automatic disclosure is not required.

B. Arrange involuntary hospitalization, since his choice must be treated as suicidal

A lawful end-of-life choice is not automatically treated as a psychiatric emergency.

C. Use discretion on breaching, guided by consultation, law, and the client's situation ✓ **KEY**

B.2.b gives counselors discretion, informed by consultation and applicable law.

D. Refer him to another counselor, since end-of-life choices fall outside counseling

Supporting end-of-life decision making is within the counselor's role.

Rationale: B.2.b gives counselors who work with terminally ill clients considering hastening death the option to maintain confidentiality, depending on applicable laws and the specific circumstances, after seeking consultation or supervision from appropriate professional and legal parties. Counselors also help clients explore options, values, and supports. State law on medical aid in dying varies.

References: ACA Code of Ethics — B.2.b Confidentiality Regarding End-of-Life Decisions · State law — Medical aid-in-dying statutes (legal in some states only)

► **Check:** Confirm the 2014 code location B.2.b for end-of-life confidentiality (the 2005 code placed end-of-life care at A.9).

☐ Approve ☐ Revise ☐ Retire Notes:

A 24-year-old client from a close-knit Mexican American family lives at home and gives part of his pay to his parents. The counselor, who values independence, begins urging him to move out to "become his own person." The client has not named this as a goal. What is the ethical concern?

A. The counselor is right to push independence, since adulthood requires leaving home

This is a culturally bound assumption, not a universal developmental requirement.

B. The counselor is imposing personal values instead of exploring the client's own ✓ **KEY**

A.4.b bars imposing the counselor's values; goals should reflect the client's worldview.

C. The counselor should refer him, since cultural differences make counseling unworkable

Cultural difference is not grounds for referral; the counselor builds competence instead.

D. The concern is billing, since family issues are not covered as a reason for counseling

Billing is unrelated to the ethical issue of value imposition.

Rationale: A.4.b directs counselors to be aware of, and avoid imposing, their own values, attitudes, and beliefs. Many collectivist families see interdependence and financial support of parents as healthy. Goals should come from the client, explored within the client's cultural context (A.2.c, A.1.c).

References: ACA Code of Ethics — A.4.b Personal Values · Sue & Sue (Counseling the Culturally Diverse) — Individualism vs collectivism and culture-bound counseling values

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor researcher is preparing a consent form for a study of coping in caregivers of people with dementia. Which statement is a required element of informed consent for research participants?

A. Participants may decline or withdraw at any time without penalty ✓ KEY

G.2.a lists the freedom to withdraw without penalty as a required element.

B. Participants must finish the study once they have signed the form

Participation is voluntary throughout; withdrawal must be allowed.

C. Participants give up the right to see results in exchange for pay

Payment does not remove participant rights, and results may be shared.

D. Participants are told the hypotheses only after data have been used

Purposes and procedures are explained up front, except in justified deception.

Rationale: G.2.a requires researchers to explain the purpose and procedures, risks, benefits, alternatives, limits of confidentiality, the format and audience for results, and that participants are free to withdraw at any time without penalty. Consent is an ongoing process, not a one-time signature.

References: ACA Code of Ethics — G.2.a Informed Consent in Research · Erford (Research and Evaluation in Counseling) — Informed consent and voluntary participation

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor in a community agency plans to survey clients about service satisfaction and later publish the findings. The agency has access to a university institutional review board (IRB). Which step is MOST appropriate before collecting data?

A. Begin the survey now, since satisfaction surveys of clients carry no risk to them

Research with clients involves risks, including pressure to participate and privacy.

B. Ask clients to put their names on surveys so their answers can be checked later

Collecting names without need raises risk; data should be de-identified when possible.

C. Collect the data first and seek a review only if the results turn out to be publishable

Review occurs before data collection, not after results are known.

D. Seek IRB review and plan how consent is obtained and how data will be kept private ✓ KEY

Section G calls for review of research and for protecting participants' consent and privacy.

Rationale: Section G directs counselors to follow applicable law and institutional review requirements, to obtain informed consent, and to protect the confidentiality of participant information. Planned publication makes this research rather than routine program evaluation, so IRB review is sought before data collection, and data are de-identified and stored securely.

References: ACA Code of Ethics — Section G, Research and Publication (institutional review and confidentiality of information) · Erford (Research and Evaluation in Counseling) — Institutional review boards and protection of participants

► **Check:** Check whether a specific G subsection (e.g., G.1.b or G.2.d) should be cited for IRB review and data confidentiality.

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor's outcome study finds that a group program her agency promotes did not reduce symptoms more than a waitlist. The agency director suggests leaving these results out of the annual report. What should the counselor do?

A. Omit the findings, since agency staff must support the programs their employer runs

Loyalty to an employer does not permit withholding results of professional value.

B. Adjust the analysis until the program shows a positive effect worth reporting on

Manipulating analyses to produce a result is a form of data distortion (G.4.a).

C. Report the results accurately, since unfavorable findings may not be withheld ✓ **KEY**

G.4.b states that results reflecting unfavorably on programs are not withheld.

D. Report only the subgroups in which scores improved and describe them as overall gains

Selective reporting misrepresents results and violates G.4.a.

Rationale: G.4.a requires counselors to plan, conduct, and report research accurately without distorting data or biasing results. G.4.b states that results of professional value are reported, and results that reflect unfavorably on institutions, programs, services, or vested interests are not withheld.

References: ACA Code of Ethics — G.4.b Obligation to Report Unfavorable Results · ACA Code of Ethics — G.4.a Accurate Results

☐ Approve ☐ Revise ☐ Retire Notes:

Intake, Assessment, and Diagnosis

48 items · NCE weight 12% (19 of 160 scored items)

49. nce-s-ass-001 · Counseling and Helping Relationships · intake interview structure · easy · In review

A counselor at a community agency begins a first session with a 34-year-old client who has been referred for "stress." Before asking about presenting concerns or history, what should the counselor address?

A. A full family genogram covering at least three generations

A genogram can be useful later, but it is not the opening task and does not secure informed consent.

B. Informed consent, including the limits of confidentiality ✓ **KEY**

Clients need to know their rights, the nature of services, and the limits of confidentiality before disclosing sensitive information.

C. A provisional DSM-5-TR diagnosis for the referral form

A diagnosis requires assessment data that have not yet been gathered; it cannot come first.

D. Long-term treatment goals the client hopes to reach

Goal setting follows assessment of the presenting problem; it is premature at the very start.

Rationale: Intake interviews typically open with informed consent and an explanation of confidentiality and its limits, then move to presenting problem, history, mental status, risk, and goals. Discussing limits first lets the client make an informed choice about what to disclose.

References: ACA Code of Ethics — A.2.a Informed Consent; B.1.d Explanation of Limitations

☐ Approve ☐ Revise ☐ Retire Notes:

50. nce-s-ass-002 · Counseling and Helping Relationships · mental status examination · hard · In review

When asked why he came in, a 61-year-old client describes his commute, the parking garage, a conversation with a neighbor, and the weather in great detail before finally explaining that he has been sleeping poorly. How should the counselor record this in the mental status examination?

A. Tangentiality, noted under thought process

Tangential speech drifts away and does not return to the original question; this client does return to it.

B. Flight of ideas, noted under thought content

Flight of ideas involves rapid, accelerated shifts, usually with pressured speech, and is a thought process finding, not content.

C. Circumstantiality, noted under thought process ✓ **KEY**

Excessive, irrelevant detail that eventually reaches the point is circumstantiality, a disturbance of thought process.

D. Perseveration, noted under perceptual disturbance

Perseveration is repetition of the same response, and it is not a perceptual finding.

Rationale: The MSE separates thought process (how ideas connect) from thought content (what the person thinks about). Circumstantial speech is roundabout but goal-directed in the end; tangential speech never gets back to the goal. Both are recorded under thought process.

References: Hays (Assessment) — Mental status examination: thought process

☐ Approve ☐ Revise ☐ Retire Notes:

During intake, a 19-year-old college student says, "Sometimes I wish I could just go to sleep and not wake up." What should the counselor do FIRST?

A. Ask directly about thoughts of suicide, plans, intent, and means ✓ KEY

Passive death wishes call for direct, specific questions about ideation, plan, intent, means, and past behavior to estimate risk.

B. Avoid the subject so as not to put the idea of suicide in her mind

Asking about suicide does not increase risk; avoiding it leaves risk unassessed.

C. Arrange an involuntary hospitalization before the session ends

Level of care is decided after assessment; a passive wish alone does not establish need for involuntary admission.

D. Refer her to a psychiatrist and end the intake for the day

Referral without assessment abandons the risk question and may leave the client without support.

Rationale: A wish to be dead is the lowest rung of suicidal ideation on structured tools such as the C-SSRS, but it requires follow-up. The counselor asks plainly about active thoughts, method, intent, plan, preparatory behavior, and prior attempts, then decides on next steps based on that picture.

References: C-SSRS — Ideation severity: wish to be dead through active ideation with plan and intent

☐ Approve ☐ Revise ☐ Retire Notes:

A 47-year-old veteran reports recurring suicidal thoughts without a current plan or intent. He mentions that he keeps a loaded handgun in his nightstand. Which step is MOST important to include in his safety plan?

A. A written promise that he will not harm himself before next week

No-harm contracts lack evidence of protective value and do not address lethal means.

B. A list of reasons for living to read when he feels hopeless

Reasons for living can help, but they do not reduce access to a highly lethal method.

C. A plan to increase his exercise and sleep routine this month

Wellness activities are reasonable adjuncts but leave the most urgent risk factor untouched.

D. Lethal means counseling to store the gun outside the home ✓ KEY

Firearm access sharply raises lethality; collaborative means restriction is a core safety-plan step.

Rationale: Firearms are the most lethal common method of suicide, and reducing access during high-risk periods saves lives. The Stanley-Brown Safety Planning Intervention includes making the environment safe, such as having a trusted person hold a firearm or storing it locked away from home.

References: Stanley-Brown SPI — Step 6 (final step): Making the environment safe, including lethal means

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor trained years ago asks a colleague where to record the Axis V Global Assessment of Functioning (GAF) score when using DSM-5-TR. What is the BEST answer?

A. On Axis V, as in earlier versions of the manual

DSM-5-TR has no Axis V; the multiaxial system is gone.

B. On Axis IV, together with psychosocial stressors

There are no axes in DSM-5-TR, so Axis IV does not exist either.

C. Nowhere; DSM-5-TR uses nonaxial documentation ✓ **KEY**

DSM-5 dropped the multiaxial system and the GAF; diagnoses are listed together without axes.

D. In a required appendix scored from zero to 100

No GAF appendix exists; the GAF was removed from the manual.

Rationale: DSM-5 and DSM-5-TR replaced the five-axis system with nonaxial documentation. Psychosocial and contextual factors are noted with codes from "Other Conditions That May Be a Focus of Clinical Attention," and the GAF was dropped; the WHODAS 2.0 is offered as a measure of disability.

References: DSM-5-TR — Section I: Use of the Manual (nonaxial documentation)

☐ Approve ☐ Revise ☐ Retire Notes:

A 28-year-old client currently meets criteria for a major depressive episode. Two years ago she had a 5-day period of elevated mood, decreased need for sleep, and rapid speech that friends noticed, with no marked impairment, hospitalization, or psychotic features. She has had no other mood episodes. Which diagnosis BEST fits?

A. Major depressive disorder, recurrent

A past hypomanic episode rules out major depressive disorder.

B. Bipolar II disorder ✓ **KEY**

At least one hypomanic episode plus at least one major depressive episode, with no manic episode, defines bipolar II.

C. Bipolar I disorder

Bipolar I requires a manic episode, which lasts at least one week (or any duration if hospitalized) and causes marked impairment, hospitalization, or psychosis.

D. Cyclothymic disorder

Cyclothymia requires two years of subthreshold symptoms and is excluded once a major depressive episode occurs.

Rationale: Hypomania lasts at least four consecutive days, is observable by others, and does not cause marked impairment or require hospitalization. Mania lasts at least a week (or any duration if hospitalized) and is severe enough to cause marked impairment, require hospitalization, or include psychotic features. A hypomanic episode plus a major depressive episode equals bipolar II.

References: DSM-5-TR — Bipolar II Disorder criteria; Hypomanic Episode Criterion A

☐ Approve ☐ Revise ☐ Retire Notes:

A test developer gives the same anxiety inventory to the same group of adults twice, two weeks apart, and correlates the two sets of scores. Which type of reliability is being estimated?

A. Test-retest reliability ✓ **KEY**

Correlating scores from the same test given twice to the same people estimates stability over time.

B. Alternate-form reliability

Alternate-form reliability uses two different but equivalent versions of the test.

C. Split-half reliability

Split-half reliability comes from one administration divided into two halves.

D. Interrater reliability

Interrater reliability is agreement between two or more scorers or observers.

Rationale: Test-retest reliability (the coefficient of stability) reflects how consistent scores are across time. The interval matters: too short invites practice and memory effects, too long lets real change lower the coefficient.

References: Hays (Assessment) — Reliability: test-retest (coefficient of stability)

☐ Approve ☐ Revise ☐ Retire Notes:

A school counselor has a single administration of a 40-item vocabulary test in which every item is scored simply right or wrong. Which statistic is BEST suited to estimate the test's internal consistency?

A. A test-retest correlation

Test-retest requires two administrations and measures stability, not internal consistency.

B. Cohen's kappa coefficient

Kappa indexes agreement between raters, corrected for chance.

C. An alternate-form coefficient

Alternate-form reliability needs a second, equivalent version of the test.

D. Kuder-Richardson Formula 20 ✓ **KEY**

KR-20 estimates internal consistency from one administration of dichotomously scored items.

Rationale: Internal consistency estimates come from one administration. KR-20 is used with dichotomous (right/wrong) items; coefficient (Cronbach's) alpha generalizes it to items with more than two score points. Split-half reliability, corrected with the Spearman-Brown formula, is another single-administration option.

References: Erford (Research and Evaluation in Counseling) — Internal consistency: KR-20 and coefficient alpha

☐ Approve ☐ Revise ☐ Retire Notes:

Developers of a new mechanical aptitude test give it to newly hired technicians and correlate those scores with supervisor performance ratings collected 18 months later. What type of validity evidence does this provide?

A. Concurrent validity

Concurrent validity uses a criterion measured at about the same time as the test.

B. Predictive validity ✓ KEY

The criterion is measured well after testing, so this is criterion-related predictive validity.

C. Content validity

Content validity concerns how well items sample the domain, judged by experts, not a criterion correlation.

D. Discriminant validity

Discriminant validity is construct evidence showing low correlation with unrelated constructs.

Rationale: Criterion-related validity has two forms that differ by timing: concurrent (criterion gathered at the same time) and predictive (criterion gathered later). Correlating test scores with later job ratings shows how well the test forecasts future performance.

References: Hays (Assessment) — Validity: criterion-related (concurrent and predictive)

☐ Approve

☐ Revise

☐ Retire

Notes:

A new depression scale correlates .78 with an established depression inventory and .06 with a measure of musical aptitude. The low correlation with musical aptitude is BEST described as evidence of which type of validity?

A. Convergent validity

Convergent evidence is the high correlation with the established depression inventory, not the low one.

B. Predictive validity

Predictive validity concerns forecasting a later criterion, which is not described here.

C. Discriminant validity ✓ KEY

A low correlation with a theoretically unrelated construct supports discriminant (divergent) validity.

D. Face validity

Face validity is whether a test looks like it measures its target; it is not shown by correlations.

Rationale: Construct validity is supported by convergent evidence (strong relations with measures of the same construct) and discriminant evidence (weak relations with measures of different constructs). The multitrait-multimethod matrix examines both together.

References: Erford (Research and Evaluation in Counseling) — Construct validity: convergent and discriminant evidence

☐ Approve

☐ Revise

☐ Retire

Notes:

A district reading test reports, for each fourth grader, whether the student has mastered at least 80% of the grade-level reading objectives, regardless of how classmates performed. What kind of test is this?

A. A norm-referenced test

Norm-referenced tests compare a score with a norm group, which this report does not do.

B. An ipsative measure

Ipsative measures compare traits within one person, not mastery of a standard.

C. A projective technique

Projective techniques use ambiguous stimuli to explore personality.

D. A criterion-referenced test ✓ KEY

Comparing performance with a fixed standard of mastery is criterion-referenced.

Rationale: Criterion-referenced tests interpret a score against a set standard or body of content (what the person can do). Norm-referenced tests interpret a score against the performance of a comparison group (how the person ranks).

References: Hays (Assessment) — Norm-referenced vs criterion-referenced interpretation

☐ Approve ☐ Revise ☐ Retire Notes:

Scores on a test are normally distributed. Approximately what percentage of test takers score between one standard deviation below the mean and two standard deviations above the mean?

A. About 82% ✓ KEY

About 34% fall between -1 SD and the mean, and about 47.5% between the mean and +2 SD, for roughly 82%.

B. About 68%

68% is the share within plus or minus one SD of the mean.

C. About 95%

95% is the share within plus or minus two SD of the mean.

D. About 48%

About 47.5% lies between the mean and +2 SD alone; the band below the mean is left out.

Rationale: On the normal curve about 68% of scores fall within 1 SD of the mean, about 95% within 2 SD, and about 99.7% within 3 SD. Halving those figures gives the share on one side: about 34% from the mean to 1 SD and about 47.5% from the mean to 2 SD.

References: Erford (Research and Evaluation in Counseling) — The normal curve: 68-95-99.7 rule

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor compares four results from different normally distributed tests. Which result reflects the HIGHEST standing relative to its norm group?

A. A z score of +1.2

A z of +1.2 is below the T score of 65, which equals $z = +1.5$.

B. A deviation IQ of 118

With mean 100 and SD 15, an IQ of 118 equals $z = +1.2$.

C. A T score of 65 ✓ KEY

With mean 50 and SD 10, a T of 65 is 1.5 SD above the mean, the highest of the four.

D. A percentile rank of 88

The 88th percentile corresponds to a z of about +1.17.

Rationale: Converting each score to z allows comparison: $z = (\text{score} - \text{mean}) / \text{SD}$. T scores have a mean of 50 and SD of 10; deviation IQs have a mean of 100 and SD of 15; the 84th percentile is about $z = +1.0$ and the 88th about $z = +1.17$.

References: Hays (Assessment) — Standard scores: z, T and deviation IQ conversions

☐ Approve ☐ Revise ☐ Retire Notes:

On a normally distributed test, one student's percentile rank rose from 50 to 60 and another student's rose from 89 to 99. Which statement about their raw-score gains is accurate?

A. Both students gained about the same number of raw-score points

Percentile ranks are not equal-interval units, so equal percentile gains do not mean equal raw gains.

B. The second student needed a much larger raw-score gain ✓ KEY

Scores thin out in the upper tail, so moving from the 89th to 99th percentile takes far more raw-score distance.

C. The first student needed a much larger raw-score gain

Scores cluster near the mean, so a 10-rank gain there takes a small raw-score change.

D. The gains cannot be compared without first converting to stanines

Stanines are not needed; converting percentiles to z scores shows the difference directly.

Rationale: Percentile ranks are ordinal. Because most scores pile up near the mean, a small raw change in the middle moves the percentile a lot, while the same percentile change in the tail requires a large raw change. Here 50 to 60 is about 0.25 SD; 89 to 99 is about 1.1 SD.

References: Hays (Assessment) — Percentile ranks: ordinal scale and interpretation limits

☐ Approve ☐ Revise ☐ Retire Notes:

A client obtains a score of 92 on a test with a standard error of measurement (SEM) of 4. Which range is the approximate 95% confidence band for the client's true score?

A. 88 to 96

Plus or minus 1 SEM gives a band of about 68% confidence.

B. 90 to 94

Plus or minus half an SEM is far narrower than any standard band.

C. 80 to 104

Plus or minus 3 SEM gives about 99.7% confidence, wider than asked.

D. 84 to 100 ✓ KEY

Plus or minus about 2 SEM (1.96×4 , roughly 8 points) gives the 95% band.

Rationale: The SEM is the standard deviation of a person's hypothetical scores around the true score. Bands of plus or minus 1 SEM, about 1.96 SEM (often rounded to 2), and 3 SEM around the obtained score give roughly 68%, 95%, and 99.7% confidence that the true score lies inside. Here $1.96 \times 4 = 7.84$, so the 95% band is about 84 to 100.

References: Hays (Assessment) — Standard error of measurement and confidence intervals

☐ Approve ☐ Revise ☐ Retire Notes:

A test has a standard deviation of 15 and a reliability coefficient of .91. What is its standard error of measurement?

A. 4.5 points ✓ KEY

$SEM = 15 \times \text{the square root of } (1 - .91) = 15 \times 0.3 = 4.5$.

B. 1.35 points

This multiplies the SD by .09 and omits the square root.

C. 13.65 points

This multiplies the SD by the reliability coefficient itself.

D. 0.3 points

This is only the square root of $(1 - .91)$, not multiplied by the SD.

Rationale: $SEM = SD \times \text{the square root of } (1 - r)$. Higher reliability means a smaller SEM and narrower confidence bands; a perfectly reliable test would have an SEM of zero.

References: Erford (Research and Evaluation in Counseling) — Standard error of measurement formula

☐ Approve ☐ Revise ☐ Retire Notes:

A school-based counselor is part of a team evaluating the intellectual functioning of a 10-year-old. Which instrument is designed for this child's age?

A. WAIS-IV

The WAIS-IV is normed for adults and older adolescents (16 and up).

B. WPPSI-IV

The WPPSI-IV is for preschool and early primary children, below age 8.

C. WISC-V ✓ KEY

The WISC-V covers ages 6 through 16, including a 10-year-old.

D. MMPI-A

The MMPI-A is an adolescent personality inventory, not an intelligence test.

Rationale: The Wechsler scales are split by age: WPPSI-IV for young children (about 2½ to 7½), WISC-V for ages 6 to 16, and WAIS-IV for ages 16 to 90. Each yields a Full Scale IQ with a mean of 100 and SD of 15.

References: Hays (Assessment) — Intelligence testing: Wechsler scales by age range

► **Check:** Check the Wechsler age ranges used in the rationale (WPPSI-IV, WISC-V, WAIS-IV).

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor wants a personality inventory built on the five-factor model (neuroticism, extraversion, openness, agreeableness, and conscientiousness). Which instrument fits this purpose?

A. Rorschach Inkblot Test

The Rorschach is a projective technique, not a five-factor inventory.

B. NEO Personality Inventory-3 ✓ KEY

The NEO-PI-3 measures the five broad domains of the five-factor model and their facets.

C. Myers-Briggs Type Indicator

The MBTI sorts people into Jungian types rather than measuring the five factors.

D. MMPI-3

The MMPI-3 focuses on psychopathology and validity scales, not the five-factor model.

Rationale: The NEO-PI-3 (Costa and McCrae) assesses neuroticism, extraversion, openness, agreeableness, and conscientiousness, each with six facets. The MMPI family is used mainly to assess psychopathology and includes validity scales for over- and underreporting.

References: Hays (Assessment) — Personality inventories: NEO-PI-3 and the five-factor model

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor shows an adolescent a series of ambiguous drawings of people in everyday situations and asks him to tell a story about each one, with a beginning, middle, and end. Which instrument is being used?

A. Rorschach Inkblot Test

The Rorschach uses inkblots, not pictures of people, and asks what the person sees.

B. House-Tree-Person

In House-Tree-Person the client makes the drawings rather than responding to them.

C. Rotter Incomplete Sentences

The Rotter uses sentence stems for the client to finish, not pictures.

D. Thematic Apperception Test ✓ KEY

The TAT presents ambiguous picture cards and asks for a story about each.

Rationale: The Thematic Apperception Test, developed by Morgan and Murray, is a projective technique in which stories told about ambiguous scenes are thought to reveal needs, motives, and conflicts. Projective tests rely on ambiguous stimuli and generally have weaker psychometric support than objective inventories.

References: Hays (Assessment) — Projective techniques: Thematic Apperception Test

☐ Approve ☐ Revise ☐ Retire Notes:

68. nce-s-ass-020 · Assessment and Testing · screening instruments (PHQ-9, GAD-7, AUDIT, CAGE) · medium · In review

On the PHQ-9, a client's total score is 6, in the mild range, but she marks "several days" on the last item, which asks about thoughts that she would be better off dead or of hurting herself. What should the counselor do next?

A. Follow up at once with a direct suicide risk assessment ✓ KEY

Any endorsement of the self-harm item calls for direct risk assessment whatever the total score.

B. Note mild depression and recheck the PHQ-9 next month

Waiting ignores a positive response on the item that flags possible suicidal thinking.

C. Give the GAD-7 to see whether anxiety explains the item

An anxiety screen does not assess or rule out suicide risk.

D. Treat the item as invalid because the total score is low

A low total does not cancel a positive item 9; risk can exist with few other symptoms.

Rationale: The PHQ-9 has nine items scored 0 to 3 (total 0 to 27). Item 9 screens for passive or active thoughts of death or self-harm; any positive answer should be followed by a clinician's direct suicide risk assessment, since a screening total alone does not measure risk.

References: Hays (Assessment) — Screening instruments: Patient Health Questionnaire-9 (PHQ-9)

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old client says that when he goes a day without drinking he gets shaky, sweaty, and cannot sleep. Using the ASAM Criteria (3rd edition), under which dimension should the counselor rate these symptoms?

A. Readiness to change

This dimension rates motivation and engagement, not physical withdrawal.

B. Recovery environment

This dimension rates the client's living situation and supports.

C. Intoxication and withdrawal ✓ KEY

Tremor, sweating, and insomnia on stopping alcohol are withdrawal signs, rated in Dimension 1.

D. Relapse potential

This dimension rates risk of continued use, not current withdrawal.

Rationale: The ASAM Criteria rate six dimensions to match level of care. Dimension 1 covers intoxication and withdrawal; in the widely taught third edition the others address biomedical conditions, emotional/behavioral/cognitive conditions, readiness to change, relapse or continued-use potential, and the recovery environment. The 2023 fourth edition renamed and reorganized several later dimensions, but intoxication and withdrawal remains part of Dimension 1. Alcohol withdrawal can be dangerous and may call for medically managed care.

References: ASAM Criteria — 3rd edition, Dimension 1: acute intoxication and/or withdrawal potential

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is asked to assess the cognitive abilities of a 13-year-old who arrived from Guatemala six months ago and is still learning English. Which approach is MOST appropriate?

A. Give a verbal English IQ test and report the score as usual

The score would mostly reflect English proficiency, so reporting it without caveats is invalid.

B. Use a nonverbal or native-language measure, interpreted with care ✓ KEY

Reducing language load and noting norm limits gives a fairer, more valid picture of ability.

C. Delay all testing until he has lived in the country for five years

A rigid wait is arbitrary and may delay needed services; fairer methods exist now.

D. Add 15 points to his score to adjust for the language barrier

Arbitrary score adjustment has no psychometric basis and distorts results.

Rationale: Counselors select instruments whose norms and language suit the client and interpret results in light of culture, language, and acculturation. When a test was not normed on the client's population, the report should say so. Nonverbal measures or measures in the client's dominant language reduce bias from limited English.

References: ACA Code of Ethics — E.8 Multicultural Issues/Diversity in Assessment · Sue & Sue (Counseling the Culturally Diverse) — Culturally biased assessment and test interpretation

☐ Approve ☐ Revise ☐ Retire Notes:

During a biopsychosocial intake, a 55-year-old client with no prior mental health history describes new anxiety, a racing heart, weight loss despite a good appetite, and feeling hot all the time. What is the counselor's BEST next step?

A. Diagnose generalized anxiety disorder and begin relaxation training

GAD requires that symptoms not be due to a medical condition, which has not been ruled out.

B. Explore childhood experiences that may underlie the new anxiety

This skips the possibility that a medical problem is driving the symptoms.

C. Reassure the client that midlife anxiety is common and expected

Reassurance without evaluation may miss a treatable physical cause.

D. Recommend a medical evaluation to rule out a physical cause ✓ KEY

Late onset with weight loss and heat intolerance suggests a possible medical cause, such as thyroid disease.

Rationale: The biological part of a biopsychosocial history covers health, medications, and physical symptoms. New-onset anxiety in midlife with physical signs such as weight loss and heat intolerance can reflect conditions like hyperthyroidism, so a medical evaluation comes before a mental disorder diagnosis.

References: DSM-5-TR — Anxiety Disorder Due to Another Medical Condition; GAD Criterion E

☐ Approve

☐ Revise

☐ Retire

Notes:

A client being assessed for alcohol use says he "only has a couple of beers on weekends." With his written consent, the counselor also interviews his spouse. What is the PRIMARY value of this collateral information?

A. It can confirm or add to self-report that may be minimized ✓ KEY

Collateral sources help check self-report, which can be limited by minimization or poor insight.

B. It removes the need for a structured diagnostic interview

Collateral data supplement, not replace, a thorough clinical interview.

C. It lets the spouse set the goals of the client's treatment

Treatment goals are set collaboratively with the client, who directs his own care.

D. It supplies the evidence that state law requires to diagnose

Diagnosis does not legally require collateral reports; they are clinically useful, not mandatory.

Rationale: Collateral information from family, prior records, or other providers broadens the assessment, especially when the client may underreport, lack insight, or have memory problems. It is gathered with the client's consent and weighed alongside the client's own account.

References: Hays (Assessment) — Collateral sources of assessment information

☐ Approve

☐ Revise

☐ Retire

Notes:

A 23-year-old graduate student wants to know how her likes and dislikes compare with those of people who are employed in, and satisfied with, a wide range of occupations. Which instrument is designed to provide this comparison?

A. Strong Interest Inventory ✓ KEY

Its Occupational Scales compare the client's responses with those of satisfied workers in many occupations.

B. Myers-Briggs Type Indicator

The MBTI sorts people into psychological types; it does not compare interests with occupational groups.

C. Differential Aptitude Tests

The DAT measures aptitudes (abilities), not interests or occupational fit.

D. Minnesota Importance Questionnaire

The MIQ measures work needs and values, not interests compared with workers in occupations.

Rationale: The Strong Interest Inventory reports General Occupational Themes (Holland's RIASEC types), Basic Interest Scales, Occupational Scales, and Personal Style Scales. The Occupational Scales are built empirically from people who are employed in an occupation and report liking it, so a high score means the client's interests resemble theirs. Interest similarity predicts satisfaction, not ability or success.

References: Sharf (Career Development Theory) — Interest inventories: Strong Interest Inventory scales · Hays (Assessment) — Career assessment: interest inventories

☐ Approve ☐ Revise ☐ Retire Notes:

A rural high school with limited counseling staff wants a career interest tool that students can take, score, and begin interpreting on their own, producing a three-letter summary code they can use to look up matching occupations. Which instrument BEST fits this need?

A. Strong Interest Inventory

The Strong is scored by the publisher and is usually interpreted with a trained counselor, not self-scored.

B. ASVAB career aptitude battery

The ASVAB measures aptitudes and is not a self-scored interest inventory.

C. Self-Directed Search ✓ KEY

Holland designed the SDS to be self-administered and self-scored, yielding a summary code linked to an occupations finder.

D. Thematic Apperception Test

The TAT is a projective personality technique, not a career interest measure.

Rationale: John Holland's Self-Directed Search asks about activities, competencies, occupations, and self-estimates, and the person totals the scores into a three-letter RIASEC summary code. The code is matched to occupations with the same code. Its self-guided design makes it practical where counselor time is limited, although counselor follow-up still improves interpretation.

References: Sharf (Career Development Theory) — Holland's theory: the Self-Directed Search

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor working with a 46-year-old who lost a long-held factory job asks him to sort cards naming occupations into "would choose," "would not choose," and "undecided" piles and then talk through why he placed each card where he did. What is the MAIN advantage of this method over a standardized interest inventory?

A. It yields norm-referenced scores that compare clients fairly

Card sorts are qualitative and generally unnormed; they do not produce norm-referenced scores.

B. It brings out the client's own reasons and personal meanings ✓ KEY

The discussion of each placement reveals the client's constructs, values, and reasoning in his own words.

C. It offers strong predictive validity for later job success

Card sorts are not designed or validated to predict job success.

D. It removes the need for counselor interpretation of results

Card sorts rely heavily on counselor-client dialogue; interpretation is the heart of the method.

Rationale: Card sorts (occupational, values, or skills sorts) are qualitative, interactive assessments. Their strength is flexibility and insight into how the client thinks about work, which is useful for adults in transition. The tradeoff is the lack of norms and standardized psychometric evidence.

References: Sharf (Career Development Theory) — Qualitative career assessment: card sorts

☐ Approve

☐ Revise

☐ Retire

Notes:

A 31-year-old client's interest profile fits her field well, yet she has left three jobs, each time saying the work offered little recognition or independence. Her counselor gives her the Minnesota Importance Questionnaire. What does this instrument primarily measure?

A. Her verbal, numerical, and spatial aptitudes for job training

Aptitudes are measured by ability batteries, not the MIQ.

B. Her personality type according to the five-factor model

Five-factor traits are measured by instruments such as the NEO-PI-3, not the MIQ.

C. How closely her likes resemble those of satisfied workers

That describes the Occupational Scales of an interest inventory such as the Strong.

D. Her work needs and values, compared with what jobs reward ✓ KEY

The MIQ assesses vocational needs and values so they can be matched with the reinforcers that occupations provide.

Rationale: The Minnesota Importance Questionnaire comes from Dawis and Lofquist's Theory of Work Adjustment, which holds that satisfaction depends on the match between a worker's needs and values and the reinforcers a job supplies. Interests alone may fit, but a values mismatch (such as low recognition or autonomy) can still lead to dissatisfaction and turnover.

References: Sharf (Career Development Theory) — Theory of Work Adjustment: Minnesota Importance Questionnaire

► **Check:** Confirm the MIQ is still described in Sharf as the TWA values/needs measure (it is now largely available through O*NET-related tools).

☐ Approve

☐ Revise

☐ Retire

Notes:

A high school junior is exploring careers. Her counselor wants an estimate of her potential to learn in several separate areas, such as mechanical comprehension, arithmetic reasoning, and word knowledge, rather than a record of what she has already mastered in class. Which type of instrument BEST fits?

A. A multiaptitude battery such as the ASVAB ✓ KEY

Multiaptitude batteries estimate potential in several distinct ability areas and are widely used in career exploration.

B. An achievement battery covering school subjects

Achievement tests measure what has already been learned, which the counselor explicitly does not want.

C. A general intelligence test with one overall IQ

A single global score does not profile separate occupational ability areas.

D. An interest inventory such as the Strong

Interest inventories measure likes and dislikes, not ability or potential.

Rationale: Aptitude tests predict future learning or performance; achievement tests measure past learning from instruction; intelligence tests estimate general cognitive ability. A multiaptitude battery such as the ASVAB or the Differential Aptitude Tests yields a profile of specific abilities, which is useful alongside interests and values in career planning.

References: Hays (Assessment) — Aptitude, achievement, and intelligence testing

☐ Approve

☐ Revise

☐ Retire

Notes:

A counselor educator runs an item analysis on a classroom test. Item 7: $p = .52$, $D = .41$. Item 12: $p = .48$, $D = -.22$. Item 19: $p = .90$, $D = .10$. Item 23: $p = .30$, $D = .35$. Which item should be revised or removed FIRST?

A. Item 7, because its difficulty sits too near the midpoint

A p near $.50$ with $D = .41$ is close to ideal; it is the strongest item here.

B. Item 23, because fewer than a third of students got it right

A hard item is acceptable when it discriminates well, and $D = .35$ is good.

C. Item 12, because low scorers outperform the high scorers ✓ KEY

A negative discrimination index means the item works backward and is likely flawed (miskeyed or misleading).

D. Item 19, because nearly every student answered it correctly

An easy item with weak but positive discrimination is a lower priority than one that discriminates negatively.

Rationale: Item difficulty (p) is the proportion of examinees answering correctly, so a higher p means an easier item. The discrimination index (D) is the proportion correct in the high-scoring group minus the proportion correct in the low-scoring group. Positive D is desired; a negative D signals a defective item and is the top priority for review.

References: Hays (Assessment) — Item analysis: item difficulty and item discrimination

☐ Approve

☐ Revise

☐ Retire

Notes:

A test developer splits a 60-item anxiety scale into odd- and even-numbered items and finds that the two halves correlate .60. Using the Spearman-Brown formula, what is the estimated reliability of the full 60-item scale?

A. .36

Squaring .60 gives shared variance, not a reliability estimate for the lengthened test.

B. .60

This is the half-test correlation, which underestimates the reliability of the full-length test.

C. .80

This overstates the correction; $2(.60)/(1 + .60)$ is .75.

D. .75 ✓ KEY

Spearman-Brown: $2r/(1 + r) = 1.20/1.60 = .75$.

Rationale: Split-half reliability correlates two halves of one administration, but each half is only half as long as the real test, and shorter tests are less reliable. The Spearman-Brown prophecy formula corrects for this: $r(\text{full}) = 2r(\text{half}) / (1 + r(\text{half}))$. The formula can also project reliability for other changes in test length.

References: Hays (Assessment) — Split-half reliability and the Spearman-Brown formula

☐ Approve ☐ Revise ☐ Retire Notes:

Two counselors independently review 50 recorded intakes and judge whether each client meets criteria for a substance use disorder. They agree on 90% of cases, but most clients fall into the "does not meet" category. Why would a researcher report Cohen's kappa instead of simple percent agreement?

A. Kappa shows how stable each rater's judgments are over time

Stability over time is test-retest (or intra-rater) reliability, not what kappa between two raters addresses.

B. Kappa adjusts for the agreement expected by chance alone ✓ KEY

When one category dominates, raters agree often by chance; kappa removes that chance agreement.

C. Kappa proves the raters' diagnoses are accurate and valid

Agreement is reliability; two raters can agree and both be wrong, so kappa does not establish validity.

D. Kappa estimates internal consistency of the rating form items

Internal consistency is estimated with coefficient alpha or KR-20, not kappa.

Rationale: Inter-rater reliability reflects how consistently different raters score the same material. Percent agreement is inflated when base rates are lopsided, because raters will often agree by chance. Cohen's kappa compares observed agreement with chance-expected agreement, giving a more honest estimate for categorical judgments.

References: Erford (Research and Evaluation in Counseling) — Inter-rater reliability: Cohen's kappa

☐ Approve ☐ Revise ☐ Retire Notes:

Clients who try out a new workplace stress inventory say, "It's obviously asking about job stress." The developer cites these comments in the manual as the main evidence that the inventory is valid. What do the comments actually reflect?

A. Face validity, which is not real psychometric evidence ✓ KEY

Face validity is how a test looks to test takers; it can aid cooperation but is not evidence of what the test measures.

B. Content validity, shown through expert review of items

Content validity requires systematic expert judgment of how well items sample the domain, not test takers' impressions.

C. Construct validity, shown by the internal factor structure

Construct validity requires accumulated empirical evidence such as factor analysis or convergent correlations.

D. Concurrent validity, shown by agreement with a criterion

Concurrent validity requires correlating scores with a criterion measured at the same time.

Rationale: Face validity is the superficial appearance that a test measures what it claims. Content validity, by contrast, is established when experts judge items against a defined content domain or table of specifications. Only content, criterion-related, and construct evidence support score interpretation; face validity does not.

References: Hays (Assessment) — Validity: content validity versus face validity

☐ Approve ☐ Revise ☐ Retire Notes:

A developer theorizes that counselor burnout has three distinct dimensions and writes items for each. A factor analysis of responses from 900 counselors shows the items clustering on three factors that match the theorized dimensions. This result is BEST described as evidence of which kind of validity?

A. Predictive validity tied to a future outcome criterion

Predictive validity requires correlating scores with a criterion measured later.

B. Test-retest reliability across repeated administrations

Factor structure from one administration says nothing about stability over time, and it is reliability, not validity.

C. Construct validity based on the test's internal structure ✓ KEY

Items grouping into the theorized dimensions supports the claim that the test measures the intended construct.

D. Content validity confirmed by the judgments of experts

Content validity rests on expert review of item coverage, not on statistical clustering of responses.

Rationale: Construct validity asks whether a test measures the theoretical trait it is meant to measure. Factor analysis provides internal-structure evidence: if items load on factors that match the theory, the score structure fits the construct. Convergent and discriminant correlations are other common sources of construct evidence.

References: Hays (Assessment) — Construct validity: factor analysis and internal structure

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor plans to give a self-esteem inventory to a 72-year-old Filipino American retiree. The manual reports norms from about 2,000 undergraduates at four Midwestern universities. What is the counselor's PRIMARY concern in interpreting her percentile score?

A. A 2,000-person sample is too small to yield stable norms

Two thousand is a substantial norm sample; size is not the problem here.

B. Self-report formats are invalid for clients over age 65

Older adults can complete self-report measures; age alone does not invalidate the format.

C. Percentile ranks first need a criterion-referenced cutoff

Percentiles are norm-referenced by definition and do not depend on a criterion cutoff.

D. The norm group does not represent clients like this one ✓ **KEY**

Her score would be compared with young college students, a group that differs from her in age, culture, and life stage.

Rationale: Norm-referenced scores mean something only relative to the norm group. Counselors check that the norm sample is representative of the client (age, culture, language, setting) and reasonably current. When the norm group does not match, percentile and standard scores can mislead, so the counselor should seek better-matched norms or interpret with explicit caution.

References: Hays (Assessment) — Norm groups: representativeness, size, and recency

☐ Approve ☐ Revise ☐ Retire Notes:

A third grader earns a grade-equivalent score of 6.2 on a mathematics achievement test given to her class. Her parent asks whether she should be moved into sixth-grade math. Which interpretation should the counselor give?

A. She has mastered most of the sixth-grade math curriculum

The test sampled third-grade content; it did not test sixth-grade skills.

B. She did very well on third-grade content, not sixth-grade ✓ **KEY**

A grade equivalent of 6.2 means her score matches what an average early sixth grader is estimated to earn on this third-grade test.

C. Her score is three standard deviations above her grade mean

Grade equivalents are not equal-interval units and cannot be converted into standard deviations this way.

D. She would score at the 62nd percentile among sixth graders

The decimal in a grade equivalent refers to the month of the school year, not a percentile.

Rationale: Grade and age equivalents are developmental scores that are easy to misread. They are often extrapolated rather than based on students actually tested at that grade, they are not equal-interval, and they do not indicate mastery of higher-grade material. Percentile ranks or standard scores within the student's own grade are better for interpretation.

References: Hays (Assessment) — Developmental scores: limits of grade and age equivalents

☐ Approve ☐ Revise ☐ Retire Notes:

A school district reports its students' achievement results as normal curve equivalents (NCEs) and as stanines. A school board member asks the counselor how to read these scores. Which statement is accurate?

A. NCEs are equal-interval scores (mean 50) that can be averaged ✓ KEY

NCEs are normalized standard scores (mean 50, SD about 21.06) on an equal-interval scale, which is why programs average them.

B. Stanines divide students into nine bands holding equal percentages

Stanine bands are equal in width (half an SD) but hold unequal percentages, about 4% at each end and 20% in the middle.

C. An NCE of 50 and a stanine of 9 describe the same relative standing

An NCE of 50 is average, matching stanine 5; stanine 9 is the top band.

D. NCEs equal percentile ranks at every point along the whole score scale

NCEs match percentile ranks only at 1, 50, and 99; elsewhere they differ because percentiles are not equal-interval.

Rationale: Stanines ("standard nines") run from 1 to 9 with a mean of 5 and an SD of about 2. Normal curve equivalents run from 1 to 99 with a mean of 50 and an SD of about 21.06, chosen so they line up with percentiles at 1, 50, and 99. Unlike percentile ranks, NCEs are equal-interval and can be averaged and compared across groups.

References: Hays (Assessment) — Standard scores: stanines and normal curve equivalents

☐ Approve ☐ Revise ☐ Retire Notes:

86. nce-s-ass-038 · Research and Program Evaluation · standard scores, percentiles and the normal curve · medium · In review

On a very difficult practice exam, most examinees earn low scores and a handful earn very high scores. How are the mean, median, and mode MOST likely to be ordered in this distribution?

A. Mean lowest, median next, and mode highest of the three

This ordering describes a negatively skewed distribution, where a few very low scores pull the mean down.

B. Mean, median, and mode fall at the same point in the curve

The three measures coincide only in a symmetrical distribution such as the normal curve.

C. Mode lowest, median next, and mean highest of the three ✓ KEY

A few very high scores create a positive skew and pull the mean toward the right tail.

D. Median lowest, with mean and mode equal and above it

The median lies between the mode and mean in a skewed distribution; it is not the lowest.

Rationale: Skew is named for the tail. When scores bunch at the low end with a tail to the right, the distribution is positively skewed, and the mean (most sensitive to extreme scores) is pulled toward the tail: mode < median < mean. In a negative skew the order reverses. The median is often preferred for describing skewed data.

References: Erford (Research and Evaluation in Counseling) — Descriptive statistics: skewness and measures of central tendency

☐ Approve ☐ Revise ☐ Retire Notes:

In a clinic sample, weekly exercise hours correlate $-.70$ with depression scores, and nightly hours of sleep correlate $+.40$ with depression scores. Which conclusion is accurate?

A. The sleep link is stronger because its coefficient is positive

The sign shows direction only; strength depends on the absolute value, and $.70$ exceeds $.40$.

B. Exercise accounts for 70% of the variance in depression scores

Shared variance is r squared, so $-.70$ corresponds to 49%, not 70%.

C. The negative coefficient shows exercise reduces depression

Correlation does not establish causation; a third variable or reverse direction could explain it.

D. The exercise link is stronger and shares 49% of the variance ✓ **KEY**

The absolute value $.70$ is larger than $.40$, and r squared ($.49$) gives the proportion of shared variance.

Rationale: A correlation coefficient's sign gives direction and its absolute value gives strength. Squaring r gives the coefficient of determination, the proportion of variance the two variables share. No correlation, however strong, demonstrates cause and effect.

References: Erford (Research and Evaluation in Counseling) — Correlation: direction, strength, and coefficient of determination

☐ Approve ☐ Revise ☐ Retire Notes: _____

88. nce-s-ass-040 · Assessment and Testing · screening instruments (PHQ-9, GAD-7, AUDIT, CAGE) · medium · In review

An elementary school principal proposes using scores from a 10-minute behavior screening checklist, completed once by each teacher, to decide which students are placed in special education services. What is the school counselor's BEST advice?

A. Accept the plan if the checklist has internal consistency above $.80$

Good reliability does not make a brief screener valid for a high-stakes placement decision.

B. Use the screener to flag students for a full multi-measure evaluation ✓ **KEY**

Screeners identify who needs further assessment; high-stakes decisions require comprehensive evaluation from multiple sources.

C. Raise the cutoff score so that fewer students are wrongly identified

Changing the cutoff trades false positives for false negatives and still leaves a single screener driving placement.

D. Keep the screener but have a second teacher rescore every checklist

Double scoring reduces clerical error but does not address the misuse of a screener for a high-stakes decision.

Rationale: Screening instruments are brief and built to cast a wide net, accepting some false positives. High-stakes decisions such as placement, diagnosis, or eligibility should rest on multiple measures, multiple informants, and qualified professional judgment rather than a single score.

References: Hays (Assessment) — Screening versus comprehensive assessment; high-stakes testing

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor at a university counseling center plans to give the Beck Depression Inventory-II (BDI-II) to new clients. Which description of the BDI-II is accurate?

A. A 21-item self-report of depressive symptoms over two weeks ✓ KEY

The BDI-II is a 21-item self-report of depressive symptom severity covering the past two weeks, including today.

B. A clinician-administered interview that yields DSM-5-TR diagnoses

The BDI-II is a self-report severity measure, not a diagnostic interview.

C. A projective technique that uncovers depressive themes in children

It is an objective self-report inventory, and it is intended for adolescents and adults.

D. A nine-item primary care screen keyed to DSM depression criteria

That describes the PHQ-9; the BDI-II has 21 items and was developed separately from primary care screening.

Rationale: The BDI-II measures the severity of depressive symptoms in adolescents and adults, with each item rated 0 to 3. It is useful for screening and tracking change but does not by itself establish a diagnosis; a clinical interview is still needed.

References: Hays (Assessment) — Assessment of depression: Beck Depression Inventory-II

☐ Approve ☐ Revise ☐ Retire Notes:

On an MMPI-3 completed during a disability evaluation, the inconsistent-responding scales are within normal limits, but the infrequent-responding (overreporting) scales are markedly elevated, as are many clinical scales. What is the MOST appropriate interpretation?

A. Random responding, so the client probably could not read the items

Normal inconsistency scales argue against random or content-unrelated responding.

B. Defensive underreporting, so true problems are likely even more severe

Underreporting is shown by the underreporting scales, not by elevated infrequent-responding scales.

C. A valid profile, so the elevations confirm severe psychopathology

Marked elevations on overreporting scales call the validity of substantive scale scores into question.

D. Possible overreporting, so the clinical scales need cautious reading ✓ KEY

Elevated infrequent-responding scales with consistent responding suggest symptom overreporting, which limits face-value interpretation.

Rationale: MMPI-3 validity scales fall into three groups: inconsistent responding (CRIN, VRIN, TRIN), overreporting (F, Fp, Fs, FBS, RBS), and underreporting (L, K). The validity scales are reviewed before any substantive scale. When overreporting indicators are markedly elevated, substantive elevations may reflect exaggeration rather than true symptoms, particularly in settings with external incentives.

References: Hays (Assessment) — MMPI-3 validity scales: inconsistency, overreporting, underreporting

► **Check:** Verify the MMPI-3 validity scale names and groupings listed in the rationale against the current manual.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor uses the SCID-5 at intake. It follows a set order of questions keyed to DSM-5 criteria but lets the counselor ask follow-up questions and use clinical judgment in rating each criterion. How is this interview format BEST classified?

A. Fully structured, read word for word with no clinical judgment

Fully structured interviews (often designed for lay interviewers) leave no room for follow-up probes or clinician judgment.

B. Unstructured, with topics guided mainly by the client's story

Unstructured interviews have no set question sequence, unlike the SCID-5.

C. Semi-structured, a set guide with room for clinician probes ✓ KEY

The SCID-5 is a semi-structured interview: standard questions plus clinician follow-up and judgment.

D. A mental status exam, since it records observed presentation

An MSE describes current presentation; the SCID-5 is a diagnostic interview organized by DSM criteria.

Rationale: Structured interviews maximize reliability and coverage but limit flexibility; unstructured interviews maximize flexibility and rapport but are less reliable and can miss areas. Semi-structured interviews such as the SCID-5 combine a standard framework with clinician probes, balancing reliability and clinical depth.

References: Hays (Assessment) — Clinical interviews: structured, semi-structured, and unstructured

☐ Approve

☐ Revise

☐ Retire

Notes:

Asked how she has been feeling, a 40-year-old client says, "Honestly, I'm good, really good." Throughout the interview she is tearful, her voice trembles, and her facial expression changes little. How should the counselor document this in the mental status examination?

A. Mood as the observed tears; affect as her statement of "good"

This reverses the terms: mood is self-reported, affect is observed.

B. Mood reported as "good"; affect tearful and mood-incongruent ✓ KEY

Mood is the client's stated emotional state; affect is the counselor's observation, here incongruent with the stated mood.

C. Thought content as denial; thought process as tearful and flat

Tearfulness is an affect finding, and thought process describes how ideas connect, not emotional expression.

D. Insight as intact, because she can describe how she is feeling

Her report conflicts with her presentation, so this does not show intact insight into her condition.

Rationale: In the MSE, mood is the sustained emotion the client reports (often quoted), and affect is the observed expression of emotion, described by range, intensity, stability, and congruence with mood and content. Thought process is how thoughts connect; thought content is what the client thinks about; insight and judgment are rated separately.

References: Hays (Assessment) — Mental status examination: mood and affect

☐ Approve

☐ Revise

☐ Retire

Notes:

A 9-year-old is referred for inattention and angry outbursts. On a behavior rating scale, the parent's ratings are highly elevated, the teacher's ratings are in the average range, and the child's self-report is low. How should the counselor interpret these results?

A. Informant differences are common and show where problems appear ✓ **KEY**

Discrepancies often reflect real differences across settings and relationships, and they guide further assessment.

B. The teacher's ratings are most objective, so set the parent's aside

No single informant is automatically correct; discarding the parent data loses information about home behavior.

C. Self-reports from children this age are invalid and add nothing

Children can provide useful information, especially about internal states, when measures fit their age.

D. The gap shows the parent is exaggerating, so assessment can end

The discrepancy is a reason to explore further, not evidence of exaggeration or a reason to stop.

Rationale: Assessment of children and adolescents relies on multiple informants (parents, teachers, the youth) and multiple methods, as in the Achenbach System of Empirically Based Assessment. Low cross-informant agreement is typical, because behavior varies by setting and each rater sees a different slice. Counselors integrate the reports with observation and history rather than choosing one.

References: Hays (Assessment) — Assessing children: multiple informants and settings

☐ Approve ☐ Revise ☐ Retire Notes:

For the past four weeks, a 33-year-old client has had depressed mood and two other depressive symptoms, causing clear impairment at work. There is no identifiable stressor, and no other mental disorder or medical cause explains the symptoms. The counselor wants the record to state why full criteria are not met. Which diagnosis BEST fits?

A. Unspecified depressive disorder, because no full criteria are met

Unspecified is used when the clinician chooses not to give the reason or lacks the information; here the counselor wants to state it.

B. Major depressive disorder, single episode, mild in severity

A major depressive episode requires at least five symptoms; this client has three.

C. Adjustment disorder with depressed mood, given the impairment

Adjustment disorder requires an identifiable stressor, which is absent.

D. Other specified depressive disorder, noting the specific reason ✓ **KEY**

Other specified is used when the clinician records the reason the presentation falls short, such as insufficient symptoms.

Rationale: DSM-5-TR offers two options for presentations that cause distress or impairment but do not meet full criteria for a disorder in a class. "Other specified" is used when the clinician chooses to communicate the specific reason (for example, a depressive episode with insufficient symptoms). "Unspecified" is used when the clinician chooses not to specify the reason, including when there is not enough information, as in an emergency setting.

References: DSM-5-TR — Use of other specified and unspecified disorders; Other Specified Depressive Disorder

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is seeing a 38-year-old client who recently immigrated and describes her distress mainly in terms of family duty and spiritual concerns. The counselor wants a DSM-5-TR tool that explores the client's own explanation of the problem, cultural identity, sources of support, and expectations of help. Which tool should the counselor use?

A. Level 1 Cross-Cutting Symptom Measure

This measure screens symptom domains; it does not explore cultural explanations or help-seeking.

B. WHO Disability Assessment Schedule 2.0

The WHODAS 2.0 assesses functioning and disability, not cultural understanding of the problem.

C. Cultural Formulation Interview ✓ KEY

The CFI asks about the cultural definition of the problem, perceived causes, cultural identity, stressors and supports, and help-seeking.

D. Structured Clinical Interview (SCID-5)

The SCID-5 is a diagnostic interview organized by criteria, not a cultural assessment.

Rationale: The DSM-5-TR Cultural Formulation Interview is a person-centered, semi-structured set of questions in Section III. It covers the cultural definition of the problem, cultural perceptions of cause, context, and support, cultural factors affecting coping and help-seeking, and current help-seeking expectations. Informant and supplementary modules extend it.

References: DSM-5-TR — Section III: Cultural Formulation Interview · Sue & Sue (Counseling the Culturally Diverse) — Culturally sensitive assessment

☐ Approve ☐ Revise ☐ Retire Notes:

96. nce-s-ass-048 · Assessment and Testing · screening instruments (PHQ-9, GAD-7, AUDIT, CAGE) · medium · In review

A 76-year-old client's daughter reports that he has trouble finding words and has missed paying several bills. On a brief cognitive screen such as the Montreal Cognitive Assessment (MoCA), he scores below the screening cutoff. What should the counselor do NEXT?

A. Diagnose a major neurocognitive disorder based on the score

A screening score cannot establish a neurocognitive diagnosis or its cause.

B. Refer for a full medical and neuropsychological evaluation ✓ KEY

A positive screen calls for comprehensive evaluation to clarify the cause, including reversible conditions.

C. Readminister the same screen next week to confirm the result

Repeating the screen delays evaluation and adds practice effects without clarifying the cause.

D. Reassure the family that these changes reflect normal aging

Word-finding problems and new difficulty with finances go beyond expected age-related change and need evaluation.

Rationale: Cognitive screens such as the MoCA or MMSE flag possible impairment but are influenced by education, language, hearing, vision, and mood. A positive screen in an older adult should prompt medical evaluation (to rule out causes such as medication effects, thyroid problems, delirium, or depression) and fuller neuropsychological testing before any diagnosis is made.

References: Hays (Assessment) — Assessment of older adults: cognitive screening · DSM-5-TR — Neurocognitive disorders: major and mild NCD

☐ Approve ☐ Revise ☐ Retire Notes:

Areas of Clinical Focus

118 items · NCE weight 29% (47 of 160 scored items)

97. nce-s-clf-001 · Assessment and Testing · depressive disorders · easy · In review

Under DSM-5-TR, what is the minimum duration for the symptoms of a major depressive episode?

A. One week

One week is the minimum duration for a manic episode, not a major depressive episode.

B. Two weeks ✓ KEY

A major depressive episode requires five or more symptoms present during the same 2-week period.

C. One month

One month is not the major depressive episode threshold; it is used for other diagnoses, such as the active phase of schizophrenia.

D. Six months

Six months is the duration for generalized anxiety disorder, not a major depressive episode.

Rationale: DSM-5-TR requires at least five of nine symptoms, including depressed mood or loss of interest or pleasure, present nearly every day during the same 2-week period and representing a change from prior functioning. Knowing duration thresholds helps counselors tell a depressive episode apart from shorter, situational low mood.

References: DSM-5-TR — Major Depressive Disorder, Criterion A

☐ Approve ☐ Revise ☐ Retire Notes:

98. nce-s-clf-002 · Assessment and Testing · depressive disorders · medium · In review

A 41-year-old client reports feeling down more days than not for the past 3 years, with low energy, poor appetite, and hopelessness. She cannot recall a stretch of more than a few weeks without these symptoms and has never had a manic or hypomanic episode. Full criteria for a major depressive episode have not been met. Which diagnosis BEST fits?

A. Cyclothymic disorder

Cyclothymic disorder requires numerous periods of hypomanic symptoms alongside depressive symptoms; she reports no elevated mood.

B. Adjustment disorder with depressed mood

Adjustment disorder follows an identifiable stressor and does not persist more than 6 months after the stressor ends; this is a chronic 3-year pattern.

C. Disruptive mood dysregulation disorder

DMDD is a childhood diagnosis (first given between ages 6 and 18, onset before 10) marked by severe temper outbursts.

D. Persistent depressive disorder ✓ KEY

Depressed mood more days than not for at least 2 years in an adult, with two or more associated symptoms and no symptom-free period longer than 2 months, fits persistent depressive disorder.

Rationale: Persistent depressive disorder requires depressed mood for most of the day, more days than not, for at least 2 years in adults (1 year in children and adolescents), plus at least two associated symptoms such as appetite change, low energy, or hopelessness. During that period the person has not been without symptoms for more than 2 months at a time, and there has never been a manic or hypomanic episode.

References: DSM-5-TR — Persistent Depressive Disorder, Criteria A–C

☐ Approve ☐ Revise ☐ Retire Notes:

A 15-year-old has shown irritable mood on most days for the past 14 months, along with low self-esteem and poor concentration. He has not gone more than a few weeks without these symptoms, has no history of severe temper outbursts, and has never met criteria for a manic or hypomanic episode. Which statement is MOST accurate?

A. Criteria for persistent depressive disorder can be met at this point ✓ KEY

For children and adolescents, mood may be irritable and the duration requirement is 1 year, so 14 months is sufficient.

B. Persistent depressive disorder requires 2 full years in any age group

The 2-year requirement applies to adults; DSM-5-TR shortens it to 1 year for children and adolescents.

C. Irritable rather than sad mood rules out a depressive disorder diagnosis

DSM-5-TR allows irritable mood to satisfy the mood criterion for depressive disorders in children and adolescents.

D. Disruptive mood dysregulation disorder is the required diagnosis here

DMDD requires severe, recurrent temper outbursts with onset before age 10, which this teen does not show.

Rationale: DSM-5-TR modifies persistent depressive disorder for youth in two ways: mood can be irritable rather than depressed, and the minimum duration is 1 year instead of 2. Counselors working with adolescents should recognize that chronic irritability can be a depressive presentation rather than simply a behavior problem.

References: DSM-5-TR — Persistent Depressive Disorder, Criterion A (children and adolescents)

☐ Approve ☐ Revise ☐ Retire Notes: _____

Unless hospitalization is required, what is the minimum duration of a manic episode in DSM-5-TR?

A. Four consecutive days

Four consecutive days is the minimum for a hypomanic episode, not a manic episode.

B. Two weeks

Two weeks is the duration requirement for a major depressive episode.

C. One week ✓ KEY

A manic episode lasts at least 1 week and is present most of the day, nearly every day, or any duration if hospitalization is needed.

D. One month

One month is not a mood episode threshold; it is too long for mania.

Rationale: A manic episode is a distinct period of abnormally and persistently elevated, expansive, or irritable mood plus increased activity or energy lasting at least 1 week (any duration if hospitalization is necessary). A hypomanic episode needs only 4 consecutive days and does not cause marked impairment or require hospitalization.

References: DSM-5-TR — Bipolar I Disorder, Manic Episode Criterion A

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 30-year-old client has had two major depressive episodes. Last spring she had 5 days of unusually elevated mood, less need for sleep, and increased productivity that friends noticed but that did not impair her work or require hospitalization. She has never had psychotic symptoms or a longer elevated period. Which diagnosis is MOST appropriate?

A. Bipolar II disorder ✓ **KEY**

At least one hypomanic episode (4+ days, no marked impairment) plus at least one major depressive episode, with no history of mania, defines bipolar II disorder.

B. Bipolar I disorder

Bipolar I requires at least one full manic episode; her elevated period was hypomanic.

C. Cyclothymic disorder

Cyclothymic disorder requires symptoms that never meet full criteria for a major depressive episode; she has had two.

D. Major depressive disorder

A history of a hypomanic episode excludes a diagnosis of major depressive disorder.

Rationale: Bipolar II disorder requires at least one hypomanic episode and at least one major depressive episode, with no lifetime manic episode. Hypomania lasts at least 4 consecutive days, is observable by others, and is not severe enough to cause marked impairment or hospitalization. Screening for past hypomania is essential before treating depression as unipolar.

References: DSM-5-TR — Bipolar II Disorder, Criteria A–B; Hypomanic Episode

☐ Approve ☐ Revise ☐ Retire Notes:

A 27-year-old with no prior psychiatric history develops grandiosity, pressured speech, almost no sleep, and reckless spending. On the third day he is psychiatrically hospitalized because his behavior has become dangerous. Substances and medical causes are ruled out. Which statement is MOST accurate?

A. The episode is hypomanic because it lasted fewer than 4 days

Hypomania itself requires at least 4 days and, by definition, does not require hospitalization.

B. No bipolar diagnosis applies until symptoms reach 1 full week

The 1-week requirement is waived when hospitalization is necessary.

C. Bipolar I also requires a prior major depressive episode on record

A single manic episode is sufficient for bipolar I; a depressive episode is common but not required.

D. The episode is manic since hospitalization waives the duration ✓ **KEY**

DSM-5-TR counts a manic episode of any duration if hospitalization is necessary, so bipolar I disorder can be diagnosed.

Rationale: The 1-week duration for a manic episode applies unless hospitalization is necessary, in which case any duration qualifies. Bipolar I disorder requires only one lifetime manic episode; major depressive episodes are common but are not required for the diagnosis.

References: DSM-5-TR — Bipolar I Disorder, Manic Episode Criterion A and Criterion for Bipolar I

☐ Approve ☐ Revise ☐ Retire Notes:

For a diagnosis of generalized anxiety disorder, excessive anxiety and worry must occur more days than not for at least how long?

A. 1 month

One month is the post-attack worry period for panic disorder, not the GAD duration.

B. 6 months ✓ KEY

GAD requires excessive anxiety and worry more days than not for at least 6 months.

C. 3 months

Three months is not the GAD threshold.

D. 2 years

Two years is the adult duration for persistent depressive disorder.

Rationale: Generalized anxiety disorder involves excessive, hard-to-control worry about a number of events or activities, occurring more days than not for at least 6 months, with three or more associated symptoms in adults (only one in children), such as restlessness, fatigue, or sleep disturbance.

References: DSM-5-TR — Generalized Anxiety Disorder, Criterion A

☐ Approve ☐ Revise ☐ Retire Notes:

A 36-year-old client has had several sudden, unexpected surges of intense fear with a pounding heart, shortness of breath, and dizziness. For the past 2 months he has worried constantly about having another attack and has stopped exercising because a racing heart might trigger one. Medical causes have been ruled out. Which diagnosis BEST fits?

A. Panic disorder ✓ KEY

Recurrent unexpected panic attacks followed by at least 1 month of persistent worry or maladaptive behavior change define panic disorder.

B. Agoraphobia

Agoraphobia requires fear or avoidance in at least two situations such as crowds or public transit; his avoidance centers on bodily sensations.

C. Illness anxiety disorder

Illness anxiety centers on a preoccupation with having a serious illness, not on recurrent unexpected panic attacks.

D. Specific phobia, situational

Specific phobia involves fear cued by a particular object or situation, whereas these attacks are unexpected.

Rationale: Panic disorder requires recurrent unexpected panic attacks, with at least one attack followed by 1 month or more of persistent concern about further attacks or their consequences, or a significant maladaptive change in behavior such as avoiding exercise. Interoceptive avoidance like this is a common target of cognitive behavioral treatment.

References: DSM-5-TR — Panic Disorder, Criteria A–B · Barlow PCT — Interoceptive avoidance and exposure

☐ Approve ☐ Revise ☐ Retire Notes:

Ten days after a serious highway collision, a 24-year-old reports intrusive memories, nightmares, feeling as if the world is unreal, avoiding driving, and trouble sleeping and concentrating. The symptoms began right after the crash and are impairing her work. Which diagnosis is MOST appropriate at this time?

A. Posttraumatic stress disorder

PTSD requires symptoms lasting more than 1 month; only 10 days have passed.

B. Adjustment disorder with anxiety

When symptoms meet criteria for a trauma-specific disorder, adjustment disorder is not the best fit.

C. Acute stress disorder ✓ KEY

Acute stress disorder applies when trauma-related symptoms last from 3 days to 1 month after exposure.

D. Generalized anxiety disorder

GAD involves diffuse worry for at least 6 months and is not tied to a traumatic event.

Rationale: Acute stress disorder is diagnosed when nine or more symptoms across intrusion, negative mood, dissociation, avoidance, and arousal last from 3 days to 1 month after trauma exposure. If symptoms persist beyond 1 month and meet PTSD criteria, the diagnosis is changed to PTSD.

References: DSM-5-TR — Acute Stress Disorder, Criteria B–C

☐ Approve ☐ Revise ☐ Retire Notes:

A firefighter developed some sleep problems and irritability soon after a fatal fire. Eight months later, after more symptoms emerged, she meets full criteria for PTSD for the first time. She does not report depersonalization or derealization. Which specifier applies?

A. With dissociative symptoms

The dissociative specifier requires persistent depersonalization or derealization, which she denies.

B. With panic attacks

The with-panic-attacks specifier requires panic attacks alongside the disorder, and none are described here.

C. With acute onset

DSM-5-TR PTSD has no acute-onset specifier; the DSM-IV acute and chronic specifiers were removed.

D. With delayed expression ✓ KEY

This specifier is used when full criteria are not met until at least 6 months after the event, even if some symptoms began earlier.

Rationale: In DSM-5-TR, PTSD "with delayed expression" is specified when full diagnostic criteria are not met until at least 6 months after the event, although the onset and expression of some symptoms may be immediate. The other PTSD specifier is "with dissociative symptoms" (depersonalization or derealization).

References: DSM-5-TR — Posttraumatic Stress Disorder, specifiers

☐ Approve ☐ Revise ☐ Retire Notes:

A client with contamination obsessions washes his hands for hours daily. He says his family will probably become seriously ill if he stops washing, although he admits he could be wrong. Which DSM-5-TR insight specifier is MOST accurate?

A. With good or fair insight

Good or fair insight means the person recognizes the beliefs are definitely or probably not true.

B. With poor insight ✓ KEY

Poor insight applies when the person thinks the OCD beliefs are probably true.

C. With absent insight

Absent insight/delusional beliefs applies when the person is completely convinced the beliefs are true; he admits doubt.

D. With tic-related features

The tic-related specifier concerns a current or past tic disorder, not the degree of insight.

Rationale: DSM-5-TR rates insight in OCD on three levels: good or fair (beliefs definitely or probably not true), poor (beliefs probably true), and absent insight/delusional beliefs (completely convinced the beliefs are true). A separate "tic-related" specifier notes a current or past tic disorder.

References: DSM-5-TR — Obsessive-Compulsive Disorder, insight specifiers

☐ Approve

☐ Revise

☐ Retire

Notes:

108. nce-s-clf-012 · Counseling and Helping Relationships · obsessive-compulsive and related disorders · easy · In review

Which psychological intervention has the strongest evidence as a first-line treatment for obsessive-compulsive disorder?

A. Insight-oriented psychodynamic therapy

Psychodynamic therapy lacks the evidence base that exposure-based CBT has for OCD.

B. Thought stopping with reassurance

Reassurance acts as a compulsion and maintains OCD; thought stopping tends to backfire.

C. Exposure and response prevention ✓ KEY

ERP, a form of CBT, exposes clients to feared cues while they refrain from compulsions and is the first-line psychological treatment.

D. Nondirective supportive counseling

Supportive counseling alone does not target the obsession–compulsion cycle.

Rationale: Exposure and response prevention has clients face feared thoughts, images, or situations while resisting compulsions, allowing anxiety to decline without the ritual and weakening the negative reinforcement that maintains OCD. Reassurance-giving by the counselor or family is itself a compulsion to be reduced.

References: NICE guidelines — CG31 Obsessive-compulsive disorder and body dysmorphic disorder: CBT including ERP

☐ Approve

☐ Revise

☐ Retire

Notes:

Over the past 12 months, a client has met five of the eleven DSM-5-TR criteria for alcohol use disorder. Which severity specifier applies?

A. Mild

Mild applies when two or three criteria are met.

B. Severe

Severe applies when six or more criteria are met.

C. Moderate ✓ KEY

Moderate applies when four or five criteria are met.

D. Subthreshold

Two criteria within 12 months are enough for a diagnosis, so five is well above threshold.

Rationale: DSM-5-TR substance use disorders require at least two of eleven criteria within a 12-month period. Severity is specified as mild (2–3 criteria), moderate (4–5), or severe (6 or more). Severity guides level-of-care decisions alongside a dimensional assessment such as the ASAM Criteria.

References: DSM-5-TR — Alcohol Use Disorder, severity specifiers

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old takes a prescribed opioid exactly as directed for chronic back pain under a physician's care. She reports needing a higher dose than at first and feeling withdrawal symptoms if a dose is late. No other use disorder criteria are present. How should the counselor understand these symptoms?

A. They are not counted toward an opioid use disorder ✓ KEY

Tolerance and withdrawal are not counted when the opioid is taken solely under appropriate medical supervision.

B. They meet criteria for mild opioid use disorder

Two counted criteria would be mild, but these two are excluded in supervised medical use.

C. They confirm a severe opioid use disorder is present

Physiological dependence alone does not establish any use disorder, let alone a severe one.

D. They point to opioid use disorder in remission

Remission requires a prior full use disorder, which has not been established.

Rationale: DSM-5-TR notes that tolerance and withdrawal are expected physiological adaptations when medications such as opioids, sedatives, or stimulants are taken as prescribed under medical supervision, and they are not counted toward a use disorder in that situation. A diagnosis requires other evidence of problematic use, such as craving or loss of control.

References: DSM-5-TR — Opioid Use Disorder, Criteria 10–11 note on medical supervision

► **Check:** The cited 'Criteria 10-11 note' detail is the writer's wording of where the DSM-5-TR note sits.

☐ Approve ☐ Revise ☐ Retire Notes:

Under DSM-5-TR, several inattentive or hyperactive-impulsive symptoms of ADHD must have been present before what age?

A. Age 7

Onset before age 7 was the DSM-IV standard; DSM-5 raised it to 12.

B. Age 12 ✓ KEY

DSM-5-TR requires several symptoms to have been present before age 12.

C. Age 16

Age 16 is not a criterion; age 17 marks where the adult symptom count of five applies.

D. Age 18

Age 18 is not the ADHD onset threshold.

Rationale: ADHD requires several inattentive or hyperactive-impulsive symptoms before age 12, present in two or more settings, with clear interference in functioning. Children need six symptoms in a domain; adolescents 17 and older and adults need five. Adult assessment therefore includes a developmental history.

References: DSM-5-TR — Attention-Deficit/Hyperactivity Disorder, Criteria A and C

☐ Approve ☐ Revise ☐ Retire Notes:

A 7-year-old struggles to take turns in conversation, misses jokes and hints, and speaks to teachers the same way he speaks to peers. A counselor is weighing autism spectrum disorder against social (pragmatic) communication disorder. Which finding would MOST support autism spectrum disorder?

A. Late onset of first spoken words

Language delay can occur in many conditions and is not a defining criterion that separates the two.

B. Trouble using language socially

Difficulty with social use of language is central to both disorders, so it does not distinguish them.

C. Below-average intellectual ability

Intellectual impairment is a possible specifier for ASD but is neither required nor distinguishing.

D. Restricted, repetitive behaviors ✓ KEY

ASD requires restricted, repetitive patterns of behavior, interests, or activities; their presence rules out SCD.

Rationale: Both ASD and social (pragmatic) communication disorder involve deficits in social communication. ASD also requires at least two kinds of restricted, repetitive patterns of behavior, interests, or activities (such as rigid routines, intense interests, or sensory differences). SCD is diagnosed only when such patterns are absent, past or present.

References: DSM-5-TR — Social (Pragmatic) Communication Disorder, Criterion D; Autism Spectrum Disorder, Criterion B

☐ Approve ☐ Revise ☐ Retire Notes:

A 22-year-old college student has had auditory hallucinations, persecutory delusions, and disorganized speech for about 3 months, including any early warning signs. There have been no mood episodes, and substance and medical causes are ruled out. Which diagnosis is MOST appropriate?

A. Schizophreniform disorder ✓ **KEY**

Schizophreniform disorder has schizophrenia-like symptoms lasting at least 1 month but less than 6 months.

B. Schizophrenia

Schizophrenia requires continuous signs of disturbance for at least 6 months.

C. Brief psychotic disorder

Brief psychotic disorder lasts at least 1 day but less than 1 month, with full return to functioning.

D. Delusional disorder

Delusional disorder involves delusions without prominent hallucinations or disorganized speech.

Rationale: The schizophrenia spectrum is partly sorted by duration: brief psychotic disorder lasts 1 day to less than 1 month; schizophreniform disorder lasts 1 month to less than 6 months; schizophrenia requires at least 6 months of disturbance, including at least 1 month of active-phase symptoms.

References: DSM-5-TR — Schizophreniform Disorder, Criteria A–B

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 26-year-old has had 5 weeks of prominent hallucinations and delusions. His family reports that for about 6 months before that, he withdrew from friends, showed flat affect, and voiced odd beliefs. No mood episodes have occurred, and substance and medical causes are excluded. Which diagnosis BEST fits?

A. Schizophreniform disorder

The prodromal period counts toward duration, so the total exceeds the less-than-6-months limit for schizophreniform disorder.

B. Schizophrenia ✓ **KEY**

More than 1 month of active symptoms plus a prodromal period gives over 6 months of continuous disturbance.

C. Brief psychotic disorder

Brief psychotic disorder lasts less than 1 month in total; the active phase alone is longer.

D. Schizoaffective disorder

Schizoaffective disorder requires a major mood episode concurrent with psychotic symptoms; none occurred.

Rationale: Schizophrenia requires continuous signs of disturbance for at least 6 months, which must include at least 1 month of active-phase symptoms and may include prodromal or residual periods with negative symptoms or attenuated positive symptoms. Collateral history from family is often essential for establishing the prodromal phase.

References: DSM-5-TR — Schizophrenia, Criterion C

☐ Approve ☐ Revise ☐ Retire Notes: _____

Which personality disorder belongs to DSM-5-TR Cluster C (the anxious or fearful cluster)?

A. Schizotypal personality disorder

Schizotypal is in Cluster A, the odd or eccentric cluster.

B. Histrionic personality disorder

Histrionic is in Cluster B, the dramatic, emotional, or erratic cluster.

C. Avoidant personality disorder ✓ KEY

Avoidant personality disorder is in Cluster C with dependent and obsessive-compulsive personality disorders.

D. Paranoid personality disorder

Paranoid is in Cluster A with schizoid and schizotypal.

Rationale: DSM-5-TR groups personality disorders into three clusters: A (paranoid, schizoid, schizotypal) for odd or eccentric patterns; B (antisocial, borderline, histrionic, narcissistic) for dramatic or erratic patterns; and C (avoidant, dependent, obsessive-compulsive) for anxious or fearful patterns.

References: DSM-5-TR — Personality Disorders, cluster groupings

☐ Approve

☐ Revise

☐ Retire

Notes:

A 20-year-old shows a pervasive pattern of deceitfulness, impulsivity, repeated fights, and lack of remorse since age 15. To diagnose antisocial personality disorder, which additional finding does DSM-5-TR require?

A. A documented history of arrest or jail

Legal involvement is common but not required; failure to conform to social norms can be shown in other ways.

B. No co-occurring substance use disorder

Substance use disorders frequently co-occur and do not rule out antisocial personality disorder.

C. Onset of the pattern after age 18 only

The pattern must be evident since age 15; onset is not restricted to adulthood.

D. Evidence of conduct disorder before 15 ✓ KEY

DSM-5-TR requires evidence of conduct disorder with onset before age 15, and the person must be at least 18.

Rationale: Antisocial personality disorder requires a pervasive pattern of disregard for and violation of the rights of others occurring since age 15, a current age of at least 18, and evidence of conduct disorder with onset before age 15. The behavior must not occur only during schizophrenia or bipolar disorder.

References: DSM-5-TR — Antisocial Personality Disorder, Criteria B–C

☐ Approve

☐ Revise

☐ Retire

Notes:

A 33-year-old describes episodes, about twice a week for 6 months, of eating far more than most people would in a short period while feeling unable to stop. She eats rapidly, eats alone out of embarrassment, and feels disgusted afterward. She does not vomit, fast, misuse laxatives, or exercise to compensate. Which diagnosis BEST fits?

A. Bulimia nervosa

Bulimia nervosa requires recurrent inappropriate compensatory behaviors, which she does not use.

B. Binge-eating disorder ✓ KEY

Recurrent binges with loss of control and marked distress, at least weekly for 3 months, without regular compensatory behavior, fit binge-eating disorder.

C. ARFID

ARFID involves restricted intake, not binge episodes.

D. Anorexia nervosa, binge/purge type

Anorexia nervosa requires restriction leading to significantly low body weight and fear of weight gain, none of which is described, and she does not purge.

Rationale: Binge-eating disorder requires recurrent binge episodes with a sense of loss of control, three or more associated features (such as eating rapidly, eating alone from embarrassment, or feeling disgusted afterward), marked distress, and a frequency of at least once a week for 3 months. Unlike bulimia nervosa, there are no recurrent compensatory behaviors.

References: DSM-5-TR — Binge-Eating Disorder, Criteria A–E

☐ Approve ☐ Revise ☐ Retire Notes:

Since choking on a piece of food 4 months ago, a 10-year-old refuses most solid foods, eats only liquids and soft foods, and has lost weight. She expresses no concern about body shape or size and wants to be "back to normal." Which diagnosis is MOST likely?

A. Anorexia nervosa

Anorexia nervosa involves fear of weight gain or disturbed body image, which she does not show.

B. Rumination disorder

Rumination disorder involves repeated regurgitation of food, not avoidance after choking.

C. ARFID ✓ KEY

ARFID covers restriction driven by fear of aversive consequences such as choking, without body image disturbance.

D. Pica

Pica involves persistent eating of nonnutritive, nonfood substances.

Rationale: Avoidant/restrictive food intake disorder involves restricted eating that leads to weight loss, nutritional deficiency, or impaired functioning, motivated by lack of interest in food, sensory aversions, or fear of aversive consequences such as choking or vomiting. It is distinguished from anorexia nervosa by the absence of fear of weight gain or disturbed body image.

References: DSM-5-TR — Avoidant/Restrictive Food Intake Disorder, Criteria A and C

☐ Approve ☐ Revise ☐ Retire Notes:

A 74-year-old retired teacher and her daughter both report a gradual decline in her memory over the past year. Testing shows a modest decline from her prior level. She still manages her own bills, medications, and shopping, although these tasks now take more effort and reminder lists. Which diagnosis BEST fits?

A. Major neurocognitive disorder

Major NCD requires substantial decline that interferes with independence in everyday activities.

B. Delirium

Delirium has an acute onset with fluctuating attention and awareness, not a gradual year-long decline.

C. Mild neurocognitive disorder ✓ KEY

Modest documented decline that does not interfere with independence, even if compensation is needed, fits mild NCD.

D. Dissociative amnesia

Dissociative amnesia involves inability to recall autobiographical information, usually traumatic, not a gradual cognitive decline.

Rationale: Mild and major neurocognitive disorder differ chiefly in functional impact. Mild NCD shows modest decline from a previous level, supported by concern and testing, without loss of independence in everyday activities, though greater effort or compensatory strategies may be needed. Major NCD involves significant decline that interferes with independence.

References: DSM-5-TR — Mild Neurocognitive Disorder, Criteria A–B

☐ Approve ☐ Revise ☐ Retire Notes:

An 81-year-old hospitalized for a urinary tract infection became confused over 2 days. Staff note she is alert one hour and disoriented the next. A counselor on the unit is asked to help distinguish delirium from major neurocognitive disorder. Which feature MOST points to delirium?

A. Acute onset with fluctuating attention ✓ KEY

Delirium develops over hours to days and fluctuates in severity, with disturbed attention and awareness.

B. Memory impairment found on screening

Memory deficits occur in both delirium and major NCD, so they do not distinguish them.

C. Onset of symptoms after the age of 65

Both conditions are more common in older adults; age does not separate them.

D. Presence of vivid visual hallucinations

Visual hallucinations can occur in delirium and in major NCD, notably with Lewy bodies.

Rationale: Delirium is a disturbance in attention and awareness that develops over a short period (hours to a few days), represents a change from baseline, and tends to fluctuate during the day. It often has a medical cause, such as infection or medication effects, and requires prompt medical evaluation.

References: DSM-5-TR — Delirium, Criteria A–B

☐ Approve ☐ Revise ☐ Retire Notes:

A 58-year-old whose wife died 7 months ago reports intense yearning for her nearly every day, feeling that part of himself has died, and difficulty re-engaging with friends. His grief is far beyond what is expected in his community and is impairing his work. Which statement about prolonged grief disorder is MOST accurate?

A. It can be diagnosed now because symptoms have lasted over 6 months

Six months is the threshold for children and adolescents, not adults.

B. It cannot yet be diagnosed because 12 months have not yet passed ✓ KEY

For adults, DSM-5-TR requires at least 12 months since the death.

C. It is not in DSM-5-TR, so his grief is coded only as a Z code

Prolonged grief disorder was added to the trauma- and stressor-related disorders in DSM-5-TR; uncomplicated bereavement is the Z code.

D. It is ruled out, and major depression must be diagnosed instead

Nothing here establishes a major depressive episode; yearning and identity disruption are grief-specific.

Rationale: DSM-5-TR added prolonged grief disorder to the trauma- and stressor-related disorders. It requires that at least 12 months have passed since the death for adults (6 months for children and adolescents), with intense yearning or preoccupation nearly every day for at least the last month, plus at least three additional symptoms such as identity disruption or emotional numbness.

References: DSM-5-TR — Prolonged Grief Disorder, Criterion A

☐ Approve ☐ Revise ☐ Retire Notes:

Six weeks after his mother died, a client reports sadness, poor sleep, and crying spells. Which feature is MORE typical of grief than of a major depressive episode?

A. Pervasive worthlessness and self-loathing

Pervasive worthlessness and self-loathing are more typical of a major depressive episode.

B. Persistent unhappiness unrelated to reminders

Mood that stays low regardless of reminders of the loss is more typical of depression.

C. Self-esteem that is generally preserved ✓ KEY

In grief, self-esteem is usually preserved, while depression often brings worthlessness.

D. Inability to anticipate any future pleasure

Pervasive anhedonia and pessimism about the future are characteristic of a depressive episode.

Rationale: DSM-5-TR notes that in grief the predominant feelings are emptiness and loss, dysphoria tends to come in waves ("pangs") linked to reminders of the deceased, humor can coexist with pain, and self-esteem is usually preserved. In a major depressive episode, mood is persistently low and is often accompanied by worthlessness and self-loathing.

References: DSM-5-TR — Major Depressive Disorder, note distinguishing grief from a major depressive episode

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor volunteers at a shelter the day after a tornado destroyed many homes. Survivors are shaken, tired, and worried about relatives. Which approach is MOST appropriate in this early phase?

A. Lead a required group debriefing on each survivor's experience

Mandatory single-session debriefing is not recommended and may increase distress for some survivors.

B. Give each survivor a structured PTSD diagnostic interview

Most survivors recover naturally; diagnostic interviews this early are premature and intrusive.

C. Start trauma-focused exposure work with the survivors who are upset

Trauma-focused therapy is for persistent symptoms after the acute phase, not as an immediate shelter intervention.

D. Offer psychological first aid for safety, comfort, and needs ✓ **KEY**

Psychological first aid promotes safety, calm, connection, and practical help, and is the recommended early response.

Rationale: In the immediate aftermath of a disaster, the recommended response is psychological first aid: ensuring safety, providing comfort, meeting practical needs, connecting people with loved ones and supports, and screening for those needing more help. Routine single-session psychological debriefing is not recommended for preventing PTSD.

References: Psychological First Aid Field Operations Guide (NCTSN/NCPTSD) — PFA core actions: safety and comfort, practical assistance, connection with social supports · NICE guidelines — NG116 Post-traumatic stress disorder: psychologically focused debriefing not offered

☐ Approve ☐ Revise ☐ Retire Notes:

A client calls the counselor sobbing after receiving an eviction notice. She says, "I just don't see the point of anything anymore." What should the counselor do FIRST?

A. Ask her directly about suicidal thoughts and intent ✓ **KEY**

A statement of hopelessness in crisis calls for direct assessment of suicidal ideation, intent, and plan before other steps.

B. Help her brainstorm options for finding new housing now

Problem solving is useful later, but safety must be assessed first.

C. Reflect feelings and save risk questions for later

Delaying risk assessment after a possible warning sign could miss imminent danger.

D. Give her the number of a local legal aid clinic

A referral addresses the stressor but skips assessment of her immediate safety.

Rationale: In crisis intervention, ensuring safety comes before problem solving. Hopeless statements such as "I don't see the point" are possible warning signs, and asking directly about suicidal thoughts does not increase risk. Structured tools such as the C-SSRS guide questions about ideation, intent, plan, and behavior.

References: C-SSRS — Screener: suicidal ideation and intent questions

☐ Approve ☐ Revise ☐ Retire Notes:

During the individual portion of a couples intake, one partner discloses that the other checks her phone, controls all the money, and shoved her last month. She says she is afraid of what will happen if he learns she told. What is the counselor's BEST next step?

A. Begin joint sessions on communication skills

Conjoint work is contraindicated while coercive control and fear are present, as it can increase danger.

B. Raise her disclosure in the next joint session

Revealing the disclosure to the partner could trigger retaliation.

C. Refer both partners to one anger management group

Anger management does not address coercive control, and joint referral may compromise her safety.

D. Assess danger with her alone and plan for safety ✓ KEY

Individual risk assessment and safety planning come first when violence and fear are disclosed.

Rationale: When intimate partner violence involving fear and coercive control is disclosed, conjoint counseling is generally contraindicated because sessions can provoke retaliation. The counselor should meet with the person alone, assess lethality and danger, develop a safety plan, and connect her with advocacy resources while protecting the confidentiality of the disclosure.

References: ACA Code of Ethics — A.4.a Avoiding Harm

► **Check:** IPV item cites general sources (ACA, Sue & Sue); no IPV-specific source on the approved list.

☐ Approve ☐ Revise ☐ Retire Notes:

A recently immigrated client discloses that her husband hits her and threatens that she will be deported if she leaves him, since her visa depends on his sponsorship. She speaks limited English and has no family nearby. Which counselor response is MOST helpful?

A. Advise her to stay until her immigration status is secure

This accepts the abuser's threat at face value and leaves her in danger without exploring legal protections.

B. Connect her with an immigration-informed IPV advocate ✓ KEY

Specialized advocates can explain protections available to abused immigrants and support culturally responsive safety planning.

C. Report the husband's conduct to immigration authorities

This is not the counselor's role, breaches confidentiality, and may put the client at greater risk.

D. Suggest couples counseling to ease conflict at home now

Couples counseling is contraindicated while there is ongoing violence and threats.

Rationale: Abusers of immigrant partners often use immigration status as a tool of control. Counselors should recognize this cultural and structural vulnerability, prioritize safety, provide interpreter and language access, and link clients with advocates who understand immigration-related protections rather than offering legal advice themselves.

References: Sue & Sue (Counseling the Culturally Diverse) — Immigrant and refugee clients: structural barriers and advocacy

► **Check:** IPV item cites general sources; the key deliberately avoids specific immigration-law (VAWA) claims.

☐ Approve ☐ Revise ☐ Retire Notes:

During a session at an elementary school, a 7-year-old tells the school counselor that her stepfather hit her with a belt last night, and the counselor sees welts on her arm. What should the counselor do FIRST?

A. Make a report to child protective services as the law requires of mandated reporters ✓ KEY

Reasonable suspicion of abuse triggers a mandated report; the counselor reports promptly and lets child protective services investigate.

B. Call the stepfather to hear his account of what happened before deciding to file any report

Contacting the alleged offender can put the child at risk and is not required before reporting; investigation belongs to CPS.

C. Examine and photograph all of the child's injuries so the evidence is complete before reporting

Counselors do not conduct forensic examinations; doing so can retraumatize the child and is the investigator's role.

D. Schedule a follow-up meeting for next week to see whether new marks appear before reporting

Waiting delays protection; the reporting duty arises at reasonable suspicion, which is already present.

Rationale: Counselors are mandated reporters in every U.S. state. The threshold is reasonable suspicion, not proof, and the report is made promptly to child protective services or law enforcement. The counselor's job is to report and support the child, not to investigate or confront the alleged perpetrator.

References: State law — Mandated reporting statutes for suspected child abuse and neglect · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes:

A 10-year-old client mentions that he is often home alone for whole weekends with little food in the house. He gives few details and changes the subject. A colleague advises the counselor not to report neglect until the boy gives a clear account. Which statement BEST reflects the reporting standard?

A. A report is required only after the child gives a clear and consistent disclosure

Children often disclose partially; the law does not require a complete or consistent disclosure before reporting.

B. A report is required only after a parent has a chance to explain the circumstances

Mandated reporters do not need to hear the caregiver's explanation first; that is part of the CPS investigation.

C. A report is required when there is reasonable suspicion; proof of neglect is not needed ✓ KEY

The legal threshold is reasonable cause to suspect abuse or neglect, which repeated lack of supervision and food can meet.

D. A report is required only once a supervisor independently confirms that neglect occurred

Consulting a supervisor is good practice, but the individual duty to report does not wait for anyone to confirm the facts.

Rationale: Mandated reporting laws use a reasonable-suspicion standard. Counselors report what they have observed or been told and leave fact-finding to child protective services. Neglect, including inadequate supervision and failure to provide food, is reportable in the same way as physical abuse.

References: State law — Child abuse and neglect reporting statutes: reasonable cause to suspect standard

☐ Approve ☐ Revise ☐ Retire Notes:

In Holland's theory, consistency describes how close the first two letters of a person's code sit on the RIASEC hexagon. Which Holland code shows the LOWEST consistency?

A. RIC (Realistic-Investigative)

Realistic and Investigative are adjacent on the hexagon, so this code is highly consistent.

B. SEA (Social-Enterprising)

Social and Enterprising are adjacent on the hexagon, so this code is highly consistent.

C. ECR (Enterprising-Conventional)

Enterprising and Conventional are adjacent on the hexagon, so this code is highly consistent.

D. RSI (Realistic-Social) ✓ KEY

Realistic and Social sit opposite each other on the hexagon, the least consistent pairing.

Rationale: Holland arranged the six types in the order R-I-A-S-E-C around a hexagon. Adjacent types share the most in common and opposite types the least, so a code whose first two letters are opposite (R-S, I-E, A-C) has low consistency. Consistency is distinct from differentiation (how sharply one type stands out) and congruence (person-environment fit).

References: Sharf (Career Development Theory) — Holland's theory of types: consistency, differentiation, congruence

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old machinist who expected to finish his career at one plant is laid off when it closes. He is now researching training programs, trying out new interests, and reconsidering what work means to him. Which concept from Super's life-span, life-space theory BEST describes this process?

A. Recycling through exploration tasks during a later life stage ✓ KEY

Super held that adults can recycle through earlier stage tasks, such as exploration, when a transition disrupts their career.

B. Circumscription of options that conflict with his social class

Circumscription belongs to Gottfredson's theory and describes early elimination of options by sex type and prestige.

C. Planned happenstance that turns the chance layoff into learning

Planned happenstance is Krumboltz's concept, not Super's, and describes using unplanned events, not a stage process.

D. Correspondence between his abilities and the reinforcers of work

Correspondence is the core idea of the theory of work adjustment (Dawis and Lofquist), not of Super's stage model.

Rationale: Super's stages (growth, exploration, establishment, maintenance, disengagement) form a maxicycle across the life span, but people can recycle through minicycles of earlier stages at transitions such as job loss. A midlife worker re-exploring options is the classic example of recycling.

References: Sharf (Career Development Theory) — Super's life-span, life-space theory: maxicycle, minicycles and recycling

☐ Approve ☐ Revise ☐ Retire Notes:

A 7-year-old girl who loves building things tells her counselor that she cannot be an engineer because "that is a job for boys." In Gottfredson's theory, what is this child doing?

A. Compromising by settling for a job that is more accessible to her

Compromise comes later, when people give up preferred options because of real-world barriers to access.

B. Circumscribing her options by ruling out jobs seen as wrong for her sex ✓ **KEY**

Ages 6 to 8 are Gottfredson's orientation to sex roles, when children eliminate jobs that seem wrong for their gender.

C. Showing low differentiation because her interests are not yet clear

Differentiation is a Holland concept about how distinct a profile is; her interest in building is actually clear.

D. Crystallizing a vocational preference as the exploration stage begins

Crystallization is a task in Super's exploration stage during adolescence, not a process of ruling options out in childhood.

Rationale: Gottfredson's theory of circumscription and compromise describes children progressively eliminating occupations: by size and power (ages 3-5), by sex type (ages 6-8), by social valuing or prestige (ages 9-13), and then by internal, unique self (14 and older). Options eliminated early are often never reconsidered, so counselors can help children revisit them.

References: Sharf (Career Development Theory) — Gottfredson's theory of circumscription and compromise: orientation to sex roles

☐ Approve ☐ Revise ☐ Retire Notes:

In couples sessions, one partner regularly rolls her eyes, mocks her spouse's tone, and calls him "pathetic." According to Gottman's research, which of the Four Horsemen is this, and the single strongest predictor of divorce?

A. Criticism

Criticism attacks the partner's character, but it lacks the superiority and disgust that mark contempt.

B. Stonewalling

Stonewalling is emotional withdrawal from the interaction, not mockery or eye-rolling.

C. Defensiveness

Defensiveness is self-protection through excuses or counterattack, not a display of disgust.

D. Contempt ✓ **KEY**

Eye-rolling, mockery and name-calling express contempt, which Gottman found to be the strongest predictor of divorce.

Rationale: Gottman's Four Horsemen are criticism, contempt, defensiveness and stonewalling. Contempt communicates superiority and disgust through sarcasm, mockery, eye-rolling and insults, and it was the single best predictor of divorce in his studies. The antidote is building a culture of appreciation and respect.

References: Gladding (Counseling: A Comprehensive Profession) — Marriage, couple and family counseling: Gottman's research on the Four Horsemen

► **Check:** Confirm the Gladding section that covers Gottman's Four Horsemen.

☐ Approve ☐ Revise ☐ Retire Notes:

A newly formed stepfamily seeks counseling because the 13-year-old daughter openly defies her new stepfather's rules, and he responds by giving stricter punishments. Which recommendation is MOST consistent with research on stepfamily adjustment?

A. Have the stepfather take over discipline so the teen accepts him as the new head of the home

Early stepparent discipline before a bond forms tends to increase conflict and resentment.

B. Have the biological parent lead discipline while the stepparent builds a warm relationship first ✓ KEY

Stepfamily research supports the biological parent handling discipline early while the stepparent focuses on connection.

C. Have both adults avoid setting any rules until the teen says she is ready to accept the stepfather

Removing structure leaves the teen without limits and puts her in charge of the family's functioning.

D. Have the teen spend more time with her other parent until she is willing to follow the new rules

Using contact with the other parent as leverage triangulates the teen and can deepen loyalty conflicts.

Rationale: Stepfamilies form after losses and bring loyalty binds. Stepparents gain authority gradually, after a relationship exists, so the biological parent usually remains the primary disciplinarian at first while the stepparent acts more like a supportive adult. The couple still agrees on house rules together.

References: Gladding (Counseling: A Comprehensive Profession) — Family counseling: blended families and stepparent roles

► **Check:** Confirm the Gladding section that covers stepfamily discipline roles.

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor is about to begin conjoint counseling with a married couple. Each partner may also call or meet with the counselor alone from time to time. What should the counselor establish at the outset?

A. A promise that any individual disclosure will stay sealed from the other partner

Promising to hold secrets can trap the counselor in a hidden alliance and undermine the couple work.

B. An agreement that each partner will be assigned their own separate counselor

Separate counselors are an option but do not answer how this counselor will handle the couple case.

C. A clear policy, agreed by both, on how information shared individually is handled ✓ KEY

Clarifying who the client is and how individual information will be handled is required at the outset of multiple-client work.

D. A rule that the counselor will refuse to speak to either partner outside sessions

Some contact outside joint sessions can be appropriate; the issue is a clear policy, not a ban.

Rationale: When counseling more than one person who share a relationship, counselors clarify at the outset who the client is and the limits of confidentiality, including whether a no-secrets or limited-secrets policy applies. Both partners agree to that policy before individual contacts happen.

References: ACA Code of Ethics — A.8 Multiple Clients; B.4.b Couples and Family Counseling

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor uses Schlossberg's transition model with a 58-year-old woman who has just retired early to care for her spouse. Which set of factors does the model use to assess her resources for coping?

A. Situation, self, support and strategies ✓ KEY

Schlossberg's 4 S's are situation, self, support and strategies, used to weigh a person's coping resources.

B. Denial, anger, bargaining and acceptance

These are stages from Kübler-Ross's grief model, not Schlossberg's factors for coping with a transition.

C. Growth, exploration, establishment and decline

These are life stages in Super's career theory, not resources for coping with a transition.

D. Shock, withdrawal, adjustment and integration

This is a generic phase sequence, not the four coping-resource factors of Schlossberg's model.

Rationale: Schlossberg defines a transition as any event or non-event that changes roles, relationships, routines and assumptions. Coping is assessed through the 4 S's: the situation (timing, control, concurrent stress), the self (personal and psychological resources), support (relationships and institutions) and strategies (ways of coping).

References: Sharf (Career Development Theory) — Adult career transitions: Schlossberg's transition model (4 S's)

► **Check:** Confirm Sharf covers Schlossberg's 4 S model (adult transitions).

☐ Approve ☐ Revise ☐ Retire Notes:

A 24-year-old client lives with his parents, has changed jobs and majors several times, and says he feels "not really a kid, but not quite an adult." He is still exploring identity, work and relationships. Which concept BEST fits this period?

A. Identity foreclosure

Foreclosure is commitment without exploration; this client is exploring and has not committed.

B. Midlife transition

Levinson's midlife transition occurs around age 40, not in the early twenties.

C. Generativity versus stagnation

Erikson's generativity stage concerns middle adulthood and guiding the next generation.

D. Emerging adulthood ✓ KEY

Arnett described emerging adulthood (about ages 18 to 25) as a time of identity exploration and feeling in-between.

Rationale: Arnett proposed emerging adulthood as a distinct period, roughly ages 18 to 25 in industrialized societies, marked by identity exploration, instability, self-focus, feeling in-between and a sense of possibilities. Counselors can frame this exploration as developmentally typical rather than as failure to launch.

References: Berk (Development Through the Lifespan) — Early adulthood: Arnett's emerging adulthood

☐ Approve ☐ Revise ☐ Retire Notes:

A 79-year-old widower in assisted living spends sessions reviewing his life, some proud moments and some regrets, and asks whether his life "added up to anything." Which of Erikson's psychosocial stages is he working through?

A. Generativity versus stagnation

Generativity is the middle-adulthood task of guiding the next generation and being productive.

B. Intimacy versus isolation

Intimacy is the young-adult task of forming close, committed relationships.

C. Integrity versus despair ✓ KEY

Late adulthood centers on accepting one's life as meaningful (integrity) rather than feeling regret (despair).

D. Identity versus role confusion

Identity versus role confusion is the adolescent task of forming a coherent sense of self.

Rationale: Erikson's final stage, integrity versus despair, involves looking back and accepting one's life as it was. Life review, described by Butler, is a structured way to help older adults integrate successes and regrets and move toward integrity.

References: Berk (Development Through the Lifespan) — Late adulthood: Erikson's ego integrity versus despair; life review

☐ Approve ☐ Revise ☐ Retire Notes:

An adult daughter brings her 74-year-old mother to counseling, worried about "early dementia." Over two months the mother has lost interest in her garden, sleeps poorly, and often says "I just can't remember anything anymore." She seems distressed by her memory lapses. What should the counselor do FIRST?

A. Screen for depression and refer for a medical evaluation of the memory complaints ✓ KEY

Depression in older adults can mimic cognitive decline, and a medical workup rules out treatable causes.

B. Tell the family that memory loss of this kind is a normal part of growing older

Noticeable decline with mood change is not normal aging and should not be dismissed.

C. Begin memory-training exercises in sessions to slow the progression of dementia

This assumes a dementia diagnosis that has not been made and skips assessment of depression.

D. Recommend that the daughter begin looking into memory-care residential placement

Placement planning is premature without assessment and could cause harm if the cause is depression.

Rationale: Late-life depression often presents with memory and concentration complaints, sometimes called pseudodementia. Clues include recent onset with mood symptoms and a person who is distressed by and emphasizes their lapses. The counselor screens for depression (for example with the Geriatric Depression Scale) and coordinates a medical evaluation.

References: DSM-5-TR — Major depressive disorder: diminished ability to think or concentrate; differential with major neurocognitive disorder

☐ Approve ☐ Revise ☐ Retire Notes:

A client who uses a wheelchair after a spinal cord injury says, "My body is not the problem. The problem is buildings without ramps and managers who assume I can't do the job." Which model of disability does her statement reflect?

A. Medical model

The medical model locates disability in the individual's impairment, to be diagnosed and treated.

B. Moral model

The moral model treats disability as a sign of sin or a test of character.

C. Rehabilitation model

This model focuses on restoring individual function, still locating the problem in the person.

D. Social model ✓ KEY

The social model locates disability in environmental and attitudinal barriers, not in the person.

Rationale: The social model distinguishes impairment (a bodily difference) from disability (exclusion created by inaccessible environments and attitudes). Counselors working from it attend to barriers, advocacy and accommodation as well as the client's adjustment.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling individuals with disabilities: medical versus social models of disability

► **Check:** Confirm Sue & Sue's disability coverage; 'rehabilitation model' distractor is a looser term than medical/moral/social.

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old client with type 1 diabetes often skips glucose checks and insulin because, she says, "thinking about it makes it too real." Her blood sugar control has worsened and she has had two ER visits. She reports no persistent low mood, worry about having other illnesses, or other psychiatric symptoms. Which DSM-5-TR diagnosis BEST fits?

A. Illness anxiety disorder of the care-avoidant type

Illness anxiety involves preoccupation with having or getting a serious illness, which she does not report.

B. Psychological factors affecting other medical conditions ✓ KEY

Her avoidance is a psychological factor that worsens a diagnosed medical condition, with no other mental disorder.

C. Somatic symptom disorder with a persistent course

Somatic symptom disorder requires distressing physical symptoms with excessive related thoughts, not avoidance of care.

D. Factitious disorder imposed on self, with recurrent episodes

Factitious disorder involves deliberately falsifying or inducing illness, which is not described here.

Rationale: Psychological factors affecting other medical conditions applies when psychological or behavioral factors, such as denial or poor adherence, adversely affect a medical condition and are not better explained by another mental disorder. It is listed with somatic symptom and related disorders but does not require distressing somatic symptoms.

References: DSM-5-TR — Psychological Factors Affecting Other Medical Conditions: criteria A-B

☐ Approve ☐ Revise ☐ Retire Notes:

A bisexual client has never experienced overt harassment, yet she hides her identity at work, constantly scans for signs of rejection, and says part of her believes something is wrong with her. In Meyer's minority stress model, these experiences are BEST described as which kind of stressors?

A. Distal stressors from discrimination

Distal stressors are external events such as harassment or discrimination, which she has not experienced.

B. General life stressors shared by all

These stressors are tied specifically to her stigmatized identity, not common to everyone.

C. Proximal stressors tied to stigma ✓ KEY

Concealment, expected rejection and internalized stigma are Meyer's proximal (internal) minority stressors.

D. Coping resources within the community

Community connection is a protective factor in the model, not a source of stress.

Rationale: Meyer's model separates distal stressors (external prejudice events, discrimination, violence) from proximal stressors (expectations of rejection, concealment and internalized homophobia or biphobia). Both add to the general stress everyone faces, and community connection and coping can buffer their effects on mental health.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling LGBTQ individuals: minority stress

☐ Approve

☐ Revise

☐ Retire

Notes:

The parents of a 16-year-old who recently came out as gay ask a counselor to "help him become straight." The teen does not want to change his orientation. How should the counselor respond?

A. Decline to attempt orientation change and offer support for the teen and family ✓ KEY

Orientation change efforts lack scientific support and carry risk of harm; family support work is appropriate.

B. Agree to the goal because parents hold legal authority over a minor's treatment

Parental consent does not make a harmful, unsupported technique ethical to use.

C. Refer the family to a provider who specializes in sexual orientation change work

Referring for a harmful practice still facilitates it and does not protect the client.

D. Explore with the teen whether his orientation might be a phase that will soon pass

Framing his identity as a phase invalidates it and reflects the parents' goal, not his own.

Rationale: The ACA Code bars techniques with substantial evidence of harm even when requested, and sexual orientation change efforts fall in that category. The counselor can decline that goal while still helping the family: providing education, supporting the parents' reactions and reducing family rejection, which is a strong risk factor for LGBTQ+ youth.

References: ACA Code of Ethics — C.7.a Scientific Basis for Treatment; C.7.c Harmful Practices

☐ Approve

☐ Revise

☐ Retire

Notes:

A man who immigrated from Vietnam 15 years ago has stopped speaking Vietnamese, no longer observes family traditions, and says he identifies only with mainstream American culture. Which of Berry's acculturation strategies does this describe?

A. Integration

Integration keeps the heritage culture while also taking part in the host culture.

B. Assimilation ✓ KEY

Assimilation means giving up heritage culture and adopting the host culture.

C. Separation

Separation keeps the heritage culture while avoiding the host culture.

D. Marginalization

Marginalization means little connection to either the heritage or host culture.

Rationale: Berry's model crosses two questions: is the heritage culture maintained, and is contact with the host society sought? Yes and yes is integration; no and yes is assimilation; yes and no is separation; no and no is marginalization.

References: Sue & Sue (Counseling the Culturally Diverse) — Acculturation: Berry's four acculturation strategies

☐ Approve ☐ Revise ☐ Retire Notes:

A refugee teen has drifted away from her parents' language and customs but also feels unwelcome and isolated among peers at her new school. In Berry's framework, which strategy does this reflect, and how does it generally compare with the others in acculturative stress?

A. Separation, generally tied to the lowest acculturative stress

Separation keeps heritage culture; she has moved away from hers, and integration, not separation, is lowest in stress.

B. Integration, generally tied to the highest acculturative stress

She does not hold both cultures, and integration is usually associated with the least stress.

C. Assimilation, generally tied to the lowest acculturative stress

She has not joined the host culture, and assimilation is not the lowest-stress strategy.

D. Marginalization, generally tied to the highest acculturative stress ✓ KEY

Losing ties to both cultures is marginalization, which Berry found carries the greatest acculturative stress.

Rationale: In Berry's research, integration is generally associated with the best adaptation and least acculturative stress, while marginalization, low engagement with both cultures, is associated with the most. Assimilation and separation fall in between. Counselors can help marginalized clients rebuild connection to one or both cultures.

References: Sue & Sue (Counseling the Culturally Diverse) — Acculturation and acculturative stress: Berry's model

☐ Approve ☐ Revise ☐ Retire Notes:

A devout client feels intense guilt about her divorce because of her faith community's teachings. The counselor is not religious and privately views the teachings as harmful. Which approach is MOST appropriate?

A. Encourage her to question the teachings that are leading her to feel guilty

Steering her away from her beliefs imposes the counselor's values on the client.

B. Explore her guilt within her own faith framework and the resources it offers ✓ **KEY**

Working within the client's belief system respects her values and can draw on her faith as a resource.

C. Refer her to clergy because religious concerns fall outside counseling scope

Spiritual concerns are within counseling scope; referral based on the topic alone is not warranted.

D. Disclose the counselor's own views so she can weigh both sides of the issue

Sharing personal views on her religion serves the counselor and risks pressuring the client.

Rationale: Counselors avoid imposing their own values, attitudes and beliefs and respect the diversity of clients' spiritual lives. Exploring guilt within the client's framework, including forgiveness or grace in her tradition, honors her worldview and can turn faith into a source of coping.

References: ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 15-year-old discloses repeated nonsuicidal self-injury and says, "When everything builds up, it is the only thing that makes the feeling stop." Which function of self-injury does her statement MOST clearly reflect?

A. A wish to end her life that has not yet been spoken aloud

NSSI by definition lacks suicidal intent; her words describe emotional relief, not a wish to die.

B. A bid to get attention from peers and adults around her

NSSI is often hidden; attributing it to attention-seeking is a common, inaccurate assumption here.

C. Relief from intense negative emotion (affect regulation) ✓ **KEY**

Affect regulation, gaining relief from overwhelming feelings, is the most common reported function of NSSI.

D. Self-punishment for having broken a rule in her family

Self-punishment is a possible function, but she describes stopping a feeling, not punishing herself.

Rationale: Research consistently finds that the most common function of nonsuicidal self-injury is affect regulation: obtaining relief from a negative feeling or cognitive state. DSM-5-TR lists this expectation in its proposed criteria for nonsuicidal self-injury disorder. Treatment builds alternative regulation skills, as in DBT.

References: DSM-5-TR — Conditions for Further Study: Nonsuicidal Self-Injury Disorder, criterion B

☐ Approve ☐ Revise ☐ Retire Notes: _____

A college student shows the counselor healed marks on her forearm and says she has hurt herself during stressful weeks for two years. She denies wanting to die. Which counselor response is BEST?

A. Ask her to sign a no-harm contract before agreeing to continue in counseling

No-harm contracts lack evidence of benefit and can shut down honest disclosure.

B. Arrange an inpatient admission because self-injury signals imminent danger

NSSI without suicidal intent or other acute risk does not by itself call for hospitalization.

C. Respond calmly, explore what the behavior does for her, and assess suicide risk ✓ **KEY**

A calm, curious stance keeps her engaged, and NSSI still raises suicide risk, so risk is assessed.

D. Avoid discussing the marks so that attention does not reinforce the behavior

Ignoring the disclosure misses assessment and can feel dismissive, discouraging further honesty.

Rationale: A respectful, nonjudgmental response to self-injury builds trust and allows functional assessment. NSSI is distinct from a suicide attempt but is a strong risk factor for later suicidal behavior, so counselors also assess suicidal ideation and behavior directly, for example with the C-SSRS.

References: C-SSRS — Suicidal behavior section: distinguishing nonsuicidal self-injurious behavior from suicide attempts

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is using cognitive behavioral therapy for insomnia (CBT-I) with a client who lies awake in bed for hours most nights. Which instruction is part of the stimulus control component?

A. Avoid caffeine after noon and keep the bedroom cool, dark and quiet

These are sleep hygiene recommendations, a separate and weaker component of insomnia care.

B. Limit time in bed to the hours actually slept, then extend it slowly

This describes sleep restriction, a different CBT-I component that consolidates sleep.

C. Practice progressive muscle relaxation for twenty minutes at bedtime

Relaxation training is a separate CBT-I component aimed at lowering arousal.

D. Get out of bed if unable to sleep and return only once feeling sleepy ✓ **KEY**

Stimulus control re-links the bed with sleep by leaving it when awake and returning when sleepy.

Rationale: Stimulus control therapy aims to restore the bed and bedroom as cues for sleep: go to bed only when sleepy, use the bed only for sleep and sex, leave the bed when unable to sleep, keep a fixed wake time and avoid daytime naps. It is combined with sleep restriction, cognitive work and education in CBT-I, the recommended first-line treatment for chronic insomnia.

References: AASM Clinical Practice Guideline (Insomnia) — Behavioral and psychological treatments for chronic insomnia (2021): CBT-I recommended; stimulus control as a component · NICE guidelines — Clinical Knowledge Summary: Insomnia, CBT-I as first-line management of long-term insomnia

☐ Approve ☐ Revise ☐ Retire Notes:

A client reports difficulty staying asleep, with daytime fatigue that affects her work. The problem is not explained by a medical condition, substance or other sleep disorder. According to DSM-5-TR, how often and for how long must the sleep difficulty occur to meet criteria for insomnia disorder?

A. At least 3 nights a week for at least 3 months ✓ KEY

DSM-5-TR requires the difficulty at least 3 nights per week, present for at least 3 months.

B. At least 5 nights a week for at least 2 weeks

This is far shorter than required; two weeks fits a brief, situational sleep problem.

C. At least 1 night a week for at least 6 months

One night a week is below the frequency threshold for insomnia disorder.

D. At least 4 nights a week for at least 1 month

One month falls short of the 3-month duration; brief insomnia is coded as other specified insomnia disorder.

Rationale: DSM-5-TR insomnia disorder requires dissatisfaction with sleep quantity or quality (trouble initiating sleep, maintaining sleep or early waking) occurring at least 3 nights per week for at least 3 months, despite adequate opportunity for sleep, with clinically significant distress or impairment.

References: DSM-5-TR — Insomnia Disorder: criteria C and D (frequency and duration)

☐ Approve ☐ Revise ☐ Retire Notes: _____

150. nce-s-clf-054 · Counseling and Helping Relationships · psychopharmacology basics for counselors · hard · In review

A client with bipolar I disorder who takes lithium had a stomach virus with vomiting for two days. In session she has a coarse hand tremor, seems confused, and walks unsteadily. What should the counselor do?

A. Reassure her that tremor is a common side effect and continue the planned session

A mild tremor can occur on lithium, but confusion and unsteady gait after dehydration suggest toxicity.

B. Suggest she skip her next lithium dose until she feels better, then call her prescriber

Advising medication changes is outside counselor scope, and the symptoms need urgent evaluation now.

C. Arrange immediate medical evaluation because the symptoms may signal lithium toxicity ✓ KEY

Dehydration can raise lithium levels; confusion, coarse tremor and ataxia warrant urgent medical care.

D. Explore whether anxiety about her illness is causing her shaking and trouble focusing

Attributing these signs to anxiety risks missing a medical emergency.

Rationale: Lithium has a narrow therapeutic range and requires regular blood-level monitoring. Vomiting, diarrhea, dehydration and some medications can raise levels into the toxic range, producing coarse tremor, confusion, ataxia and gastrointestinal symptoms. Counselors do not adjust medication but recognize warning signs and arrange urgent medical evaluation.

References: NICE guidelines — Bipolar disorder (CG185): lithium monitoring and toxicity

☐ Approve ☐ Revise ☐ Retire Notes: _____

A client with major depressive disorder started an SSRI prescribed by her physician 10 days ago. She says, "It is not doing anything, so I am going to stop taking it." What is the counselor's BEST response?

A. Agree that stopping makes sense since there has been no improvement

Ten days is too early to judge, and stopping without the prescriber is not the counselor's call to endorse.

B. Explain SSRIs often take weeks to work; urge a talk with the prescriber ✓ **KEY**

SSRI benefit usually builds over several weeks, and medication decisions belong with the prescriber.

C. Suggest she ask her physician for a benzodiazepine to work more quickly

Recommending a specific drug is outside counselor scope, and benzodiazepines carry dependence risk.

D. Advise her to take half the dose for a week to see if she feels different

Counselors do not advise dose changes; that is a prescriber decision.

Rationale: SSRIs typically take several weeks to produce a full antidepressant effect, although side effects can appear sooner. Counselors provide basic education, support adherence and coordinate with the prescriber, but they do not recommend starting, stopping or changing specific medications.

References: APA CPG — Depression across the lifespan: second-generation antidepressants · ACA Code of Ethics — C.2.a Boundaries of Competence

☐ Approve ☐ Revise ☐ Retire Notes:

152. nce-s-clf-056 · Human Growth and Development · school and academic concerns · medium · In review

A 9-year-old with ADHD performs at grade level when given extended test time and a seat near the teacher. He does not need specially designed instruction. Which school plan BEST fits his needs?

A. An Individualized Education Program under IDEA

An IEP is for students who need specially designed instruction, which he does not.

B. A gifted education plan for advanced placement

Nothing indicates a need for gifted programming.

C. A behavior intervention plan for disruptive acts

A behavior plan targets problem behaviors; the need described is for accommodations.

D. A Section 504 plan for classroom accommodations ✓ **KEY**

Section 504 provides accommodations for a disability that limits learning without special education.

Rationale: Section 504 of the Rehabilitation Act covers students with a disability that substantially limits a major life activity such as learning, and provides accommodations such as extended time or preferential seating. IDEA provides an IEP only for students in eligible categories who need specially designed instruction.

References: Gladding (Counseling: A Comprehensive Profession) — School counseling: Section 504 plans and IEPs under IDEA

☐ Approve ☐ Revise ☐ Retire Notes:

An 8-year-old has missed most of three weeks of school, crying and complaining of stomachaches each morning that ease once his mother lets him stay home. He worries something bad will happen to her while he is away. Pediatric causes have been ruled out. Which plan is MOST appropriate?

A. Arrange home-based instruction until his worry about his mother eases on its own

Staying home relieves anxiety in the short term but reinforces avoidance and lets the fear grow.

B. A gradual return to school with exposure, coping skills and school collaboration ✓ **KEY**

Graduated exposure with parent and school coordination is the evidence-based approach to anxiety-based school refusal.

C. Tell his mother to keep him home on any morning that he has physical complaints

Making symptoms the ticket to stay home rewards them and maintains the cycle.

D. Delay any school plan until weekly play therapy has resolved the underlying fear

Each week out of school makes return harder; return should begin alongside treatment.

Rationale: School refusal driven by separation anxiety is maintained by avoidance, because staying home brings immediate relief. Treatment uses CBT with graduated exposure to attending school, coping skills, parent training to reduce accommodation, and close collaboration with school staff to support a prompt, stepwise return.

References: DSM-5-TR — Separation Anxiety Disorder: reluctance or refusal to go to school; physical complaints on anticipated separation

☐ Approve ☐ Revise ☐ Retire Notes:

A 31-year-old Army veteran meets criteria for PTSD after combat deployments. He is willing to engage in therapy and asks what treatment has the strongest support. According to the VA/DoD clinical practice guideline, which approach is recommended as first-line?

A. Individual, manualized trauma-focused therapy such as PE or CPT ✓ **KEY**

The VA/DoD guideline recommends manualized trauma-focused psychotherapies such as PE, CPT and EMDR first.

B. A benzodiazepine to reduce his hyperarousal and improve sleep

The guideline recommends against benzodiazepines for PTSD because of harm and lack of benefit.

C. Single-session psychological debriefing about his deployments

Psychological debriefing is not supported for PTSD and may worsen outcomes.

D. Nondirective supportive counseling without trauma processing

Supportive counseling is less effective than trauma-focused therapy and is not first-line.

Rationale: The VA/DoD PTSD guideline recommends individual, manualized trauma-focused psychotherapies, chiefly prolonged exposure (PE), cognitive processing therapy (CPT) and EMDR, over pharmacotherapy. It recommends against benzodiazepines for PTSD. Counselors working with veterans should know these options and refer when they are not trained to deliver them.

References: VA/DoD CPG — Management of PTSD and Acute Stress Disorder: trauma-focused psychotherapy recommendations

► **Check:** How VA/DoD 2023 grades debriefing and supportive counseling (distractors) is from memory; verify.

☐ Approve ☐ Revise ☐ Retire Notes:

A Marine veteran says his worst memory is not being in danger himself but following an order that led to civilian deaths. He is consumed by guilt and shame, believes he is "a bad person," and has pulled away from his faith community. Which concept BEST describes his distress?

A. Moral injury from acts that violated his own deeply held moral beliefs ✓ KEY

Moral injury describes guilt, shame and loss of trust after perpetrating or witnessing acts that violate one's moral code.

B. Compassion fatigue from caring for wounded civilians and fellow troops

Compassion fatigue affects helpers worn down by caregiving; his distress centers on his own actions.

C. Survivor guilt from having lived through events in which comrades died

Survivor guilt centers on surviving when others did not; his guilt is about what he did.

D. Adjustment disorder triggered by the stress of returning to civilian life

His distress is tied to a specific morally violating combat event, not to the transition home.

Rationale: Moral injury, described by Litz and colleagues, is lasting distress after perpetrating, failing to prevent or witnessing acts that transgress deeply held moral beliefs. It shows up mainly as guilt, shame, self-condemnation and spiritual or relational withdrawal rather than fear. It overlaps with PTSD criterion D (negative beliefs about oneself, guilt, shame) but is not a DSM diagnosis.

References: DSM-5-TR — PTSD criterion D: persistent negative beliefs about oneself and persistent guilt or shame

► **Check:** Moral injury (Litz et al.) is not a DSM construct; the DSM-5-TR reference points to PTSD criterion D only.

☐ Approve ☐ Revise ☐ Retire Notes:

A 44-year-old with major depressive disorder has had a depressive episode each of the last three winters, with full remission every spring. A counselor is considering the "with seasonal pattern" specifier. Which finding would argue MOST strongly AGAINST using it?

A. She sleeps more and craves carbohydrates in her episodes

Hypersomnia and carbohydrate craving are common in winter-pattern episodes and are consistent with the specifier.

B. She lives far north, where winter daylight is quite short

Higher latitude is associated with winter-pattern depression and does not argue against the specifier.

C. Her construction job lays her off each winter, without pay ✓ KEY

DSM-5-TR excludes cases in which an obvious seasonal psychosocial stressor, such as regular seasonal unemployment, explains the timing.

D. Each episode has ended completely by the following April

Full remission at a characteristic time of year is one of the requirements for the specifier.

Rationale: The seasonal pattern specifier requires a regular temporal relationship between episode onset and a particular time of year, full remission (or a switch to mania or hypomania) at a characteristic time, two seasonal episodes in the last 2 years with no nonseasonal episodes in that period, and seasonal episodes substantially outnumbering nonseasonal ones over the lifetime. It does not apply when an obvious season-linked psychosocial stressor, such as being unemployed every winter, accounts for the pattern.

References: DSM-5-TR — Specifiers for Depressive Disorders: with seasonal pattern

☐ Approve ☐ Revise ☐ Retire Notes:

A 29-year-old developed a major depressive episode during the second trimester of her first pregnancy. Her midwife assumed the "with peripartum onset" specifier could only be used after delivery. Which statement is accurate under DSM-5-TR?

A. It applies to onset in pregnancy or within 4 weeks after delivery ✓ KEY

DSM-5-TR applies the peripartum onset specifier when mood symptoms begin during pregnancy or in the 4 weeks following delivery.

B. It applies only to episodes that begin after the baby is delivered

DSM-5 replaced the older "postpartum onset" specifier with "peripartum onset" to include onset during pregnancy.

C. It applies to any onset up to one year following the delivery date

One year is used in some research and screening, but the DSM-5-TR specifier window is 4 weeks after delivery.

D. It can be used only when the episode includes psychotic features

Psychotic features are not required; peripartum episodes may occur with or without psychosis.

Rationale: DSM-5-TR uses "with peripartum onset" when the onset of mood symptoms occurs during pregnancy or in the 4 weeks following delivery. A substantial share of postpartum major depressive episodes actually begin during pregnancy, which is why the earlier postpartum-only specifier was broadened. Peripartum episodes should be distinguished from the brief, milder "baby blues."

References: DSM-5-TR — Specifiers for Depressive Disorders: with peripartum onset

☐ Approve ☐ Revise ☐ Retire Notes:

A client in a current major depressive episode also says she feels keyed up and tense most days and has a nagging fear that something awful is about to happen. She does not meet criteria for an anxiety disorder. Which specifier BEST captures these features?

A. With mixed features

"With mixed features" refers to manic or hypomanic symptoms during a depressive episode, not anxiety.

B. With anxious distress ✓ KEY

Feeling keyed up or tense and fearing that something awful may happen are two of the five anxious distress symptoms; two are required.

C. With melancholic features

Melancholic features center on near-total loss of pleasure, early-morning awakening, and marked psychomotor change.

D. With atypical features

Atypical features involve mood reactivity plus symptoms such as hypersomnia, leaden paralysis, and rejection sensitivity.

Rationale: The anxious distress specifier requires at least two of five symptoms during most days of the episode: feeling keyed up or tense, feeling unusually restless, difficulty concentrating because of worry, fear that something awful may happen, and feeling one might lose control. Anxious distress is linked to higher suicide risk, longer illness, and poorer treatment response, so it is worth assessing even when no separate anxiety disorder is present.

References: DSM-5-TR — Specifiers for Depressive Disorders: with anxious distress

☐ Approve ☐ Revise ☐ Retire Notes:

For about 3 years, a 34-year-old graphic designer has swung between stretches of high energy, talkativeness, and big plans and stretches of low mood and fatigue. Neither the highs nor the lows have ever met full criteria for a hypomanic, manic, or major depressive episode, and she is rarely symptom-free for more than a month. Which diagnosis BEST fits?

A. Bipolar II disorder

Bipolar II requires at least one full hypomanic episode and at least one major depressive episode.

B. Persistent depressive disorder

Persistent depressive disorder involves chronic depressed mood without the recurring hypomanic symptoms she reports.

C. Bipolar I disorder, mild

Bipolar I requires at least one full manic episode, which has never occurred.

D. Cyclothymic disorder ✓ KEY

Two or more years of numerous subthreshold hypomanic and depressive periods, with no gap over 2 months and no full episodes, fits cyclothymic disorder.

Rationale: Cyclothymic disorder requires at least 2 years (1 year in children and adolescents) of numerous periods with hypomanic symptoms and numerous periods with depressive symptoms that never meet full episode criteria. Symptoms are present at least half the time, the person is never symptom-free for more than 2 months, and criteria for a major depressive, manic, or hypomanic episode have never been met.

References: DSM-5-TR — Cyclothymic Disorder, Criteria A–C

☐ Approve ☐ Revise ☐ Retire Notes:

Three weeks after starting an antidepressant for depression, a 38-year-old develops elevated mood, grandiosity, little need for sleep, and reckless spending. His prescriber stops the medication, but the full manic syndrome continues well beyond the drug's expected physiological effect. Which statement is MOST accurate?

A. Mania that starts on an antidepressant is coded only as medication-induced

Onset during antidepressant treatment does not by itself settle the question; persistence beyond the drug effect matters.

B. Persisting at full level past the drug effect supports bipolar I disorder ✓ KEY

DSM-5-TR states that a full manic episode persisting beyond the physiological effect of antidepressant treatment is sufficient evidence for bipolar I.

C. Bipolar II should be diagnosed because a medication triggered the mood

Bipolar II requires hypomanic, not manic, episodes; the trigger does not change a full manic syndrome into hypomania.

D. No bipolar diagnosis can be made until a medication-free year has passed

DSM-5-TR sets no medication-free waiting period; the criterion is persistence beyond the physiological effect.

Rationale: DSM-5-TR notes that a full manic episode emerging during antidepressant treatment (medication or electroconvulsive therapy) that persists at a fully syndromal level beyond the physiological effect of that treatment is sufficient evidence for a manic episode and therefore a bipolar I diagnosis. If the symptoms resolve as the drug effect wears off, a substance/medication-induced bipolar and related disorder is more appropriate.

References: DSM-5-TR — Bipolar I Disorder, Manic Episode Criterion D note on antidepressant treatment

☐ Approve ☐ Revise ☐ Retire Notes:

For 8 months, a 47-year-old has avoided buses, crowded grocery stores, and long checkout lines. She fears that if she suddenly felt faint or lost bladder control, escape would be hard and no one would help. She has never had an unexpected panic attack and is not worried about being judged by others. Which diagnosis BEST fits?

A. Specific phobia, situational type

Specific phobia centers on one circumscribed situation; fear across several situations for this escape-related reason points to agoraphobia.

B. Social anxiety disorder

Social anxiety centers on fear of scrutiny or negative evaluation, which she denies.

C. Agoraphobia ✓ KEY

Fear in two or more agoraphobic situations because escape may be difficult or help unavailable if incapacitating symptoms occur fits agoraphobia.

D. Panic disorder

Panic disorder requires recurrent unexpected panic attacks, which she has never had.

Rationale: Agoraphobia requires marked fear or anxiety about at least two of five situations: public transportation, open spaces, enclosed spaces, standing in line or being in a crowd, and being outside the home alone. The fear is driven by thoughts that escape might be difficult or help unavailable if panic-like or other incapacitating or embarrassing symptoms occur. Since DSM-5, agoraphobia can be diagnosed without panic disorder, and it typically lasts 6 months or more.

References: DSM-5-TR — Agoraphobia, Criteria A–B

☐ Approve ☐ Revise ☐ Retire Notes:

A 6-year-old has not spoken at school for 4 months, although she talks freely with her parents at home. Her teacher refers her for evaluation of selective mutism. Which additional finding would make selective mutism LESS likely as the explanation?

A. Her family moved recently and she is still learning the school language ✓ KEY

DSM-5-TR excludes failure to speak that is due to lack of knowledge of, or comfort with, the language required in that setting.

B. She whispers to one close friend at recess but to no one else there

Limited speech with one trusted peer is common in selective mutism and does not argue against it.

C. Her silence has lasted well beyond the first month of the school year

Duration of at least 1 month, not limited to the first month of school, is a requirement and supports the diagnosis.

D. Her parents describe her as very shy and anxious in new situations

Shyness and social anxiety are commonly associated with selective mutism.

Rationale: Selective mutism is a consistent failure to speak in specific social situations where speech is expected, such as school, despite speaking in other situations. It must last at least 1 month (not limited to the first month of school), interfere with achievement or communication, and not be due to lack of knowledge of or comfort with the spoken language. Counselors working with newly arrived immigrant children should consider normal second-language "silent periods" before diagnosing.

References: DSM-5-TR — Selective Mutism, Criteria A–D

☐ Approve ☐ Revise ☐ Retire Notes:

Five weeks after being unexpectedly laid off from a job of 20 years, a 56-year-old reports sadness, frequent replaying of the termination meeting, poor sleep, and withdrawal from friends. His distress is out of proportion to what is typical and is affecting family life. Criteria for a major depressive episode are not met. Which diagnosis is MOST appropriate?

A. Posttraumatic stress disorder

PTSD requires exposure to actual or threatened death, serious injury, or sexual violence; a layoff does not meet Criterion A.

B. Acute stress disorder

Acute stress disorder has the same Criterion A trauma requirement as PTSD, which a layoff does not satisfy.

C. Major depressive disorder

The stem states that criteria for a major depressive episode are not met.

D. Adjustment disorder ✓ KEY

Distressing symptoms beginning within 3 months of an identifiable, non-traumatic stressor, out of proportion and impairing, fit adjustment disorder.

Rationale: Adjustment disorder involves emotional or behavioral symptoms that begin within 3 months of an identifiable stressor, are out of proportion to the stressor or impair functioning, and do not meet criteria for another mental disorder. Unlike PTSD and acute stress disorder, the stressor need not involve death, serious injury, or sexual violence. Symptoms should resolve within 6 months after the stressor or its consequences end.

References: DSM-5-TR — Adjustment Disorders, Criteria A–E

☐ Approve ☐ Revise ☐ Retire Notes:

A 4-year-old was adopted at age 2 after living in an understaffed orphanage. Her adoptive parents say she happily wanders off with strangers at the park, climbs onto unfamiliar adults' laps, and does not look back to check on them. Which diagnosis BEST fits this pattern?

A. Reactive attachment disorder, persistent

Reactive attachment disorder involves an inhibited, emotionally withdrawn child who rarely seeks or responds to comfort.

B. Disinhibited social engagement disorder ✓ KEY

Overly familiar behavior with strangers, wandering off without checking back, and a history of insufficient care define this disorder.

C. Attention-deficit/hyperactivity disorder

ADHD impulsivity is not specific to indiscriminate social approach and is not tied to a history of social neglect.

D. Autism spectrum disorder, level 1

Autism involves deficits in social communication and restricted interests, not eager, indiscriminate approach to strangers.

Rationale: Both reactive attachment disorder and disinhibited social engagement disorder follow a pattern of extremely insufficient care, such as neglect or institutional rearing. Reactive attachment disorder is an inhibited, withdrawn pattern with minimal comfort seeking, while disinhibited social engagement disorder is a pattern of culturally inappropriate, overly familiar behavior with unfamiliar adults and diminished checking back with caregivers. The child must have a developmental age of at least 9 months.

References: DSM-5-TR — Disinhibited Social Engagement Disorder, Criteria A–C

☐ Approve ☐ Revise ☐ Retire Notes:

A 22-year-old man who lifts weights daily is convinced his body looks small and weak, although friends describe him as very muscular. He checks mirrors for hours, wears baggy layers to hide his build, and skips social events. He is not focused on body fat or thinness. Which diagnosis BEST fits?

A. Anorexia nervosa, restricting type

Anorexia nervosa involves significantly low body weight and fear of weight gain, not a belief of being too small.

B. Obsessive-compulsive disorder

When obsessions and rituals center only on perceived appearance flaws, body dysmorphic disorder is the more specific diagnosis.

C. Body dysmorphic disorder ✓ KEY

Preoccupation with a perceived appearance flaw plus repetitive checking fits BDD; the belief that his build is too small calls for the muscle dysmorphia specifier.

D. Illness anxiety disorder

Illness anxiety disorder centers on fear of having a serious illness, not on perceived appearance flaws.

Rationale: Body dysmorphic disorder is preoccupation with perceived defects in appearance that are not observable or appear slight to others, along with repetitive behaviors such as mirror checking or comparing, causing distress or impairment. The "with muscle dysmorphia" specifier applies when the person believes his or her build is too small or insufficiently muscular. DSM-5-TR also specifies the degree of insight.

References: DSM-5-TR — Body Dysmorphic Disorder, Criteria A–D and muscle dysmorphia specifier

☐ Approve ☐ Revise ☐ Retire Notes:

A 66-year-old describes himself as "a collector." Which finding MOST clearly points to hoarding disorder rather than ordinary collecting?

A. Clutter fills living areas, and parting with items causes distress ✓ KEY

Persistent difficulty discarding, distress at parting with items, and clutter that blocks the use of living spaces are the core of hoarding disorder.

B. He owns several hundred items of one specific type in his home

A large number of items is common in collecting; what matters is whether clutter compromises living areas.

C. He spends a good share of his spare money on additions each year

Spending on a hobby is not diagnostic; collectors often budget substantial amounts.

D. He feels warm sentimental attachment to items that were gifts

Sentimental attachment to meaningful objects is normal and not in itself a sign of hoarding disorder.

Rationale: Hoarding disorder is persistent difficulty discarding or parting with possessions, regardless of their actual value, due to a perceived need to save them and distress associated with discarding. The result is an accumulation that congests and clutters active living areas and substantially compromises their intended use. Collecting, by contrast, tends to be organized and intentional and does not block the use of living space. A specifier notes whether excessive acquisition is present.

References: DSM-5-TR — Hoarding Disorder, Criteria A–C

☐ Approve ☐ Revise ☐ Retire Notes:

Two days after stopping several weeks of heavy methamphetamine use, a client reports feeling deeply down and exhausted. He is sleeping for long stretches, has vivid unpleasant dreams, and is eating much more than usual. Which withdrawal syndrome do these symptoms MOST closely match?

A. Opioid withdrawal

Opioid withdrawal features flu-like signs such as tearing, runny nose, yawning, dilated pupils, muscle aches, and diarrhea.

B. Cannabis withdrawal

Cannabis withdrawal typically brings irritability, anxiety, restlessness, trouble sleeping, and decreased appetite.

C. Alcohol withdrawal

Alcohol withdrawal involves autonomic hyperactivity, tremor, insomnia, and possible seizures, not hypersomnia with increased appetite.

D. Stimulant withdrawal ✓ KEY

Dysphoric mood with fatigue, vivid unpleasant dreams, sleep change, and increased appetite matches stimulant withdrawal.

Rationale: Stimulant withdrawal requires dysphoric mood plus at least two of: fatigue; vivid, unpleasant dreams; insomnia or hypersomnia; increased appetite; and psychomotor retardation or agitation, developing within hours to days after stopping. The "crash" can bring marked depression, so counselors should screen for suicidal thinking during early stimulant withdrawal.

References: DSM-5-TR — Stimulant Withdrawal, Criteria A–B

☐ Approve ☐ Revise ☐ Retire Notes:

After losing a large sum at an online casino, a client logs back in the next morning, certain that a few more bets will win back what he lost. Which DSM-5-TR gambling disorder criterion does this behavior illustrate?

A. Tolerance

Tolerance in gambling disorder is needing to gamble with increasing amounts of money to achieve the desired excitement.

B. Chasing losses ✓ KEY

Returning another day to get even after losing money is the "chasing" criterion of gambling disorder.

C. Relying on bailouts

This criterion refers to relying on others for money to relieve a desperate financial situation caused by gambling.

D. Withdrawal

Withdrawal refers to restlessness or irritability when attempting to cut down or stop gambling.

Rationale: Gambling disorder is the only behavioral (non-substance) addiction in the DSM-5-TR substance-related and addictive disorders chapter. It requires at least four of nine criteria within 12 months, including chasing losses, needing larger amounts to get the desired excitement, restlessness when cutting down, lying to conceal gambling, and relying on others for bailouts. Chasing is considered one of the hallmark features of problem gambling.

References: DSM-5-TR — Gambling Disorder, Criterion A6 (chasing losses)

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old with opioid use disorder says he is not ready to stop using but wants to keep seeing the counselor. Which counselor action BEST reflects a harm reduction approach?

A. Require a period of abstinence before further sessions are offered

Making abstinence a condition of care pushes away clients who are not ready and conflicts with harm reduction.

B. Confront his denial each session until he admits he needs to quit

Confrontational approaches tend to raise resistance and are not part of harm reduction or motivational work.

C. Help him get naloxone and plan not to use alone, while exploring change ✓ **KEY**

Reducing overdose risk now, while keeping him engaged and exploring change, is the essence of harm reduction.

D. Refer him to a mutual-help group and end individual counseling for now

Mutual-help groups can help, but ending counseling removes support when he is open to engagement.

Rationale: Harm reduction meets clients where they are, aiming to reduce the health and social harms of substance use without requiring abstinence first. For opioid use, this includes naloxone access, not using alone, and sterile supplies, alongside offering medications for opioid use disorder. Staying engaged also leaves room for motivational work that may later lead to reduced use or recovery.

References: SAMHSA TIP 63 — Medications for Opioid Use Disorder: overdose prevention and naloxone

► **Check:** Confirm that SAMHSA TIP 63 is the best approved citation for naloxone distribution as a harm reduction practice.

☐ Approve ☐ Revise ☐ Retire Notes:

170. nce-s-clf-074 · Assessment and Testing · neurodevelopmental disorders · easy · In review

Under DSM-5-TR, the severity level of intellectual developmental disorder (mild, moderate, severe, profound) is determined mainly by which factor?

A. Adaptive functioning in conceptual, social and practical areas ✓ **KEY**

DSM-5-TR defines severity by adaptive functioning, because that determines the level of support needed.

B. The full-scale IQ score range obtained on standardized testing

IQ is part of assessing intellectual deficits, but DSM-5 moved severity away from IQ ranges.

C. The age at which the intellectual deficits were first observed

Onset during the developmental period is required, but age of onset does not set severity.

D. Whether a specific genetic or medical cause has been identified

Etiology may be noted, but it does not determine the severity level.

Rationale: Intellectual developmental disorder requires deficits in intellectual functions and in adaptive functioning, with onset during the developmental period. DSM-5-TR bases the severity specifiers on adaptive functioning across conceptual, social, and practical domains rather than IQ scores, because adaptive functioning determines the level of supports required. IQ scores remain part of the assessment but are less valid at the lower end of the range.

References: DSM-5-TR — Intellectual Developmental Disorder, severity specifiers

☐ Approve ☐ Revise ☐ Retire Notes:

An 11-year-old has had eye blinking and shoulder shrugging since age 9, and over the past year he has also developed frequent throat clearing and sniffing. The tics wax and wane but have never stopped for long. He has never used obscene words. Which diagnosis BEST fits?

A. Persistent (chronic) motor tic disorder

Persistent tic disorder involves motor or vocal tics, but not both; he has both.

B. Provisional tic disorder

Provisional tic disorder applies when tics have been present for less than 1 year.

C. Stereotypic movement disorder

Stereotypies are rhythmic, seemingly purposeless movements that begin earlier and lack the vocal tics described.

D. Tourette's disorder ✓ KEY

Multiple motor tics and at least one vocal tic, present for more than 1 year with onset before 18, meet criteria for Tourette's disorder.

Rationale: Tourette's disorder requires both multiple motor tics and one or more vocal tics at some time during the illness, although not necessarily at the same time. Tics may wax and wane but must have persisted for more than 1 year since the first tic, with onset before age 18. Coprolalia (uttering obscenities) occurs in only a minority of cases and is not required.

References: DSM-5-TR — Tourette's Disorder, Criteria A–D

☐ Approve ☐ Revise ☐ Retire Notes:

For 8 months, a 58-year-old accountant has been certain that a neighbor is poisoning his garden and reading his mail. He has filed police reports and installed cameras. He reports no hallucinations, his speech is organized, and he continues to work effectively. No mood episodes or substance causes are present. Which diagnosis is MOST appropriate?

A. Schizophrenia

Schizophrenia requires two or more Criterion A symptoms and usually marked functional decline; he has a single fixed delusion and functions well.

B. Delusional disorder ✓ KEY

One or more delusions lasting 1 month or longer, without other Criterion A symptoms and with functioning not markedly impaired, fits delusional disorder (persecutory type).

C. Paranoid personality disorder

Paranoid personality disorder involves pervasive distrust and suspiciousness, not a fixed false belief held with delusional conviction.

D. Brief psychotic disorder

Brief psychotic disorder lasts at least 1 day but less than 1 month, and he has had symptoms for 8 months.

Rationale: Delusional disorder requires one or more delusions lasting at least 1 month, Criterion A for schizophrenia never having been met, and functioning that is not markedly impaired apart from the impact of the delusion. Behavior is not obviously bizarre. Subtypes include persecutory, jealous, erotomanic, grandiose, and somatic, and DSM-5-TR adds a "with bizarre content" specifier.

References: DSM-5-TR — Delusional Disorder, Criteria A–E and persecutory type

☐ Approve ☐ Revise ☐ Retire Notes:

Over 2 years of illness, a 33-year-old has had major depressive episodes for most of the time, often with voices and persecutory beliefs. Between episodes there was a 3-week stretch when the voices and delusions continued while her mood was stable. She has never had mania, and substances are ruled out. Which diagnosis is MOST appropriate?

A. Major depressive disorder with psychotic features

In MDD with psychotic features, psychosis occurs only during mood episodes; here it persisted for weeks without one.

B. Schizophrenia with comorbid depressive episodes

Schizophrenia fits when mood episodes are present for only a minority of the illness; hers spanned most of it.

C. Schizoaffective disorder, depressive type ✓ KEY

Psychosis for 2 or more weeks without a mood episode, with mood episodes present for most of the illness, fits schizoaffective disorder.

D. Bipolar I disorder with psychotic features

Bipolar I requires a manic episode, which has never occurred.

Rationale: Schizoaffective disorder requires an uninterrupted period of illness in which a major mood episode co-occurs with Criterion A of schizophrenia, plus delusions or hallucinations for 2 or more weeks in the absence of a major mood episode at some point in the lifetime illness. Mood episodes must be present for the majority of the total duration of the active and residual illness. The depressive type applies when only major depressive episodes have occurred.

References: DSM-5-TR — Schizoaffective Disorder, Criteria A–C

☐ Approve ☐ Revise ☐ Retire Notes:

A 26-year-old seeks counseling for "bipolar mood swings." She describes intense anger, chronic emptiness, and fear of being left by her partner. A counselor is weighing borderline personality disorder against a bipolar disorder. Which finding MOST supports borderline personality disorder?

A. Impulsive spending sprees during periods of high distress

Impulsive spending occurs in both borderline personality disorder and manic or hypomanic episodes, so it does not discriminate.

B. Mood shifts lasting hours, set off by perceived rejection ✓ KEY

Reactive mood shifts lasting hours and triggered by interpersonal events are typical of BPD, whereas bipolar episodes last days or longer.

C. Several past periods of low mood with fatigue and poor focus

Depressive periods occur in both conditions and do not distinguish them.

D. A biological parent with a documented bipolar I diagnosis

A family history of bipolar disorder raises the likelihood of a bipolar diagnosis rather than supporting BPD.

Rationale: Affective instability in borderline personality disorder involves marked reactivity of mood, with episodes usually lasting a few hours and rarely more than a few days, often triggered by interpersonal stress or perceived abandonment. Bipolar mood episodes are sustained, distinct periods (at least 4 days for hypomania, 1 week for mania) with changes such as decreased need for sleep that are not simply reactions to relationships. The two can co-occur, so careful history taking matters.

References: DSM-5-TR — Borderline Personality Disorder, Differential Diagnosis (bipolar disorders)

☐ Approve ☐ Revise ☐ Retire Notes:

A 35-year-old has few close friends, dresses in an unusual way, and speaks in vague, odd phrases. She believes she can sense what strangers are thinking and that song lyrics on the radio carry special messages for her. Her social anxiety does not ease as she gets to know people. Which personality disorder BEST fits?

A. Schizoid personality disorder

Schizoid personality disorder involves detachment and restricted emotion without the odd beliefs and perceptual distortions described.

B. Avoidant personality disorder

Avoidant personality disorder (Cluster C) involves fear of rejection without magical thinking or ideas of reference.

C. Paranoid personality disorder

Paranoid personality disorder centers on distrust and suspicion of others' motives rather than eccentric beliefs and speech.

D. Schizotypal personality disorder ✓ KEY

Ideas of reference, magical thinking, odd speech and appearance, and persistent social anxiety define schizotypal personality disorder.

Rationale: Schizotypal personality disorder, in Cluster A, combines acute discomfort with close relationships and cognitive or perceptual distortions and eccentric behavior. Features include ideas of reference, odd beliefs or magical thinking, unusual perceptual experiences, odd speech, eccentric appearance, and excessive social anxiety that does not diminish with familiarity. Schizoid personality disorder shares the social detachment but lacks the cognitive and perceptual oddities.

References: DSM-5-TR — Schizotypal Personality Disorder, Criterion A

☐ Approve ☐ Revise ☐ Retire Notes:

A 19-year-old reports recurrent binge-eating episodes followed by purging, along with an intense fear of gaining weight. Her physician notes that her body weight is significantly low for her age, sex, and developmental trajectory. Which diagnosis is MOST appropriate?

A. Anorexia nervosa ✓ KEY

Significantly low body weight with fear of weight gain and recurrent binge-purge behavior fits anorexia nervosa, binge-eating/purging type.

B. Bulimia nervosa

Bulimia nervosa cannot be diagnosed when the disturbance occurs exclusively during episodes of anorexia nervosa.

C. Binge-eating disorder

Binge-eating disorder excludes regular compensatory behaviors such as purging.

D. Purging disorder

Purging disorder (an other specified presentation) involves purging without binge eating, and she does binge; she also meets full criteria for anorexia nervosa.

Rationale: The key feature separating anorexia nervosa from bulimia nervosa is significantly low body weight. Binge eating and purging can occur in both, but when they happen in a person with significantly low weight and fear of weight gain, the diagnosis is anorexia nervosa, binge-eating/purging type. Bulimia nervosa Criterion E states the disturbance does not occur exclusively during episodes of anorexia nervosa.

References: DSM-5-TR — Anorexia Nervosa subtypes; Bulimia Nervosa, Criterion E

☐ Approve ☐ Revise ☐ Retire Notes:

A client in outpatient counseling for bulimia nervosa says her purging has increased over the past month. Today she mentions feeling dizzy and noticing her heart "fluttering" at times. What should the counselor do?

A. Teach slow breathing, since palpitations often reflect anxiety

Attributing cardiac symptoms to anxiety before medical evaluation risks missing a dangerous physical complication.

B. Keep the session focused on the binge triggers and revisit later

Delaying attention to new physical warning signs is unsafe in an eating disorder with frequent purging.

C. Arrange prompt medical evaluation of her physical warning signs ✓ KEY

Frequent purging can cause electrolyte disturbances that affect heart rhythm, so dizziness and palpitations warrant prompt medical assessment.

D. Suggest she adjust her own fluid intake and recheck how she feels

Self-management advice is outside the counselor's scope and does not address possible electrolyte or cardiac problems.

Rationale: Bulimia nervosa carries significant medical risk; repeated purging can lead to fluid and electrolyte disturbances that may cause dangerous heart rhythm problems. Counselors should coordinate with medical providers and refer for prompt evaluation when clients report physical warning signs such as dizziness, fainting, or palpitations. Eating disorder care is best delivered by a team that includes medical monitoring.

References: NICE guidelines — NG69 Eating disorders: physical health assessment and monitoring

☐ Approve ☐ Revise ☐ Retire Notes:

A 72-year-old has had a gradual decline in thinking over the past year. His wife reports that he is clear one day and very confused the next, often sees detailed images of children in the house who are not there, and shouts and kicks while dreaming. Mild stiffness and a shuffling gait appeared months after the cognitive changes. Which diagnosis BEST fits?

A. Major NCD due to Alzheimer's disease

Alzheimer's disease usually begins with memory and learning decline; fluctuations, early visual hallucinations, and REM sleep behavior are not core features.

B. Major frontotemporal NCD

Frontotemporal disorder presents with early behavioral disinhibition, apathy, or language decline rather than these features.

C. Major vascular NCD

Vascular disorder is tied to cerebrovascular events and often shows stepwise decline in attention and executive function.

D. Major NCD with Lewy bodies ✓ KEY

Fluctuating cognition, recurrent detailed visual hallucinations, and later parkinsonism are core Lewy body features; REM sleep behavior disorder is a suggestive feature.

Rationale: In DSM-5-TR, the core features of neurocognitive disorder with Lewy bodies are fluctuating cognition with marked variations in attention and alertness, recurrent well-formed and detailed visual hallucinations, and spontaneous parkinsonism beginning after cognitive decline. Suggestive features include REM sleep behavior disorder and severe sensitivity to antipsychotic (neuroleptic) medications, which is why accurate identification matters for care coordination.

References: DSM-5-TR — Major or Mild Neurocognitive Disorder With Lewy Bodies, Criteria C–D

☐ Approve ☐ Revise ☐ Retire Notes:

Which presentation MOST suggests vascular neurocognitive disorder rather than neurocognitive disorder due to Alzheimer's disease?

A. Stepwise drops in attention and planning after small strokes ✓ KEY

Onset tied to cerebrovascular events and prominent decline in complex attention and executive function point to vascular NCD.

B. Slow, steady loss of recent memory that began without warning

Insidious onset with gradual, early memory and learning decline is typical of Alzheimer's disease.

C. Early disinhibition and loss of empathy with intact memory

Early behavioral disinhibition and loss of empathy suggest the behavioral variant of frontotemporal NCD.

D. Recurrent visual hallucinations with alertness that fluctuates

Recurrent visual hallucinations with fluctuating alertness are core features of NCD with Lewy bodies.

Rationale: Vascular neurocognitive disorder is suggested when onset is temporally related to one or more cerebrovascular events, or when decline is prominent in complex attention (including processing speed) and frontal-executive function. Its course is often stepwise. Alzheimer's disease, by contrast, has an insidious onset and gradual progression, with memory and learning typically affected early.

References: DSM-5-TR — Major or Mild Vascular Neurocognitive Disorder, Criterion B

☐ Approve ☐ Revise ☐ Retire Notes:

Fourteen months after her husband died, a 67-year-old client accepts the death and has let herself feel its pain. Now she is struggling to manage the finances he always handled, to drive on the highway alone, and to think of herself as "single." Which of Worden's tasks of mourning is she MOST working on?

A. Accepting the reality of the loss

The stem states she already accepts that the death has happened.

B. Processing the pain of the grief

She has allowed herself to feel the pain; her current struggle is with new roles and identity.

C. Adjusting to a world without him ✓ KEY

Taking on roles the deceased filled and redefining her sense of self are the external and internal adjustments of this task.

D. Finding an enduring connection

This task involves keeping a bond with the deceased while moving forward, not mastering new skills and roles.

Rationale: Worden described four tasks of mourning: accepting the reality of the loss, processing the pain of grief, adjusting to a world without the deceased, and finding an enduring connection with the deceased while embarking on a new life. Adjustment includes external changes (new roles and skills), internal changes (sense of self), and spiritual changes (beliefs about the world). Unlike stage models, tasks are active and can be revisited.

References: Berk (Development Through the Lifespan) — Ch. 19: Bereavement, the experience of grieving

► **Check:** Verify that the cited Berk chapter covers Worden's four tasks; Worden's own text is not on the approved source list.

☐ Approve ☐ Revise ☐ Retire Notes:

Six months after his teenage son died, a father has some days filled with looking at photos and crying, and other days focused on returning to work and learning to cook for his family. He worries that this "back and forth" means he is grieving the wrong way. Which framework BEST normalizes his experience?

A. Kübler-Ross's stages of dying, read as a fixed sequence

Kübler-Ross described reactions of dying patients; read as a fixed sequence, it would frame his back-and-forth as a problem.

B. Stroebe and Schut's dual process model of coping with loss ✓ KEY

The dual process model describes healthy oscillation between loss-oriented and restoration-oriented coping.

C. Bowlby's phases of mourning, ending in reorganization

Bowlby's phases describe a general progression and do not center on alternating between two kinds of coping.

D. Caplan's crisis theory, with its time-limited disequilibrium

Crisis theory addresses short-term disequilibrium after an event, not the ongoing rhythm of bereavement coping.

Rationale: Stroebe and Schut's dual process model proposes that bereaved people oscillate between loss-oriented coping (grief work, yearning, reviewing the relationship) and restoration-oriented coping (attending to life changes, new roles, distraction). Moving back and forth, including taking breaks from grief, is considered adaptive. Kübler-Ross's stages (denial, anger, bargaining, depression, acceptance) came from work with dying patients and are not a required sequence for the bereaved.

References: Berk (Development Through the Lifespan) — Ch. 19: Bereavement, the dual-process model of coping with loss

► **Check:** Berk is cited as "Ch. 19"; confirm the chapter number for the edition in use.

☐ Approve ☐ Revise ☐ Retire Notes:

According to Caplan's crisis theory, which statement BEST describes a crisis?

A. A time-limited upset when usual coping fails, which may lead to growth ✓ KEY

Caplan described crisis as a temporary disequilibrium when customary problem solving fails, with potential for growth or deterioration.

B. A lasting pattern of poor coping rooted in long-standing personality traits

Crisis is an acute, time-limited state, not an enduring personality pattern.

C. A mental disorder that calls for formal diagnosis and long-term therapy

A crisis is a normal human reaction, not a mental disorder, and calls for brief, focused intervention.

D. A reaction set by how severe the event is, apart from how it is perceived

Crisis theory stresses the person's perception of the event and coping resources, not the event alone.

Rationale: Gerald Caplan described a crisis as a state of psychological disequilibrium that occurs when a person faces an obstacle to important life goals that cannot be overcome with customary problem-solving methods. Crises are time-limited, typically resolving within weeks, and the outcome can be either growth and improved coping or deterioration. This is why early, brief intervention is emphasized.

References: Gladding (Counseling: A Comprehensive Profession) — Crisis counseling: the nature of crises

► **Check:** Confirm Gladding's crisis counseling coverage of Caplan; no crisis-theory text (e.g., James & Gilliland) is on the approved list.

☐ Approve ☐ Revise ☐ Retire Notes:

Four months after a flood, the volunteers and news crews have left a small town. Residents who once worked side by side now voice anger at slow insurance payouts and agency red tape, and many feel forgotten. In the commonly used phases of community response to disaster, which phase is the town MOST likely in?

A. Heroic phase

The heroic phase occurs during and right after impact, when people act to save lives and property.

B. Honeymoon phase

The honeymoon phase brings community bonding and optimism as aid arrives, which has faded here.

C. Reconstruction phase

Reconstruction comes later, as residents take responsibility for rebuilding and find new meaning.

D. Disillusionment phase ✓ KEY

As outside support withdraws and bureaucratic delays mount, frustration, anger, and feeling abandoned mark disillusionment.

Rationale: Disaster mental health models describe community phases: pre-disaster, impact, heroic, honeymoon, disillusionment, and reconstruction. The disillusionment phase can last months or longer, as optimism gives way to fatigue and anger when aid slows and the reality of losses sets in. Counselors can anticipate rising stress during this phase and increase outreach.

References: Gladding (Counseling: A Comprehensive Profession) — Crisis and disaster counseling: phases of disaster response

► **Check:** Phase names follow the SAMHSA/Zunin and Myers disaster model; confirm the Gladding citation or substitute a SAMHSA disaster source if one is added.

☐ Approve ☐ Revise ☐ Retire Notes:

A week after her partner assaulted her, a client tells the counselor that he cried, brought flowers, promised it would never happen again, and has been "the man I fell in love with." She feels hopeful. In Walker's cycle of violence, which phase is this?

A. Tension-building phase

Tension building involves rising strain, minor incidents, and the victim trying to keep the peace.

B. Acute battering phase

The acute battering phase is the explosion of violence itself, which has already passed.

C. Honeymoon phase ✓ KEY

Apologies, gifts, promises, and loving behavior after an assault mark the honeymoon (loving contrition) phase.

D. Resolution phase

Walker's cycle has no resolution phase; without intervention, the cycle tends to repeat.

Rationale: Lenore Walker's cycle of violence has three phases: tension building, an acute battering incident, and loving contrition (the honeymoon phase). The honeymoon phase reinforces hope that the abuse will end and helps explain why many survivors stay. Over time the honeymoon phase often shortens or disappears, so counselors should use this period to support safety planning.

References: Gladding (Counseling: A Comprehensive Profession) — Intimate partner violence: the cycle of violence

► **Check:** Walker's cycle is cited via Gladding; confirm coverage, since no IPV-specific text is on the approved list.

☐ Approve ☐ Revise ☐ Retire Notes:

A client says her husband controls all their money, reads her messages, has cut her off from friends, and threatens to take the children if she leaves. She adds, "It isn't really abuse, because he has never hit me." Which framework would BEST help her see these behaviors as connected forms of abuse?

A. Walker's cycle of violence

The cycle of violence describes phases around violent episodes, and she reports no physical violence.

B. Duluth Power and Control Wheel ✓ KEY

The wheel shows tactics such as economic abuse, isolation, and using children as parts of one pattern of coercive control.

C. Gottman's Four Horsemen

The Four Horsemen describe destructive communication in conflict, not coercive control or abuse.

D. Bowen's triangulation concept

Triangulation describes drawing a third party into a dyad's anxiety, not a pattern of abusive control.

Rationale: The Power and Control Wheel, developed by the Domestic Abuse Intervention Project in Duluth, Minnesota, shows abuse as a pattern of coercive control. Its spokes are tactics such as intimidation, isolation, economic abuse, using children, and threats, held together by a rim of physical and sexual violence. Coercive control can be dangerous even without physical assault, and conjoint couples counseling is generally contraindicated while it is ongoing.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling women: intimate partner violence and coercive control

► **Check:** The Duluth model is cited via Sue & Sue; confirm coverage or substitute a better approved source.

☐ Approve

☐ Revise

☐ Retire

Notes:

A 46-year-old warehouse supervisor lost his job four months ago. A generous severance package covers his bills for the next year, yet he reports feeling aimless, sleeping late, missing his coworkers, and no longer knowing "who I am." Which framework BEST explains why his distress persists despite financial security?

A. Holland's congruence between personality type and setting

Congruence concerns fit between a person's type and a work environment; it does not explain distress from losing work altogether.

B. Krumboltz's planned happenstance and openness to chance

Planned happenstance is a stance for using unplanned events, not an account of why job loss is distressing.

C. Jahoda's latent deprivation model of unemployment ✓ KEY

Jahoda argued that work supplies latent benefits such as time structure, social contact, purpose, status and activity; losing them causes distress even when income continues.

D. Gottfredson's compromise among sex type, prestige and field

Compromise describes how people give up preferred options when choosing; it does not address the psychological functions of employment.

Rationale: Jahoda's latent deprivation model holds that employment has a manifest function (income) and latent functions: time structure, regular social contact, shared purpose, status and identity, and regular activity. Unemployed people lose these latent benefits even when money is not a problem, which helps explain aimlessness and identity loss. Counselors can help clients rebuild structure, contact and purpose while job searching.

References: Sharf (Career Development Theory) — Career counseling for unemployment and job loss; Jahoda's latent deprivation model

► **Check:** Verify that the cited Sharf edition discusses Jahoda's latent deprivation model; the model itself (Jahoda, 1981-1982) is well established.

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 34-year-old client has completed several interest inventories, researched dozens of occupations, and shadowed workers in two fields, yet still cannot choose. She describes the same agonizing paralysis when picking an apartment, a car, or even a restaurant, and says any choice feels like it could ruin her life. Which approach BEST fits her concern?

A. Address the anxiety behind her pervasive indecisiveness ✓ KEY

Indecision that spans life domains and persists despite ample information reflects chronic indecisiveness, which calls for work on anxiety and personal concerns, not more data.

B. Give more occupational information and a new inventory

More information helps a developmentally undecided client who lacks data; this client already has extensive information and remains stuck.

C. Treat her concern as normal, developmental undecidedness

Developmental undecidedness is situational and resolves with exploration; her paralysis is pervasive and long-standing.

D. Have her pick the highest-rated field and commit to it now

Pushing a fast commitment ignores the anxiety driving her indecisiveness and is likely to increase her distress.

Rationale: Career literature distinguishes career indecision (undecidedness), a common developmental state that usually responds to information and exploration, from indecisiveness, a chronic trait-like difficulty making decisions across many areas of life, often tied to anxiety, perfectionism or fear of commitment. Indecisive clients benefit from attention to these personal factors alongside career work.

References: Sharf (Career Development Theory) — Career indecision versus chronic indecisiveness in career counseling

☐ Approve ☐ Revise ☐ Retire Notes:

An emergency room nurse works predictable shifts and is home every evening. She says the fatigue, tension and irritability she carries home from chaotic shifts leave her snapping at her children and too drained to enjoy family time. Which form of work-family conflict does this BEST illustrate?

A. Time-based conflict over hours that work takes from home

Time-based conflict occurs when hours spent in one role make it hard to meet another role's demands; her schedule leaves her home every evening.

B. Behavior-based conflict from work habits that misfit home

Behavior-based conflict involves role behaviors, such as a brisk clinical style, that are inappropriate at home; her problem is spillover of strain.

C. Family-to-work conflict in which home duties disrupt her job

This reverses the direction; here work is interfering with family life, not the other way around.

D. Strain-based conflict as work stress spills into home life ✓ KEY

Strain-based conflict occurs when stress, fatigue or irritability from one role impairs functioning in another, as her work strain does at home.

Rationale: Greenhaus and Beutell described three forms of work-family conflict: time-based, strain-based and behavior-based, each of which can run work-to-family or family-to-work. Identifying the form guides intervention; strain-based conflict points toward recovery routines, stress management and boundary rituals between roles rather than schedule changes.

References: Sharf (Career Development Theory) — Work-family conflict and life roles: time-based, strain-based and behavior-based conflict

► **Check:** Confirm the cited Sharf edition covers the Greenhaus and Beutell (1985) three-form typology of work-family conflict.

☐ Approve ☐ Revise ☐ Retire Notes:

A 27-year-old client with well-controlled epilepsy has a job interview next week. She asks her counselor whether she is required to tell the interviewer about her seizure disorder. Under the Americans with Disabilities Act, which statement is MOST accurate?

A. She must disclose it at the interview so the employer can judge risk

The ADA does not require applicants to disclose a disability at an interview; employers may ask only about the ability to do job functions.

B. Disclosure is her choice; it is needed if she seeks accommodation ✓ **KEY**

Disclosure under the ADA is generally voluntary; an employer needs to know about the disability only when an accommodation is requested.

C. Disclosure is only required if the job involves operating machinery

Job tasks do not create a duty to disclose at the interview; safety concerns are handled through job-related standards after an offer.

D. The employer may ask about any medical condition before an offer

Before a conditional job offer, the ADA bars disability-related questions and medical exams.

Rationale: The ADA prohibits disability-related inquiries and medical examinations before a conditional job offer and does not require applicants to disclose. Disclosure becomes necessary mainly when a person requests a reasonable accommodation. Career counselors help clients with disabilities weigh whether, when and how to disclose, and how to describe needed accommodations in terms of job performance.

References: Sharf (Career Development Theory) — Career development of people with disabilities; ADA disclosure and reasonable accommodation

► **Check:** Confirm the ADA pre-offer inquiry rules as stated and that the cited source covers disability disclosure; Sharf may not discuss ADA specifics.

☐ Approve ☐ Revise ☐ Retire Notes:

A 19-year-old first-generation college student from a Filipino American family says her parents expect her to become a nurse, though she is drawn to graphic design. She values her family's sacrifices and wants their blessing. Her counselor is trained in a decision model that stresses individual choice. What is the MOST culturally responsive approach?

A. Encourage her to choose design and assert independence from family

This imposes an individualistic value and ignores that family harmony and approval matter to her.

B. Advise her to follow her parents' wishes to keep peace in the family

This dismisses her own interests and is just as directive as pushing independence.

C. Explore her values and include family input in the decision process ✓ **KEY**

Honoring collectivist values while exploring her interests, and inviting family into the process when she wishes, respects her worldview.

D. Use the standard model as written, since good models fit all clients

Career models built on Western individualism may not fit clients from collectivist backgrounds and need cultural adaptation.

Rationale: Many career theories assume individualistic decision making. With clients from collectivist backgrounds, career choice may be a family decision tied to obligation and gratitude. Culturally responsive counselors explore the client's own values, acknowledge family expectations, and may involve family in the process rather than framing autonomy as the goal.

References: Sue & Sue (Counseling the Culturally Diverse) — Collectivism, family roles and culturally responsive career counseling

☐ Approve ☐ Revise ☐ Retire Notes:

Parents of a 15-year-old argue constantly with their son about curfew, friends and privacy, while also arranging care for the father's ailing mother. In Carter and McGoldrick's family life cycle, which key emotional task does the family face in this stage?

A. Increasing the flexibility of family boundaries for independence ✓ KEY

The families-with-adolescents stage calls for more flexible boundaries to allow teen independence and to absorb grandparents' frailties.

B. Accepting new members into the system as the couple has children

Accepting new members is the task of families with young children.

C. Accepting many exits from and entries into the family system

Accepting exits and entries is the task of launching children and moving on.

D. Accepting the shifting of roles from one generation to the next

Accepting shifting generational roles is the task of families in later life.

Rationale: Carter and McGoldrick describe stages of the family life cycle, each with a key emotional process. In families with adolescents, boundaries must become more flexible so teens can move in and out of the system, while parents often begin caring for aging grandparents. Stress tends to rise when a family remains rigid at a transition point.

References: Gladding (Counseling: A Comprehensive Profession) — Family life cycle stages and key emotional tasks (Carter & McGoldrick)

☐ Approve ☐ Revise ☐ Retire Notes:

A divorcing couple with children aged 6 and 10 asks a counselor what will MOST affect how well their children adjust over the coming years. Based on research on divorce and child outcomes, which factor should the counselor highlight?

A. Whether the children are told about the divorce at the same time

How the news is shared matters, but a single conversation is not a major long-term predictor of adjustment.

B. Whether a sole or a joint legal custody arrangement is chosen

Custody type alone predicts outcomes less than the quality of parenting and the level of conflict children face.

C. Whether either parent remarries within the first year after

Remarriage brings its own adjustments, but it is not the strongest predictor of children's post-divorce adjustment.

D. How much ongoing conflict between parents the children see ✓ KEY

Exposure to persistent, hostile interparental conflict is among the strongest predictors of poorer child adjustment after divorce.

Rationale: Research by Hetherington, Amato and others shows that most children of divorce adjust well within a few years. Poorer outcomes are most strongly linked to ongoing interparental conflict, especially when children are caught in the middle, along with declines in parenting quality and economic hardship. Counselors help parents shield children from conflict and keep warm, consistent parenting.

References: Berk (Development Through the Lifespan) — Divorce: factors in children's adjustment, interparental conflict and parenting quality

☐ Approve ☐ Revise ☐ Retire Notes:

During a heated discussion in couples counseling, a husband pauses, smiles, and says, "Okay, I'm getting defensive. Can I try saying that again?" His wife's tone softens. In Gottman's research, what is the husband's statement called?

A. Stonewalling

Stonewalling is withdrawing from the interaction, one of the Four Horsemen; he is re-engaging, not shutting down.

B. A repair attempt ✓ **KEY**

A repair attempt is any statement or action that de-escalates conflict and keeps negativity from spiraling.

C. A harsh startup

A harsh startup is opening a discussion with criticism or contempt; his comment de-escalates.

D. Flooding

Flooding is physiological overwhelm during conflict; his calm self-correction shows the opposite.

Rationale: Gottman found that stable couples argue too, but they make and accept repair attempts, such as humor, apologies or requests to start over, that stop negativity from escalating. Whether repairs succeed depends largely on the strength of the couple's friendship. Counselors teach partners to offer and recognize repair attempts.

References: Gladding (Counseling: A Comprehensive Profession) — Couples counseling: Gottman's research on repair attempts and the Four Horsemen

► **Check:** Gottman's own texts are not on the approved source list; confirm the cited Gladding edition covers repair attempts.

☐ Approve ☐ Revise ☐ Retire Notes:

A 20-year-old college student raised in a close-knit congregation tells her counselor she has begun questioning teachings she once accepted without thought. She is reading widely, testing beliefs against her own reasoning, and wants a faith she has chosen for herself. Which of Fowler's stages of faith does this BEST describe?

A. Synthetic-conventional

Synthetic-conventional faith conforms to the beliefs of one's group without critical examination, which she is now moving beyond.

B. Mythic-literal

Mythic-literal faith, typical of school-age children, takes stories and symbols literally.

C. Individuative-reflective ✓ **KEY**

This stage involves critically examining inherited beliefs and taking personal responsibility for one's own commitments.

D. Conjunctive

Conjunctive faith, usually in midlife or later, embraces paradox and other traditions after the reflective stage.

Rationale: Fowler proposed stages of faith development: primal (undifferentiated), intuitive-projective, mythic-literal, synthetic-conventional, individuative-reflective, conjunctive and universalizing. The shift from synthetic-conventional to individuative-reflective often occurs in late adolescence or young adulthood, when people move from group-held belief to a self-examined faith. Counselors can normalize this questioning rather than treat it as a crisis.

References: Gladding (Counseling: A Comprehensive Profession) — Spirituality in counseling: Fowler's stages of faith development

☐ Approve ☐ Revise ☐ Retire Notes:

After a long illness in his family, a 63-year-old deacon says he has lost his sense that God listens and feels unmoored from his faith. He is sad at times but sleeps, eats and works normally, and his distress is in proportion to his experience. He meets criteria for no mental disorder. How should the counselor document the focus of treatment?

A. Adjustment disorder with depressed mood after the stressor

Adjustment disorder requires distress out of proportion to the stressor or significant impairment; neither is present.

B. Persistent depressive disorder, given his low periods

He has occasional sadness with normal functioning, not depressed mood most days for two years.

C. No code, since spiritual concerns fall outside counseling

Spiritual concerns can be a legitimate focus of counseling and have a code for documentation.

D. Religious or spiritual problem as a focus of attention ✓ KEY

DSM-5-TR lists religious or spiritual problem (Z65.8) for distress such as loss or questioning of faith that is a focus of care.

Rationale: DSM-5-TR includes Religious or Spiritual Problem (Z65.8) among Other Conditions That May Be a Focus of Clinical Attention. It covers distressing experiences such as loss or questioning of faith, conversion, or conflicts with a religious institution. It lets counselors document a legitimate focus of care without pathologizing a spiritual struggle that does not meet criteria for a mental disorder.

References: DSM-5-TR — Other Conditions That May Be a Focus of Clinical Attention: Religious or Spiritual Problem (Z65.8)

☐ Approve ☐ Revise ☐ Retire Notes:

A mother tells her counselor that her 11-year-old's new volunteer tutor is "amazing." He buys her daughter small gifts, offers rides so the mother can rest, texts the girl privately at night, and recently told her some of their talks are "just between us." Which feature MOST strongly suggests possible grooming?

A. Private contact paired with a request to keep secrets ✓ KEY

Grooming typically moves toward private access and secrecy, which isolate the child and lower the chance of disclosure.

B. His popularity with the parents in the neighborhood

Groomers may win over parents, but being well liked is common among safe adults and is not a specific warning sign.

C. His strong interest in the girl's grades and reading

Academic interest is expected of a tutor and is not by itself a warning sign.

D. His volunteering without pay for a nonprofit program

Unpaid volunteering is ordinary and does not distinguish grooming from safe mentoring.

Rationale: Grooming is a gradual process in which an offender builds trust with a child and often the family, gives special attention or gifts, tests boundaries, increases private access and introduces secrecy. Secret-keeping and isolated contact are the most telling signs. Counselors educate parents about these patterns and follow mandated reporting law when abuse is reasonably suspected.

References: Gladding (Counseling: A Comprehensive Profession) — Child sexual abuse: grooming patterns and warning signs

► **Check:** Confirm the cited source covers grooming; the concept itself is standard in child-protection literature.

☐ Approve ☐ Revise ☐ Retire Notes:

A school counselor notices bruises on an active 6-year-old who plays rough sports at recess. Which bruise location should raise the GREATEST concern about possible physical abuse?

A. Scattered bruises over both shins and knees

Shins and knees are bony areas that are commonly bruised in normal childhood play.

B. A bruise on the forehead after a playground fall

The forehead is a common site of accidental injury in falls, especially with a matching account.

C. Bruises on the ears, cheeks and the upper back ✓ KEY

Ears, cheeks and the back are protected, soft areas rarely bruised by accident, so injuries there suggest inflicted harm.

D. Bruises on the elbows of both arms from falling

Elbows are bony prominences that often take the impact of accidental falls.

Rationale: Accidental bruises in children usually appear over bony prominences such as the shins, knees, elbows and forehead. Bruises on soft or protected areas such as the ears, cheeks, neck, torso, back, buttocks or upper arms, or bruises in the shape of an object or hand, are more concerning for abuse. Reasonable suspicion triggers a mandated report.

References: Gladding (Counseling: A Comprehensive Profession) — Recognizing physical indicators of child abuse · State law — Child abuse mandated reporting statutes: reasonable-suspicion standard

► **Check:** Confirm the cited Gladding edition lists physical indicators of abuse; the injury-location pattern is standard pediatric guidance.

☐ Approve ☐ Revise ☐ Retire Notes:

A middle school counselor is preparing a parent workshop on cyberbullying. Which feature MOST clearly sets cyberbullying apart from traditional in-person bullying?

A. It involves a real or perceived imbalance of power

A power imbalance is part of the definition of all bullying, not a distinguishing feature of cyberbullying.

B. It can reach the child at home, at any hour, by many ✓ KEY

Online bullying follows children home around the clock and can be seen and shared by large audiences, often anonymously.

C. It is intentional and is repeated over a period of time

Intent and repetition define bullying in general, both online and in person.

D. It harms the target's mood and school performance

Both forms of bullying are linked to depression, anxiety and academic problems.

Rationale: Bullying is unwanted aggressive behavior that is intentional, repeated or likely to be repeated, and involves a power imbalance. Cyberbullying shares these features but differs in that it can occur at any time and place, reach a wide audience, persist as a permanent record, and allow anonymity, so home is no longer a refuge.

References: Gladding (Counseling: A Comprehensive Profession) — School counseling: bullying and cyberbullying

☐ Approve ☐ Revise ☐ Retire Notes:

For several weeks, two larger eighth graders have mocked a smaller classmate, taken his lunch, and posted edited photos of him. The principal asks the school counselor to hold a peer mediation session so the three boys can "work it out together." What is the counselor's BEST response?

A. Agree, and set clear ground rules so each boy can share equally

Ground rules do not remove the power imbalance that makes mediation unsafe and unfair for a bullied student.

B. Agree, but ask the target to suggest ways he could avoid the boys

This places responsibility on the target and implies he shares blame for being bullied.

C. Decline, and leave the matter to discipline by the administration

Discipline may be needed, but the counselor still has a role in supporting the target and working with the students who bully.

D. Explain why mediation fits poorly and meet with the boys separately ✓ KEY

Because bullying involves a power imbalance, separate meetings that support the target and hold the others accountable are preferred.

Rationale: Peer mediation assumes two parties with roughly equal power and shared responsibility for a conflict. Bullying is repeated, intentional aggression with a power imbalance, so mediation can retraumatize the target and send the message that both are to blame. Recommended practice is to support the target, address the behavior of those who bully separately, and involve families and school-wide prevention.

References: Gladding (Counseling: A Comprehensive Profession) — School counseling: responding to bullying; limits of peer mediation

► **Check:** Confirm the cited source states that peer mediation is not recommended for bullying (a position also taken by federal StopBullying guidance).

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor reviewing a new referral for a 77-year-old with low mood, poor sleep and withdrawal thinks, "Anyone her age would feel down. Therapy probably won't change much at this point." What does the counselor's reaction BEST reflect?

A. Ageist bias that risks undertreating treatable depression ✓ KEY

Treating depression as a normal part of aging, and assuming older adults cannot change, is ageism and leads to undertreatment.

B. Sound clinical judgment about late-life treatment outcomes

Older adults respond well to psychotherapy for depression; the assumption is not supported.

C. Appropriate humility about his limits with older clients

Humility would lead to consultation or training, not a belief that therapy is futile.

D. A correct view of depression as part of normal aging

Depression is common in later life but is not a normal part of aging.

Rationale: Ageism includes stereotypes that older adults are rigid, unable to change, or naturally depressed. In clinicians it can show up as therapeutic nihilism and missed diagnoses. Depression is not a normal part of aging, and evidence-based therapies such as CBT and problem-solving therapy work well for older adults.

References: Berk (Development Through the Lifespan) — Late adulthood: ageism, stereotypes of aging and late-life depression

☐ Approve ☐ Revise ☐ Retire Notes:

An 84-year-old client mentions that her nephew, who now manages her finances, has her sign blank checks, and that her utilities were shut off last month. She seems anxious when he is mentioned and asks the counselor not to "make trouble." What should the counselor do?

A. Wait to act until she gives proof that funds were taken

Reporting standards rest on reasonable suspicion; counselors do not need proof before acting.

B. Call the nephew to ask him to explain how he uses the money

Confronting a suspected abuser can raise the risk to the client and breaches confidentiality.

C. Follow state law and report suspected exploitation to APS ✓ **KEY**

Financial exploitation of an older adult is reportable abuse in many states; counselors follow their state's law and report to Adult Protective Services.

D. Respect her wish and only document what she has disclosed

Her wishes matter, but many states mandate reports of suspected elder abuse regardless of consent.

Rationale: Elder abuse includes physical, emotional, sexual and financial abuse and neglect; perpetrators are often family members or trusted caregivers. Signs of financial exploitation include unpaid bills despite adequate income and unusual control of money. Reporting laws vary, so counselors know their state's requirements, report suspected abuse to Adult Protective Services when required, and involve the client as much as possible.

References: State law — Elder and vulnerable adult abuse reporting statutes (requirements vary by state) · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 29-year-old nonbinary client, assigned male at birth, has lived in line with their gender identity for years and feels settled and content with it. They seek counseling for work stress after a promotion. Which statement about a gender dysphoria diagnosis is MOST accurate?

A. It applies, since gender identity differs from assigned sex

Incongruence alone is not a disorder; DSM-5-TR also requires clinically significant distress or impairment.

B. It does not apply without related distress or impairment ✓ **KEY**

Gender dysphoria requires clinically significant distress or impairment linked to the incongruence, which this client does not report.

C. It applies, since the incongruence has lasted over 6 months

Duration is only part of the criteria; distress or impairment is also required.

D. It does not apply, since nonbinary identities are excluded

DSM-5-TR criteria include a desire to be or be treated as an alternative gender, which covers nonbinary identities.

Rationale: In DSM-5-TR, gender dysphoria requires a marked incongruence between experienced or expressed gender and assigned gender lasting at least 6 months, plus clinically significant distress or impairment. The diagnosis centers on distress, not identity; being transgender or nonbinary is not itself a mental disorder. Counselors avoid diagnosing identity and focus on the client's presenting concerns.

References: DSM-5-TR — Gender Dysphoria in adolescents and adults: Criterion A duration and Criterion B distress or impairment

☐ Approve ☐ Revise ☐ Retire Notes: _____

In the third session, a 17-year-old client tells the counselor that he is transgender, goes by Eli, and uses he/him pronouns. He has not told his parents. Which counselor response is MOST affirming?

A. Ask him to wait a few months before changing his name

Delaying use of his name invalidates his identity and can weaken trust in the relationship.

B. Use his legal name in session until his parents know

Withholding his name in session is not affirming; safety concerns apply to records and outside contacts, not to how the counselor addresses him.

C. Explore whether this is a phase before using his name

Treating his identity as a phase to be tested is not affirming practice.

D. Use Eli and he/him, and ask where it is safe to do so ✓ **KEY**

Affirming care uses the client's name and pronouns and plans together where they are used, such as in records or with family, to avoid outing him.

Rationale: Affirming counseling with transgender clients means using their chosen name and pronouns, treating gender identity as valid, and collaborating on safety, including when names appear in records or communications that parents or others may see. This respects the client's autonomy and protects against unwanted disclosure.

References: ACA Code of Ethics — A.1.a Primary Responsibility; C.5 Nondiscrimination · Sue & Sue (Counseling the Culturally Diverse) — Counseling LGBTQ+ individuals: affirming practice

☐ Approve ☐ Revise ☐ Retire Notes:

A 45-year-old Somali refugee who resettled two years ago works steadily and cares for his children. He grieves the loss of his land, his community's rituals, and his parents' graves, feels guilty for leaving, and sometimes dreams that his ancestors are calling him. Which concept BEST describes his experience?

A. Cultural bereavement after losing homeland and community ✓ **KEY**

Cultural bereavement is grief over the loss of one's homeland, social structures and cultural identity, a normal response to uprooting.

B. Culture shock from early contact with an unfamiliar setting

Culture shock is the early disorientation of contact with a new culture; his grief centers on what was lost.

C. Prolonged grief disorder after the death of a close person

Prolonged grief disorder requires the death of someone close and impairing grief; his loss is of culture and place, and he functions well.

D. Posttraumatic stress disorder with prominent survivor guilt

He reports no intrusion, avoidance or arousal symptoms; guilt and longing alone do not indicate PTSD.

Rationale: Eisenbruch described cultural bereavement as the grief of uprooted people for lost land, culture and social bonds, which can include guilt over leaving, longing, and dreams or visits from ancestors. He cautioned that such experiences may be misread as pathology. Counselors validate the loss and draw on cultural and community resources for mourning.

References: Sue & Sue (Counseling the Culturally Diverse) — Refugees and immigrants: cultural bereavement and migration-related loss

► **Check:** Verify the Eisenbruch (1991) attribution and that the cited Sue & Sue edition discusses cultural bereavement.

☐ Approve ☐ Revise ☐ Retire Notes:

A 12-year-old regularly interprets for her Spanish-speaking parents at medical visits and the bank. At the start of a family counseling session about the father's job loss, the parents ask her to interpret for them. What should the counselor do?

A. Let her interpret, since the family trusts her the most

Trust does not offset the role reversal and exposure to adult concerns that interpreting places on a child.

B. Hold the session in English and simplify the language

Simplifying English does not give the parents full access to the session.

C. Arrange a qualified interpreter so she can be a child ✓ **KEY**

A qualified interpreter gives the parents full access and frees the child from a parental role in adult matters.

D. Ask her to sum up each part instead of translating it

Summaries still make the child the gatekeeper of adult content and risk distortion.

Rationale: Language brokering, in which children of immigrants interpret for parents, can build skills but also brings role reversal, stress and exposure to adult worries. In counseling, using a child as interpreter compromises accuracy and confidentiality and burdens the child. The ACA Code directs counselors to arrange qualified interpreters when clients have difficulty understanding the language used.

References: ACA Code of Ethics — A.2.c Developmental and Cultural Sensitivity (qualified interpreters) · Sue & Sue (Counseling the Culturally Diverse) — Language barriers, language brokering and the use of interpreters

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 19-year-old with a three-year history of superficial cutting to "calm down" tells her counselor that last weekend she swallowed a bottle of pills because she "wanted to never wake up." She slept for 12 hours and woke without treatment. Which statement is MOST accurate?

A. The overdose is part of her NSSI pattern, given her history

NSSI is defined by the absence of suicidal intent; she clearly stated she wanted to die.

B. The overdose was a suicide attempt, and NSSI raises her risk ✓ **KEY**

Intent to die makes the overdose a suicide attempt, and a history of NSSI is a known risk factor for suicidal behavior.

C. Her low-lethality outcome shows the act was mainly a gesture

Outcome does not define intent; calling it a gesture minimizes a real suicide attempt.

D. Her NSSI lowers suicide risk because it works as coping for her

NSSI may regulate emotion short term, but it is associated with higher, not lower, suicide risk.

Rationale: NSSI and suicidal behavior are distinguished mainly by intent: NSSI is self-inflicted harm without intent to die, usually to relieve distress, whereas a suicide attempt involves at least some intent to die. The two often co-occur, and NSSI history is a robust predictor of later suicide attempts. Each act should be assessed for intent, and safety planning updated.

References: DSM-5-TR — Conditions for Further Study: Nonsuicidal Self-Injury Disorder and Suicidal Behavior Disorder · C-SSRS — Distinguishing suicidal from non-suicidal self-injurious behavior by intent

☐ Approve ☐ Revise ☐ Retire Notes: _____

The DSM-5-TR proposed criteria for nonsuicidal self-injury disorder, a condition for further study, require intentional self-inflicted injury without suicidal intent on how many days in the past year?

A. At least 2 days

Two days is below the proposed threshold.

B. At least 12 days

Twelve days exceeds the proposed threshold.

C. At least 30 days

Thirty days is far above the proposed threshold.

D. At least 5 days ✓ KEY

The proposed criteria require self-injury on 5 or more days in the past year.

Rationale: In DSM-5-TR Section III, the proposed nonsuicidal self-injury disorder requires intentional self-inflicted damage to the body on 5 or more days in the past year, with the expectation of only minor or moderate harm. DSM-5-TR also added codes for current or past nonsuicidal self-injury among conditions that may be a focus of clinical attention.

References: DSM-5-TR — Conditions for Further Study: Nonsuicidal Self-Injury Disorder, Criterion A

☐ Approve

☐ Revise

☐ Retire

Notes:

A 16-year-old cannot fall asleep before 2 a.m. on school nights and is exhausted in morning classes. On weekends he sleeps soundly from 2 a.m. to 11 a.m. and wakes refreshed. Months of sleep hygiene tips such as no caffeine and a cool, dark room have not helped. Which explanation BEST fits?

A. A circadian rhythm disorder of the delayed sleep phase type ✓ KEY

Sleep that is normal in quality but shifted later, causing conflict with required schedules, fits delayed sleep phase type.

B. An insomnia disorder needing more strict sleep hygiene rules

He sleeps well when allowed his own schedule, so the problem is timing, and sleep hygiene alone has not helped.

C. A hypersomnolence disorder marked by excessive sleep need

His total sleep is normal and he wakes refreshed on weekends, which argues against hypersomnolence.

D. Narcolepsy with sleep attacks in his early morning classes

Narcolepsy involves recurrent irresistible sleep episodes regardless of schedule, not a simple shift in timing.

Rationale: Circadian rhythm sleep-wake disorders arise from a mismatch between the internal clock and required schedules. Delayed sleep phase type, common in adolescents, causes trouble falling asleep and waking at conventional times, while sleep at the preferred time is normal. Sleep hygiene alone rarely fixes a timing problem; approaches such as a consistent wake time and morning light, coordinated with a medical provider, target the clock itself.

References: DSM-5-TR — Circadian Rhythm Sleep-Wake Disorders: delayed sleep phase type

☐ Approve

☐ Revise

☐ Retire

Notes:

A 58-year-old client with schizophrenia has taken a first-generation antipsychotic for many years. The counselor notices repetitive lip smacking, tongue thrusting and facial grimacing that the client does not seem aware of. Which side effect do these signs MOST likely reflect?

A. Akathisia, an inner restlessness and urge to keep moving

Akathisia is subjective restlessness with pacing or fidgeting, not involuntary oral and facial movements.

B. Acute dystonia, with sudden and sustained muscle spasms

Acute dystonia appears early in treatment as painful sustained contractions, not repetitive movements after years.

C. Tardive dyskinesia from long-term antipsychotic use ✓ KEY

Involuntary repetitive movements of the mouth, tongue and face after long-term antipsychotic use are classic tardive dyskinesia.

D. Neuroleptic malignant syndrome, a fever-and-rigidity crisis

NMS is an acute emergency with high fever, severe rigidity and confusion, not chronic oral-facial movements.

Rationale: Tardive dyskinesia is a late-emerging, sometimes irreversible movement disorder linked to long-term dopamine-blocking medications, especially first-generation antipsychotics. Counselors who notice possible signs should promptly inform the prescriber. Second-generation antipsychotics carry lower TD risk but greater metabolic risks, such as weight gain, diabetes and lipid changes, which also warrant medical monitoring.

References: DSM-5-TR — Medication-Induced Movement Disorders: Tardive Dyskinesia

☐ Approve ☐ Revise ☐ Retire Notes:

A client who takes phenelzine, a monoamine oxidase inhibitor, mentions he is planning a celebration dinner of aged cheeses, cured salami and draft beer. The counselor encourages him to check with his prescriber first. Which reaction is the main concern?

A. Agranulocytosis

Agranulocytosis is a blood disorder linked mainly to clozapine, not to diet with MAOIs.

B. Hypertensive crisis ✓ KEY

Tyramine in aged, cured and fermented foods can trigger a dangerous blood pressure spike in people taking MAOIs.

C. Lithium toxicity

Lithium toxicity relates to lithium levels, not to tyramine or MAOIs.

D. Tardive dyskinesia

Tardive dyskinesia is a movement disorder from long-term antipsychotics, not a dietary reaction.

Rationale: MAOIs block the breakdown of tyramine, found in aged cheeses, cured meats, some tap or draft beers and other fermented foods. Tyramine buildup can cause a hypertensive crisis with severe headache, stiff neck and rapid blood pressure rise. MAOIs also interact dangerously with many drugs, including other antidepressants, raising the risk of serotonin syndrome. Counselors reinforce prescriber guidance rather than giving medical advice.

References: Gladding (Counseling: A Comprehensive Profession) — Psychopharmacology for counselors: MAOIs and tyramine dietary restrictions

► **Check:** Confirm the cited source covers MAOI dietary restrictions; the pharmacology is standard.

☐ Approve ☐ Revise ☐ Retire Notes:

A 20-year-old college student without an ADHD diagnosis tells her counselor she takes a friend's prescription stimulant before exams to "focus better." She says it is safe "because doctors prescribe it to kids." What is the counselor's BEST response?

A. Tell her she must stop now or the counselor will alert the school

Threats damage the alliance, and confidential disclosure of past use does not by itself permit a report.

B. Agree it is low risk if used only during the exam period

Nonmedical stimulant use carries real risks even when occasional, so this response gives false reassurance.

C. Advise her to seek her own prescription so she can use it safely

Steering her to obtain a stimulant without a clinical need endorses misuse and is outside the counselor's role.

D. Explore her use nonjudgmentally and discuss its health risks ✓ **KEY**

A nonjudgmental, MI-consistent discussion of risks such as dependence and cardiac strain respects autonomy and invites change.

Rationale: Nonmedical use of prescription stimulants is common among college students and carries risks such as elevated heart rate and blood pressure, anxiety, insomnia and stimulant use disorder. Counselors respond with motivational interviewing: exploring the student's reasons, offering accurate information with permission, and addressing underlying academic stress, rather than lecturing or threatening.

References: Miller & Rollnick (MI) — Exchanging information with permission (elicit-provide-elicite) and avoiding the righting reflex · DSM-5-TR — Stimulant Use Disorder

☐ Approve ☐ Revise ☐ Retire Notes:

212. nce-s-clf-116 · Counseling and Helping Relationships · military and veteran populations · medium · In review

Two months after her husband returned from a 9-month deployment, a military spouse says he keeps overriding household routines she ran on her own, the children test his rules, and they feel like "strangers sharing a house." In the emotional cycle of deployment, which phase is the family in?

A. Post-deployment, as the family renegotiates its roles ✓ **KEY**

Post-deployment (reintegration) involves renegotiating roles, routines and intimacy after the service member returns.

B. Sustainment, as the family settles into new routines

Sustainment occurs during deployment, when the family at home builds new routines and support.

C. Redeployment, as the family anticipates his return

Redeployment is the period just before return, marked by excitement and apprehension.

D. Pre-deployment, as the family prepares for the leave

Pre-deployment covers preparation and anticipatory loss before departure.

Rationale: The emotional cycle of deployment is often described in phases: pre-deployment, deployment, sustainment, redeployment and post-deployment. Post-deployment reintegration can take months as families renegotiate roles the at-home partner took on, rebuild intimacy and adjust parenting. Counselors normalize this friction and help families communicate about changed roles.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling military families: emotional cycle of deployment and reintegration

► **Check:** Verify the five-phase deployment cycle (Pincus et al.) and that the cited Gladding edition covers it.

☐ Approve ☐ Revise ☐ Retire Notes:

A 38-year-old Air Force veteran discloses that she was sexually assaulted by a fellow service member during her enlistment. She never reported it, has no VA disability rating, and assumes VA will not help her without proof. Which statement BEST guides the counselor's response?

A. She must first file a report so the assault can be verified

VA does not require that the assault was reported or documented to provide MST-related care.

B. She can get care only after a service-connection rating

MST-related care does not require a service-connected disability rating.

C. VA offers free MST care without a report or a rating ✓ **KEY**

VA provides free care for mental and physical conditions related to military sexual trauma without requiring a prior report or a rating.

D. MST must first be diagnosed as a disorder to get services

MST is an experience, not a diagnosis; care addresses conditions related to it.

Rationale: Military sexual trauma (MST) is the VA term for sexual assault or threatening sexual harassment during military service. It is an experience, not a diagnosis. VA screens veterans for MST and provides free MST-related treatment even when the veteran never reported the event and has no service-connected rating. Knowing this helps counselors connect survivors to care and reduce barriers.

References: VA/DoD CPG — Management of PTSD and acute stress disorder: military sexual trauma and access to care

► **Check:** Verify current VA MST eligibility rules (free MST-related care without documentation or rating) and whether the VA/DoD CPG is the best citation; a VA program source may fit better.

☐ Approve ☐ Revise ☐ Retire Notes:

The mother of a 9-year-old with cerebral palsy says she accepted her son's diagnosis long ago and loves their life, yet waves of sadness return each time his classmates reach milestones he cannot, such as riding a bike. She asks whether something is wrong with her. Which concept BEST frames her experience?

A. Prolonged grief disorder from an unresolved earlier loss

Prolonged grief disorder follows a death and involves persistent, impairing grief; her sadness is episodic and she functions well.

B. Chronic sorrow, a normal and recurring grief response ✓ **KEY**

Chronic sorrow describes periodic, recurring grief triggered by milestones in parents of children with lasting disabilities; it is not pathological.

C. Denial, the first stage that she has not yet moved beyond

She openly acknowledges the disability; renewed sadness is not denial, and stage models are not strictly linear.

D. A major depressive episode with a seasonal recurring pattern

Brief, triggered waves of sadness with good functioning do not meet criteria for a depressive episode.

Rationale: Olshansky introduced chronic sorrow to describe the recurring sadness of parents of children with lifelong disabilities. Grief resurfaces at milestones, transitions and reminders of the gap between expected and actual life, while day-to-day functioning and love for the child remain intact. Counselors normalize this pattern instead of framing it as failed acceptance.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling families of people with disabilities: chronic sorrow

► **Check:** Confirm the cited Gladding edition discusses chronic sorrow (Olshansky, 1962).

☐ Approve ☐ Revise ☐ Retire Notes:

Treatment Planning

34 items · NCE weight 9% (14 of 160 scored items)

215. nce-s-trt-001 · Counseling and Helping Relationships · SMART goals and measurable objectives · easy · In review

A counselor is writing a treatment plan for a 34-year-old client with social anxiety. Which objective is BEST written according to SMART criteria?

A. Client will feel much more confident in social settings by the end of treatment.

Feeling "more confident" is not observable or measurable, and "end of treatment" is not a specific time frame.

B. Client will start one conversation with a coworker at least 3 days a week within 6 weeks. ✓ **KEY**

This is specific, measurable (frequency count), achievable, relevant to social anxiety, and time-bound (6 weeks).

C. Client will understand the cognitive roots of social anxiety and work through them fully.

"Understand" and "work through" are vague internal processes with no measurable criterion or deadline.

D. Client will attend weekly counseling sessions and complete homework as the counselor assigns.

Attendance and compliance describe the treatment process, not a measurable change in the presenting problem.

Rationale: SMART objectives are Specific, Measurable, Achievable, Relevant and Time-bound. A good objective names an observable behavior, a criterion for success, and a deadline, so progress can be tracked and the plan revised. Vague feeling states and process statements (attendance) do not meet the standard.

References: Wiger (Documentation) — Treatment plans: behaviorally measurable goals and objectives

► **Check:** Wiger reference detail describes the concept, not an exact chapter title.

☐ Approve ☐ Revise ☐ Retire Notes:

A 41-year-old client with an alcohol use disorder has relapsed twice while attending weekly outpatient sessions. There is no withdrawal risk, no acute medical or psychiatric concern, and the client has stable housing and a supportive partner. Which level of care is MOST appropriate?

A. Medically managed intensive inpatient treatment in a hospital setting

Hospital-level care is for severe, unstable withdrawal or acute medical/psychiatric needs, none of which are present.

B. Long-term residential treatment with 24-hour structure and support

Residential care is more restrictive than needed for a client with stable housing and a supportive recovery environment.

C. An intensive outpatient program with several sessions per week ✓ **KEY**

Repeated relapse despite weekly outpatient care, with no withdrawal, medical or housing barriers, points to a step up to the next more intensive outpatient level.

D. Continue weekly outpatient sessions with no change to the treatment plan

Repeated relapse at the current intensity signals that the present level is not meeting the client's needs.

Rationale: Placement criteria such as the ASAM Criteria match clients to the least restrictive level of care that safely meets their needs across multiple dimensions (withdrawal, medical, emotional, readiness, relapse potential, recovery environment). When a client is not progressing at one level and has no acute risks, the usual step is to the next more intensive level, not a jump to residential or hospital care.

References: ASAM Criteria — Multidimensional assessment and least restrictive level of care; outpatient vs intensive outpatient

► **Check:** ASAM Criteria 4th ed. (2023) renamed some levels; confirm the level-of-care wording is current.

☐ Approve ☐ Revise ☐ Retire Notes:

A 29-year-old veteran meets criteria for posttraumatic stress disorder after a combat deployment. According to major clinical practice guidelines, which psychotherapy approach has the strongest research support as a first-line treatment?

A. Trauma-focused therapy such as cognitive processing therapy or prolonged exposure ✓ **KEY**

APA and VA/DoD guidelines give their strongest recommendations to trauma-focused therapies such as CPT and PE.

B. A single session of psychological debriefing held soon after returning home

Single-session debriefing is not recommended; it has not been shown to prevent or treat PTSD and may be harmful.

C. Open-ended nondirective supportive counseling focused on current life stressors

Nondirective supportive counseling is often used as a control condition and is not a first-line PTSD treatment.

D. Progressive muscle relaxation training used alone as the primary treatment

Relaxation may help with arousal, but used alone it lacks the support of trauma-focused treatments for PTSD.

Rationale: Evidence-based treatment selection integrates the best research evidence with clinical expertise and client preferences. For PTSD, the APA (2017) and VA/DoD guidelines recommend trauma-focused psychotherapies such as cognitive processing therapy and prolonged exposure as first-line treatments.

References: APA CPG — Clinical Practice Guideline for the Treatment of PTSD (2017): CPT, PE, CBT and CT strongly recommended · VA/DoD CPG — Management of PTSD and Acute Stress Disorder: trauma-focused psychotherapy as first-line treatment

► **Check:** VA/DoD 2023 reference detail is paraphrased; confirm wording.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor has completed an intake with a 52-year-old Korean American client who reports grief and insomnia after her husband's death. What is the BEST way for the counselor to develop the treatment plan?

- A.** Write the plan alone based on the diagnosis, then review it with the client next week
Presenting a finished plan treats the client as a recipient rather than a partner and misses her priorities.
- B.** Use a standard grief protocol so the client receives the same plan as other clients
A one-size-fits-all plan ignores individual goals, cultural values and preferences.
- C.** Ask the client's adult children what goals they would like their mother to work on
Family input may be welcome with consent, but the client herself is the primary partner in setting her goals.
- D.** Work with the client to identify her goals and the methods she finds acceptable ✓ **KEY**
Collaborative planning engages the client in choosing goals and methods, respecting her values and culture.

Rationale: Treatment plans should be developed jointly with the client. Collaboration respects autonomy, incorporates cultural values and preferences, and increases engagement and follow-through. The ACA Code directs counselors and clients to work jointly in devising counseling plans that offer reasonable promise of success.

References: ACA Code of Ethics — A.1.c Counseling Plans

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor gives a 23-year-old client the PHQ-9 at every session. After eight sessions of treatment for depression, the client's scores have not meaningfully changed from intake. What should the counselor do NEXT?

- A.** Stop giving the PHQ-9, since repeated testing seems to be discouraging the client
Dropping the measure removes the very data that revealed the lack of progress.
- B.** Review the scores with the client and revise the treatment plan together ✓ **KEY**
Measurement-based care uses nonresponse as a signal to discuss progress with the client and adjust the plan.
- C.** Continue the same approach unchanged, since depression often improves slowly
Ignoring eight sessions of flat scores delays a needed change in treatment.
- D.** Refer the client out at once, since the lack of change shows a poor counselor fit
Flat scores call for review and adjustment first; immediate referral is premature.

Rationale: Routine outcome monitoring with a standardized measure lets the counselor detect clients who are not responding. Feedback on scores should be shared with the client and used to revisit goals, interventions or level of care. Measurement-based care has been shown to improve outcomes, especially for clients at risk of treatment failure.

References: Hays (Assessment) — Outcome assessment and progress monitoring with standardized measures

☐ Approve ☐ Revise ☐ Retire Notes:

During a first session, a 47-year-old client mentions that she also sees a psychiatrist who prescribes her antidepressant. What should the counselor do to coordinate care?

A. Call the psychiatrist right away to share the intake findings and diagnosis

Sharing information without the client's authorization breaches confidentiality.

B. Ask the client to stop seeing the psychiatrist to avoid conflicting treatment

Medication management and counseling are complementary; there is no basis for asking her to stop.

C. Request the client's written consent to inform and consult with the psychiatrist ✓ KEY

The ACA Code directs counselors to request a release so they can inform the other professional and collaborate.

D. Note the psychiatrist in the file and leave all contact entirely up to the client

This is passive; counselors are expected to seek permission to coordinate with other providers.

Rationale: When counselors learn that a client is in a professional relationship with another mental health professional, they request a release to inform that professional and work toward a collaborative relationship. Coordination improves safety and consistency of care, while written consent protects confidentiality.

References: ACA Code of Ethics — A.3 Clients Served by Others

☐ Approve ☐ Revise ☐ Retire Notes: _____

A program director is training new counselors at a community mental health agency. When should discharge and aftercare planning ideally begin for a new client?

A. At intake, revisiting the plan throughout the course of treatment ✓ KEY

Planning for discharge from the start keeps treatment goal-directed and prepares the client for transition.

B. In the final session, once the client has met every treatment goal

Waiting until the last session leaves too little time to arrange supports and aftercare.

C. When the insurer signals that authorized sessions are running out

Letting payer limits drive discharge timing is reactive and can leave gaps in care.

D. Only after the client asks to end treatment or stops attending

This leaves aftercare to chance and misses the chance to plan termination together.

Rationale: Discharge and aftercare planning should begin at admission and be updated as treatment progresses. Early planning defines what successful completion looks like, identifies community supports and step-down services, and reduces the risk of relapse or abrupt termination.

References: Wiger (Documentation) — Discharge planning and the discharge summary

► **Check:** Wiger reference detail describes the concept, not an exact chapter title.

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor is completing a Stanley-Brown Safety Planning Intervention with a 19-year-old college student who has recent suicidal ideation. Which step does the safety plan address FIRST?

A. Listing professionals and crisis lines to contact in an emergency

Professional and crisis contacts come later in the plan, after self-directed and social steps.

B. Making the environment safe by limiting access to lethal means

Means safety is a key part of the plan but is addressed as the final step.

C. Naming people the student can ask directly for help in a crisis

Reaching out to trusted people for help follows warning signs and internal coping.

D. Recognizing personal warning signs that a crisis may be developing ✓ KEY

The plan begins with identifying warning signs so the client knows when to use the plan.

Rationale: The Stanley-Brown Safety Plan proceeds in order: (1) warning signs, (2) internal coping strategies, (3) people and social settings that provide distraction, (4) people to ask for help, (5) professionals and agencies to contact, and (6) making the environment safe. Recognizing warning signs comes first because it tells the client when to begin using the plan.

References: Stanley-Brown SPI — Six-step safety plan: Step 1, recognizing warning signs

☐ Approve ☐ Revise ☐ Retire Notes:

223. nce-s-trt-009 · Counseling and Helping Relationships · stages of change and treatment readiness · easy · In review

A 38-year-old client says, "I know my drinking is hurting my marriage, and part of me wants to cut back, but I'm just not sure I'm ready to give it up." Which stage of change does this statement BEST reflect?

A. Precontemplation

In precontemplation the person does not see a problem or intend to change; this client recognizes the problem.

B. Contemplation ✓ KEY

Acknowledging the problem while feeling ambivalent about changing is the hallmark of contemplation.

C. Preparation

Preparation involves a commitment to act soon and small steps toward change, which are absent here.

D. Action

Action involves actively modifying the behavior, which this client has not begun.

Rationale: In the Transtheoretical Model, contemplation is marked by awareness that a problem exists and serious thought about change, combined with ambivalence and no firm commitment. Counselors often use motivational interviewing to explore and resolve that ambivalence.

References: Transtheoretical Model — Stages of change: contemplation and ambivalence

☐ Approve ☐ Revise ☐ Retire Notes:

A 60-year-old client stopped smoking four months ago. He uses nicotine gum, avoids places where he used to smoke, and calls a friend when cravings hit. According to Prochaska and DiClemente, which stage is he in?

A. Preparation

Preparation precedes the behavior change; this client has already quit.

B. Maintenance

Maintenance generally begins after about six months of sustained change; he is at four months.

C. Action ✓ KEY

He has changed the behavior within the past six months and is actively using change strategies.

D. Contemplation

Contemplation involves thinking about change without acting; he has already acted.

Rationale: The action stage covers roughly the first six months after a person has made an overt change in behavior, when effort and relapse risk are highest. Maintenance begins once the change has been sustained for more than about six months and focuses on preventing relapse.

References: Transtheoretical Model — Action vs maintenance: the six-month criterion

► **Check:** Action vs. maintenance rests on the conventional ~6-month line in the Transtheoretical Model.

☐ Approve ☐ Revise ☐ Retire Notes:

Three months into a new school-based anger management group, a counselor gathers feedback from students and teachers so she can refine the sessions while the program is still running. This is an example of which type of evaluation?

A. Formative evaluation ✓ KEY

Formative evaluation occurs during implementation to improve the program as it runs.

B. Summative evaluation

Summative evaluation judges overall effectiveness or outcomes after the program is completed.

C. Needs assessment

A needs assessment happens before a program is designed, to identify gaps and priorities.

D. Cost-benefit analysis

Cost-benefit analysis compares program costs with monetary value of outcomes, not process feedback.

Rationale: Formative evaluation is conducted while a program is under way to give feedback for improvement. Summative evaluation is conducted at the end to judge whether the program achieved its goals and should be continued, expanded or ended.

References: Erford (Research and Evaluation in Counseling) — Program evaluation: formative vs summative evaluation

☐ Approve ☐ Revise ☐ Retire Notes:

Before creating any new services, a rural counseling center surveys residents, local physicians and school staff to find out which mental health concerns are most common and which services are missing. What is the center conducting?

A. An outcome evaluation

Outcome evaluation measures the effects of an existing program; no program exists yet.

B. A process evaluation

Process evaluation examines how an existing program is being delivered.

C. A fidelity review

A fidelity review checks whether a program is delivered as designed, which requires a program in place.

D. A needs assessment ✓ KEY

Gathering data on community problems and service gaps before planning services is a needs assessment.

Rationale: A needs assessment systematically identifies the gap between current conditions and desired conditions in a population, using surveys, key informants, focus groups or existing data. It is typically the first step in program planning and guides which services to develop.

References: Erford (Research and Evaluation in Counseling) — Needs assessment as the first phase of program evaluation

☐ Approve ☐ Revise ☐ Retire Notes: _____

A school counselor records a student's disruptive outbursts during baseline, introduces a token system, removes it, and then reintroduces it (an ABAB design). What is the main purpose of the withdrawal phase?

A. To let the student rest so the token system does not lose its appeal

Withdrawal is not about preventing satiation; it is a test of experimental control.

B. To show that behavior changes when and only when the treatment changes ✓ KEY

A return toward baseline when treatment is removed, and improvement when it returns, supports a causal link.

C. To compare the student's behavior with a randomly assigned control group

Single-subject designs use the person as his or her own control; there is no separate group.

D. To produce enough data points to compute a reliable correlation value

The goal of the reversal is to demonstrate experimental control, not to compute a correlation.

Rationale: In an ABAB (reversal or withdrawal) single-subject design, behavior is measured at baseline (A), during treatment (B), after treatment is withdrawn (A), and after it is reinstated (B). If behavior worsens on withdrawal and improves again on reinstatement, the counselor has stronger evidence that the intervention, not some other factor, caused the change.

References: Erford (Research and Evaluation in Counseling) — Single-subject designs: AB and ABAB (withdrawal) designs

☐ Approve ☐ Revise ☐ Retire Notes: _____

A researcher delivers a new career-readiness curriculum to one intact ninth-grade class and uses a second existing class at the same school as a comparison. Both classes complete pretests and posttests. Which design is this?

A. A true experimental pretest-posttest control group design

A true experiment requires random assignment of participants to conditions, which did not occur.

B. A single-subject multiple-baseline design across settings

Single-subject designs track individuals over repeated phases, not two classes compared once.

C. A quasi-experimental nonequivalent control group design ✓ KEY

Using intact groups without random assignment, with pre- and posttests, is a nonequivalent group quasi-experiment.

D. A correlational design examining two naturally linked variables

The researcher manipulated an independent variable (the curriculum), so this is not correlational.

Rationale: The defining feature of a true experiment is random assignment to conditions. When a researcher manipulates an independent variable but uses pre-existing groups, the design is quasi-experimental. Pretests help assess initial group differences, but selection remains a threat to internal validity.

References: Erford (Research and Evaluation in Counseling) — Experimental vs quasi-experimental designs; random assignment

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor selects the 15 students with the highest scores on a test anxiety scale for a six-week group. There is no comparison group, all students complete the group, and nothing notable happens at school during that time. Posttest scores are lower. Which threat to internal validity MOST likely accounts for part of this drop?

A. Experimental mortality, or attrition of group members

All students completed the group, so loss of participants cannot explain the change.

B. History, or outside events occurring during the study period

The stem rules out notable outside events during the six weeks.

C. Diffusion of treatment between groups being compared

Diffusion requires a comparison group that is exposed to the treatment; there is none.

D. Statistical regression of extreme scores toward the mean ✓ KEY

Participants selected for extreme scores tend to score closer to the mean on retest even without treatment.

Rationale: Statistical regression is a threat to internal validity whenever participants are chosen because of extreme scores. Because no measure is perfectly reliable, extreme scorers tend to move toward the mean on retesting, which can be mistaken for a treatment effect. A comparison group selected the same way helps rule this out.

References: Erford (Research and Evaluation in Counseling) — Threats to internal validity: statistical regression

☐ Approve ☐ Revise ☐ Retire Notes:

A researcher sets alpha at .05 and finds $p = .03$ when comparing a new intervention with treatment as usual, so she rejects the null hypothesis. If the intervention in fact has no real effect, what has occurred?

A. A Type I error, a false positive finding ✓ KEY

Rejecting a null hypothesis that is actually true is a Type I error; its probability is set by alpha.

B. A Type II error, a false negative finding

A Type II error is failing to reject a false null, missing a real effect; here the null was rejected.

C. A correct decision, since p fell below alpha

A p -value below alpha does not guarantee a true effect; if there is none, rejecting the null was an error.

D. A power failure caused by too small a sample

Low power raises the risk of Type II errors, not of falsely detecting an effect that does not exist.

Rationale: A Type I error (alpha) is rejecting a true null hypothesis, concluding an effect exists when it does not. A Type II error (beta) is failing to reject a false null. The p -value is the probability of results at least this extreme if the null is true; it is not the probability that the null is true, so a significant p can still be a false positive.

References: Erford (Research and Evaluation in Counseling) — Hypothesis testing: Type I and Type II errors, alpha and p -values

☐ Approve ☐ Revise ☐ Retire Notes:

A study of 4,000 clients finds that a brief online module reduces anxiety scores compared with a waitlist, $p < .001$, with Cohen's $d = 0.08$. How should a counselor BEST interpret these results?

A. The module has a large effect, as shown by the very small p -value

The p -value reflects sample size as well as effect; it does not indicate magnitude.

B. The difference is statistically significant but practically trivial ✓ KEY

With a very large sample, tiny effects reach significance; $d = 0.08$ is well below even a small effect (0.2).

C. The result is likely a Type II error because the sample was so large

A Type II error is missing a real effect; here an effect was detected, and large samples reduce Type II risk.

D. The module causes anxiety to drop by about 8 percent for most clients

Cohen's d is a difference in standard deviation units, not a percentage reduction.

Rationale: Statistical significance tells whether an effect is unlikely under the null hypothesis; effect size tells how large it is. Cohen's guidelines for d are about 0.2 small, 0.5 medium and 0.8 large. Large samples can make trivial differences significant, so counselors should look at effect size to judge clinical or practical importance.

References: Erford (Research and Evaluation in Counseling) — Effect size (Cohen's d) vs statistical significance

☐ Approve ☐ Revise ☐ Retire Notes:

A survey of 600 adolescents finds that hours spent on social media correlate $r = .45$ with scores on a depressive symptom scale. Which conclusion is BEST supported by this finding?

A. Heavy social media use causes depressive symptoms in teenagers

A correlation cannot establish causation; depression could drive use, or a third variable could affect both.

B. Teens who use social media more tend to report more depressive symptoms ✓ KEY

A positive correlation means higher values on one variable tend to go with higher values on the other, without implying cause.

C. About 45 percent of depressive symptoms are explained by social media use

Shared variance is r squared ($.45 \times .45$, about 20 percent), and even that describes association, not explanation by cause.

D. Reducing social media time will lower depression scores for most teens

Predicting the effect of changing one variable is a causal claim that requires an experimental design.

Rationale: Correlational designs show whether and how strongly two variables move together, but not why. The direction of influence is unknown, and a third variable (such as loneliness or poor sleep) may account for both. The coefficient of determination (r squared) gives the proportion of shared variance; here about 20 percent.

References: Erford (Research and Evaluation in Counseling) — Correlational research: correlation vs causation and the coefficient of determination

☐ Approve ☐ Revise ☐ Retire Notes: _____

A statistics instructor asks counseling students to classify several variables by scale of measurement. Which variable is measured on an interval scale?

A. Clients' ranked order of improvement

Ranks show order but not equal distances between positions, which makes them ordinal.

B. Number of sessions each client attended

A count has equal intervals and a true zero (no sessions), which makes it a ratio scale.

C. Temperature in degrees Fahrenheit ✓ KEY

Fahrenheit has equal intervals but an arbitrary zero point, the defining feature of an interval scale.

D. Clients' primary diagnostic category

Diagnostic categories are named groups with no inherent order, which makes them nominal.

Rationale: The four scales build on each other: nominal (categories), ordinal (rank order), interval (equal units but no true zero), and ratio (equal units plus a true zero). Because interval data lack a true zero, ratios are not meaningful; 80 degrees Fahrenheit is not twice as hot as 40 degrees.

References: Erford (Research and Evaluation in Counseling) — Scales of measurement: nominal, ordinal, interval and ratio

☐ Approve ☐ Revise ☐ Retire Notes: _____

Annual household incomes of clients at a community counseling agency are mostly low, but a small number of clients report very high incomes. Which statement about this distribution is accurate?

A. It is positively skewed, and the mean is higher than the median ✓ KEY

A tail of a few very high values is a positive skew, and the mean is pulled toward that tail, above the median.

B. It is negatively skewed, and the mean is lower than the median

A negative skew has its tail toward low values; here the tail is toward high incomes.

C. It is positively skewed, and the mode is higher than the mean

In a positive skew the mode sits at the peak on the low end, below both the median and the mean.

D. It is symmetrical, so the mean, median and mode are about equal

A few extreme high values make the distribution asymmetrical, so the three measures separate.

Rationale: Skew is named for the direction of the tail. In a positively skewed distribution, the typical order is mode < median < mean, because the mean is sensitive to extreme scores. For skewed data such as income, the median is usually the best single measure of central tendency.

References: Erford (Research and Evaluation in Counseling) — Descriptive statistics: measures of central tendency and skewed distributions

☐ Approve ☐ Revise ☐ Retire Notes:

Scores on a depression inventory are normally distributed with a mean of 50 and a standard deviation of 10. A client scores 60. Approximately what percentage of the norm group scored at or below this client?

A. 34 percent

About 34 percent of scores fall between the mean and +1 SD; this omits the 50 percent below the mean.

B. 50 percent

Fifty percent falls at or below the mean of 50, not at a score one SD above it.

C. 68 percent

About 68 percent fall within plus or minus 1 SD of the mean, which is a band, not a cumulative percentage.

D. 84 percent ✓ KEY

A score of 60 is $z = +1$; 50 percent fall below the mean plus about 34 percent between the mean and +1 SD.

Rationale: The standard deviation describes how spread out scores are around the mean. In a normal distribution, about 68 percent of scores fall within 1 SD of the mean, 95 percent within 2 SD, and 99.7 percent within 3 SD. A score 1 SD above the mean ($z = +1$, $T = 60$) sits at about the 84th percentile.

References: Erford (Research and Evaluation in Counseling) — Standard deviation, the normal curve and percentile ranks

☐ Approve ☐ Revise ☐ Retire Notes:

A researcher randomly assigns 90 clients to one of three conditions (individual CBT, a mindfulness group, or a waitlist) and compares the groups on posttest anxiety scores measured on an interval scale. Which statistical test is MOST appropriate?

A. An independent-samples t-test

A t-test compares only two group means; running several t-tests inflates the Type I error rate.

B. A one-way analysis of variance ✓ **KEY**

One categorical independent variable with three levels and one interval dependent variable calls for a one-way ANOVA.

C. A chi-square test of independence

Chi-square tests relationships between categorical variables (frequencies), not differences in mean scores.

D. A Pearson product-moment correlation

Pearson r measures the linear relationship between two continuous variables, not group differences.

Rationale: Here the independent variable is treatment condition (three levels) and the dependent variable is the anxiety score. A one-way ANOVA compares three or more group means in a single test while holding the Type I error rate at alpha; post hoc tests then show which groups differ. A t-test is used for two groups, chi-square for categorical outcomes, and Pearson r for relationships between two continuous variables.

References: Erford (Research and Evaluation in Counseling) — Selecting inferential statistics: t-test, ANOVA, chi-square and correlation

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor-researcher is planning a study comparing two interventions and wants to reduce the risk of missing a true but modest difference between them. Which change would MOST directly increase statistical power?

A. Lowering alpha from .05 to .01 to guard against false positives

A stricter alpha makes it harder to reject the null, which lowers power and raises Type II risk.

B. Using a smaller sample so each participant gets more attention

Smaller samples increase sampling error and reduce power to detect a real effect.

C. Increasing the number of participants assigned to each condition ✓ **KEY**

Larger samples reduce standard error, making a true effect of a given size more likely to be detected.

D. Switching from a one-tailed test to a two-tailed test of the effect

Splitting alpha across two tails lowers power for an effect in the predicted direction.

Rationale: Power (1 - beta) is the probability of correctly rejecting a false null hypothesis. It rises with larger sample size, larger effect size, a more lenient alpha, a one-tailed test (when the direction is correct), and more reliable measures. Researchers run a power analysis before a study, often targeting .80, to choose a sample size that can detect the expected effect.

References: Erford (Research and Evaluation in Counseling) — Statistical power, sample size and Type II error

☐ Approve ☐ Revise ☐ Retire Notes:

A trial finds that a new anxiety program works well. Participants knew they were in a closely watched study, received extra attention from research staff, and liked being part of something new. Critics doubt that the results will hold in routine agency practice. Which threat are the critics MOST likely describing?

A. Reactive effects of the experimental arrangements ✓ KEY

Changes caused by knowing one is studied (a Hawthorne-type effect) limit generalization to ordinary settings, a threat to external validity.

B. Statistical regression of extreme scores toward the mean

Regression is an internal validity threat tied to selecting extreme scorers, which the stem does not describe.

C. Instrumentation changes in how outcomes were measured

Instrumentation is an internal validity threat involving shifts in measurement, not generalizability.

D. Selection differences between the groups being compared

Selection is an internal validity threat about nonequivalent groups, not about generalizing to practice.

Rationale: External validity concerns whether results generalize to other people, settings and times. Reactive arrangements, such as the Hawthorne effect, occur when the research situation itself (attention, novelty, awareness of being studied) contributes to the outcome, so effects may shrink in routine care. Internal validity threats such as history, regression, selection and instrumentation instead question whether the treatment caused the change at all.

References: Erford (Research and Evaluation in Counseling) — Threats to external validity: reactive effects of experimental arrangements

☐ Approve ☐ Revise ☐ Retire Notes:

A school counselor wants to show that a social skills intervention caused improvement for three elementary students on the autism spectrum. Once learned, the skills are not expected to fade, and withdrawing the intervention would be ethically questionable. Which design BEST fits these conditions?

A. An AB design with a single baseline and treatment phase

An AB design lacks replication, so it cannot rule out other explanations such as maturation or history.

B. An ABAB reversal design that withdraws the intervention

Reversal requires withdrawing treatment and expects behavior to return to baseline, neither of which fits here.

C. A one-group pretest-posttest design with all three students

This pre-experimental design has no control for threats such as maturation or history.

D. A multiple-baseline design staggered across the students ✓ KEY

Starting the intervention at different times for each student shows control without withdrawing treatment.

Rationale: In a multiple-baseline design, baselines begin together but the intervention is introduced at staggered points across participants, behaviors or settings. If each behavior changes only when the intervention starts for it, the counselor gains evidence of causation without a reversal. It is the design of choice when behavior is not reversible or withdrawal would be unethical; a simple AB design shows change but not experimental control.

References: Erford (Research and Evaluation in Counseling) — Single-subject designs: AB vs multiple-baseline designs

☐ Approve ☐ Revise ☐ Retire Notes:

A researcher conducts in-depth interviews with 12 parents who have lost a child to overdose. Her goal is to describe the essence of how parents live through this kind of grief, setting aside her own assumptions as she analyzes the data. Which qualitative tradition does this study reflect?

A. Grounded theory

Grounded theory aims to generate a theory of a process through constant comparison and theoretical sampling.

B. Phenomenology ✓ KEY

Describing the essence of a lived experience and bracketing researcher assumptions are hallmarks of phenomenology.

C. Ethnography

Ethnography studies the shared patterns of a culture-sharing group through prolonged fieldwork and observation.

D. Case study

A case study examines one or a few bounded systems in depth using multiple sources of data.

Rationale: Phenomenology seeks the essential structure of a lived experience shared by several people, typically through in-depth interviews, with the researcher bracketing (epoche) personal assumptions. Grounded theory builds theory from data, ethnography describes a culture, and a case study explores a bounded case in its context.

References: Erford (Research and Evaluation in Counseling) — Qualitative traditions: phenomenology, grounded theory, ethnography and case study

☐ Approve ☐ Revise ☐ Retire Notes:

After coding interviews with first-generation college students, a counselor-researcher shares her preliminary themes with the participants and asks whether the themes accurately capture their experiences. Which trustworthiness strategy is she using?

A. Triangulation

Triangulation compares multiple data sources, methods, investigators or theories, not participant review of findings.

B. Peer debriefing

Peer debriefing involves a colleague, not the participants, reviewing and questioning the analysis.

C. Member checking ✓ KEY

Returning interpretations to participants to confirm their accuracy is member checking.

D. Audit trail

An audit trail documents decisions throughout the study so an outside reviewer can follow them.

Rationale: Trustworthiness is the qualitative counterpart of validity and reliability, with criteria such as credibility, transferability, dependability and confirmability (Lincoln and Guba). Member checking is a key credibility strategy; triangulation, peer debriefing, prolonged engagement, thick description and audit trails support other criteria.

References: Erford (Research and Evaluation in Counseling) — Trustworthiness in qualitative research: member checking and triangulation

☐ Approve ☐ Revise ☐ Retire Notes:

A state counseling association plans to survey its members and wants school, clinical mental health and rehabilitation counselors represented in the sample in the same proportions as in the full membership, with members chosen at random. Which sampling method BEST meets this goal?

A. Stratified random sampling ✓ KEY

Dividing the population into subgroups and randomly sampling from each guarantees proportional representation.

B. Simple random sampling

Simple random sampling gives everyone an equal chance but does not guarantee each subgroup's proportion.

C. Convenience sampling

Convenience sampling uses whoever is easily available and is neither random nor proportional.

D. Snowball sampling

Snowball sampling recruits through referrals from participants and is used for hard-to-reach groups.

Rationale: Stratified random sampling divides a population into strata on a key characteristic and then randomly selects from each stratum, often in proportion to its size. It improves representativeness on that characteristic compared with simple random sampling, while convenience and snowball samples are nonprobability methods that limit generalization.

References: Erford (Research and Evaluation in Counseling) — Probability vs nonprobability sampling: stratified random sampling

☐ Approve ☐ Revise ☐ Retire Notes: _____

A researcher locates 48 published and unpublished studies of group counseling for adolescent depression and statistically combines their effect sizes to estimate an overall effect. What type of study is this?

A. A narrative literature review

A narrative review summarizes studies in words without statistically pooling their results.

B. A longitudinal cohort study

A cohort study follows one group of participants over time and collects new data.

C. A randomized controlled trial

An RCT randomly assigns its own participants to conditions rather than combining prior studies.

D. A meta-analysis ✓ KEY

Quantitatively pooling effect sizes across many studies to estimate an overall effect is meta-analysis.

Rationale: Meta-analysis converts the results of many studies to a common effect size metric (such as Cohen's d) and combines them, weighting larger studies more heavily. Including unpublished studies helps reduce publication bias. Meta-analyses of randomized trials are often placed at the top of evidence hierarchies used in evidence-based practice.

References: Erford (Research and Evaluation in Counseling) — Meta-analysis and effect sizes

☐ Approve ☐ Revise ☐ Retire Notes: _____

A college counseling center builds a logic model for a new peer-support program. Which entry belongs in the outcomes column of the model rather than in the outputs column?

A. Number of peer-support sessions held each semester
Sessions delivered are an output, a count of activity, not a change in participants.

B. Number of students who attended at least one session
Attendance counts are outputs that show reach, not program effects.

C. Lower loneliness scores among participating students ✓ KEY
A change in participants' loneliness is an outcome, the intended effect of the program.

D. Number of peer supporters who finished the training course
Trained supporters are a product of program activities, which makes this an output.

Rationale: A logic model links inputs (resources) to activities, outputs (direct products such as sessions held and people served), and outcomes (changes in knowledge, behavior or well-being). Process evaluation examines whether activities and outputs are delivered as planned; outcome evaluation examines whether the intended changes occurred.

References: Erford (Research and Evaluation in Counseling) — Program evaluation: logic models, process vs outcome evaluation

☐ Approve ☐ Revise ☐ Retire Notes:

The parents and teacher of an 8-year-old boy report frequent tantrums when classwork becomes frustrating. Which objective for his treatment plan is MOST measurable?

A. He will use his calm-down card instead of a tantrum in 4 of 5 hard tasks by March ✓ KEY
This names an observable replacement behavior, a success criterion (4 of 5 tasks) and a deadline.

B. He will gain better control of his emotions and feel happier at school this year
Emotional control and happiness are not defined in observable terms and lack a success criterion.

C. He will learn why tantrums upset classmates and grow more empathy toward them
Understanding and empathy are internal states with no observable measure or time frame.

D. He will meet with the school counselor every week to talk through angry feelings
Attending sessions describes the intervention, not a measurable change in the problem behavior.

Rationale: Objectives for children should describe a specific, observable behavior that teachers or parents can count, a criterion for success, and a target date. Framing the objective as a replacement behavior (using a coping tool) rather than only the absence of tantrums also gives the child something concrete to practice and adults something to reinforce.

References: Wiger (Documentation) — Treatment plans: observable, measurable objectives

► **Check:** Wiger reference detail describes the concept, not an exact chapter title.

☐ Approve ☐ Revise ☐ Retire Notes:

A 26-year-old man with schizophrenia was discharged from a psychiatric unit two weeks ago. He is taking his medication and has no suicidal or homicidal ideation, but he has missed several outpatient appointments and struggles to manage daily tasks on his own. Consistent with the least restrictive environment, which plan is MOST appropriate?

A. Readmit him to the inpatient unit until his attendance becomes consistent

He has no acute risk; hospitalizing him for missed appointments is far more restrictive than needed.

B. Add assertive community treatment or case management to support him at home ✓ **KEY**

Bringing intensive supports to him in the community addresses engagement and daily functioning in the least restrictive setting.

C. Arrange long-term placement in a locked residential facility for his safety

A locked setting is unjustified for a client who is stable and not dangerous to himself or others.

D. Continue monthly outpatient visits and close the case if he misses another

This ignores clear signs that his current level of support is not enough and risks disengagement.

Rationale: The least restrictive environment principle calls for providing care in the most integrated, least confining setting that can safely meet the client's needs. When a client is stable but struggling to engage and function, the first step is to intensify community-based supports, such as assertive community treatment or case management, rather than move him to a more restrictive setting.

References: Gladding (Counseling: A Comprehensive Profession) — Clinical mental health counseling: community-based care and the least restrictive setting

► **Check:** Gladding reference detail is a concept paraphrase; confirm a better-matched source if available.

☐ Approve ☐ Revise ☐ Retire Notes:

A 33-year-old client finishing treatment for alcohol use disorder is building a relapse prevention plan with her counselor. Based on the cognitive-behavioral relapse prevention model, which element is MOST central to the plan?

A. Agreeing that any single lapse means treatment failed and must start over

This all-or-nothing view feeds the abstinence violation effect, which turns lapses into full relapses.

B. Avoiding all talk of cravings so that the urges lose their power over time

Avoiding discussion of cravings leaves her unprepared; the model teaches recognizing and coping with urges.

C. Focusing the plan mainly on the past events that first led her to drink

Relapse prevention is present- and future-focused, centered on upcoming risks and skills.

D. Identifying her high-risk situations and rehearsing coping responses ✓ **KEY**

Spotting personal high-risk situations and practicing coping skills for them is the core of relapse prevention.

Rationale: Marlatt and Gordon's relapse prevention model holds that relapse risk rises when a person meets a high-risk situation without an effective coping response. Plans therefore identify triggers, build and rehearse coping skills, increase self-efficacy, and treat a lapse as a learning opportunity rather than a failure, countering the abstinence violation effect.

References: Corey (Theory & Practice) — Cognitive behavioral approaches: relapse prevention, high-risk situations and coping skills

► **Check:** Confirm the Corey edition in use covers relapse prevention; Marlatt & Gordon is the primary source but is not on the approved list.

☐ Approve ☐ Revise ☐ Retire Notes:

An interdisciplinary team at an adolescent residential program decides that clients who miss group will lose family phone calls for a week. The counselor on the team believes this decision raises ethical concerns. What should the counselor do FIRST?

A. Report the team to the state licensing board for unethical practice

Outside avenues are pursued only after attempts to resolve the concern within the team have failed.

B. Tell the clients' families privately that she disagrees with the team

Going around the team to families undermines the team and does not address the decision itself.

C. Raise the concern with the team and try to resolve it within the team ✓ **KEY**

The ACA Code directs counselors to first attempt to resolve ethical concerns about team decisions within the team.

D. Follow the decision without comment, since team consensus outranks her

Counselors keep their own ethical obligations on a team and may not simply defer to consensus.

Rationale: Counselors on interdisciplinary teams keep the focus on how best to serve clients and clarify the professional and ethical obligations of the team and its members. When a team decision raises ethical concerns, they first try to resolve the concern within the team; if that fails, they pursue other avenues consistent with client well-being.

References: ACA Code of Ethics — D.1.d Establishing Professional and Ethical Obligations

► **Check:** Confirm the section number D.1.d for resolving team ethical concerns within the team (vs D.1.c Interdisciplinary Teamwork).

☐ Approve ☐ Revise ☐ Retire Notes: _____

Counseling Skills and Interventions

120 items · NCE weight 30% (48 of 160 scored items)

249. nce-s-cou-001 · Counseling and Helping Relationships · psychodynamic and psychoanalytic theory · easy · In review

After a tense meeting in which she felt belittled by her supervisor, a client goes home and yells at her teenage son over a minor chore. Which defense mechanism does this BEST illustrate?

A. Projection

Projection attributes one's own unacceptable impulses to someone else; she is not accusing her son of her feelings.

B. Displacement ✓ **KEY**

Displacement redirects an impulse from a threatening target (the supervisor) to a safer one (the son).

C. Reaction formation

Reaction formation expresses the opposite of the true impulse, such as excessive friendliness toward the supervisor.

D. Sublimation

Sublimation channels an impulse into a socially valued outlet, such as vigorous exercise; yelling at her son is not that.

Rationale: In Freudian theory the ego uses defense mechanisms to reduce anxiety. Displacement shifts feelings away from the person who aroused them toward a less threatening target, as when anger at a boss is discharged at a family member.

References: Corey (Theory & Practice) — Psychoanalytic Therapy: ego-defense mechanisms (displacement)

☐ Approve ☐ Revise ☐ Retire Notes:

250. nce-s-cou-002 · Counseling and Helping Relationships · psychodynamic and psychoanalytic theory · medium · In review

Several months into treatment, a client begins reacting to the counselor's neutral comments with hurt and anger, saying, "You sound just like my father, always judging me." From a psychoanalytic perspective, the counselor should view this reaction as:

A. Countertransference that the counselor ought to disclose to the client

Countertransference is the counselor's own emotional reaction to the client; here the reaction is the client's.

B. Resistance that the counselor should confront firmly and directly

Analysts explore resistance rather than confront it firmly, and this reaction is better understood as transference.

C. A realistic response to the counselor's own critical style

The comments were neutral, so the client is reacting to a figure from the past rather than to the counselor's actual behavior.

D. Transference that offers material for analysis and interpretation ✓ **KEY**

The client is redirecting feelings about a significant past figure onto the counselor, which analysts treat as central therapeutic material.

Rationale: Transference occurs when a client unconsciously shifts feelings and attitudes from significant early figures onto the therapist. In psychoanalytic work, analyzing and interpreting transference helps clients gain insight into unresolved conflicts and how they repeat in current relationships.

References: Corey (Theory & Practice) — Psychoanalytic Therapy: transference and its interpretation

☐ Approve ☐ Revise ☐ Retire Notes:

An Adlerian counselor tells a client that his frequent helplessness seems to get his siblings to handle his responsibilities for him. Once the hidden payoff is named, the behavior becomes less appealing to the client. This technique is known as:

A. Spitting in the client's soup ✓ KEY

Exposing the purpose or payoff of a behavior "spoils" it, so the client can no longer use it as innocently as before.

B. Acting as if

Acting as if asks the client to behave as though already the person he wishes to be; it does not expose a payoff.

C. Catching oneself

Catching oneself has the client notice self-defeating behavior in the moment; here the counselor names its purpose.

D. The push-button technique

The push-button technique has clients alternate pleasant and unpleasant images to see that they create their feelings.

Rationale: Adlerian therapy assumes behavior is purposeful. "Spitting in the client's soup" means pointing out the hidden goal or payoff of a self-defeating behavior; the client may continue it, but it no longer tastes as good.

References: Corey (Theory & Practice) — Adlerian Therapy: techniques of the reorientation phase

☐ Approve ☐ Revise ☐ Retire Notes:

In Adler's view of psychological birth order, which child is MOST associated with the experience of being "dethroned" when a new sibling arrives?

A. The second-born child

Adler described the second child as racing to catch up with the older sibling, not as dethroned.

B. The youngest child

The youngest is described as the "baby" who may be pampered; no later sibling displaces them.

C. The oldest child ✓ KEY

The firstborn enjoys undivided attention until a sibling is born and must then adjust to losing that favored position.

D. The only child

An only child is never displaced by a sibling, although Adler noted they may struggle with sharing.

Rationale: Adler emphasized psychological (not merely ordinal) birth position within the family constellation. The oldest child is initially the center of attention and is "dethroned" by a sibling's birth, which can shape a lifestyle focused on responsibility, achievement, or regaining status.

References: Corey (Theory & Practice) — Adlerian Therapy: birth order and the family constellation

☐ Approve ☐ Revise ☐ Retire Notes:

Carl Rogers proposed that clients move toward growth when the counselor offers congruence, accurate empathic understanding, and which other core condition?

A. Interpretation of defenses

Interpreting defenses is a psychoanalytic technique, not a Rogerian core condition.

B. Structured homework assignments

Homework is typical of cognitive and behavioral approaches, not person-centered therapy.

C. Unconditional positive regard ✓ KEY

Rogers named unconditional positive regard, nonjudgmental acceptance of the client, as a core condition.

D. Confrontation of irrational beliefs

Disputing irrational beliefs is central to REBT, not to the person-centered approach.

Rationale: Rogers held that congruence (genuineness), unconditional positive regard, and accurate empathic understanding are necessary and sufficient conditions for therapeutic change. The counselor's attitudes, rather than techniques, release the client's actualizing tendency.

References: Corey (Theory & Practice) — Person-Centered Therapy: core therapeutic conditions

☐ Approve ☐ Revise ☐ Retire Notes:

A college student tells her counselor she feels worthwhile only when she earns top grades, the one achievement her parents consistently praised while growing up. From a person-centered perspective, her distress MOST likely stems from:

A. Internalized conditions of worth that distort her self-concept ✓ KEY

Rogers held that conditional acceptance from significant others leads people to value themselves only when meeting those conditions, creating incongruence.

B. An unresolved conflict between her id impulses and her superego

Id-superego conflict is a psychoanalytic formulation, not a person-centered one.

C. Irrational demands that she must perform well to be acceptable

Absolutistic "musts" are the REBT explanation of disturbance, not Rogers's.

D. A mistaken private logic formed in her early family constellation

Private logic and the family constellation are Adlerian concepts.

Rationale: Rogers described how children who receive only conditional positive regard introject conditions of worth. They then deny or distort experiences that do not fit, producing incongruence between self-concept and experience, which shows up as anxiety and vulnerability.

References: Corey (Theory & Practice) — Person-Centered Therapy: conditions of worth and incongruence

☐ Approve ☐ Revise ☐ Retire Notes:

Which theorist founded logotherapy, drawing on his experiences in Nazi concentration camps to argue that people can find meaning even in unavoidable suffering?

A. Rollo May

May was an American existential psychologist known for his work on anxiety, freedom, and love, not logotherapy.

B. Irvin Yalom

Yalom described the existential givens and group therapeutic factors but did not found logotherapy.

C. Fritz Perls

Perls founded gestalt therapy.

D. Viktor Frankl ✓ **KEY**

Frankl developed logotherapy, centered on the will to meaning, and wrote Man's Search for Meaning.

Rationale: Viktor Frankl's logotherapy holds that the primary human motivation is the will to meaning. Meaning can be found through creative work, experiences and relationships, and the attitude one takes toward unavoidable suffering.

References: Corey (Theory & Practice) — Existential Therapy: Frankl and the search for meaning

☐ Approve ☐ Revise ☐ Retire Notes:

Irvin Yalom described four "ultimate concerns," or givens of existence, that are central to existential psychotherapy. Which set lists them?

A. Anxiety, guilt, shame, and existential despair

These are emotional states that may arise from facing the givens, not the givens themselves.

B. Death, freedom, isolation, and meaninglessness ✓ **KEY**

Yalom identified death, freedom (and responsibility), existential isolation, and meaninglessness as the ultimate concerns.

C. Survival, belonging, power, freedom, and fun

These are Glasser's five basic needs from choice theory.

D. Safety, love, esteem, and self-actualization

These resemble levels of Maslow's hierarchy of needs.

Rationale: Yalom organized existential therapy around four givens: death, freedom, existential isolation, and meaninglessness. Anxiety arises when people confront these givens, and therapy helps clients face them authentically rather than through defensive avoidance.

References: Corey (Theory & Practice) — Existential Therapy: Yalom's ultimate concerns

☐ Approve ☐ Revise ☐ Retire Notes:

A client grieving her late father says she never told him how angry she was about his drinking. The counselor invites her to imagine her father sitting in an empty chair and to speak to him directly, in the present tense. This intervention is MOST characteristic of:

A. Gestalt therapy ✓ KEY

The empty chair technique is a hallmark gestalt experiment for working through unfinished business in the here and now.

B. Rational emotive behavior therapy

REBT disputes irrational beliefs; it does not rely on enactment with an empty chair.

C. Reality therapy

Reality therapy focuses on current behavior and the WDEP system, not on enacted dialogues.

D. Solution-focused brief therapy

SFBT emphasizes exceptions and preferred futures rather than expressing unfinished feelings.

Rationale: Gestalt therapy, associated with Fritz and Laura Perls, uses experiments such as the empty chair to bring unfinished business into present awareness. Speaking directly to the imagined person helps the client experience and complete blocked feelings.

References: Corey (Theory & Practice) — Gestalt Therapy: unfinished business and the empty-chair technique

☐ Approve ☐ Revise ☐ Retire Notes:

When a coworker criticizes him, a client reports that he wants to shout back but instead clenches his jaw and digs his fingernails into his own palms. In gestalt terms, this pattern of turning back onto oneself what one would like to do to others is called:

A. Introjection

Introjection is uncritically swallowing others' values or beliefs without assimilating them.

B. Confluence

Confluence is blurring the boundary between self and environment so that differences are not experienced.

C. Retroflexion ✓ KEY

Retroflexion is doing to oneself what one wants to do to someone else, such as turning anger inward.

D. Deflection

Deflection is avoiding contact through distraction, humor, or vagueness.

Rationale: Gestalt therapy describes several contact-boundary disturbances: introjection, projection, retroflexion, deflection, and confluence. Retroflexion turns an impulse meant for the environment back against the self, often seen in self-harm, muscle tension, or self-blame.

References: Corey (Theory & Practice) — Gestalt Therapy: contact and resistances to contact

☐ Approve ☐ Revise ☐ Retire Notes:

Joseph Wolpe's systematic desensitization pairs deep relaxation with gradual imagined exposure to a fear hierarchy. The technique is based on which principle?

A. Operant extinction

Operant extinction removes reinforcement for a behavior; desensitization targets a conditioned anxiety response.

B. Reciprocal inhibition ✓ KEY

Wolpe reasoned that a response incompatible with anxiety, such as relaxation, inhibits anxiety when paired with the feared stimulus.

C. Negative reinforcement

Negative reinforcement increases behavior by removing an aversive stimulus; it is not the basis of the technique.

D. Vicarious learning

Vicarious learning through modeling comes from Bandura's social learning theory.

Rationale: Systematic desensitization is a classical-conditioning procedure. The client learns relaxation, builds an anxiety hierarchy, and then imagines each step while relaxed. Because relaxation and anxiety are incompatible, the relaxed state reciprocally inhibits the fear response.

References: Corey (Theory & Practice) — Behavior Therapy: systematic desensitization and reciprocal inhibition

☐ Approve ☐ Revise ☐ Retire Notes:

To reduce a 7-year-old's hitting, his parents remove his tablet for the evening each time he hits his sister, and the hitting decreases. Which operant procedure are they using?

A. Positive reinforcement

Positive reinforcement adds a desirable stimulus to increase a behavior.

B. Negative reinforcement

Negative reinforcement removes an aversive stimulus to increase a behavior.

C. Positive punishment

Positive punishment adds an aversive stimulus, such as extra chores, to decrease a behavior.

D. Negative punishment ✓ KEY

Removing a desired item (the tablet) to decrease a behavior is negative punishment.

Rationale: In operant conditioning, "positive" means adding and "negative" means removing a stimulus; reinforcement increases behavior and punishment decreases it. Taking away a privilege to reduce hitting is therefore negative punishment (sometimes called response cost).

References: Corey (Theory & Practice) — Behavior Therapy: operant conditioning techniques

☐ Approve ☐ Revise ☐ Retire Notes:

Aaron Beck's cognitive triad of depression describes a negative view of which three areas?

A. The self, others, and the past

The triad concerns the future, not the past, and the world rather than others alone.

B. Thoughts, feelings, and behaviors

This is the general CBT model of interacting domains, not the cognitive triad.

C. The past, present, and future

The triad is not a time sequence; it includes the self and the world.

D. The self, the world, and the future ✓ KEY

Beck described depressed clients as viewing themselves, their world, and their future negatively.

Rationale: Beck proposed that depression is maintained by a negative cognitive triad: "I am inadequate" (self), "Everything goes wrong" (world), and "It will never get better" (future). Cognitive therapy helps clients test these appraisals against evidence.

References: Corey (Theory & Practice) — Cognitive Behavior Therapy: Beck's cognitive triad

☐ Approve

☐ Revise

☐ Retire

Notes:

A mother tells her counselor that her son's soccer team lost the championship because she had to miss the game for work. In Beck's cognitive therapy, this is an example of which cognitive distortion?

A. Magnification

Magnification exaggerates the importance of an event rather than wrongly taking responsibility for it.

B. Personalization ✓ KEY

She attributes an external event to herself without a basis for making the connection.

C. Dichotomous thinking

Dichotomous thinking sorts experiences into all-or-nothing categories.

D. Selective abstraction

Selective abstraction draws conclusions from one detail while ignoring the larger context.

Rationale: Personalization is the tendency to relate external events to oneself when there is no basis for doing so. Beck catalogued distortions such as arbitrary inference, selective abstraction, overgeneralization, magnification and minimization, personalization, and dichotomous thinking.

References: Corey (Theory & Practice) — Cognitive Behavior Therapy: Beck's systematic errors in reasoning

☐ Approve

☐ Revise

☐ Retire

Notes:

In Albert Ellis's ABCDE model of rational emotive behavior therapy, what does the "D" represent?

A. Describing the activating event

The activating event is the "A" in the model.

B. Developing new behaviors

New behaviors may follow, but "D" is a cognitive step, not behavior development.

C. Disputing irrational beliefs ✓ KEY

D is disputing, the active challenge of the irrational beliefs ("B") that cause disturbed consequences ("C").

D. Defining emotional consequences

Emotional and behavioral consequences are the "C" in the model.

Rationale: Ellis's model holds that an Activating event does not directly cause emotional Consequences; Beliefs about the event do. The counselor Disputes irrational beliefs, producing an Effective new philosophy (E) and healthier feelings.

References: Corey (Theory & Practice) — Rational Emotive Behavior Therapy: the A-B-C framework

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, "I must get this promotion. If I don't, it will be awful, I won't be able to stand it, and it will prove I'm worthless." According to Ellis, which element is the CORE of this irrational belief, from which the others derive?

A. Demandingness, or absolutistic "musts" ✓ KEY

Ellis held that rigid demands ("I must") are primary, and awfulizing, low frustration tolerance, and self-downing derive from them.

B. Catastrophic misinterpretation

Catastrophic misinterpretation of bodily sensations is a cognitive model of panic, not Ellis's core belief.

C. Internalized conditions of worth

Conditions of worth are a person-centered concept.

D. Introjected parental messages

Introjection is a gestalt contact disturbance, not the REBT core of irrationality.

Rationale: REBT identifies demandingness ("must," "should," "ought" held as absolutes) as the core of irrational thinking. Awfulizing, "I-can't-stand-it-itis" (low frustration tolerance), and global self-rating are derivatives that follow from rigid demands.

References: Corey (Theory & Practice) — Rational Emotive Behavior Therapy: musturbation and derivative beliefs

☐ Approve ☐ Revise ☐ Retire Notes:

Reality therapy is often taught through the WDEP system (Wants, Doing and direction, Evaluation, Planning). Who developed and expanded this system?

A. Robert Wubbolding ✓ **KEY**

Wubbolding developed the WDEP system as a practical framework for applying Glasser's reality therapy.

B. Albert Ellis

Ellis founded REBT and the ABCDE model.

C. Steve de Shazer

De Shazer co-developed solution-focused brief therapy.

D. Michael White

White co-developed narrative therapy.

Rationale: William Glasser founded reality therapy and choice theory; Robert Wubbolding extended it with the WDEP system. Counselors explore what clients want, what they are doing, whether it is working, and a simple, attainable plan.

References: Corey (Theory & Practice) — Reality Therapy: the WDEP system

☐ Approve ☐ Revise ☐ Retire Notes:

A high school student says he skips class because "nobody there likes me anyway." After exploring what he wants and what he is currently doing, the reality therapist asks, "Is skipping class helping you get the friendships you want?" This question reflects which part of WDEP?

A. Wants

Wants were explored earlier; this question judges whether behavior serves those wants.

B. Doing and direction

Doing describes current behavior; it does not ask the client to judge its effectiveness.

C. Evaluation ✓ **KEY**

Asking whether current behavior is getting the client what he wants is self-evaluation, the core of reality therapy.

D. Planning

Planning comes after evaluation and creates a specific, attainable course of action.

Rationale: In Wubbolding's WDEP system, Evaluation asks clients to judge whether their total behavior is meeting their needs and wants. Glasser's five basic needs are survival, love and belonging, power, freedom, and fun; here the student is pursuing belonging ineffectively.

References: Corey (Theory & Practice) — Reality Therapy: self-evaluation in the WDEP system

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor asks a client, "Suppose that tonight, while you sleep, the problem that brought you here is solved. When you wake up, what will be the first thing you notice that tells you things are different?" This is an example of:

A. An exception question

Exception questions ask about past times the problem was absent or less severe.

B. The miracle question ✓ KEY

Imagining the problem gone overnight and describing the difference is the classic miracle question.

C. A scaling question

Scaling questions ask clients to rate progress or confidence on a numerical scale.

D. A coping question

Coping questions ask how the client has managed to keep going despite difficulties.

Rationale: The miracle question, developed within solution-focused brief therapy by Steve de Shazer and Insoo Kim Berg, invites clients to describe a preferred future in concrete, observable terms. Those details then become the basis for goals and small next steps.

References: Corey (Theory & Practice) — Postmodern Approaches: solution-focused brief therapy, the miracle question

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor says, "On a scale from 0 to 10, where 10 means the problem is fully resolved, you said you are at a 4. What would tell you that you had moved to a 5?" This technique and the approach it comes from are MOST closely associated with:

A. Michael White and David Epston

White and Epston developed narrative therapy, known for externalizing conversations.

B. Jay Haley and Cloé Madanes

Haley and Madanes are associated with strategic family therapy and directives.

C. William Miller and Stephen Rollnick

Miller and Rollnick developed motivational interviewing; MI uses importance rulers, but this framing is SFBT scaling.

D. Steve de Shazer and Insoo Kim Berg ✓ KEY

Scaling questions focused on small next steps are a signature solution-focused technique from de Shazer and Berg.

Rationale: Solution-focused brief therapy, developed by Steve de Shazer and Insoo Kim Berg, uses scaling questions to make progress concrete and to identify the next small, observable step. The focus is on what is already working rather than on the problem's history.

References: Corey (Theory & Practice) — Postmodern Approaches: solution-focused brief therapy, scaling questions

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor working with a 10-year-old who refuses to go to school asks, "When does the Worry show up, and how does it try to convince you to stay home?" The counselor is using which technique?

A. Reframing

Reframing offers a new meaning for a behavior; it does not separate the problem from the person.

B. Paradoxical intention

Paradoxical intention asks the client to deliberately do or wish for the feared behavior.

C. Externalizing the problem ✓ KEY

Naming the Worry as a separate entity that acts on the child externalizes the problem.

D. Cognitive restructuring

Cognitive restructuring identifies and replaces distorted thoughts rather than personifying the problem.

Rationale: Narrative therapy, developed by Michael White and David Epston, holds that "the person is not the problem; the problem is the problem." Externalizing conversations separate clients from problem-saturated identities so they can examine and resist the problem's influence.

References: Corey (Theory & Practice) — Postmodern Approaches: narrative therapy, externalizing conversations

☐ Approve ☐ Revise ☐ Retire Notes:

A narrative counselor learns that a client who describes himself as "a hopeless addict" turned down a drink at a family wedding last month. The counselor explores this event in detail to begin building an alternative story. In narrative therapy, such events are called:

A. Unique outcomes ✓ KEY

White used "unique outcomes" for moments that contradict the dominant problem story and seed re-authoring.

B. Exceptions

"Exceptions" is the solution-focused term for times the problem is absent; the question asks for narrative terminology.

C. Peak experiences

Peak experiences are Maslow's term for moments of intense fulfillment.

D. Corrective emotional experiences

This psychodynamic term refers to re-experiencing old conflicts with a new, healthier outcome in therapy.

Rationale: Narrative therapists listen for unique outcomes: times when the client acted outside the problem-saturated story. Thickening these accounts helps re-author an alternative, preferred identity story.

References: Corey (Theory & Practice) — Postmodern Approaches: narrative therapy, unique outcomes and re-authoring

► **Check:** 'Exceptions' (SFBT) sits beside the key 'unique outcomes' (narrative); confirm one best answer reads clearly.

☐ Approve ☐ Revise ☐ Retire Notes:

Which statement BEST reflects a central principle of feminist therapy?

A. Distress mainly reflects unresolved intrapsychic conflict

Feminist therapy looks beyond intrapsychic causes to social, political, and cultural context.

B. The personal is political, so context shapes distress ✓ **KEY**

"The personal is political" holds that individual problems are linked to social structures and oppression.

C. The counselor holds expert authority to direct the client

Feminist therapy emphasizes an egalitarian relationship that minimizes power differences.

D. A diagnosis is required before exploring social context

Feminist therapists are cautious about diagnostic labels and prioritize the client's context.

Rationale: Feminist therapy rests on principles such as "the personal is political," an egalitarian counseling relationship, valuing women's experiences, and a commitment to social change. Symptoms are often reframed as understandable responses to oppressive conditions.

References: Corey (Theory & Practice) — Feminist Therapy: key principles

☐ Approve

☐ Revise

☐ Retire

Notes:

A counselor helps a client identify the messages she received growing up about what women "should" be, where those messages came from, and how they influence her guilt about setting limits with family today. This feminist intervention is called:

A. Lifestyle assessment

Lifestyle assessment is an Adlerian technique exploring family constellation and early recollections.

B. Genogram construction

A genogram maps multigenerational family patterns, a Bowenian tool.

C. Cognitive restructuring

Cognitive restructuring targets distorted thoughts without centering gender socialization.

D. Gender-role analysis ✓ **KEY**

Gender-role analysis examines the impact of gender-role expectations on a client's well-being.

Rationale: Gender-role analysis helps clients recognize gender-role messages, their sources, and their consequences, and then decide which to keep or change. Along with power analysis, it is a signature feminist therapy intervention.

References: Corey (Theory & Practice) — Feminist Therapy: gender-role analysis and intervention

☐ Approve

☐ Revise

☐ Retire

Notes:

In motivational interviewing, the core skills are summarized by the acronym OARS: open questions, affirmations, reflections, and what?

A. Solutions

MI avoids prematurely supplying solutions; the client voices reasons for change.

B. Summaries ✓ KEY

Summaries collect and link what the client has said, often highlighting change talk.

C. Silence

Silence can be useful, but it is not part of the OARS acronym.

D. Self-disclosure

Self-disclosure is not one of the OARS core skills.

Rationale: Miller and Rollnick describe OARS (open questions, affirming, reflecting, summarizing) as the foundational communication skills of MI. These skills support engaging, focusing, evoking, and planning.

References: Miller & Rollnick (MI) — Core interviewing skills: OARS

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, "I know I should cut back on drinking, but it's the only thing that helps me relax after work." Which counselor response is MOST consistent with motivational interviewing?

A. "You need to understand that drinking is harming your health."

Unsolicited warnings reflect the righting reflex and tend to elicit sustain talk.

B. "Why haven't you been able to cut back on your drinking yet?"

Asking why the client has not changed invites defense of the status quo.

C. "It sounds like you are in denial about how serious this is."

Labeling and confronting denial increases discord, which MI avoids.

D. "It helps you unwind, and you also see reasons to cut back." ✓ KEY

A double-sided reflection acknowledges both sides of ambivalence and ends on the change side.

Rationale: MI treats ambivalence as normal. A double-sided reflection captures both sustain talk and change talk, typically ending with the change side, so the client hears his own reasons for change without feeling pushed. Confrontation and advice-giving tend to strengthen sustain talk.

References: Miller & Rollnick (MI) — Responding to sustain talk and discord: double-sided reflections

☐ Approve ☐ Revise ☐ Retire Notes:

A DBT skills group is teaching TIPP skills for crisis survival and practicing radical acceptance of painful realities that cannot be changed right now. These skills belong to which DBT module?

A. Distress tolerance ✓ KEY

Crisis survival skills such as TIPP and reality-acceptance skills such as radical acceptance are taught in distress tolerance.

B. Emotion regulation

Emotion regulation focuses on understanding emotions, reducing vulnerability, and changing unwanted emotions.

C. Interpersonal effectiveness

Interpersonal effectiveness teaches asking for what one needs and saying no (for example, DEAR MAN).

D. Core mindfulness

Mindfulness teaches observing, describing, and participating nonjudgmentally (wise mind).

Rationale: Marsha Linehan's DBT skills training has four modules: mindfulness, distress tolerance, emotion regulation, and interpersonal effectiveness. Distress tolerance helps clients survive crises without making things worse and accept reality as it is.

References: Corey (Theory & Practice) — Behavior Therapy: dialectical behavior therapy skills modules

► **Check:** Corey's placement of DBT (third-wave section of Behavior Therapy) may differ by edition.

☐ Approve ☐ Revise ☐ Retire Notes:

Marsha Linehan developed dialectical behavior therapy initially for chronically suicidal clients. The central dialectic of the approach is the balance between:

A. Insight and catharsis

Insight and catharsis are associated with psychodynamic and experiential approaches.

B. Structure and spontaneity

This is not the organizing dialectic of DBT.

C. Acceptance and change ✓ KEY

DBT balances validation and acceptance of the client with behavioral change strategies.

D. Thinking and feeling

DBT integrates reason and emotion in "wise mind," but its central dialectic is acceptance and change.

Rationale: DBT integrates cognitive-behavioral change strategies with acceptance practices drawn from Zen and mindfulness. Linehan developed it for chronically suicidal clients, many meeting criteria for borderline personality disorder, who experienced pure change-focused therapy as invalidating.

References: Corey (Theory & Practice) — Behavior Therapy: dialectical behavior therapy, acceptance and change

► **Check:** Corey's placement of DBT (third-wave section of Behavior Therapy) may differ by edition.

☐ Approve ☐ Revise ☐ Retire Notes:

Two parents in a tense marriage each confide in their 15-year-old daughter about their frustrations with the other, and the daughter now feels responsible for keeping the peace. A Bowenian family counselor would MOST likely describe this pattern as:

A. Triangulation ✓ KEY

Bowen described how anxious two-person systems draw in a third person to stabilize tension, forming a triangle.

B. Emotional cutoff

Emotional cutoff is managing unresolved fusion by distancing from family, the opposite of being drawn in.

C. Sibling position

Sibling position concerns how birth order shapes functioning, not parental conflict.

D. Societal regression

Societal regression applies Bowen's concepts to society as a whole.

Rationale: In Bowen family systems theory, the triangle is the smallest stable relationship unit; dyads under stress pull in a third person. Bowenian counselors help members increase differentiation of self, often using genograms to trace multigenerational patterns, so they can stay connected without being triangled.

References: Corey (Theory & Practice) — Family Systems Therapy: Bowen's triangulation and differentiation of self

☐ Approve ☐ Revise ☐ Retire Notes:

Parents tell a family counselor they always agree on rules. The counselor asks them to decide, right now in session, whether their son may stay out late Saturday, then watches as the son interrupts and his mother sides with him. In Minuchin's structural family therapy, this technique is called:

A. Prescribing the symptom

Prescribing the symptom is a paradoxical directive from strategic therapy (Haley, Madanes).

B. Enactment ✓ KEY

Enactment has the family act out a transaction in session so the counselor can observe and modify its structure.

C. Detriangulation

Detriangulation is a Bowenian goal of removing a third person from a two-person conflict.

D. Unbalancing

Unbalancing is a structural technique in which the counselor temporarily sides with one member to shift power; that is not what is described.

Rationale: Minuchin's structural family therapy focuses on subsystems, boundaries, and hierarchy. In an enactment, the counselor has family members interact in session rather than report on their interactions, revealing patterns such as a cross-generational coalition that the counselor can then help restructure.

References: Corey (Theory & Practice) — Family Systems Therapy: structural family therapy, enactment

► **Check:** 'Unbalancing' is also a Minuchin technique; confirm the scenario reads clearly as an enactment.

☐ Approve ☐ Revise ☐ Retire Notes:

In the third session of a community anger-management group, members begin arguing about the attendance rule, and two members openly question whether the leader is qualified to run the group. According to Tuckman's model, the group is MOST likely in which stage?

A. Forming

Forming is the orientation stage of politeness, dependence on the leader and tentative participation, before open conflict emerges.

B. Storming ✓ KEY

Storming is marked by conflict, challenges to the leader and struggles over control and rules as members test the group.

C. Norming

Norming follows storming; members have settled conflicts, agreed on norms and developed cohesion.

D. Performing

Performing is the productive stage in which the group works on its goals with little energy spent on conflict.

Rationale: Tuckman (1965; adjourning added with Jensen in 1977) described forming, storming, norming, performing and adjourning. Conflict over rules and direct challenges to the leader's authority are the hallmark of storming, which leaders should treat as a normal developmental step rather than a sign the group is failing.

References: Gladding (Counseling: A Comprehensive Profession) — Group work: Tuckman stages of group development

☐ Approve ☐ Revise ☐ Retire Notes:

During the transition stage of a counseling group, a member crosses his arms and says to the leader, 'Honestly, this group is a waste of my time.' Other members fall silent. Which leader response BEST fits Corey's guidance for this stage?

A. Remind him that the group rules ask members to stay positive and to avoid discouraging the others in the room.

Invoking a rule to suppress the comment shuts down a challenge the group needs to work through and models avoidance of conflict.

B. Move to a structured exercise so the silence ends quickly and the group can return to the planned topic of the day.

Changing the subject bypasses the conflict; unaddressed resistance in the transition stage tends to go underground and erode trust.

C. Ask the other members whether they agree with him so that the group can vote on whether to keep meeting each week.

Polling members turns a personal reaction into a referendum, can isolate the member and puts the group itself on trial.

D. Respond without defensiveness and invite him to say more about what isn't working for him in the group so far.

✓ KEY

Corey advises leaders to meet challenges openly and non-defensively, treating resistance as useful information and modeling direct, respectful handling of conflict.

Rationale: In Corey's model the transition stage is characterized by anxiety, defensiveness, resistance and struggles for control, including challenges to the leader. The leader's task is to recognize and explore these reactions directly and non-defensively, which builds the trust needed to move into the working stage.

References: Corey (Theory & Practice) — Group stages: transition stage, dealing with resistance and challenges to the leader

☐ Approve ☐ Revise ☐ Retire Notes:

A high school counselor plans a six-week group in which students learn and practice specific study and test-anxiety management skills, with brief lessons and structured activities each week. Under the ASGW classification of group work, this is BEST described as which type of group?

A. Psychoeducation group ✓ KEY

Psychoeducation groups emphasize teaching information and skills, often to prevent problems, using structured content and activities.

B. Task group

Task and work groups focus on accomplishing an organizational goal, such as a committee or planning team, rather than member learning.

C. Counseling group

Counseling groups address interpersonal and developmental concerns through member interaction, with less didactic structure.

D. Psychotherapy group

Psychotherapy groups address more severe or chronic problems and personality reconstruction, which this skills group does not target.

Rationale: The ASGW identifies four group types: task/work, psychoeducation, counseling and psychotherapy. A time-limited, structured group whose main aim is teaching skills to a nonclinical population is a psychoeducation group.

References: Gladding (Counseling: A Comprehensive Profession) — Group work: ASGW types of groups

☐ Approve ☐ Revise ☐ Retire Notes:

After a member of a grief group shares that she still sets a place at the table for her late husband, three other members say they have done something similar. She replies, 'I thought I was the only one.' Which of Yalom's therapeutic factors is MOST evident?

A. Instillation of hope

Instillation of hope comes from seeing others improve or cope well, not from discovering that others share the same experience.

B. Altruism

Altruism is the benefit of helping others; the member here is the one receiving relief rather than giving help.

C. Universality ✓ KEY

Universality is the relief of learning that one is not alone or unique in one's problems, thoughts or feelings.

D. Catharsis

Catharsis is the release of strong emotion; the key experience described is recognizing shared experience.

Rationale: Yalom identified 11 therapeutic factors. Universality, the discovery that others share one's 'secret' struggles, often appears early in groups and reduces shame and isolation, which is why the member's statement that she thought she was the only one points to it.

References: Yalom & Leszcz (Group Psychotherapy) — Ch. 1: The therapeutic factors, universality

☐ Approve ☐ Revise ☐ Retire Notes:

In a young adult counseling group, two members repeatedly press a quieter member to 'finally tell us what really happened' with her family, even after she says she is not ready. Which leadership skill is MOST needed at this moment?

A. Linking, by connecting her reluctance to similar feelings that others in the group have described

Linking builds connection among members but does not stop the pressure being placed on her right now.

B. Drawing out, by inviting the quiet member to share a little more so the others feel heard

Drawing her out would join the pressure; she has already stated a boundary about disclosure.

C. Blocking, by stopping the pressure and affirming her right to decide what and when to share ✓ **KEY**

Blocking protects members from counterproductive behavior such as coercive questioning while respecting their autonomy over disclosure.

D. Summarizing, by restating the main points of the discussion before moving to another member

Summarizing may move the group along but does not address the coercion or protect the member.

Rationale: Group leaders have a duty to protect members from undue pressure and from behaviors that threaten safety. Blocking intervenes in harmful patterns, such as pressuring, gossiping or breaking confidences, without attacking the members doing it.

References: Corey (Theory & Practice) — Group leadership skills: blocking and protecting members · ACA Code of Ethics — A.9.b Protecting Clients (group work)

☐ Approve ☐ Revise ☐ Retire Notes:

In a long-term interpersonal group, a man reacts to the male co-leader with the same deference and hidden resentment he felt toward his father, and competes with an older member as he did with his brother. Over months he recognizes the pattern and responds to them differently. Which of Yalom's therapeutic factors does this process BEST illustrate?

A. Corrective recapitulation of the primary family group ✓ **KEY**

The group re-creates early family dynamics, and the member relives and then reworks those patterns in a new, corrective way.

B. Imitative behavior modeled on the co-leader and members

Imitative behavior means adopting behaviors seen in others; the core change here is reworking old family-based roles.

C. Socializing techniques learned through group role-plays

Socializing techniques are basic social skills learned in the group, not the reworking of family-of-origin patterns.

D. Existential factors, such as accepting final responsibility

Existential factors involve accepting responsibility, mortality and life's unfairness, not reliving family roles.

Rationale: Yalom describes groups as resembling a family in many ways, so members often relate to leaders and peers as they did to parents and siblings. When those patterns are not merely relived but explored and corrected, the factor is corrective recapitulation of the primary family group.

References: Yalom & Leszcz (Group Psychotherapy) — Ch. 1: Corrective recapitulation of the primary family group

☐ Approve ☐ Revise ☐ Retire Notes:

In Egan's SOLER guideline for nonverbal attending, what does the letter 'L' stand for?

A. Listen without interrupting

Uninterrupted listening matters, but SOLER describes physical posture, and L is not listening.

B. Look at the client's face

Eye contact is the E in SOLER, not the L.

C. Limit note-taking in session

Note-taking is not one of the SOLER behaviors.

D. Lean slightly toward the client ✓ KEY

L stands for leaning toward the client, a posture that signals involvement and interest.

Rationale: SOLER stands for Squarely face the client, Open posture, Lean toward the client, Eye contact and Relaxed manner. Egan presents these as flexible guidelines; the counselor adapts them to the client's culture and comfort, since direct eye contact or close posture is not welcome in every context.

References: Gladding (Counseling: A Comprehensive Profession) — Building the counseling relationship: attending behavior (SOLER)

☐ Approve

☐ Revise

☐ Retire

Notes:

286. nce-s-cou-038 · Counseling and Helping Relationships · basic attending and listening skills · medium · In review

A practicum student is eager to start using confrontation and interpretation with clients. According to Ivey's microskills hierarchy, which skills should the student master FIRST?

A. Reflection of meaning and the interpretation of core themes

Reflection of meaning and interpretation sit higher in the hierarchy and rely on the listening foundation.

B. Attending behavior and the basic listening sequence ✓ KEY

Attending and the basic listening sequence are the foundation on which higher-level influencing skills are built.

C. Influencing skills such as directives and feedback

Influencing skills are later in the hierarchy and are effective only after the client feels heard.

D. Skill integration and a personal style of counseling

Skill integration and personal style form the top of the hierarchy, reached after the other skills are mastered.

Rationale: Ivey's hierarchy rests on ethics, multicultural competence and wellness, then attending behavior, then the basic listening sequence (questions, observation, encouraging, paraphrasing, summarizing, reflecting feeling). Focusing, empathic confrontation, reflection of meaning, influencing skills and skill integration build on that base.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: Ivey microskills hierarchy

► **Check:** Ivey's editions differ slightly on the hierarchy's base; the key holds across editions.

☐ Approve

☐ Revise

☐ Retire

Notes:

A client says, 'My supervisor criticized my report in front of the whole team again, and I just sat there and said nothing.' Which counselor response is the BEST example of a reflection of feeling?

A. You felt embarrassed, and maybe angry with yourself for not speaking up. ✓ KEY

This names the likely emotions behind the statement, which is the defining feature of reflecting feeling.

B. Your supervisor criticized your report in front of the team once again.

This restates the content of the message, which makes it a paraphrase rather than a reflection of feeling.

C. What do you think stopped you from saying something to your supervisor?

This is an open question that explores reasons; it does not reflect the emotion being expressed.

D. Next time, it might help to ask your supervisor to talk with you privately.

This is advice-giving, an influencing response that skips past the client's feelings.

Rationale: A reflection of feeling identifies and names the emotion in the client's message, often tentatively, so the client feels understood and can explore it further. A paraphrase captures content; questions and advice are different skills.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: reflection of feeling versus paraphrasing

☐ Approve ☐ Revise ☐ Retire Notes:

Midway through a session, a client has moved quickly among worries about money, conflict with her sister and trouble sleeping. The counselor wants to pull these threads together, check accuracy and help the client choose a focus. Which skill is MOST suited to this purpose?

A. Paraphrasing

Paraphrasing restates the content of a single statement rather than tying together several themes across the session.

B. Reflection of meaning

Reflection of meaning explores values and deeper significance, not the organization of several topics into a focus.

C. Summarizing ✓ KEY

Summarizing condenses several statements or themes, checks understanding and helps set direction or transition.

D. Encouraging

Minimal encouragers keep the client talking; they do not integrate content across topics.

Rationale: Summarizing draws together the main content and feelings from a longer stretch of talk. It is used to begin or end sessions, to make transitions and to help a client who has raised many concerns decide what to work on.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: summarizing

☐ Approve ☐ Revise ☐ Retire Notes:

A client who is tearful and twisting a tissue says, 'I'm completely fine with the divorce. It's no big deal.' Which response BEST illustrates empathic confrontation?

A. You aren't being honest with me or yourself, because you are clearly upset right now.

This accuses and judges the client, which is likely to raise defensiveness rather than invite exploration.

B. Divorce is one of the most stressful life events, so being upset would be quite normal.

This normalizes and reassures, but it does not name the discrepancy between the words and the behavior.

C. Why do you think you are telling me it is no big deal when it seems to matter a lot?

"Why" questions tend to put clients on the defensive and ask for justification rather than reflection.

D. You're saying the divorce is no big deal, and you're also in tears as we talk about it. ✓ **KEY**

This nonjudgmentally places the verbal message beside the nonverbal one and lets the client explore the mixed message.

Rationale: Empathic confrontation identifies discrepancies, mixed messages or conflicts (often with an "on the one hand ... and on the other" format) in a supportive, nonjudgmental way. Its aim is to help the client notice and resolve the incongruity, not to win an argument.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: confrontation of discrepancies

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor says, 'I notice that whenever we get close to talking about your father, you change the subject and glance at me as if checking my reaction. I'm wondering what is happening between us right now.' This statement is BEST described as:

A. Self-disclosure

Self-disclosure shares information about the counselor's own life; here the focus is the current relationship.

B. Immediacy ✓ **KEY**

Immediacy addresses what is occurring in the here-and-now between counselor and client.

C. Interpretation

Interpretation offers a new frame for the client's behavior; this response invites discussion of the present interaction instead.

D. Reframing

Reframing recasts a situation in a different light; it does not focus on the counselor-client relationship.

Rationale: Immediacy is the counselor's direct, present-tense comment on the counseling relationship or on a pattern unfolding in the session. It is useful when trust, tension, dependence or avoidance in the relationship is blocking progress.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: immediacy

☐ Approve ☐ Revise ☐ Retire Notes:

In a child-centered play therapy session, a 6-year-old grows frustrated and raises a wooden block as if to throw it at the counselor. Using Landreth's ACT model of limit setting, which response is BEST?

A. If you throw that block, our playtime is over for today and we will go back out.

Leading with a consequence skips acknowledging the feeling and offering an alternative; consequences come only if limits are repeatedly broken.

B. We don't throw blocks in here. Can you please put it back on the shelf for me?

This states a limit but ignores the child's feeling and gives no acceptable outlet for it.

C. You're really mad. I'm not for throwing. You can throw the soft ball at the wall. ✓ KEY

This acknowledges the feeling, communicates the limit and targets an acceptable alternative, the three steps of ACT.

D. It sounds like you are feeling frustrated with how the game is going for you today.

Reflecting the feeling alone leaves the counselor and child unprotected; a limit is needed when safety is at stake.

Rationale: Landreth's ACT model: Acknowledge the child's feeling, Communicate the limit and Target an acceptable alternative. Limits in child-centered play therapy are few, are stated calmly and briefly, and protect physical safety and the relationship while leaving the feeling itself accepted.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling children: child-centered play therapy and limit setting

► **Check:** The key's wording is in Landreth's style, not a direct quote.

☐ Approve ☐ Revise ☐ Retire Notes:

A play therapist spends early sessions building an egalitarian relationship with a 9-year-old, then gathers information about the child's lifestyle and mistaken beliefs, helps the child gain insight, and finally uses reorientation to practice new ways of belonging. This sequence is characteristic of which approach?

A. Adlerian play therapy ✓ KEY

Kottman's Adlerian play therapy proceeds through relationship building, exploring lifestyle, gaining insight and reorientation/reeducation.

B. Child-centered play therapy

Child-centered play therapy is nondirective and does not assess lifestyle or work toward planned reeducation phases.

C. Theraplay

Theraplay is a directive, attachment-based approach involving caregivers in structured engagement and nurture activities.

D. Filial therapy

Filial therapy trains parents to conduct child-centered play sessions; it is not organized around lifestyle analysis.

Rationale: Terry Kottman's Adlerian play therapy integrates Adlerian concepts (lifestyle, mistaken beliefs, the Crucial Cs, encouragement) with play. Its four phases are building an egalitarian relationship, exploring the child's lifestyle, helping the child gain insight and reorientation/reeducation.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling children: approaches to play therapy (Adlerian, child-centered, filial)

► **Check:** Kottman's Adlerian play therapy phases are cited via Gladding, which may not cover Kottman in depth.

☐ Approve ☐ Revise ☐ Retire Notes:

A college student arrives at the counseling center hours after learning his scholarship was revoked, saying he 'can't see a way forward.' Following Roberts' seven-stage crisis intervention model, what should the counselor do FIRST?

A. Help him generate options for funding and plan his next steps with financial aid

Generating and exploring alternatives is stage 5, after safety, rapport, problems and feelings have been addressed.

B. Conduct a thorough psychosocial and lethality assessment, including suicide risk ✓ **KEY**

Stage 1 of Roberts' model is a biopsychosocial and lethality assessment; his hopeless statement makes this urgent.

C. Explore and validate his feelings of shock, anger and shame about the revocation

Dealing with feelings and emotions is stage 4 of the model, not the first step.

D. Identify the main problems and the precipitating event that led him to come in today

Identifying major problems and precipitants is stage 3, after assessment and rapport building.

Rationale: Roberts' seven stages are: (1) plan and conduct a thorough biopsychosocial and lethality assessment, (2) establish rapport, (3) identify major problems and precipitants, (4) deal with feelings and emotions, (5) generate and explore alternatives, (6) implement an action plan and (7) follow up. Stages can overlap, but safety assessment comes first.

References: Gladding (Counseling: A Comprehensive Profession) — Crisis counseling: Roberts' seven-stage crisis intervention model

☐ Approve ☐ Revise ☐ Retire Notes:

A family is seen by a crisis counselor two days after their apartment was destroyed in a fire. Which goal BEST reflects the primary aim of crisis intervention?

A. Uncover early losses that make the family vulnerable to grief and resolve them

Exploring early history is a goal of longer-term therapy and can overwhelm people in acute crisis.

B. Restructure family roles and boundaries that may have contributed to stress

Structural change in family organization is a family therapy goal, not the immediate aim of crisis work.

C. Complete a diagnostic evaluation of each member to begin long-term treatment

Crisis intervention is brief and focused on coping; a crisis reaction is not by itself a disorder requiring diagnosis.

D. Restore the family to at least its pre-crisis level of coping and functioning ✓ **KEY**

Crisis intervention seeks to stabilize people and restore equilibrium, at least to the pre-crisis level of functioning.

Rationale: A crisis is a time-limited state in which usual coping is overwhelmed. Crisis intervention is brief, active and present-focused, aiming to ensure safety, mobilize resources and coping and return people to at least their pre-crisis level of functioning; deeper personality or system change is left for later work if needed.

References: Gladding (Counseling: A Comprehensive Profession) — Crisis counseling: goals of crisis intervention

☐ Approve ☐ Revise ☐ Retire Notes:

On Holland's RIASEC hexagon, which pair of personality types is MOST consistent, meaning the two types share the most characteristics?

A. Realistic and Investigative ✓ KEY

Realistic and Investigative are adjacent on the hexagon, so they share the most traits and form a consistent pair.

B. Realistic and Social

Realistic and Social sit opposite each other on the hexagon and are the least consistent pairing.

C. Artistic and Conventional

Artistic and Conventional are opposite types, one valuing self-expression and the other order and structure.

D. Investigative and Enterprising

Investigative and Enterprising are opposite each other on the hexagon, so they are inconsistent.

Rationale: Holland arranged the six types in the order R-I-A-S-E-C around a hexagon. Adjacent types are most consistent, alternate types are intermediate and opposite types (R-S, I-E, A-C) are least consistent. Holland's related ideas include congruence (fit between person and environment) and differentiation.

References: Sharf (Career Development Theory) — Holland's theory of types: the hexagon and consistency

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old nurse manager tells a counselor that her main concerns are holding on to the position she has built, keeping her clinical skills current and innovating within her role. In Super's life-span, life-space theory, these concerns BEST fit which career stage?

A. Establishment

Establishment (about ages 25 to 44) centers on stabilizing in and advancing within a chosen field.

B. Exploration

Exploration (about ages 15 to 24) involves crystallizing, specifying and implementing an occupational choice.

C. Maintenance ✓ KEY

Maintenance (about ages 45 to 64) involves holding, updating and innovating to preserve one's position.

D. Disengagement

Disengagement (about 65 and older) involves decelerating, retirement planning and retirement living.

Rationale: Super's stages are growth, exploration, establishment, maintenance and disengagement, each with developmental tasks; the maintenance tasks are holding, updating and innovating. Super treated the stage ages as approximate, and he stressed that people recycle through stages and that work is one of several life roles shown in the life-career rainbow.

References: Sharf (Career Development Theory) — Super's life-span, life-space theory: maintenance stage tasks

☐ Approve ☐ Revise ☐ Retire Notes:

A client is anxious because she has no clear five-year career plan. The counselor helps her see that past opportunities arose from unexpected events, and encourages curiosity, persistence, flexibility, optimism and risk taking to create and use future chance events. This approach reflects:

A. Theory of work adjustment

The theory of work adjustment (Dawis and Lofquist) addresses correspondence between worker needs and job reinforcers.

B. Planned happenstance theory ✓ KEY

Planned happenstance (Mitchell, Levin and Krumboltz) teaches clients to create and capitalize on unplanned events.

C. Circumscription and compromise

Gottfredson's theory explains how children narrow options by sex type and prestige, not how to use chance events.

D. Cognitive information processing

CIP (Sampson, Peterson and colleagues) focuses on the pyramid of information processing and the CASVE decision cycle.

Rationale: Planned happenstance grew out of Krumboltz's social learning theory of career decision making. It reframes indecision as open-mindedness and builds five skills (curiosity, persistence, flexibility, optimism and risk taking) so that clients can turn unplanned events into career opportunities.

References: Sharf (Career Development Theory) — Krumboltz's social learning theory and planned happenstance

☐ Approve ☐ Revise ☐ Retire Notes:

A 4-year-old eagerly plans games and starts projects, but his parents often scold him for 'making messes' when he tries new activities. In Erikson's theory, this child is working on which psychosocial crisis?

A. Autonomy vs. shame and doubt

Autonomy vs. shame and doubt is the toddler stage (about 1 to 3 years), centered on self-control and independence.

B. Industry vs. inferiority

Industry vs. inferiority is the school-age stage (about 6 to 12 years), centered on competence and productivity.

C. Trust vs. mistrust

Trust vs. mistrust is the infancy stage (birth to about 1 year), centered on reliable caregiving.

D. Initiative vs. guilt ✓ KEY

Initiative vs. guilt (about 3 to 6 years) involves planning and starting activities; harsh criticism can foster guilt.

Rationale: Erikson's eight stages are trust vs. mistrust (infancy), autonomy vs. shame and doubt (toddlerhood), initiative vs. guilt (preschool), industry vs. inferiority (school age), identity vs. role confusion (adolescence), intimacy vs. isolation (young adulthood), generativity vs. stagnation (middle adulthood) and integrity vs. despair (late adulthood).

References: Berk (Development Through the Lifespan) — Erikson's psychosocial theory: initiative vs. guilt in early childhood

☐ Approve ☐ Revise ☐ Retire Notes:

299. nce-s-cou-051 · Human Growth and Development · human development theories (Erikson, Piaget, Kohlberg) · easy · In review

An 8-year-old correctly explains that a ball of clay has the same amount of clay after it is rolled into a long snake, but she struggles with a purely hypothetical question about what would happen if people had no need for sleep. In Piaget's theory, she is MOST likely in which stage?

A. Concrete operational ✓ KEY

The concrete operational child (about 7 to 11) masters conservation and logical operations on concrete objects but not abstract hypotheticals.

B. Preoperational

Preoperational children (about 2 to 7) typically fail conservation tasks because of centration.

C. Formal operational

Formal operational thinkers (about 11 and older) can reason about abstract and hypothetical propositions, which she cannot yet do.

D. Sensorimotor

The sensorimotor stage (birth to about 2) involves learning through senses and actions, before symbolic thought.

Rationale: Piaget's four stages are sensorimotor (birth to 2), preoperational (2 to 7), concrete operational (7 to 11) and formal operational (11 and up). Conservation marks concrete operations; abstract, hypothetico-deductive reasoning marks formal operations.

References: Berk (Development Through the Lifespan) — Piaget's cognitive-developmental theory: concrete operational stage

☐ Approve ☐ Revise ☐ Retire Notes:

300. nce-s-cou-052 · Human Growth and Development · human development theories (Erikson, Piaget, Kohlberg) · medium · In review

Asked whether a man should steal an unaffordable medicine to save his wife, a 16-year-old answers, 'No, because if everyone broke the law when they felt like it, society would fall apart.' This reasoning BEST fits which Kohlberg stage?

A. Stage 2: Instrumental exchange

Stage 2 (preconventional) reasoning focuses on self-interest and fair deals, not on maintaining social order.

B. Stage 3: Interpersonal conformity

Stage 3 reasoning centers on being seen as good and winning the approval of others.

C. Stage 4: Law and order orientation ✓ KEY

Stage 4 (conventional) reasoning upholds laws and duties because they maintain the social system.

D. Stage 5: Social contract

Stage 5 (postconventional) sees laws as changeable agreements that can be set aside to protect basic rights such as life.

Rationale: Kohlberg described three levels with two stages each: preconventional (1 punishment and obedience, 2 instrumental purpose), conventional (3 interpersonal conformity, 4 social-order maintenance or law and order) and postconventional (5 social contract, 6 universal ethical principles). Stage is judged by the reasoning given, not by the yes-or-no answer.

References: Berk (Development Through the Lifespan) — Kohlberg's theory of moral development: conventional level

☐ Approve ☐ Revise ☐ Retire Notes:

In the Strange Situation, a 14-month-old stays close to her mother and explores little. She becomes very distressed when her mother leaves, and on reunion she clings to her mother while also pushing away and crying angrily. Which attachment classification BEST fits?

A. Secure

Securely attached infants explore using the parent as a base and are readily comforted on reunion.

B. Anxious-ambivalent (resistant) ✓ KEY

Resistant infants explore little, are highly distressed at separation and both seek and resist contact on reunion.

C. Avoidant

Avoidant infants show little distress at separation and ignore or avoid the parent on reunion.

D. Disorganized/disoriented

Disorganized infants show contradictory, confused behaviors such as freezing or approaching with the head averted.

Rationale: Ainsworth's Strange Situation identified secure, avoidant and anxious-ambivalent (resistant) patterns; Main and Solomon later added disorganized/disoriented. Mixing angry resistance with contact seeking on reunion is the signature of the resistant pattern, often linked to inconsistent caregiving.

References: Berk (Development Through the Lifespan) — Attachment: Ainsworth Strange Situation classifications

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor reviewing an infant assessment reads that, on reunion with a parent, the 12-month-old approached with his face turned away, then froze with a dazed expression. Which statement about this pattern is MOST accurate?

A. It is Ainsworth's original avoidant category, linked mainly to warm but busy caregivers

Avoidant infants show organized avoidance, and the pattern is linked to rejecting, not warm-but-busy, caregiving.

B. It reflects a secure pattern in a temperamentally shy infant who needs time to warm up

Freezing and approach-avoidance are not features of secure attachment, regardless of temperament.

C. It is the resistant pattern, defined by clinging mixed with anger toward the caregiver

Resistant infants show organized ambivalence; the freezing and averted approach described here is different.

D. It fits the disorganized category added by Main and Solomon, tied to frightening care ✓ KEY

Disorganized attachment involves contradictory or dazed behavior and is associated with frightened or frightening caregiving.

Rationale: Main and Solomon identified disorganized/disoriented attachment in infants whose behavior did not fit Ainsworth's three organized patterns. The infant lacks a coherent strategy, often because the caregiver is a source of both comfort and fear, as in maltreatment or unresolved parental trauma.

References: Berk (Development Through the Lifespan) — Attachment: disorganized/disoriented attachment (Main & Solomon)

☐ Approve ☐ Revise ☐ Retire Notes:

The original Sue, Arredondo and McDavis multicultural competencies were organized around attitudes and beliefs, knowledge and skills. Which additional aspirational competency did the Multicultural and Social Justice Counseling Competencies (MSJCC) add?

A. Action ✓ KEY

The MSJCC framework adds action (AKSA), calling counselors to engage in advocacy and intervention at multiple levels.

B. Awareness

Awareness of one's own cultural values and biases was already central to the original competencies.

C. Assessment

Culturally appropriate assessment is part of skills, not a separate aspirational competency in the MSJCC.

D. Accreditation

Accreditation is a program-level standard, not one of the aspirational competencies.

Rationale: The MSJCC (Ratts and colleagues, endorsed by ACA in 2015) uses quadrants of privileged and marginalized counselor and client identities and four developmental domains: counselor self-awareness, client worldview, the counseling relationship, and counseling and advocacy interventions. Each domain includes attitudes and beliefs, knowledge, skills and action.

References: Sue & Sue (Counseling the Culturally Diverse) — Multicultural and Social Justice Counseling Competencies (MSJCC)

► **Check:** MSJCC cited via Sue & Sue; consider the primary source (Ratts et al., 2016) when one is added.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor notices she feels impatient with clients who defer major decisions to their extended families, and she begins examining how her own upbringing shapes that reaction. In the Sue, Arredondo and McDavis framework, this work MOST directly reflects which competency area?

A. Understanding the worldview of the culturally different client

Understanding the client's worldview is a separate area; her focus here is on her own conditioning.

B. Developing culturally appropriate intervention strategies

Intervention strategies concern what the counselor does in session, not examining her own reactions.

C. Awareness of her own cultural values, assumptions and biases ✓ KEY

Examining how her own background shapes reactions to clients is counselor self-awareness, the first competency area.

D. Knowledge of family structures common among collectivist cultures

Knowledge of client cultures matters, but the activity described is reflection on her own biases.

Rationale: Sue, Arredondo and McDavis (1992) described three characteristics of the culturally skilled counselor: awareness of one's own assumptions, values and biases; understanding the worldview of culturally different clients; and developing appropriate intervention strategies. Each is assessed across beliefs and attitudes, knowledge and skills.

References: Sue & Sue (Counseling the Culturally Diverse) — Multicultural counseling competencies: counselor awareness of own assumptions, values and biases

☐ Approve ☐ Revise ☐ Retire Notes:

Months after a racist incident on campus shook her belief that race did not matter, a Black college student now surrounds herself with Black-centered organizations, studies Black history intensely and expresses anger toward White people. In Cross's Nigrescence model, she is MOST likely in which stage?

A. Pre-encounter

Pre-encounter involves low race salience or pro-White attitudes, which she has moved beyond.

B. Immersion-emersion ✓ KEY

Immersion-emersion involves intense involvement in Black culture and often anger toward White people.

C. Encounter

The encounter is the event, here the incident, that challenged her prior worldview; she has moved past it.

D. Internalization

Internalization brings a secure, less reactive Black identity and openness to other groups.

Rationale: Cross's model describes pre-encounter, encounter, immersion-emersion and internalization (with internalization-commitment for sustained action). After an encounter disrupts the old identity, the person often immerses in Black culture and withdraws from or rejects White culture before reaching a more secure, internalized identity.

References: Sue & Sue (Counseling the Culturally Diverse) — Racial/cultural identity development: Cross Nigrescence model

☐ Approve ☐ Revise ☐ Retire Notes:

After a multicultural course, a White counseling student feels guilt and confusion. She now sees racism around her, yet feels torn between loyalty to family members who make racist jokes and her new beliefs. In Helms' White racial identity model, this experience BEST fits which status?

A. Contact

Contact involves obliviousness to racism and a naive, color-blind view, which she has moved past.

B. Reintegration

Reintegration resolves the conflict by retreating to beliefs in White superiority and blaming people of color.

C. Pseudo-independence

Pseudo-independence is an intellectual commitment to change that comes after the conflict of disintegration.

D. Disintegration ✓ KEY

Disintegration involves guilt, confusion and conflict between new awareness of racism and loyalty to one's own group.

Rationale: Helms' White racial identity statuses are contact, disintegration, reintegration, pseudo-independence, immersion/emersion and autonomy. Disintegration is the uncomfortable phase in which new awareness of racism collides with prior beliefs and group loyalties, producing guilt, shame and moral dilemmas.

References: Sue & Sue (Counseling the Culturally Diverse) — White racial identity development: Helms' model

☐ Approve ☐ Revise ☐ Retire Notes:

At an intake, a client with a history of assault becomes visibly tense when the counselor closes the office door and moves her chair to sit between the client and the exit. Which response BEST reflects trauma-informed practice?

A. Name what she noticed, explain the reason for privacy and ask how the client wants the room set up ✓ **KEY**

This offers transparency, voice and choice, restoring a sense of safety and control, which are core trauma-informed principles.

B. Continue the intake as planned so the client can see the office is safe and her anxiety will fade

Pressing on ignores a cue of distress and puts the counselor's agenda ahead of the client's sense of safety.

C. Begin a detailed account of the assault now so that the tension can be processed early in treatment

Pushing for trauma details at intake, before safety and stabilization, risks retraumatization.

D. Reassure her that the building is secure and that no one can come in while the door is fully closed

Reassurance alone does not give the client any control and may increase her sense of being trapped.

Rationale: SAMHSA's trauma-informed approach rests on safety; trustworthiness and transparency; peer support; collaboration and mutuality; empowerment, voice and choice; and attention to cultural, historical and gender issues. Small environmental choices, explained openly, help clients regain control and prevent retraumatization.

References: Gladding (Counseling: A Comprehensive Profession) — Trauma and crisis counseling: principles of trauma-informed care

► **Check:** SAMHSA's six trauma-informed principles are cited via Gladding (SAMHSA TIP 57 is not on the approved list).

☐ Approve ☐ Revise ☐ Retire Notes:

A client has met her goals for managing panic after 14 sessions, and she and the counselor agree to end counseling. Which approach to the final sessions is BEST?

A. End at the next session with a brief goodbye to avoid fostering dependence on counseling

An abrupt ending skips consolidation and relapse planning and gives no room to process feelings about ending.

B. Review her gains, anticipate setbacks, plan to keep progress and discuss feelings about ending ✓ **KEY**

Effective termination consolidates learning, prepares for relapse, addresses feelings and clarifies how to return if needed.

C. Begin work on new goals so that she stays engaged and does not lose momentum after her progress

Adding goals once the agreed goals are met and the client no longer needs services is not in her interest.

D. Reduce session length gradually while avoiding talk about the ending so she is not made anxious

Avoiding the topic of ending denies the client a chance to process it and to plan for maintaining gains.

Rationale: Termination is a planned phase, not an event. Counselors review progress, help clients attribute change to their own efforts, anticipate challenges and relapse, explore feelings about ending and leave the door open, consistent with ACA guidance to terminate when services are no longer needed and to provide pretermination counseling.

References: ACA Code of Ethics — A.11.c Appropriate Termination · Gladding (Counseling: A Comprehensive Profession) — Termination of counseling relationships

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is interviewing applicants for a 10-week interpersonal process group. Under the ACA Code of Ethics, the main purpose of this pre-group screening is to:

A. confirm that each applicant has a diagnosis that the group can be billed for

Billing eligibility is an administrative matter; the Code frames screening around fit, group process and member welfare.

B. collect a detailed history so the leader can interpret members in session

Some history may be gathered, but screening exists to select suitable members, not to prepare interpretations.

C. select members whose goals fit the group and who will not be harmed by it ✓ **KEY**

A.9.a directs counselors to select members whose needs and goals are compatible with the group, who will not impede it and whose well-being will not be jeopardized.

D. make sure that every member shares the same cultural background and age range

Homogeneity can be useful for some groups, but it is not the ethical purpose of screening, and diversity often enriches a group.

Rationale: ACA Code A.9.a requires counselors to screen prospective group members and, to the extent possible, select those whose needs and goals are compatible with the group's goals, who will not impede the group process and whose well-being will not be jeopardized by the experience. The ASGW Best Practice Guidelines likewise treat screening and orientation as part of group planning.

References: ACA Code of Ethics — A.9.a Screening · Corey (Theory & Practice) — Group work: pre-group screening and selection of members

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor on an acute inpatient psychiatric unit, where the average stay is five days, is designing a daily coping-skills group. Which group format is MOST appropriate?

A. An open group that admits new members as patients arrive ✓ **KEY**

With rapid turnover, an open group lets patients join and leave as they are admitted and discharged, and each session can stand alone.

B. A closed group that keeps the same members for eight weeks

A closed eight-week group is impossible when patients stay about five days; most would be discharged after a few sessions.

C. A closed group with a waiting list for the next full cycle

A waiting list would keep most short-stay patients from attending at all before discharge.

D. A marathon group that meets once for a full weekend session

A single marathon session does not fit a daily skills program or the unpredictable timing of admissions.

Rationale: Open groups replace members as they leave, which suits settings with high turnover such as inpatient units, though new arrivals can slow the growth of cohesion. Closed groups keep the same members from start to finish, which supports stability and cohesion but requires members who can commit for the full duration.

References: Corey (Theory & Practice) — Group work: open versus closed groups

☐ Approve ☐ Revise ☐ Retire Notes: _____

An elementary school counselor is forming a counseling group for 7-year-olds whose parents recently divorced. Which group size is generally recommended for a group of children this age?

A. 8 to 10 members

Eight to ten is closer to the size often suggested for adult counseling groups; young children need more individual attention.

B. 3 to 5 members ✓ KEY

Young children have short attention spans and need frequent leader attention, so very small groups are recommended.

C. 12 to 15 members

A group this large would leave young children little time to talk and make the group hard to manage.

D. 20 to 25 members

This is a classroom guidance size, suited to psychoeducation lessons rather than a counseling group.

Rationale: Group size depends on members' age, the leader's experience and the type of group. Groups for young children are kept small (often about 3 to 5) and sessions short; adolescent groups are somewhat larger, and an ongoing adult counseling group of about 8 members with one leader is often cited as workable.

References: Corey (Theory & Practice) — Group work: practical considerations, group size and composition

► **Check:** Verify the recommended size range for young children's groups in the current Corey or Gladding edition (commonly cited as about 3 to 5).

☐ Approve ☐ Revise ☐ Retire Notes:

312. nce-s-cou-064 · Group Counseling and Group Work · group leadership skills and therapeutic factors · medium · In review

Two co-leaders of an adult support group notice that members have begun bringing complaints about one leader to the other, and the leaders have started to feel competitive with each other. Which step BEST addresses this co-leadership problem?

A. Divide the members so that each leader is responsible for half of the group

Splitting the group between leaders formalizes the division and leaves the competition unaddressed.

B. Let the more experienced co-leader run the sessions while the other observes

Sidelining one leader avoids the conflict instead of resolving it and wastes the benefits of co-leadership.

C. Agree to hide any differences of opinion from the members during sessions

Respectfully voiced differences can model healthy conflict; concealing them tends to feed the splitting.

D. Meet regularly between sessions to talk openly about their working relationship ✓ KEY

Regular meetings to process their relationship let co-leaders address rivalry and present a united, coordinated approach.

Rationale: Co-leadership offers shared perspective, modeling of a healthy relationship, coverage for absences and mutual support. Its main risks are poor communication, competition and members splitting the leaders. Corey stresses that co-leaders should meet before and after sessions to plan and to process their own relationship.

References: Corey (Theory & Practice) — Group leadership: advantages and disadvantages of co-leadership

☐ Approve ☐ Revise ☐ Retire Notes:

In the fourth session of a men's support group, one member has again taken most of the time telling long stories about his week, while others glance at the clock. Which leader response is MOST appropriate?

A. "You've given us a lot to think about. I'd like to hear how others are reacting to what you said." ✓ KEY

This acknowledges the member without attacking him, blocks the monopolizing and opens the floor to the rest of the group.

B. "Let's keep listening, since he clearly needs the group's time today more than anyone else does."

Letting the pattern continue reinforces monopolizing and ignores the frustration building among other members.

C. "You talk too much in here, and it is unfair to the others. Please stay quiet for the rest of today."

A harsh, shaming confrontation can humiliate the member and makes others fear the same treatment.

D. "I'll speak with you privately after group about whether this group is a good fit for your needs."

Raising removal ignores a workable in-group intervention and treats a common group behavior as grounds for exclusion.

Rationale: Monopolizing is one of the most common problem behaviors in groups. Leaders intervene early and respectfully, blocking the behavior while protecting the member's dignity, and they often invite the group to share reactions so members learn to address the pattern directly.

References: Corey (Theory & Practice) — Group work: problem behaviors, the monopolizing member

☐ Approve ☐ Revise ☐ Retire Notes:

In a teen group, several members begin blaming one boy for the group's lack of progress, citing his lateness and jokes, and they bring him up whenever the discussion becomes tense. Which leader response is BEST?

A. Ask the boy to explain to the group why he keeps arriving late and making jokes

Putting him on the spot joins the group's blaming and deepens his role as the scapegoat.

B. Allow the criticism to continue, since honest feedback helps him change his conduct

Collective blaming is not constructive feedback, and letting it continue fails the leader's duty to protect members.

C. Block the blaming and ask the group to look at what is happening among them now ✓ KEY

The leader protects the targeted member and turns attention to the group process, including the feelings members may be displacing onto him.

D. Move the boy to a different group so that the others can resume productive work

Removing the scapegoat leaves the underlying dynamic in place, and the group often finds a new target.

Rationale: Scapegoating occurs when a group displaces its tension onto one member. The leader blocks the attack to protect that member and then explores what the group is avoiding, which usually involves members' own anxieties or conflicts with the leader or one another.

References: Corey (Theory & Practice) — Group work: scapegoating and protecting members · ACA Code of Ethics — A.9.b Protecting Clients (group work)

☐ Approve ☐ Revise ☐ Retire Notes:

A group leader says, "I notice that each time someone starts to talk about anger, the group quickly shifts to joking. What do you make of that?" This statement is an example of which leader focus?

A. A content focus on the topic under discussion

A content focus addresses what is being discussed, such as the anger itself, rather than how the group handles it.

B. A process focus on how the members interact ✓ KEY

The leader is commenting on the pattern of interaction in the room, which is a process observation.

C. An educational focus on the facts about anger

An educational focus would teach information about anger rather than reflect a pattern back to the group.

D. An individual focus on one member's history

The observation is about the group as a whole, not about any one member's background.

Rationale: Content is what the group talks about; process is how members relate to one another while they talk, including patterns of avoidance, alliances and shifts in mood. Process comments help members see their here-and-now behavior and are central to interpersonal group work.

References: Yalom & Leszcz (Group Psychotherapy) — Ch. 6: The here-and-now and process illumination

☐ Approve

☐ Revise

☐ Retire

Notes:

A leader says, "Amara, what you just described about feeling invisible at work sounds a lot like what Dev shared last week about his family. Dev, how does Amara's story land for you?" Which group leadership skill is the leader using?

A. Cutting off

Cutting off stops a member who is rambling or going off track; nothing is being stopped here.

B. Drawing out

Drawing out invites a quiet member to participate; the main move here is connecting two members' concerns.

C. Summarizing

Summarizing pulls together the main points of a discussion rather than connecting two members.

D. Linking ✓ KEY

Linking connects what one member says or feels to another member's concerns, promoting member-to-member interaction.

Rationale: Linking builds cohesion and universality by pointing out common themes between members and encouraging them to speak to each other rather than only to the leader. Related skills include drawing out quieter members and cutting off, or redirecting, members who stray from the group's focus.

References: Corey (Theory & Practice) — Group leadership skills: linking

☐ Approve

☐ Revise

☐ Retire

Notes:

Which statement about group cohesiveness is MOST consistent with Yalom's account of it?

A. Highly cohesive groups can express and work through more conflict and hostility. ✓ KEY

Yalom notes that members of cohesive groups feel secure enough to voice anger and disagreement, which can then be worked through.

B. Cohesiveness is mostly fixed by the end of the first session and changes little later.

Cohesion develops and fluctuates over the life of a group; it is not set in the first meeting.

C. Cohesive groups tend to have poorer attendance because members feel less accountable.

Cohesion is linked to better attendance, greater participation and lower dropout, not poorer attendance.

D. Cohesion is a distinct outcome rather than a condition for other therapeutic factors.

Yalom treats cohesiveness as a precondition for other factors, comparable to the relationship in individual therapy.

Rationale: Yalom describes cohesiveness as the group analogue of the therapeutic relationship: the members' sense of belonging and acceptance. It is associated with better attendance, more self-disclosure, greater risk-taking and better outcomes, and cohesive groups can tolerate open conflict because members trust the group to survive it.

References: Yalom & Leszcz (Group Psychotherapy) — Ch. 3: Group cohesiveness

☐ Approve ☐ Revise ☐ Retire Notes:

318. nce-s-cou-070 · Group Counseling and Group Work · group stages and dynamics · medium · In review

Midway through a closed 12-week counseling group, a member emails the leader that she has decided to quit the group and will not return. What is the BEST response from the leader?

A. Tell her she agreed to finish all 12 weeks and must attend until the group ends

Members have the right to leave a group; the leader can encourage but not compel continued attendance.

B. Accept her decision and avoid mentioning her absence so the others stay focused

Unexplained departures affect remaining members, who may wonder whether they caused it; silence leaves this unprocessed.

C. Respect her right to leave while encouraging her to talk it over with the group ✓ KEY

Corey advises leaders to respect members' freedom to leave while encouraging them to discuss the decision with the group rather than simply disappear.

D. Replace her at once with a new member from the waiting list to keep the size stable

Adding a member midway changes the closed format and does nothing to address the departure itself.

Rationale: Members have the right to leave a group, but an abrupt exit can leave both the departing member and the remaining members with unfinished business. Leaders explain this during orientation and, when a member wants to leave, encourage discussion of the decision in the group and process the loss with those who remain.

References: Corey (Theory & Practice) — Group work ethics: freedom to leave a group

☐ Approve ☐ Revise ☐ Retire Notes:

In a training group, members persistently treat the leader as all-knowing, wait passively for direction and become resentful when the leader does not supply answers. In Bion's theory of group functioning, this pattern BEST reflects which basic assumption?

A. Fight-flight

Fight-flight is shown when the group attacks something or escapes the task, for example through hostility or changing the subject.

B. Dependency ✓ KEY

In basic assumption dependency the group acts as if a strong leader will provide for and protect it, and members become passive.

C. Pairing

Pairing is a shared hope that two members will produce a future savior or solution, not reliance on the leader.

D. Work group

The work group is Bion's term for the rational, task-focused mode of functioning, not a basic assumption.

Rationale: Wilfred Bion, working at the Tavistock Clinic, proposed that groups alternate between a work group focused on the task and three unconscious basic assumptions: dependency, fight-flight and pairing. Recognizing a basic assumption lets the leader interpret the group-as-a-whole behavior instead of simply meeting the demand.

References: Yalom & Leszcz (Group Psychotherapy) — Group-as-a-whole approaches: Bion's basic assumptions (Tavistock)

► **Check:** Verify that the cited Yalom & Leszcz chapter (group-as-a-whole/Tavistock discussion) covers Bion's basic assumptions; Corey or Gladding may be a better reference.

☐ Approve ☐ Revise ☐ Retire Notes:

Frank Parsons described wise vocational choice as resting on a clear understanding of oneself, knowledge of the requirements and conditions of different jobs, and which third element?

A. Exploration of chance events that may reshape one's path

Using chance events is central to Krumboltz's planned happenstance, not to Parsons' model.

B. Implementation of the self-concept through a series of roles

Implementing the self-concept in work roles is Super's idea, developed decades after Parsons.

C. Resolution of early parent-child needs through one's work

Linking occupational choice to early parent-child relations is Anne Roe's theory.

D. True reasoning on the relations between these two groups ✓ KEY

Parsons' third step was "true reasoning" about how the person's traits match the requirements of occupations.

Rationale: In *Choosing a Vocation* (1909), Parsons set out three steps: understand yourself, know the world of work and use true reasoning to match the two. This matching approach became trait-factor theory and underlies later person-environment fit models such as Holland's.

References: Sharf (Career Development Theory) — Trait and factor theory: Parsons' three-part model

☐ Approve ☐ Revise ☐ Retire Notes:

A client's interest inventory shows a very high Investigative score, while her other five RIASEC scores are all low and close together. In Holland's theory, this profile is BEST described as showing a high degree of:

A. Differentiation ✓ KEY

Differentiation is the degree to which a profile has clear peaks and valleys; one high score above flat low scores is well differentiated.

B. Consistency

Consistency concerns how close a person's top types sit on the hexagon, which needs at least two high types to judge.

C. Congruence

Congruence is the fit between a person's type and the work environment, which a profile alone does not show.

D. Calculus

Calculus is Holland's assumption that hexagon distances reflect type relationships, not a property of one profile.

Rationale: Holland's secondary constructs include differentiation (how distinct the profile is), consistency (how related the top types are on the hexagon), congruence (person-environment fit), identity (clarity and stability of goals) and calculus. A flat profile is undifferentiated and is often associated with less clear career direction.

References: Sharf (Career Development Theory) — Holland's theory: differentiation, consistency and congruence

☐ Approve ☐ Revise ☐ Retire Notes:

Super used career maturity to describe an adolescent's readiness to make age-appropriate career decisions. Because adults recycle through stages rather than progress in a fixed order, which term did Super propose for readiness to cope with changing work and working conditions in adulthood?

A. Career salience

Salience refers to the importance a person gives to the worker role relative to other life roles.

B. Vocational identity

Vocational identity, the clarity and stability of one's goals and interests, is associated with Holland.

C. Career adaptability ✓ KEY

Super introduced career adaptability as the adult counterpart of career maturity; Savickas later made it central to career construction theory.

D. Career crystallizing

Crystallizing is a developmental task of the exploration stage, not an index of adult readiness.

Rationale: Career maturity, measured by tools such as the Career Development Inventory, compares a young person's coping with developmental tasks to that of peers. Because adult careers involve recycling and transitions, Super suggested career adaptability for adults, a concept Savickas expanded into the adapt-abilities of concern, control, curiosity and confidence.

References: Sharf (Career Development Theory) — Super's theory: career maturity and career adaptability

► **Check:** Confirm attribution: Super proposed career adaptability for adults (early 1980s), later developed by Savickas.

☐ Approve ☐ Revise ☐ Retire Notes:

According to Anne Roe's theory, an adult who grew up in a cold, rejecting or neglectful family climate would be MOST likely to gravitate toward which kind of occupation?

A. Person-oriented fields such as service and business contact

Roe linked warm, accepting or overprotective climates, not cold ones, with orientation toward people.

B. Non-person-oriented fields such as technology and science ✓ KEY

Roe proposed that avoidant (rejecting or neglecting) climates lead toward work that does not center on people.

C. Highly creative fields chosen to win approval from parents

Roe did not tie avoidant climates to approval-seeking creative work; arts and entertainment is a field, not this mechanism.

D. Any field at all, since family climate predicts only level

In Roe's model, family climate shapes orientation toward or away from people; level depends on ability and status factors.

Rationale: Roe classified occupations into eight fields and six levels and proposed that early parent-child relations (emotional concentration, avoidance or acceptance) create needs that steer people toward or away from person-oriented work. Research gave her theory limited support, but her field-and-level classification influenced later occupational systems.

References: Sharf (Career Development Theory) — Roe's personality theory of career choice: parent-child relations

☐ Approve ☐ Revise ☐ Retire Notes:

324. nce-s-cou-076 · Career Development · career development theories (Holland, Super, Krumboltz) · easy · In review

A 14-year-old who once wanted to be an astronaut now talks about whether she has the grades and abilities for engineering and whether the work fits what she values. In Ginzberg's theory, she is in which period?

A. Fantasy period

The fantasy period (to about age 11) involves play-based choices made without regard to ability or reality.

B. Realistic period

The realistic period (about 17 into early adulthood) involves exploration, crystallization and specification of a choice.

C. Crystallization

Crystallization is a substage of the later realistic period, when a clear occupational pattern emerges.

D. Tentative period ✓ KEY

The tentative period (about 11 to 17) moves through interest, capacity, values and transition substages.

Rationale: Ginzberg, Ginsburg, Axelrad and Herma described three periods: fantasy, tentative (interest, capacity, values, transition) and realistic (exploration, crystallization, specification). Ginzberg later revised the theory to view career choice as a lifelong, open-ended process rather than one fixed by early adulthood.

References: Sharf (Career Development Theory) — Ginzberg's developmental theory: fantasy, tentative and realistic periods

☐ Approve ☐ Revise ☐ Retire Notes:

A client must make a major compromise because her preferred occupations are not available locally. According to Gottfredson's theory of circumscription and compromise, which aspect of her self-concept is she likely to protect MOST when compromising?

A. Sextype, how suitable the job seems for her gender ✓ KEY

Gottfredson proposed that sextype, the earliest and most core element to be circumscribed, is the last to be given up in major compromises.

B. Interest field, how well the work matches her likes

Interests develop later and are more peripheral, so they tend to be sacrificed before prestige or sextype in major compromises.

C. Prestige, the social standing of the occupation

Prestige is protected over interests but is given up before sextype when the compromise is severe.

D. Pay level, the salary that the job typically offers

Salary is not one of the self-concept dimensions in Gottfredson's model of circumscription and compromise.

Rationale: Gottfredson described circumscription in four stages: orientation to size and power, to sex roles, to social valuation and to the internal, unique self. Because sextype is incorporated earliest, it is the most protected aspect; in major compromises people tend to give up interest fit first, then prestige, and sextype last.

References: Sharf (Career Development Theory) — Gottfredson's theory of circumscription and compromise

► **Check:** Gottfredson's 1996/2002 revision makes the order depend on how severe the compromise is; verify the key against the current Sharf wording.

☐ Approve ☐ Revise ☐ Retire Notes:

326. nce-s-cou-078 · Career Development · career development theories (Holland, Super, Krumboltz) · medium · In review

A counselor helps a client set a career goal, clarify her values, generate several alternatives and gather occupational information. In Krumboltz's social learning theory of career decision making, the counselor is building which factor?

A. Genetic endowment and special abilities

Genetic endowment covers inherited qualities such as physical traits and talents, which counseling does not build.

B. Environmental conditions and events

Environmental conditions, such as job markets and policies, are largely outside the individual's control.

C. Task-approach skills ✓ KEY

Task-approach skills are the learned skills, such as goal setting, values clarification and information seeking, used to approach career tasks.

D. Associative learning experiences

Associative learning links neutral stimuli with positive or negative ones; it is not the set of decision skills described.

Rationale: Krumboltz identified four influences on career decisions: genetic endowment and special abilities, environmental conditions and events, learning experiences (instrumental and associative) and task-approach skills. Their interaction produces self-observation generalizations, worldview generalizations and the actions people take.

References: Sharf (Career Development Theory) — Krumboltz's social learning theory: task-approach skills

☐ Approve ☐ Revise ☐ Retire Notes:

A college senior knows her strengths and has researched several occupations, but each time she nears a choice she tells herself, "I always make the wrong decision," and stalls. In the cognitive information processing (CIP) approach, the counselor should focus FIRST on which domain of the pyramid?

A. Self-knowledge domain

She already knows her strengths; her difficulty is not a lack of self-knowledge.

B. Executive processing domain ✓ KEY

Executive processing covers metacognitions such as self-talk and self-monitoring; her negative self-talk is blocking the process.

C. Occupational knowledge domain

She has already researched occupations, so information about options is not the main barrier.

D. Decision-making skills domain

The CASVE cycle may need work later, but the dysfunctional self-talk at the top of the pyramid is what stalls her.

Rationale: Peterson, Sampson, Reardon and Lenz describe a pyramid with knowledge domains (self and occupations) at the base, decision-making skills (the CASVE cycle: communication, analysis, synthesis, valuing, execution) in the middle and executive processing at the top. Negative career thoughts, measured by the Career Thoughts Inventory, operate at the executive level.

References: Sharf (Career Development Theory) — Cognitive information processing: pyramid and executive processing

☐ Approve ☐ Revise ☐ Retire Notes:

A first-generation college student is confident she can master the coursework for an engineering degree, but she doubts that the degree will lead to a good job for someone without family connections in the field. In social cognitive career theory, her doubt BEST reflects low:

A. Self-efficacy beliefs

Self-efficacy is belief in one's ability to perform; she is confident she can do the work.

B. Personal goals

Goals are intentions to pursue an activity or outcome; her doubt concerns expected results, not intentions.

C. Interest development

In SCCT interests grow from efficacy and outcome expectations; the stem describes a belief about results.

D. Outcome expectations ✓ KEY

Outcome expectations are beliefs about the consequences of an action, such as whether a degree will pay off.

Rationale: Lent, Brown and Hackett's social cognitive career theory builds on Bandura and centers on self-efficacy, outcome expectations and personal goals. Contextual supports and barriers, such as lack of professional networks, shape these beliefs and help explain why capable people may avoid a field.

References: Sharf (Career Development Theory) — Social cognitive career theory: self-efficacy, outcome expectations and goals

☐ Approve ☐ Revise ☐ Retire Notes:

An accountant consistently receives excellent performance reviews, but he tells his counselor that the job offers none of the autonomy or variety he values and he dreads going to work. In the theory of work adjustment, his situation is BEST described as:

A. high satisfactoriness and low satisfaction ✓ KEY

His employer is pleased with his performance (satisfactoriness), but his own needs and values are not being met (satisfaction).

B. high satisfaction and low satisfactoriness

This reverses the terms; satisfaction is the worker's view of the job and satisfactoriness is the employer's view of the worker.

C. low satisfaction and low satisfactoriness

His excellent reviews show that the employer finds him satisfactory.

D. high satisfaction and high satisfactoriness

His dread and unmet values show that he is not satisfied with the job.

Rationale: Davis and Lofquist's theory of work adjustment holds that tenure depends on correspondence between person and environment. Satisfaction reflects whether the job's reinforcers meet the worker's needs and values; satisfactoriness reflects whether the worker's abilities meet the job's requirements. Low satisfaction predicts that the worker may leave even when performance is strong.

References: Sharf (Career Development Theory) — Theory of work adjustment: satisfaction and satisfactoriness

☐ Approve ☐ Revise ☐ Retire Notes:

A 24-year-old has interests and options but says his parents should decide his career, and he puts off every decision he could make himself. In Savickas's career construction theory, which career adapt-ability is MOST in need of development?

A. Concern

Concern is orientation toward and planning for the future; the main problem described is lack of ownership of decisions.

B. Curiosity

Curiosity is exploring self and options; he already has interests and options to choose from.

C. Control ✓ KEY

Control is taking personal responsibility for one's career; deferring decisions and indecision signal a control deficit.

D. Confidence

Confidence is belief in one's ability to solve problems; the stem points to handing decisions to others rather than self-doubt.

Rationale: Savickas identifies four career adapt-abilities: concern (future orientation), control (decisiveness and ownership), curiosity (exploration) and confidence (self-efficacy for problem solving). Career construction counseling also uses the Career Construction Interview to identify life themes that give meaning to career choices.

References: Sharf (Career Development Theory) — Savickas's career construction theory: career adapt-abilities

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor wants a free online database, sponsored by the U.S. Department of Labor, that replaced the Dictionary of Occupational Titles and describes occupations by tasks, skills, knowledge, work context and interest codes. Which resource fits this description?

A. Occupational Outlook Handbook

The Bureau of Labor Statistics handbook focuses on job outlook, pay and training, and it did not replace the DOT.

B. Occupational Information Network ✓ KEY

*O*NET replaced the DOT and describes occupations with detailed descriptors, including RIASEC interest profiles.*

C. Standard Occupational Classification

The SOC is a federal coding system for classifying jobs in statistical data, not a descriptive career database.

D. Guide for Occupational Exploration

The GOE grouped occupations by interest area and was a DOT companion, not its replacement.

Rationale: O*NET, the Occupational Information Network, is the Department of Labor's primary source of occupational information and replaced the DOT. It links occupations to Holland codes and pairs with labor market information, such as the Occupational Outlook Handbook's projections and wage data.

References: Gladding (Counseling: A Comprehensive Profession) — Career counseling: occupational information sources (O*NET, OOH)

☐ Approve ☐ Revise ☐ Retire Notes:

332. nce-s-cou-084 · Human Growth and Development · human development theories (Erikson, Piaget, Kohlberg) · easy · In review

A 9-year-old spends his energy on school, sports and friendships with same-sex peers, and sexual interests seem quiet. In Freud's psychosexual theory, he is in which stage?

A. Phallic stage

The phallic stage (about 3 to 6) centers on the Oedipal conflict and identification with the same-sex parent.

B. Genital stage

The genital stage begins at puberty, when sexual impulses reawaken and turn toward mature relationships.

C. Anal stage

The anal stage (about 1 to 3) centers on toilet training and control.

D. Latency stage ✓ KEY

In latency (about 6 to puberty) sexual drives are dormant and energy goes into skills, school and same-sex friendships.

Rationale: Freud's psychosexual stages are oral (birth to 1), anal (1 to 3), phallic (3 to 6), latency (6 to 11) and genital (adolescence). Erikson's industry vs. inferiority covers the same school-age years, with emphasis on competence rather than sexual drives.

References: Berk (Development Through the Lifespan) — Freud's psychosexual theory: latency stage

☐ Approve ☐ Revise ☐ Retire Notes:

A 19-year-old says he will take over his family's restaurant, as his parents always planned, and that he has never seriously considered any other path. In Marcia's framework, his identity status is:

A. Identity foreclosure ✓ KEY

Foreclosure is commitment without exploration, often accepting goals and values chosen by parents.

B. Identity moratorium

Moratorium is active exploration without commitment yet; he has committed without exploring.

C. Identity achievement

Achievement is commitment that follows a period of exploration, which he has not had.

D. Identity diffusion

Diffusion is neither exploring nor committed; he holds a firm commitment.

Rationale: Marcia extended Erikson's identity stage by crossing two dimensions, exploration and commitment, to form four statuses: achievement (explored, committed), moratorium (exploring, not committed), foreclosure (committed, not explored) and diffusion (neither). Statuses can change over time, and foreclosure is not unhealthy in every cultural context.

References: Berk (Development Through the Lifespan) — Marcia's identity statuses

☐ Approve ☐ Revise ☐ Retire Notes:

A 42-year-old architect is questioning whether the youthful vision he built his career around is worth pursuing, reappraising his marriage and wondering what legacy he will leave. In Levinson's seasons of life, he is MOST likely in the:

A. early adult transition

The early adult transition (about 17 to 22) involves leaving adolescence and forming a first sense of the Dream.

B. midlife transition ✓ KEY

The midlife transition (about 40 to 45) involves reappraising the Dream and the life structure built in early adulthood.

C. age 30 transition

The age 30 transition (about 28 to 33) involves adjusting the first adult life structure, not midlife reappraisal.

D. late adult transition

The late adult transition (about 60 to 65) involves facing aging and moving into late adulthood.

Rationale: Levinson described adulthood as alternating periods of building a life structure and transitions that question it. Key concepts include the Dream, a vision of one's adult future, and the mentor; at the midlife transition, people reappraise the Dream and rework the polarities of young-old, destruction-creation, masculine-feminine and attachment-separateness.

References: Berk (Development Through the Lifespan) — Levinson's seasons of life: the midlife transition

► **Check:** Levinson's period ages are approximate; confirm the midlife transition is given as about 40 to 45.

☐ Approve ☐ Revise ☐ Retire Notes:

335. nce-s-cou-087 · Human Growth and Development · human development theories (Erikson, Piaget, Kohlberg) · hard · In review

An 8-year-old's mother is moved to night shifts by her employer, and the child now spends evenings with less supervision and less help with homework. The child has never been to the workplace. In Bronfenbrenner's ecological systems theory, the employer's decision operates at which level?

A. Microsystem

The microsystem includes settings the child directly takes part in, such as home and school.

B. Mesosystem

The mesosystem is the connections between the child's microsystems, such as home-school links.

C. Exosystem ✓ **KEY**

The exosystem includes settings the child does not take part in, such as a parent's workplace, that still affect the child.

D. Macrosystem

The macrosystem is the broad cultural values, laws and customs that shape all the other levels.

Rationale: Bronfenbrenner nested the child within the microsystem, mesosystem, exosystem and macrosystem, with the chronosystem adding change over time. A parent's workplace policy is a classic exosystem influence: it shapes the child's daily life even though the child never enters that setting.

References: Berk (Development Through the Lifespan) — Bronfenbrenner's ecological systems theory: the exosystem

☐ Approve ☐ Revise ☐ Retire Notes:

336. nce-s-cou-088 · Human Growth and Development · human development theories (Erikson, Piaget, Kohlberg) · easy · In review

A tutor gives a first-grader step-by-step hints on a puzzle he cannot yet solve alone, then gradually withdraws the hints as he becomes able to solve it himself. This adjustment of support is BEST described as:

A. Accommodation

Accommodation is Piaget's term for changing a mental scheme to fit new information.

B. Private speech

Private speech is a child's self-directed talk used to guide thinking, not the adult's support.

C. Centration

Centration is the preoperational tendency to focus on one aspect of a situation.

D. Scaffolding ✓ **KEY**

Scaffolding adjusts the level of support to the child's current performance and fades it as competence grows.

Rationale: Vygotsky's zone of proximal development is the range of tasks a child cannot yet do alone but can do with help from a more skilled partner. Scaffolding, a term introduced by Wood, Bruner and Ross and applied to Vygotsky's theory, describes how that help is fine-tuned and withdrawn as the child masters the task.

References: Berk (Development Through the Lifespan) — Vygotsky's sociocultural theory: zone of proximal development and scaffolding

☐ Approve ☐ Revise ☐ Retire Notes:

A woman who has long put everyone else's needs ahead of her own comes to believe that caring responsibly must include caring for herself, and that avoiding harm applies to her as well as to others. In Gilligan's ethic of care, this reasoning BEST reflects which level?

A. Morality of nonviolence ✓ KEY

The third level integrates care for self and others, with a principle of not hurting anyone, including oneself.

B. Goodness as self-sacrifice

This second level equates goodness with meeting others' needs at one's own expense, which she is moving beyond.

C. Orientation to survival

The first level focuses on the self and its survival, without concern for responsibility to others.

D. Social contract reasoning

Social contract reasoning is Kohlberg's stage 5, based on justice and rights rather than care.

Rationale: Gilligan argued that Kohlberg's justice-based stages underrepresented an ethic of care. Her levels move from orientation to individual survival, through goodness as self-sacrifice, to the morality of nonviolence, with transitions from selfishness to responsibility and from goodness to truth about one's own needs.

References: Berk (Development Through the Lifespan) — Gilligan's ethic of care and critique of Kohlberg

► **Check:** Confirm Gilligan's level names (orientation to individual survival, goodness as self-sacrifice, morality of nonviolence) as given in Berk.

☐ Approve ☐ Revise ☐ Retire Notes:

A school counselor is planning supports for children growing up with poverty and parental substance use. Longitudinal research on resilience, such as Werner and Smith's Kauai study, MOST consistently identifies which protective factor?

A. Avoiding all stressful experiences throughout early childhood

Resilience develops through coping with manageable stress; complete avoidance of stress is neither possible nor protective.

B. A warm, stable relationship with at least one caring adult ✓ KEY

A close bond with a competent, caring adult, who need not be a parent, is among the most consistent protective factors.

C. An above-average IQ score as measured in the preschool years

Good cognitive skills can help, but a high IQ alone is not the most consistent protective factor.

D. Living in a family that has no history of mental illness

This describes the absence of a risk factor rather than a protective factor that buffers existing risk.

Rationale: Resilience research identifies protective factors at several levels: personal traits such as an easygoing temperament, a warm relationship with a caring adult, and community supports such as schools and mentors. Werner and Smith found that high-risk children who thrived usually had at least one stable, supportive adult in their lives.

References: Berk (Development Through the Lifespan) — Resilience: protective factors in high-risk children

☐ Approve ☐ Revise ☐ Retire Notes:

A strategic therapist tells a client with chronic insomnia that on any night he is still awake at midnight, he must get up and wax the kitchen floor until morning. The task is deliberately designed to be more unpleasant than the symptom itself. Which intervention and originator does this illustrate?

A. Ordeal therapy, associated with Jay Haley ✓ KEY

Haley's ordeal therapy assigns a task more burdensome than the symptom, so giving up the symptom becomes the easier choice.

B. Unbalancing, associated with Salvador Minuchin

Unbalancing is a structural technique in which the therapist temporarily sides with one family member to shift the hierarchy.

C. Paradoxical intention, from Viktor Frankl

Frankl's paradoxical intention would ask the client to try to stay awake; it does not attach an aversive, effortful task to the symptom.

D. Detriangulation, associated with Murray Bowen

Detriangulation is a Bowenian process of removing oneself from a three-person emotional triangle, unrelated to symptom tasks.

Rationale: Jay Haley described ordeal therapy as a strategic directive in which the client must perform an ordeal, ideally one that is good for the person (exercise, chores), every time the symptom occurs. Because the ordeal costs more than the symptom, the client gives up the symptom. It reflects strategic therapy's focus on directives and on changing behavior rather than producing insight.

References: Gladding (Counseling: A Comprehensive Profession) — Strategic family therapy: Haley, directives and ordeals

☐ Approve ☐ Revise ☐ Retire Notes:

A brief therapy team trained at the Mental Research Institute (MRI) in Palo Alto notices that a mother's repeated lectures to her withdrawn son lead him to withdraw further, which prompts more lectures. In the MRI model, what is the PRIMARY target of intervention?

A. The son's unresolved early childhood conflicts

MRI brief therapy deliberately avoids historical, insight-oriented exploration of early conflicts.

B. The family's level of differentiation of self

Differentiation of self is the central construct of Bowen family systems theory, not the MRI model.

C. The attempted solution maintaining the problem ✓ KEY

The MRI team holds that well-meant attempted solutions (here, lecturing) keep the problem going, so interrupting them is the target.

D. The hierarchy and boundaries between subsystems

Restructuring hierarchy and subsystem boundaries is the focus of Minuchin's structural family therapy.

Rationale: In MRI brief therapy (Watzlawick, Weakland, Fisch), problems persist because of the very solutions people repeatedly apply to them. "More of the same" produces only first-order change within the system's existing rules; the therapist aims for second-order change by blocking or reversing the attempted solution, often through reframing or paradoxical directives.

References: Gladding (Counseling: A Comprehensive Profession) — Strategic family therapy: MRI brief therapy, attempted solutions, first- and second-order change

☐ Approve ☐ Revise ☐ Retire Notes:

341. nce-s-cou-093 · Counseling and Helping Relationships · family systems theories (Bowen, structural, strategic) · easy · In review

During a family session about a teenager's failing grades, her father answers every emotional comment with charts, rules and calm, logical analysis, showing little feeling for himself or for others. Which Satir communication stance is he displaying?

A. Blaming

The blamer finds fault and attacks others to protect the self, disregarding others' feelings rather than all feelings.

B. Placating

The placater pleases and appeases others at the expense of the self; the father is not deferring to anyone.

C. Irrelevant

The irrelevant (distracting) stance changes the subject or clowns around, ignoring self, others and context.

D. Super-reasonable ✓ KEY

The super-reasonable (computer) stance relies on cool logic and data while discounting the feelings of self and others.

Rationale: Virginia Satir described four incongruent survival stances people adopt under stress: placating, blaming, super-reasonable and irrelevant. Her goal was congruent communication, in which words, feelings and body language match and self, other and context are all honored.

References: Gladding (Counseling: A Comprehensive Profession) — Experiential family therapy: Satir communication stances

☐ Approve ☐ Revise ☐ Retire Notes:

342. nce-s-cou-094 · Counseling and Helping Relationships · family systems theories (Bowen, structural, strategic) · medium · In review

A family therapist insists early on that he decides who attends and how sessions are run, but then leaves it to the family to take responsibility for what will change. He relies on spontaneity, playfulness and co-therapy, and calls these struggles the "battle for structure" and the "battle for initiative." Which approach is this?

A. Symbolic-experiential (Whitaker) ✓ KEY

Carl Whitaker described the battle for structure and the battle for initiative and emphasized the therapist's spontaneous, often absurd use of self.

B. Structural therapy (Minuchin)

Minuchin focused on joining, mapping and restructuring subsystems and boundaries rather than these two "battles."

C. Bowen systems therapy (Bowen)

Bowen's approach is calm, coaching and theory-driven, aimed at differentiation, not playful or provocative.

D. Validation model (Satir)

Satir was experiential too, but her model centers on self-esteem, communication stances and congruence.

Rationale: Carl Whitaker's symbolic-experiential family therapy values growth and creativity over technique. The therapist wins the battle for structure (control over the conditions of therapy) but must lose the battle for initiative, so that the family owns its decisions about change. Whitaker often worked with a co-therapist and used humor and "craziness" to unsettle rigid patterns.

References: Gladding (Counseling: A Comprehensive Profession) — Experiential family therapy: Whitaker, symbolic-experiential approach

☐ Approve ☐ Revise ☐ Retire Notes:

A man who prides himself on generosity becomes intensely angry at a coworker he calls "greedy and selfish," although his friends say he can be quite stingy himself. A Jungian counselor would MOST likely view his reaction as a projection of which archetype?

A. The persona

The persona is the public mask a person presents; his generous image is persona, but what he projects is the disowned side.

B. The shadow ✓ KEY

The shadow holds the traits a person rejects in himself; they are often projected onto others and met with strong emotion.

C. The anima

The anima is the feminine side of a man's psyche, not the store of disowned negative traits.

D. The Self

The Self is the archetype of wholeness and integration toward which individuation moves.

Rationale: Carl Jung proposed a collective unconscious containing archetypes shared across humanity, including the persona, shadow, anima/animus and Self. Disowned qualities in the shadow are commonly projected onto others, and a strong emotional charge toward someone is a clue. Jungian work aims at individuation, partly by recognizing and integrating the shadow.

References: Corey (Theory & Practice) — Psychoanalytic therapy: Jung's analytical psychology, archetypes and the shadow

☐ Approve ☐ Revise ☐ Retire Notes:

A client raised by emotionally unavailable parents eagerly reports each accomplishment to the counselor, seems to need admiring responses, and feels deflated and empty when the counselor responds neutrally. In Kohut's self psychology, the client is MOST likely seeking which kind of response?

A. An idealizing selfobject response

Idealizing needs involve merging with a calm, admired figure; the client wants to be admired, not to admire.

B. A twinship selfobject response

Twinship (alter ego) needs involve feeling essentially alike with another person, not being admired.

C. A mirroring selfobject response ✓ KEY

Mirroring needs are met by others who reflect back and admire the person's worth and achievements.

D. Splitting of good and bad objects

Splitting is an object relations concept (Klein, Kernberg) about keeping all-good and all-bad images apart, not a need for admiration.

Rationale: Heinz Kohut argued that a cohesive self develops when caregivers serve as selfobjects meeting three needs: mirroring (being admired), idealizing (merging with a strong, calm figure) and twinship (feeling alike). Unmet mirroring needs can leave adults hungry for admiration and fragile in self-esteem. Kohut's self psychology grew out of, and is often grouped with, object relations theory.

References: Corey (Theory & Practice) — Psychoanalytic therapy: contemporary trends, self psychology and object relations

☐ Approve ☐ Revise ☐ Retire Notes:

A counseling student is comparing Freud's psychosexual theory with Erikson's psychosocial theory. Which statement MOST accurately describes a key difference between them?

A. Erikson's stages extend across the entire life span ✓ KEY

Erikson's eight stages run from infancy through late adulthood, whereas Freud's stages end with the genital stage at puberty.

B. Erikson made sexual drives the core of every stage

Erikson shifted the emphasis from sexual drives to social and cultural tasks and the growth of identity.

C. Freud proposed eight stages ending in late adulthood

Freud described five psychosexual stages; the eight life-span stages belong to Erikson.

D. Freud stressed social crises more than instinctual drives

This reverses the theories; Freud stressed instinctual drives, and Erikson stressed psychosocial crises.

Rationale: Freud's five psychosexual stages (oral, anal, phallic, latency, genital) center on instinctual drives and end at adolescence. Erikson kept a stage framework but emphasized ego development, social relationships and culture, and he extended development across the whole life span with eight psychosocial crises from trust vs. mistrust to integrity vs. despair.

References: Berk (Development Through the Lifespan) — Psychoanalytic perspective: Freud's psychosexual and Erikson's psychosocial theories

☐ Approve ☐ Revise ☐ Retire Notes:

Describing a mistake she made at work, a client says aloud to herself, in a scolding tone that echoes her mother, "You should have known better. How careless can you be?" In Eric Berne's transactional analysis, which ego state is she speaking from?

A. Adult

The Adult ego state processes information objectively in the here and now; it does not scold.

B. Critical Parent ✓ KEY

The Critical (Controlling) Parent holds the rules, judgments and criticisms absorbed from caregivers.

C. Adapted Child

The Adapted Child complies with or rebels against parental messages; it receives the scolding rather than delivering it.

D. Free Child

The Free (Natural) Child is spontaneous, playful and expressive, not judgmental.

Rationale: Berne's transactional analysis describes three ego states: Parent (Critical and Nurturing), Adult and Child (Free and Adapted). Self-critical messages copied from caregivers come from the Critical Parent. TA also analyzes transactions between ego states, psychological games with hidden payoffs, and life scripts formed in childhood.

References: Corey (Theory & Practice) — Transactional analysis: ego states, transactions, games and life scripts

☐ Approve ☐ Revise ☐ Retire Notes:

Arnold Lazarus's multimodal therapy assesses clients across seven modalities summarized by the acronym BASIC I.D. What do the final letters "I.D." represent?

A. Insight and defense mechanisms

Insight and defenses are psychodynamic concepts and are not BASIC I.D. modalities.

B. Identity and dream content

Neither identity nor dreams is one of Lazarus's seven modalities.

C. Interpersonal and drugs/biology ✓ KEY

The last two modalities are Interpersonal relationships and Drugs/biological functions.

D. Imagery and developmental history

Imagery is the earlier "I" in BASIC; developmental history is not a modality.

Rationale: Lazarus's technically eclectic multimodal therapy assesses Behavior, Affect, Sensation, Imagery, Cognition, Interpersonal relationships and Drugs/biology. Counselors then choose techniques from any orientation to address problems in each modality, guided by research on what works rather than by a single theory.

References: Corey (Theory & Practice) — Behavior therapy: Lazarus's multimodal therapy and the BASIC I.D.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor prepares a newly hired paramedic for high-stress calls in three phases: first building an understanding of how stress works, then learning and rehearsing coping skills such as coping self-statements and relaxation, and finally applying them under graded, increasingly realistic stressors. Which approach is the counselor using?

A. Systematic desensitization

Wolpe's systematic desensitization pairs relaxation with an imagined fear hierarchy; it lacks the conceptualization and coping-skills phases.

B. Rational emotive imagery

Rational emotive imagery is an REBT technique for changing emotions by imagining a feared event while feeling a healthier emotion.

C. Cognitive processing therapy

Cognitive processing therapy treats PTSD by challenging stuck points about a past trauma, not by inoculating against future stress.

D. Stress inoculation training ✓ KEY

Meichenbaum's stress inoculation training moves through conceptualization, skills acquisition and rehearsal, and application and follow-through.

Rationale: Donald Meichenbaum's stress inoculation training "inoculates" clients against stress by exposing them to manageable doses after they have learned coping skills. Its three phases are conceptualization, skills acquisition and rehearsal, and application and follow-through. It builds on his self-instructional training, which teaches clients to replace self-defeating inner speech with coping self-statements.

References: Corey (Theory & Practice) — Cognitive behavior therapy: Meichenbaum's cognitive behavior modification and stress inoculation training

☐ Approve ☐ Revise ☐ Retire Notes:

A 46-year-old client returning to the workforce doubts she can learn the spreadsheet software her new job requires. According to Bandura, which source of self-efficacy is generally the MOST powerful way to strengthen her belief that she can succeed?

A. Watching a peer of similar age master the software

Vicarious experience (modeling) raises self-efficacy, especially with similar models, but less strongly than one's own success.

B. Encouragement from her counselor that she can do it

Verbal persuasion helps but is weaker and easily undone by later failure.

C. Succeeding herself at small, graded spreadsheet tasks ✓ KEY

Mastery (performance) experiences are the most powerful source of self-efficacy because they provide direct evidence of capability.

D. Feeling physically calmer when she sits down to work on it

Physiological and emotional states shape self-efficacy judgments, but they are the least direct of the four sources.

Rationale: Albert Bandura identified four sources of self-efficacy: mastery experiences, vicarious experiences (modeling), verbal persuasion, and physiological and emotional states. Mastery experiences are the strongest. Counselors therefore break goals into graded steps the client can succeed at, often after modeling and with encouragement.

References: Corey (Theory & Practice) — Behavior therapy: Bandura's social learning theory, modeling and self-efficacy

☐ Approve ☐ Revise ☐ Retire Notes:

A slot machine pays out after an unpredictable number of plays, averaging about one win every 20 pulls. Which reinforcement schedule is this, and how does it typically affect extinction?

A. Fixed interval; quick to extinguish once payouts stop

A fixed-interval schedule reinforces the first response after a set time; payouts here depend on the number of plays.

B. Variable ratio; highly resistant to extinction ✓ KEY

Reinforcement after an unpredictable number of responses is a variable-ratio schedule, which produces high, steady responding that is very hard to extinguish.

C. Fixed ratio; very resistant because it is predictable

A fixed ratio pays after a set number of responses; its predictability makes it less resistant to extinction than a variable ratio.

D. Variable interval; extinguishes faster than continuous

Payouts here depend on responses, not time, and intermittent schedules resist extinction more than continuous reinforcement.

Rationale: Skinner's intermittent schedules are fixed ratio, variable ratio, fixed interval and variable interval. Variable-ratio reinforcement produces the highest, steadiest response rates and the greatest resistance to extinction, which is why gambling is so persistent. Continuous reinforcement speeds learning but extinguishes fastest.

References: Corey (Theory & Practice) — Behavior therapy: operant conditioning and schedules of reinforcement

☐ Approve ☐ Revise ☐ Retire Notes:

After being bitten by a neighbor's German shepherd, a 7-year-old now becomes frightened around other large dogs, yet he stays relaxed around small dogs and cats. In classical conditioning terms, his fear of other large dogs and his calm around small dogs reflect, respectively, which two processes?

A. Spontaneous recovery; stimulus generalization

Spontaneous recovery is the return of an extinguished response after a rest period; nothing here was extinguished.

B. Stimulus discrimination; second-order conditioning

This reverses the first process, and second-order conditioning pairs a new neutral stimulus with an existing conditioned stimulus.

C. Stimulus generalization; stimulus discrimination ✓ KEY

Fear spreading to similar stimuli is generalization; responding differently to dissimilar stimuli is discrimination.

D. Higher-order conditioning; response extinction

His calm around small dogs is not extinction, since that fear was never conditioned and then weakened.

Rationale: In Pavlov's model the bite is the unconditioned stimulus, fear the unconditioned response, and the German shepherd becomes a conditioned stimulus. Stimulus generalization occurs when stimuli similar to the conditioned stimulus also elicit the response; stimulus discrimination occurs when the person responds only to stimuli close enough to the original.

References: Corey (Theory & Practice) — Behavior therapy: classical conditioning, generalization and discrimination

☐ Approve ☐ Revise ☐ Retire Notes:

To teach a 4-year-old with a language delay to say "water," a therapist first reinforces any vocal sound, then only "wa," then "wa-ter," and finally only the complete word. Which behavioral technique is the therapist using?

A. Chaining

Chaining links a series of already-learned behaviors into a sequence, such as the steps of handwashing.

B. Token economy

A token economy delivers conditioned reinforcers (tokens) that are later exchanged for backup rewards; it is not defined by successive approximations.

C. Premack principle

The Premack principle uses a high-frequency activity to reinforce a lower-frequency one.

D. Shaping ✓ KEY

Shaping reinforces successive approximations of a target behavior, raising the criterion step by step.

Rationale: Shaping builds a new behavior by reinforcing closer and closer approximations to the goal while no longer reinforcing earlier, cruder forms. It is often combined with other operant tools, such as token economies, in which tokens act as generalized conditioned reinforcers exchanged later for backup reinforcers.

References: Corey (Theory & Practice) — Behavior therapy: operant techniques, shaping and token economies

☐ Approve ☐ Revise ☐ Retire Notes:

A client keeps telling herself, "I'm a failure." Rather than disputing the thought, her counselor asks her to restate it as "I'm having the thought that I'm a failure" and to notice the words without arguing with them. Which acceptance and commitment therapy (ACT) process is the counselor targeting?

A. Cognitive restructuring

Cognitive restructuring is a traditional CBT strategy that tests and replaces the thought; ACT changes the client's relationship to the thought instead.

B. Cognitive defusion ✓ KEY

Defusion helps clients see thoughts as passing words and images rather than literal truths, loosening their grip on behavior.

C. Committed action

Committed action is taking concrete steps in line with chosen values, not changing how one relates to a thought.

D. Values clarification

Values work identifies what matters most to the client; this exercise targets her relationship to a thought.

Rationale: Steven Hayes's ACT targets psychological flexibility through six hexaflex processes: acceptance, cognitive defusion, contact with the present moment, self-as-context, values and committed action. Defusion techniques, such as adding "I'm having the thought that," create distance from thoughts without trying to change their content.

References: Corey (Theory & Practice) — Third-wave behavior therapies: acceptance and commitment therapy, cognitive defusion

☐ Approve ☐ Revise ☐ Retire Notes:

Mindfulness-based cognitive therapy (MBCT), developed by Segal, Williams and Teasdale, was designed primarily for which purpose?

A. Preventing relapse in people with recurrent depression ✓ KEY

MBCT was built to reduce relapse in people who have had repeated major depressive episodes and are currently in remission.

B. Treating acute mania during inpatient hospitalization

MBCT is a group program for people who are currently well, not an acute treatment for mania.

C. Reducing self-harm in clients with borderline traits

This describes dialectical behavior therapy, which also uses mindfulness but was developed by Linehan.

D. Replacing exposure as the core treatment for specific phobias

Exposure remains the core treatment for specific phobias; MBCT was not developed for them.

Rationale: MBCT combines mindfulness practice, adapted from Kabat-Zinn's MBSR, with cognitive therapy in an 8-week group format. It teaches people with recurrent depression to notice early negative thinking and relate to it from a decentered stance, interrupting the rumination that can trigger relapse. Its strongest evidence is for people with three or more prior episodes.

References: NICE guidelines — Depression in adults (NG222): relapse prevention with mindfulness-based cognitive therapy

☐ Approve ☐ Revise ☐ Retire Notes:

In William Glasser's choice theory, all behavior is an attempt to satisfy five basic needs. Which list names them?

A. Safety, esteem, belonging, purpose, and self-actualization

This resembles Maslow's hierarchy of needs, not Glasser's five basic needs.

B. Survival, attachment, mastery, autonomy, and meaning

This mixes concepts from attachment and self-determination theories; Glasser's list includes power, freedom and fun.

C. Love, work, play, spirituality, and self-direction

These echo Adlerian life tasks and wellness models rather than choice theory.

D. Survival, love and belonging, power, freedom, and fun ✓ **KEY**

Glasser named survival, love and belonging, power (achievement), freedom and fun as the five basic needs.

Rationale: Choice theory holds that people choose behaviors to meet five genetically based needs: survival, love and belonging, power or achievement, freedom, and fun. Glasser saw love and belonging as central. Reality therapy helps clients evaluate whether what they are doing is meeting these needs and plan more effective choices.

References: Corey (Theory & Practice) — Reality therapy: choice theory and the five basic needs

☐ Approve ☐ Revise ☐ Retire Notes:

356. nce-s-cou-108 · Counseling and Helping Relationships · psychodynamic and psychoanalytic theory · hard · In review

A counselor uses a time-limited psychodynamic approach (such as those of Mann or Strupp and Binder) with a client who has recurring conflicts with authority figures. How does this approach MOST differ from traditional long-term psychoanalysis?

A. Transference is avoided so the work stays short-term

Brief dynamic therapists actively use transference, especially as it relates to the focal pattern.

B. Free association becomes the counselor's main tool

Unstructured free association is typical of classical analysis; brief therapy is more focused and active.

C. Work centers on one focal conflict or relational pattern ✓ **KEY**

Time-limited dynamic therapies select a single central issue or cyclical maladaptive pattern and keep the work on it.

D. The ending date is left open until the client feels ready

A set time limit, often agreed at the start (Mann used 12 sessions), is a defining feature, and the ending itself is used therapeutically.

Rationale: Brief psychodynamic therapies keep psychoanalytic ideas such as transference and unconscious conflict but limit the number of sessions, have the counselor take a more active role, and select a circumscribed focus, such as Mann's central issue or Strupp and Binder's cyclical maladaptive pattern. The fixed ending is used to work through issues of separation and loss.

References: Corey (Theory & Practice) — Psychoanalytic therapy: brief psychodynamic therapy and time-limited dynamic psychotherapy

☐ Approve ☐ Revise ☐ Retire Notes:

A reluctant 16-year-old says only that last night "was bad" because of an argument with his mother. The counselor wants to invite him to tell the story in his own words. Which counselor question BEST serves that goal?

A. Were you upset by the argument?

A closed question invites a yes or no and limits how much he says.

B. Why did you yell at her?

"Why" questions can sound accusatory and prompt defensiveness; it also assumes he yelled.

C. Was it about curfew or grades?

An either/or question is closed and imposes the counselor's guesses about the topic.

D. What happened during the argument? ✓ KEY

An open "what" question invites him to describe events in his own words and at his own depth.

Rationale: Open questions, often starting with "what" or "how," encourage clients to elaborate and lead the content. Closed questions are useful for specific facts but narrow the conversation, and "why" questions often provoke defensiveness. Once a client is talking, minimal encouragers such as "mm-hm" help keep the story going.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: open and closed questions

☐ Approve

☐ Revise

☐ Retire

Notes:

A grieving client finishes describing her mother's final days in hospice, then stops speaking and looks down with tears in her eyes. What is the MOST helpful counselor response in that moment?

A. Allow a silence so she can stay with what she feels ✓ KEY

A respectful, attentive silence gives her room to feel and reflect and signals that the counselor can stay present with her pain.

B. Ask a new question so the pause does not grow awkward

Filling the pause usually serves the counselor's discomfort and pulls her away from her feelings.

C. Offer reassurance that her mother is no longer suffering

Premature reassurance can minimize her grief and close down the emotion she is experiencing.

D. Summarize the session so far and move to coping planning

Shifting to planning at a moment of deep feeling is mistimed and can feel dismissive.

Rationale: Purposeful silence is an attending skill. It allows clients to experience emotion, gather thoughts and decide what to say next, and it communicates acceptance. Beginning counselors often rush to fill silences; the skill lies in reading whether a silence is productive (reflective) or a sign the client is stuck or confused.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: use of silence and attending

☐ Approve

☐ Revise

☐ Retire

Notes:

A client in early recovery asks his counselor, "Have you ever struggled with addiction yourself?" The counselor has been in recovery for many years. Which guideline BEST governs whether and how she self-discloses?

A. Share her own recovery story in full detail, since shared experience builds credibility

Lengthy disclosure shifts the focus to the counselor and can burden the client, whatever its intent.

B. Disclose only if it serves the client, keep it brief, and return the focus to him ✓ KEY

Helpful self-disclosure is purposeful, brief, relevant to the client's goals, and followed by a quick return of the focus to the client.

C. Decline every personal question, because any disclosure is a boundary violation

Self-disclosure is a recognized skill; used sparingly and for the client's benefit, it is not a boundary violation.

D. Disclose only after he asks a second time, to confirm that his interest is sincere

Making him ask twice is not a recognized guideline; the test is whether disclosure serves his welfare.

Rationale: Counselor self-disclosure should be infrequent, brief, relevant and offered for the client's benefit rather than the counselor's needs. After disclosing, the counselor returns the focus to the client, for example by asking how the disclosure lands. Counselors should consider the client's culture, timing and whether the disclosure could blur roles.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: counselor self-disclosure

☐ Approve ☐ Revise ☐ Retire Notes:

360. nce-s-cou-112 · Counseling and Helping Relationships · family systems theories (Bowen, structural, strategic) · hard · In review

A father calls his 15-year-old "stubborn," and the counselor replies, "He sounds determined to stand by what he believes." Later, the counselor suggests the son's school refusal may be his way of keeping an eye on a parent he worries about. Which statement BEST distinguishes the two techniques?

A. The first is reframing, which recasts a whole situation; the second is relabeling a single trait

This reverses the terms; swapping one adjective is relabeling, and recasting the meaning of a situation is reframing.

B. The first is interpretation of a defense; the second is a paradoxical prescription

Neither statement interprets a defense or tells anyone to continue the symptom.

C. Both are confrontations of discrepancies between the family's words and actions

Confrontation points out a discrepancy; these statements offer new, more positive meanings instead.

D. The first relabels a negative trait; the second reframes what the behavior means ✓ KEY

Relabeling replaces a negative descriptor with a positive one, while reframing places a behavior in a new context that changes its meaning.

Rationale: Both techniques change perception without changing facts. Relabeling attaches a more positive label to a person or trait ("stubborn" becomes "determined"). Reframing offers a different frame for a whole pattern, often giving a symptom a protective or caring function, which can open new options for the family. Reframing is especially associated with strategic and MRI approaches.

References: Gladding (Counseling: A Comprehensive Profession) — Family counseling techniques: reframing and relabeling

► **Check:** Verify the reframing vs. relabeling distinction against the Gladding glossary, since some texts treat relabeling as a subtype of reframing.

☐ Approve ☐ Revise ☐ Retire Notes:

Pamela Hays's ADDRESSING framework helps counselors consider the many cultural influences shaping a client's identity. In this acronym, what does the first "D" stand for?

A. Developmental disabilities ✓ **KEY**

The first D is developmental disabilities; the second D is disabilities acquired later in life.

B. Dominant-group status

Privilege runs through the whole framework, but dominant-group status is not a letter in the acronym.

C. Diagnostic history

Diagnostic history is a clinical, not cultural, factor and is not part of ADDRESSING.

D. Dialect and language

Language is considered under national origin and ethnicity; it is not one of the letters.

Rationale: ADDRESSING stands for Age and generational influences, Developmental disabilities, Disabilities acquired later in life, Religion and spiritual orientation, Ethnic and racial identity, Socioeconomic status, Sexual orientation, Indigenous heritage, National origin and Gender. Hays designed it to help counselors see clients' intersecting identities and to examine where the counselor holds privilege.

References: Sue & Sue (Counseling the Culturally Diverse) — Multidimensional identity: Hays's ADDRESSING framework

► **Check:** Confirm that the cited Sue & Sue edition discusses Hays's ADDRESSING framework; if not, cite a text that does.

☐ Approve ☐ Revise ☐ Retire Notes:

After years of being passed over at work despite strong reviews, a client says he has little control over his career because of systemic discrimination, and he places responsibility for his situation on that system rather than on himself. In Derald Wing Sue's worldview model, which quadrant BEST fits?

A. Internal control, internal responsibility

IC-IR reflects the mainstream U.S. belief that people control their outcomes and are to blame for failures.

B. External control, external responsibility ✓ **KEY**

EC-ER describes someone who sees outcomes as controlled by others and attributes problems to systemic forces such as discrimination.

C. External control, internal responsibility

EC-IR describes someone who has little control yet blames himself, often linked to internalized oppression.

D. Internal control, external responsibility

IC-ER describes someone who feels able to shape his life while recognizing systemic barriers, not one who feels little control.

Rationale: Sue crossed locus of control (internal or external) with locus of responsibility (internal or person-centered vs. external or system-centered) to form four worldviews. Counselors who assume an IC-IR view may misread an EC-ER client as resistant or blaming. Understanding the client's worldview, and the real barriers behind it, guides culturally responsive goals such as advocacy.

References: Sue & Sue (Counseling the Culturally Diverse) — Worldview: locus of control and locus of responsibility quadrants

☐ Approve ☐ Revise ☐ Retire Notes:

At intake, a counselor briefly asks a Latina client whether she minds working with a counselor of a different ethnicity. He never returns to culture, even when she later describes ethnic bias from her supervisor. In Day-Vines and colleagues' broaching continuum, which style does this MOST reflect?

A. Avoidant

An avoidant counselor does not raise race or culture at all; this counselor did raise it once.

B. Infusing

An infusing counselor builds attention to culture and social justice into all of their work, well beyond the session.

C. Isolating ✓ KEY

An isolating counselor broaches culture in a superficial, one-time way to satisfy an obligation, then drops it.

D. Integrated/congruent

An integrated/congruent counselor broaches routinely and links the client's cultural experiences to the presenting concerns.

Rationale: Broaching is the counselor's consistent, open invitation to explore how race, ethnicity and culture bear on the client's concerns. Day-Vines and colleagues described a continuum of broaching styles: avoidant, isolating, continuing/incongruent, integrated/congruent and infusing. Raising culture once and never connecting it to the client's experience is the isolating style.

References: Sue & Sue (Counseling the Culturally Diverse) — Broaching race and culture in counseling: Day-Vines broaching continuum

► **Check:** Verify the broaching style names and order against Day-Vines et al. (2007) and confirm Sue & Sue covers them.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor deployed to a family assistance center after a mass shooting will use Psychological First Aid (PFA) as developed by the National Child Traumatic Stress Network and National Center for PTSD. Which practice is NOT one of the PFA core actions?

A. Linking survivors with needed collaborative services

Linkage with collaborative services is one of the eight PFA core actions.

B. Gathering information on current needs and worries

Information gathering about current needs and concerns is a PFA core action.

C. Offering information on coping with stress reactions

Providing information on coping is a PFA core action.

D. Asking survivors to recount the event in detail ✓ KEY

PFA does not require survivors to describe the event in detail; pressing for a detailed account can be distressing and resembles debriefing, which PFA avoids.

Rationale: PFA's eight core actions are contact and engagement, safety and comfort, stabilization, information gathering on current needs and concerns, practical assistance, connection with social supports, information on coping, and linkage with collaborative services. It is evidence-informed, flexible and non-intrusive, and it does not push survivors to retell or process their trauma.

References: Psychological First Aid Field Operations Guide (NCTSN/NCPTSD) — The eight PFA core actions

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor using trauma-focused cognitive behavioral therapy (TF-CBT) with a 9-year-old has completed psychoeducation and parenting skills, relaxation, affective modulation and cognitive coping. Following the PRACTICE sequence, which component comes NEXT?

A. In vivo mastery of trauma reminders

In vivo mastery follows the trauma narrative and is used only when the child avoids safe reminders.

B. Conjoint child-parent sessions

Conjoint sessions, often used to share the narrative with the caregiver, come after the narrative is developed.

C. Enhancing future safety

Enhancing safety is the final component of the PRACTICE sequence.

D. Trauma narrative and processing ✓ KEY

After the stabilization skills, the child gradually develops and processes a narrative of the trauma.

Rationale: TF-CBT, developed by Cohen, Mannarino and Deblinger, follows the PRACTICE components: Psychoeducation and Parenting skills, Relaxation, Affective modulation, Cognitive coping, Trauma narrative and processing, In vivo mastery, Conjoint child-parent sessions and Enhancing safety. Coping skills are built first so the child can tolerate the gradual exposure of the trauma narrative.

References: NICE guidelines — PTSD (NG116): trauma-focused CBT for children and young people

► **Check:** The PRACTICE order is from Cohen, Mannarino and Deblinger; confirm NICE NG116 is an acceptable citation or swap to a source that describes the model.

☐ Approve ☐ Revise ☐ Retire Notes:

After a year at a child advocacy center, a counselor notices lasting changes in her own beliefs: she now sees the world as unsafe, distrusts adults around her own children, and has intrusive images drawn from clients' accounts. Which term BEST describes her experience?

A. Burnout

Burnout is exhaustion, cynicism and reduced effectiveness from workload and work conditions; it need not involve trauma-related changes in beliefs.

B. Vicarious trauma ✓ KEY

Vicarious trauma is a cumulative change in the helper's inner world, such as beliefs about safety and trust, from empathic engagement with clients' trauma.

C. Compassion satisfaction

Compassion satisfaction is the positive fulfillment helpers draw from their work, the opposite of her experience.

D. Countertransference

Countertransference is the counselor's reaction to a particular client rooted in her own history, not a cumulative shift in worldview.

Rationale: Vicarious trauma (McCann and Pearlman) is a lasting shift in a helper's beliefs about safety, trust and control that results from repeated exposure to clients' trauma. Compassion fatigue (Figley) emphasizes the emotional cost of caring, including secondary traumatic stress, while burnout stems from work conditions. Remedies include supervision, caseload balance, peer support and personal self-care.

References: Gladding (Counseling: A Comprehensive Profession) — Counselor wellness: burnout, compassion fatigue and vicarious trauma · ACA Code of Ethics — C.2.g Impairment

☐ Approve ☐ Revise ☐ Retire Notes:

While talking about an assault, a client begins to dissociate: she stares blankly, speaks in a monotone, and says the room feels "far away." What should the counselor do FIRST?

A. Guide her to name things she can see and feel her feet on the floor ✓ KEY

Grounding techniques that engage the senses and body bring her back into the present moment and her window of tolerance.

B. Encourage her to keep going so the memory can be fully processed today

Continuing while she is dissociated is unlikely to be therapeutic and can overwhelm her further.

C. Ask her to close her eyes and picture the memory in greater detail

Closing her eyes and deepening the memory increases the risk of further dissociation.

D. Interpret her dissociation as resistance to the work of the session

Dissociation is a protective response to overwhelming arousal, not resistance to be interpreted.

Rationale: When a client dissociates or becomes flooded, trauma-informed counselors first re-establish safety and present-moment awareness through grounding, such as the 5-4-3-2-1 senses exercise, feeling the feet on the floor, holding an object or naming the date and place. Trauma processing resumes only when the client is back within her window of tolerance.

References: Gladding (Counseling: A Comprehensive Profession) — Crisis and trauma counseling: grounding and stabilization

☐ Approve ☐ Revise ☐ Retire Notes:

In eye movement desensitization and reprocessing (EMDR), the client holds the disturbing image, negative belief and body sensations in mind while the counselor leads sets of eye movements, and distress ratings are taken until they fall toward zero. In Shapiro's eight-phase protocol, which phase is this?

A. Preparation

Preparation builds the relationship, explains EMDR and teaches calming skills before memories are processed.

B. Assessment

Assessment identifies the target image, negative and positive cognitions and baseline ratings without bilateral stimulation.

C. Desensitization ✓ KEY

Desensitization uses sets of bilateral stimulation while the client attends to the target, until the SUD rating approaches 0.

D. Reevaluation

Reevaluation happens at the start of the next session to check whether earlier gains have held.

Rationale: Francine Shapiro's EMDR protocol has eight phases: history taking, preparation, assessment, desensitization, installation, body scan, closure and reevaluation. Bilateral stimulation (eye movements, taps or tones) is used during desensitization, installation of the positive cognition and the body scan. EMDR is recommended by several PTSD practice guidelines.

References: APA CPG — PTSD guideline (2017): EMDR as a conditionally recommended psychotherapy

☐ Approve ☐ Revise ☐ Retire Notes:

Core Counseling Attributes

32 items · NCE weight 8% (13 of 160 scored items)

369. nce-s-cor-001 · Counseling and Helping Relationships · empathy and its levels · medium · In review

A 34-year-old client says, "My boss praised my project in front of everyone, but then gave the promotion to someone else. I don't know what to think." Which counselor response is an interchangeable (Level 3) response on Carkhuff's empathy scale?

A. You should ask your boss directly why you were passed over this time.

Advice-giving ignores the client's feelings entirely; it is subtractive (Level 1 or 2), not interchangeable.

B. You feel confused because the praise and the promotion decision don't match. ✓ **KEY**

This captures the feeling (confusion) and its stated cause at the same depth the client expressed, which is the definition of a Level 3 interchangeable response.

C. Underneath the confusion, you may fear that your hard work goes unrecognized.

This goes beyond what the client expressed to an underlying feeling and meaning, making it additive (Level 4), not interchangeable.

D. Lots of people get passed over at work, and it usually works out fine in the end.

Reassurance minimizes the experience and shifts away from the client's feelings, a subtractive response.

Rationale: Carkhuff's scale rates counselor responses from 1 to 5. Level 3 is the minimally facilitative, interchangeable level: the counselor reflects essentially the same feeling and meaning the client expressed, adding nothing and taking nothing away. Levels 1 and 2 subtract from the client's message, and Levels 4 and 5 add depth beyond it.

References: Gladding (Counseling: A Comprehensive Profession) — Carkhuff's scale of empathic understanding, Levels 1-5

☐ Approve ☐ Revise ☐ Retire Notes:

In a sixth session, a graduate student says, "I keep telling everyone I'm fine with moving home after graduation. My parents are happy, so I guess I should be too." Which counselor response BEST illustrates additive (Level 4) empathy?

A. So you're moving home after graduation, and your parents are pleased with it.

This paraphrases the content but omits feeling; it is at best Level 2, slightly subtractive.

B. You feel fine about moving home, since your parents are happy with the plan.

This repeats the client's surface statement and misses the doubt in "I guess I should be"; it does not add depth.

C. Why do you think your parents' happiness matters so much in this decision?

A why-question invites intellectualizing and does not communicate understanding of the feeling, so it is not an empathic response.

D. You sound unsure, as if your own wishes got set aside to keep your parents happy. ✓ KEY

This names the implicit ambivalence and the meaning beneath it (setting aside one's own wishes), adding accurately to what the client said: Level 4.

Rationale: Additive empathy (Carkhuff Levels 4-5) goes beyond the client's explicit words to reflect feelings and meanings the client implied but did not fully state, which helps the client explore at a deeper level. To be additive rather than inaccurate, the reflection must stay grounded in cues the client gave, such as the hedged "I guess I should be."

References: Gladding (Counseling: A Comprehensive Profession) — Advanced (additive) empathy, Carkhuff Levels 4-5

► **Check:** Option A is rated Level 2 in its rationale; raters may disagree on the exact level (it is clearly not additive).

☐ Approve ☐ Revise ☐ Retire Notes:

On Carkhuff's five-level scale of empathic understanding, a counselor response rated at Level 1 or Level 2 is described as which of the following?

A. Subtractive, detracting from what the client expressed ✓ KEY

Levels 1 and 2 are subtractive: they ignore or only partly capture the client's feelings and meaning.

B. Interchangeable, matching the client's expressed feeling

Interchangeable, minimally facilitative responses are Level 3.

C. Additive, reflecting feelings below the client's words

Additive responses that reach underlying feelings are Levels 4 and 5.

D. Facilitative, the baseline goal for beginning counselors

Level 3 is the minimally facilitative baseline counselors aim to reach consistently; Levels 1-2 fall below it.

Rationale: Carkhuff's scale runs from Level 1 (the response ignores or contradicts the client's feelings) to Level 5 (the response adds significantly to the client's awareness). Levels 1-2 are subtractive, Level 3 is interchangeable and minimally facilitative, and Levels 4-5 are additive.

References: Gladding (Counseling: A Comprehensive Profession) — Carkhuff's scale of empathic understanding

☐ Approve ☐ Revise ☐ Retire Notes:

A client tearfully describes the recent death of her dog. Which counselor response reflects empathy rather than sympathy?

A. I'm so sorry; I lost my own dog last year and I still cry about it sometimes too.

This shifts the focus to the counselor's own loss and expresses pity or shared distress, which is sympathy and self-disclosure, not empathy.

B. That is such a sad thing to go through, and I feel terrible for you today.

Feeling sorry for the client is sympathy; it conveys the counselor's feeling rather than understanding of the client's.

C. Losing her has left a real emptiness in your days, and the grief feels heavy. ✓ **KEY**

This communicates accurate understanding of the client's own experience from her frame of reference, which is empathy.

D. At least she had a long life with you, and you gave her a very good home.

"At least" reassurance minimizes the loss and moves away from the client's feelings.

Rationale: Empathy is understanding the client's world as if it were one's own, without losing the "as if" quality, and communicating that understanding back. Sympathy centers on the helper's own feelings of sorrow or pity for the client, which can shift the focus away from the client.

References: Corey (Theory & Practice) — Person-centered therapy: accurate empathic understanding

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old client says, "I yelled at my teenage son last night and called him useless. I'm ashamed to even tell you." Which counselor response BEST demonstrates unconditional positive regard?

A. That was harsh, but I'm sure you are a good parent most of the time anyway.

Reassurance about being a "good parent" is conditional praise; it evaluates the client rather than accepting the client.

B. Calling him useless can do real damage, so let's plan what to say instead.

This jumps to evaluation and correction before the client feels accepted; it is not an expression of UPR.

C. It took courage to share that, and I'm glad you trusted me with it here. ✓ **KEY**

This accepts the client as a person, without approving or condemning the behavior, and values the client's openness, which is UPR.

D. Don't be ashamed, since all parents lose their temper and say things they regret.

Telling the client not to feel shame dismisses the feeling and normalizes too quickly; it is reassurance, not acceptance.

Rationale: Rogers described unconditional positive regard as a warm acceptance of the client as a person, without conditions of worth. UPR does not mean approving of every behavior; it means the client's worth is not contingent on what is disclosed, which lowers defensiveness and supports self-exploration.

References: Corey (Theory & Practice) — Person-centered therapy: unconditional positive regard and acceptance

☐ Approve ☐ Revise ☐ Retire Notes:

Midway through a session, a client asks, "You look distracted. Are you even listening to me?" The counselor realizes her attention had drifted. Which response BEST demonstrates congruence?

A. You're right, my mind wandered for a moment. I'm with you now; please go on. ✓ KEY

The counselor's words match her inner experience, she owns it briefly, and she returns the focus to the client, which is congruence.

B. Of course I'm listening, and I have been following everything you've said.

Denying a lapse the counselor knows occurred is incongruent; outward words do not match inner experience.

C. It sounds like you worry people don't listen to you. Tell me more about that.

Reflecting the client's concern here deflects from the counselor's actual lapse and avoids genuineness.

D. I've had a long, stressful day with other clients, so I'm tired and distracted.

This is honest but over-discloses and shifts the focus to the counselor's needs rather than the relationship.

Rationale: Congruence (genuineness) means the counselor's outward behavior is consistent with inner experience, without a professional facade. Rogers viewed it as the most basic of the core conditions. Being genuine does not require disclosing every feeling; the counselor shares what is relevant to the relationship and keeps the focus on the client.

References: Corey (Theory & Practice) — Person-centered therapy: congruence or genuineness

☐ Approve ☐ Revise ☐ Retire Notes:

Bordin's pantheoretical model of the working alliance consists of which three components?

A. Empathy, congruence, and positive regard

These are Rogers' core conditions, not Bordin's alliance components.

B. Rapport, insight, and corrective feedback

These are general therapy processes, not the components Bordin named.

C. Trust, transference, and client motivation

Transference is a psychodynamic concept; Bordin's model was explicitly pantheoretical.

D. Agreement on goals, tasks, and the bond ✓ KEY

Bordin defined the working alliance as agreement on goals, agreement on tasks, and the emotional bond between counselor and client.

Rationale: Bordin (1979) proposed that the working alliance has three parts: agreement on the goals of counseling, agreement on the tasks used to reach them, and the affective bond between counselor and client. Because it is pantheoretical, the model applies across approaches, and alliance strength is one of the most consistent predictors of outcome.

References: Gladding (Counseling: A Comprehensive Profession) — The working alliance: goals, tasks, and bond

☐ Approve ☐ Revise ☐ Retire Notes:

In the fourth session, a client who has been completing CBT thought records says flatly, "I did the homework again, I guess. Whatever helps you." The client has arrived late twice. What should the counselor do FIRST?

A. Review the thought record in detail and praise the client for completing it

Proceeding with the task ignores the withdrawal cues and leaves a possible rupture unaddressed.

B. Invite the client to talk about how the work together is feeling so far ✓ **KEY**

Openly exploring the relationship addresses a likely withdrawal rupture and checks agreement on tasks and goals before continuing.

C. Set a lateness policy so that attendance does not continue to slip further

Focusing on rules treats the lateness as a compliance issue and can deepen the rupture.

D. Switch to a different intervention, since thought records are not working

Changing tasks unilaterally skips collaboration; the counselor does not yet know what the client wants or is reacting to.

Rationale: Flat compliance ("whatever helps you") and lateness suggest a withdrawal rupture in the alliance, often reflecting disagreement about tasks or goals or strain in the bond. Research on rupture and repair supports raising the relationship directly and nondefensively, which lets the counselor renegotiate tasks and goals collaboratively. Successfully repaired ruptures are associated with better outcomes.

References: Gladding (Counseling: A Comprehensive Profession) — Working alliance: goals, tasks, bond; addressing alliance ruptures

► **Check:** Rupture-repair evidence (Safran et al.) is cited to a general Gladding alliance reference.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor who has completed many multicultural trainings begins work with a Hmong American client. According to Hook and colleagues' model of cultural humility, which stance BEST reflects cultural humility?

A. Ask the client what their culture means to them and stay open to being corrected ✓ **KEY**

Cultural humility is an other-oriented, curious stance that treats the client as the expert on their own cultural identity.

B. Draw on prior training about Hmong customs to anticipate the client's values

Applying general group knowledge as a template risks stereotyping; it reflects a knowledge-based, not humble, stance.

C. Set cultural issues aside unless the client raises them first in the session

Waiting for the client to raise culture places the burden on the client and can signal that culture is unwelcome.

D. Treat the training already completed as sufficient competence for this client

Viewing competence as a finished achievement contradicts the ongoing, lifelong nature of humility.

Rationale: Hook et al. (2013) described cultural humility as an interpersonal stance that is other-oriented rather than self-focused, marked by respect, openness, and lack of superiority regarding the client's cultural background. It is an ongoing process of self-reflection rather than a fixed body of knowledge, and clients' perceptions of it are associated with a stronger alliance.

References: Sue & Sue (Counseling the Culturally Diverse) — Cultural humility as an other-oriented stance

► **Check:** Confirm the cited Sue & Sue edition discusses Hook et al.'s cultural humility model.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor notices that she schedules a client who lives in public housing for shorter sessions and assigns fewer homework tasks, assuming the client "won't follow through." What is the MOST appropriate next step?

A. Refer the client to a counselor who has more experience with low-income clients

Referral based on the counselor's bias, rather than competence limits, abandons the client and avoids the counselor's own work.

B. Continue as planned, since shorter sessions may reduce the client's burden

This rationalizes the bias instead of examining it and keeps treating the client differently.

C. Examine the assumption in supervision and restore an equitable treatment plan ✓ **KEY**

Recognizing an implicit bias, exploring it in supervision, and correcting its effect on care reflects counselor self-awareness.

D. Tell the client about the bias so the client can decide whether to continue

Disclosing the bias burdens the client with the counselor's issue; the counselor should first address it through supervision.

Rationale: Counselor self-awareness of attitudes, beliefs, and biases is the first dimension of multicultural competence. Biases about social class can shape clinical decisions without conscious intent. When a counselor notices differential treatment, the ethical response is to examine it, often through supervision or consultation, and correct its effect on services.

References: Sue & Sue (Counseling the Culturally Diverse) — Counselor awareness of own assumptions, values, and biases · ACA Code of Ethics — C.5 Nondiscrimination

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor notices he feels unusually irritated and impatient with an older male client who reminds him of his own critical father. This reaction is BEST described as

A. transference by the client

Transference is the client's projection of past relationships onto the counselor; here the reaction is the counselor's.

B. countertransference ✓ **KEY**

The counselor's emotional reaction to the client, shaped by his own unresolved history, is countertransference.

C. resistance in the client

Resistance refers to client behavior that impedes change, not the counselor's feelings.

D. compassion fatigue

Compassion fatigue is cumulative exhaustion from trauma exposure, not a reaction tied to a specific personal history.

Rationale: Countertransference refers to the counselor's emotional reactions to a client, particularly those rooted in the counselor's own unresolved conflicts or past relationships. It is not inherently harmful, but unexamined countertransference can distort the counselor's perceptions and responses, so recognizing it is the first step to managing it.

References: Corey (Theory & Practice) — Psychoanalytic therapy: transference and countertransference

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor whose sister died by overdose finds herself giving extra session time and her personal cell number to a young adult client in early recovery. What is the BEST course of action?

A. Keep the extra contact, since it clearly helps the client stay engaged

This keeps a boundary crossing driven by the counselor's needs rather than the client's welfare.

B. Transfer the client right away to avoid any further personal reactions

Abrupt transfer is premature; countertransference can often be managed and abrupt referral may harm the client.

C. Disclose the sister's death so the client understands her strong concern

This self-disclosure serves the counselor's needs and burdens the client.

D. Seek supervision on the reaction and restore usual session boundaries ✓ **KEY**

Recognizing the countertransference, processing it in supervision, and restoring boundaries protects the client and the work.

Rationale: Countertransference can show up as overinvolvement, rescuing, or boundary crossings that meet the counselor's needs. Effective management involves self-awareness, supervision or personal counseling, and returning to boundaries that serve the client. Referral is reserved for cases where the counselor cannot remain objective after these steps.

References: Corey (Theory & Practice) — Countertransference and the counselor as a person · ACA Code of Ethics — A.6.b Extending Conventional Boundaries

☐ Approve ☐ Revise ☐ Retire Notes:

A supervisor tells a trainee, "Every feeling you have toward a client, not just the conflicts from your own past, can be useful information about the client and the relationship." This statement reflects which view of countertransference?

A. Classical view, treating it as an obstacle to be eliminated

The classical (Freudian) view limits countertransference to the counselor's unresolved conflicts and treats it as interference.

B. Behavioral view, treating it as a learned response to extinguish

Countertransference is not framed as a conditioned response in behavioral theory; this distractor misattributes the concept.

C. Complementary view, limited to the client's projected roles

The complementary conception frames countertransference as the counselor's reaction to the interpersonal pull of the client's style, a narrower idea than all feelings toward the client.

D. Totalistic view, including all counselor reactions to clients ✓ **KEY**

The totalistic view defines countertransference as all of the counselor's emotional reactions and sees them as potentially useful data.

Rationale: The classical view, rooted in Freud, defines countertransference narrowly as the counselor's unconscious, conflict-based reactions to the client's transference and treats it as an impediment. The totalistic view broadens it to all of the counselor's emotional responses and treats them as a source of understanding. Contemporary integrative views keep the focus on conflict-based reactions while stressing that, once understood, they can inform the work.

References: Corey (Theory & Practice) — Psychoanalytic therapy: countertransference and its therapeutic use

► **Check:** Classical/totalistic/complementary views come from Gelso & Hayes; confirm Corey covers the totalistic view or swap the source.

☐ Approve ☐ Revise ☐ Retire Notes:

A college student says, "I've been hooking up with different people most weekends, and my roommate thinks I'm a mess." Which counselor response BEST reflects a nonjudgmental stance?

A. Your roommate may have a point, so let's look at the risks you're taking.

Siding with the roommate conveys the counselor's judgment before understanding the client's view.

B. Your roommate sees it one way. I'm curious how you feel about these weekends. ✓ **KEY**

This neither condemns nor endorses the behavior and invites the client's own perspective, which is nonjudgmental.

C. There's nothing wrong with that at all, and your roommate should not judge you.

Approval is also an evaluation; it imposes the counselor's values in the opposite direction.

D. What do you think is missing in your life that leads you to keep doing this?

This presumes the behavior signals a deficit, a judgment embedded in the question.

Rationale: A nonjudgmental stance means the counselor suspends both disapproval and approval and seeks to understand the client's experience from the client's frame of reference. Counselors avoid imposing their own values on clients, which allows clients to explore their choices without becoming defensive.

References: ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes:

A 70-year-old widower arrives early for his first session and seems nervous. Which counselor behavior BEST conveys warmth and respect?

A. Greet him by name, ask how he prefers to be addressed, and offer him a seat ✓ **KEY**

This shows genuine interest and respects his preferences and dignity, communicating warmth and respect.

B. Call him "sweetie" and speak slowly and loudly so he can follow along easily

Endearments and exaggerated speech are elderspeak; they are patronizing and disrespectful.

C. Hand him the intake forms and ask him to wait until his appointment time

This is procedural and distant, and it ignores his visible nervousness.

D. Tell him there is no need to be nervous because most clients feel fine later

This dismisses his feelings and offers reassurance in place of attunement.

Rationale: Warmth is conveyed through friendly, caring verbal and nonverbal behavior, and respect is shown by treating the client as a capable person with preferences and dignity. Asking how a client wishes to be addressed is a simple, culturally responsive way to communicate both, and avoiding ageist communication such as elderspeak is part of respect.

References: Gladding (Counseling: A Comprehensive Profession) — Building a counseling relationship: warmth and respect

☐ Approve ☐ Revise ☐ Retire Notes:

A White counselor working with a Black client hears the client say, "I wasn't sure I could talk about racism at work with you." Which counselor response BEST shows awareness of power and privilege in the relationship?

A. I don't really see color, so you can talk to me about anything you need to here.

Color-blind statements deny the relevance of race and the client's concern, which is a microinvalidation.

B. I've had many Black clients, so I understand a lot about what you face at work.

Claiming understanding based on experience with other clients overstates the counselor's knowledge and dismisses the concern.

C. As a White counselor I may miss things, and I'd like to hear it if you're willing. ✓ **KEY**

This names the racial difference and the limits of the counselor's perspective, and invites the client's experience without pressure.

D. Let's focus on what you can control at work, since we can't change the people there.

Redirecting away from racism minimizes a systemic experience and avoids the topic the client raised.

Rationale: Counselors from privileged groups hold both professional and social power. Broaching racial and cultural differences directly and nondefensively, and acknowledging the limits of one's own perspective, signals that the client's experiences of oppression are welcome. Color-blind or minimizing responses are common microaggressions that weaken trust.

References: Sue & Sue (Counseling the Culturally Diverse) — Power, privilege, and microaggressions in counseling; color-blindness

☐ Approve ☐ Revise ☐ Retire Notes:

385. nce-s-cor-017 · Counseling and Helping Relationships · empathy and its levels · medium · In review

A 41-year-old man ordered to counseling after a road-rage incident sits down and says, "This is a joke. The other guy cut me off and I'm the one stuck here wasting my Thursday." Which counselor response BEST conveys empathy?

A. The court decided you need to be here, so let's make good use of the hour today.

This sides with the court and moves to the agenda without acknowledging the client's feelings, a subtractive response.

B. Getting angry on the road is risky, and that is what we need to look at together.

This confronts the behavior before the client feels heard, which tends to increase defensiveness in a mandated client.

C. You're frustrated, because it feels unfair that you're the one paying the price. ✓ **KEY**

This accurately reflects the feeling (frustration) and its meaning (unfairness) from the client's frame of reference without agreeing or arguing.

D. I hear that, but many people find these sessions useful once they give them a try.

"I hear that, but" discounts the feeling and shifts to persuasion, which is reassurance rather than empathy.

Rationale: Empathy with an angry or mandated client means communicating understanding of the client's feelings and perspective, not defending the referral source or confronting the behavior first. An accurate reflection lowers defensiveness and builds the alliance needed for later work on the behavior that led to the referral.

References: Gladding (Counseling: A Comprehensive Profession) — Empathy and working with reluctant or resistant clients

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, "My sister borrowed money again and didn't pay it back. She's completely selfish, and anyone in my place would cut her off for good." Which counselor response communicates empathy WITHOUT signaling agreement with the client's judgment?

A. You feel hurt and fed up, and cutting her off seems like the only way to stop it. ✓ KEY

This reflects the client's feelings and how the situation looks to the client, conveying understanding without endorsing the conclusion that the sister is selfish.

B. She does sound selfish, and you'd be right to protect yourself from her by now.

This agrees with the client's evaluation of the sister, confusing empathy with agreement and taking sides.

C. Maybe your sister is struggling in ways that you might not know about yet, though.

Offering the sister's side challenges the client prematurely and communicates that the counselor does not understand the client.

D. Cutting off family is a big step, so let's think carefully before you decide.

This shifts to cautionary advice and skips reflecting the client's feelings, so it is not an empathic response.

Rationale: Empathy is understanding the client's world "as if" it were one's own while keeping the "as if" quality. It reflects the client's feelings and perceptions as the client experiences them; it neither agrees with nor disputes the client's judgments. Distinguishing empathy from agreement lets the counselor stay with the client without becoming an ally against third parties.

References: Corey (Theory & Practice) — Person-centered therapy: accurate empathic understanding and the "as if" quality

☐ Approve ☐ Revise ☐ Retire Notes:

A 29-year-old client who left a stable nursing job to work as a touring musician says she is happy with the choice, though money is tight. The counselor privately thinks the decision is unwise. Which approach BEST reflects unconditional positive regard and respect for client autonomy?

A. Point out the financial risks so the client can reconsider before it is too late

Steering the client toward the counselor's preferred choice imposes the counselor's values on a decision the client has not asked to revisit.

B. Accept her choice as hers to make and explore the goals she brings to counseling ✓ KEY

Valuing the client without conditions and following her own goals honors UPR and self-determination, even when the counselor would choose differently.

C. Praise the decision warmly so the client feels supported in following her dream

Praise is an evaluation too; UPR is acceptance of the person, not approval of choices, and the praise would be incongruent with the counselor's view.

D. Share the honest concern about the career change since genuineness matters most

Genuineness does not require voicing every personal opinion; volunteering disapproval of a lawful life choice imposes values.

Rationale: Unconditional positive regard means prizing the client as a person regardless of whether the counselor agrees with the client's choices. Combined with respect for self-determination, it means counselors do not impose personal values on clients' lawful life decisions and instead work on the goals clients bring.

References: Corey (Theory & Practice) — Person-centered therapy: unconditional positive regard and acceptance · ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes:

Rogers held that when a counselor provides unconditional positive regard, empathy, and congruence, clients move toward growth because of an innate drive to develop their capacities. He called this drive the

A. striving for superiority

Striving for superiority (or significance) is Adler's concept of the movement toward overcoming felt inferiority.

B. will to meaning

The will to meaning is Frankl's existential concept, central to logotherapy.

C. hierarchy of needs

The hierarchy of needs is Maslow's model of motivation, not Rogers' term for the growth drive.

D. actualizing tendency ✓ **KEY**

Rogers described the actualizing tendency as the organism's inherent tendency to develop its capacities in ways that maintain and enhance it.

Rationale: Person-centered theory rests on trust in the actualizing tendency, an inborn movement toward growth and fulfillment. The counselor does not direct change; instead, the core conditions create a climate in which the client's own actualizing tendency can operate and conditions of worth lose their hold.

References: Corey (Theory & Practice) — Person-centered therapy: the actualizing tendency

☐ Approve ☐ Revise ☐ Retire Notes:

A client describes years of failed fertility treatments. The counselor went through a similar loss and feels a wave of sadness. Which response BEST reflects genuineness and intentional use of self without inappropriate self-disclosure?

A. I'm moved hearing how much hope and heartbreak these years have held for you. ✓ **KEY**

The counselor shares an authentic, here-and-now reaction that is relevant to the client and keeps the focus on the client's experience.

B. I went through fertility treatments too, so I know exactly how painful this is.

This disclosure shifts focus to the counselor and claims identical experience, which can silence the client's own version of the loss.

C. Let's look at the coping strategies that have helped you through these years.

Moving straight to coping sidesteps the emotion in the room; the counselor masks her reaction rather than using it.

D. Many couples face this, and there are other paths to parenthood worth exploring.

Normalizing and problem-solving prematurely minimizes the grief and does not reflect a genuine, attuned response.

Rationale: Congruence means the counselor's outward expression matches inner experience, without a professional facade. Genuineness does not require telling one's personal history; intentional use of self means sharing reactions only when they serve the client. Disclosures of the counselor's own life should be brief, purposeful, and returned quickly to the client.

References: Corey (Theory & Practice) — Person-centered therapy: congruence; therapist self-disclosure

☐ Approve ☐ Revise ☐ Retire Notes:

During a video session, a client says, "On this screen it's hard to tell whether what I say really lands with you." Which counselor adjustment BEST supports genuineness in telehealth?

A. Ask more questions so the client can see the counselor is paying close attention

More questions do not convey the counselor's reactions and may feel like an interview rather than a genuine relationship.

B. Reassure the client that the counselor is listening carefully even if it is hard to see

Reassurance asks the client to trust without changing anything about how the counselor's presence comes across.

C. Put more reactions into words, since the camera filters out many nonverbal cues ✓ **KEY**

Verbalizing reactions compensates for the reduced nonverbal channel so the client can experience the counselor as present and authentic.

D. Suggest switching to phone sessions so that video issues stop being a distraction

Phone sessions remove even more nonverbal information and avoid the client's concern rather than addressing it.

Rationale: Video sessions reduce the nonverbal cues, such as posture, small facial changes, and eye contact, that usually convey a counselor's presence. To remain genuine and transparent, counselors can name their reactions more explicitly, look toward the camera, and check in about how the connection feels to the client.

References: Gladding (Counseling: A Comprehensive Profession) — Genuineness; technology and distance counseling

► **Check:** Telehealth genuineness guidance is general practice wisdom; confirm the cited Gladding chapter addresses technology-assisted counseling or swap the source.

☐ Approve ☐ Revise ☐ Retire Notes:

A 15-year-old brought to counseling by his mother crosses his arms and says, "She's the one who wanted this. I don't have anything to talk about." Which counselor response is MOST likely to begin building a working alliance?

A. Your mom is really worried about you, so it would help to tell me what has been going on.

This aligns the counselor with the parent's agenda and pressures the teen, weakening the bond.

B. Coming here wasn't your idea. I'm curious if anything would make this worth it to you. ✓ **KEY**

This acknowledges the teen's lack of choice and invites his own goals, building agreement on goals and the bond.

C. That's fine. We can just sit quietly until you decide you're ready to start talking.

Passive waiting leaves the teen without a reason to engage and does nothing to establish shared goals.

D. Most teens feel that way at first, but they usually end up liking counseling a lot.

Generalized reassurance dismisses his stated position and can sound like the counselor is selling something.

Rationale: Adolescents brought by parents often arrive reluctant. The alliance grows when the counselor acknowledges the teen's autonomy, separates from the parent's agenda, and invites the teen to name goals of his own. In Bordin's terms, this begins building agreement on goals and the bond.

References: Gladding (Counseling: A Comprehensive Profession) — Working alliance with reluctant clients and adolescents

☐ Approve ☐ Revise ☐ Retire Notes:

A court-mandated client on probation asks at intake, "So whatever I say in here goes straight to my probation officer, right?" The client has signed a release limited to attendance and progress reports. What is the BEST response?

- A. Reassure the client that sessions are confidential so that he will speak more freely
Promising full confidentiality is inaccurate given the signed release and would undermine trust when reports are sent.
- B. Tell the client that the probation officer will decide what is shared from sessions
The scope of disclosure is set by the release and law, not by the officer; this also cedes the client's informed role.
- C. Suggest that the client avoid sharing anything that could get him into legal trouble
This advises concealment and undermines the purpose of counseling instead of clarifying the actual limits.
- D. Explain exactly what the release permits to be shared and invite questions about it ✓ KEY
Transparent, specific information about the limits of confidentiality acknowledges the power imbalance and supports informed participation.

Rationale: Mandated clients face a real power imbalance: the counselor reports to a system that holds consequences over them. Counselors address this by explaining clearly, at the start and as needed, what will and will not be disclosed and to whom. Honest transparency about the counselor's dual accountability is the foundation for trust in mandated work.

References: ACA Code of Ethics — B.1.d Explanation of Limitations

☐ Approve ☐ Revise ☐ Retire Notes:

393. nce-s-cor-025 · Social and Cultural Diversity · cultural humility · medium · In review

In a first session, a counselor told a U.S.-born Korean American client, "Your English is excellent." The client became quiet for the rest of the hour. At the next session, what should the counselor do FIRST?

- A. Wait to see whether the client brings up the comment before addressing it in session
Waiting puts the burden of raising the harm on the client and lets the rupture persist.
- B. Explain that the remark was meant as a compliment and was not intended to offend
Defending intent centers the counselor and minimizes the impact on the client.
- C. Name the comment, take responsibility for its impact, and invite the client's view ✓ KEY
Nondefensively acknowledging a microaggression and inviting the client's experience is the core of rupture repair with cultural humility.
- D. Focus on the presenting problem so the session is not sidetracked by the earlier slip
Moving on avoids the rupture; unaddressed microaggressions are linked to weaker alliances and dropout.

Rationale: Remarks that assume an Asian American is a foreigner ("alien in one's own land") are a recognized microaggression. When a counselor commits one, cultural humility calls for owning the impact rather than defending intent, and openly inviting the client's reaction. Repairing such ruptures can strengthen the relationship.

References: Sue & Sue (Counseling the Culturally Diverse) — Racial microaggressions: alien in one's own land

☐ Approve ☐ Revise ☐ Retire Notes:

A second-generation Nigerian American counselor begins working with a 24-year-old woman who immigrated from Nigeria two years ago. The counselor thinks, "I grew up with this, so I already know what her parents expect of her." Which stance BEST reflects cultural humility?

A. Draw on shared heritage to explain the family expectations the client is facing

Using one's own background as a template assumes sameness and replaces the client's account with the counselor's.

B. Ask how she sees her family's expectations and stay open to differences from her own ✓ KEY

This treats the client as the expert on her experience and stays alert to differences in generation, region, ethnicity, and immigration history.

C. Refer her to a counselor of another culture to avoid any bias from the shared heritage

A shared background is not grounds for referral; the task is to examine assumptions, not avoid the client.

D. Disclose the shared background early so the client knows she will be understood

Promising understanding on the basis of shared heritage reinforces the assumption of sameness.

Rationale: Cultural humility applies even when counselor and client share a cultural background. Within-group differences in generation, acculturation, region, language, religion, and personal values can be large, and assuming sameness can lead to blind spots or overidentification. A humble, curious stance invites the client to define what her culture means to her.

References: Sue & Sue (Counseling the Culturally Diverse) — Within-group differences and the dangers of assumed similarity

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor with strong religious convictions works with a client who wants help deciding whether to leave her faith community. The counselor notices a pull to encourage the client to stay. What is the MOST appropriate action?

A. Refer the client elsewhere, because the counselor's beliefs conflict with the goal

The ACA Code bars referral based solely on the counselor's personal values; the counselor must first work to bracket them.

B. Share the counselor's own beliefs openly so the client can weigh them in deciding

Offering personal religious views on the client's decision imposes the counselor's values.

C. Gently point out what the client may lose by leaving her supportive community

Emphasizing losses of only one option steers the client toward the counselor's preference.

D. Seek consultation, set aside personal views, and follow the client's own process ✓ KEY

Recognizing the pull, getting consultation, and keeping the focus on the client's goals reflect self-awareness and A.4.b.

Rationale: Self-awareness includes noticing when personal religious or political values press on clinical work. The ACA Code directs counselors to be aware of their own values and avoid imposing them, seeking training where they are at risk of doing so, and it prohibits referral based solely on the counselor's personal values.

References: ACA Code of Ethics — A.4.b Personal Values · ACA Code of Ethics — A.11.b Values Within Termination and Referral

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor whose parents had a bitter divorce works with a 16-year-old caught in a custody dispute. She finds herself angry at the client's father and urging the teen to "stand up to him." This pattern is BEST described as

A. overidentification ✓ KEY

The counselor is merging her own history with the client's, losing objectivity and taking sides, a form of countertransference.

B. parallel process

Parallel process refers to dynamics in counseling being replicated in supervision, or the reverse.

C. advocacy

Advocacy addresses systemic barriers on behalf of a client; it is not driven by the counselor's unresolved personal history.

D. client transference

Transference is the client's projection onto the counselor; here the reaction originates in the counselor.

Rationale: Overidentification occurs when a counselor sees the client's situation through the lens of his or her own experience, losing the "as if" boundary of empathy. It is a common form of countertransference that can lead the counselor to take sides or push the client toward choices the counselor wished for, so it calls for self-reflection and supervision.

References: Corey (Theory & Practice) — The counselor as a person: countertransference and overidentification

► **Check:** Confirm Corey (Theory & Practice) discusses overidentification by name; Corey's Issues & Ethics may be the better home for it.

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, "I still smoke weed most nights to fall asleep. Go ahead and give me the lecture." Which counselor response BEST reflects a nonjudgmental stance?

A. No lecture. I'd like to understand what it does for you and how it's working. ✓ KEY

This sets aside evaluation and invites the client's own view of benefits and costs, a nonjudgmental and collaborative stance.

B. I won't lecture, but you know nightly use can make your sleep worse over time.

The "but" delivers the lecture anyway, conveying judgment and likely increasing defensiveness.

C. It sounds like you already know it's a problem, or you wouldn't expect a lecture.

This interprets the comment as an admission of a problem, imposing the counselor's conclusion.

D. Lots of people use cannabis for sleep now, so it isn't a big concern for me.

Dismissing the topic is approval, another kind of judgment, and closes off exploration.

Rationale: A nonjudgmental stance suspends both disapproval and approval in favor of curiosity about the client's experience. With substance use, exploring what the substance does for the client and how it is working invites honest self-evaluation and is consistent with motivational interviewing's spirit of acceptance.

References: Miller & Rollnick (MI) — Spirit of MI: acceptance (absolute worth, autonomy support)

☐ Approve ☐ Revise ☐ Retire Notes:

A 33-year-old single mother mentions in passing that she does sex work to pay her bills, then says she came to counseling because of anxiety about her son's problems at school. What should the counselor do?

A. Shift the focus to her work, since it is likely the underlying source of her anxiety

Assuming the work is the real problem overrides the client's stated goal and reflects the counselor's judgment.

B. Follow her stated concern while leaving room to discuss work or safety if she wishes ✓ **KEY**

Respecting the client's agenda without ignoring possible safety needs reflects a nonjudgmental, client-centered stance.

C. Give her referrals for job training so she has other options to support her family

Unrequested job referrals imply her work must change, imposing values she did not ask the counselor to apply.

D. Avoid any mention of her work so that she does not feel judged in this session

Treating the topic as unspeakable can also signal judgment and may miss safety concerns she wants to raise.

Rationale: A nonjudgmental stance means the counselor does not treat a stigmatized behavior as the presumed problem. The client sets the agenda, and the counselor stays open to discussing work, health, or safety when the client chooses, without pathologizing or avoiding the topic.

References: ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes:

A client arrives sobbing and breathing rapidly after learning she is being evicted at the end of the week. Which counselor response BEST conveys warmth in this moment?

A. Hand her a list of housing resources so she can start making calls right away

Resources may help later, but moving to tasks while she is flooded ignores her emotional state.

B. Ask her to describe the eviction notice in detail to see what her options are

Detailed fact-gathering before she is calmer is task-focused and can add to her distress.

C. Tell her firmly that panicking will not help and that she needs to calm down

A directive to stop panicking is cold and dismissive, and it rarely reduces distress.

D. Sit with her, speak in a calm, steady voice, and acknowledge how scared she is ✓ **KEY**

A calm, caring presence and acknowledgment of her fear communicate warmth and help her regulate before problem-solving.

Rationale: Warmth is communicated largely through nonverbal behavior, including tone of voice, pacing, and attentive presence, together with acknowledgment of the client's feelings. In acute distress, a warm and steady presence helps the client regulate enough to engage in the practical problem-solving that follows.

References: Gladding (Counseling: A Comprehensive Profession) — Warmth and nonverbal communication in the helping relationship

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is meeting with a Spanish-speaking client through a trained professional interpreter. Which practice BEST preserves the client's voice and respects the counseling relationship?

A. Ask the interpreter to summarize what the client says to keep sessions brief

Summaries filter the client's words through the interpreter and lose emotional content.

B. Ask the client's bilingual teenage son to interpret, since the family trusts him

Family members, especially children, compromise accuracy and confidentiality and shift family power roles.

C. Speak directly to the client in the first person rather than to the interpreter ✓ **KEY**

Addressing the client directly keeps her at the center of the relationship and the interpreter in a conduit role.

D. Speak with the interpreter after sessions to learn the cultural meaning of her words

Relying on the interpreter as cultural expert behind the client's back can sideline her own account of her experience.

Rationale: Interpreted sessions add a third person and can shift power away from the client. Best practice is to use a trained interpreter rather than family, brief the interpreter beforehand, and speak directly to the client in the first person so that the relationship remains between counselor and client.

References: ACA Code of Ethics — A.2.c Developmental and Cultural Sensitivity · Sue & Sue (Counseling the Culturally Diverse) — Language barriers and use of interpreters

☐ Approve

☐ Revise

☐ Retire

Notes: